

Thyrocare Tech (THYROCARE)

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Call: May 2023

May 26, 2023

The National Stock Exchange of India Limited BSE Limited
Exchange Plaza, Phiroze Jeejeebhoy Towers
Bandra Kurla Complex, Dalal Street,
Bandra (E), Mumbai - 400 051 Mumbai- 400 001
(SYMBOL: THYROCare) (SCRIP CODE 539871)

Dear Sir/Madam,

Sub: Transcript of post results earnings
conference call held on May 23, 2023 with Analysts
/ Investors

Ref: Disclosure under Regulation 30 of the Securities and Exchange Board of India
(Listing Obligations and Disclosure Requirements) Regulations, 2015 ("SEBI Listing
Regulations")

Pursuant to Regulation 30(6) and 46(2) read with Part A of Schedule III of the SEBI Listing
Regulations, please find enclosed herewith transcript of earnings conference call with Analysts
and Investors held on May 23, 2023.

We wish to add that the same has also been made available on the website of the Company
(www.thyrocare.com).

This is for your information and records.

Yours Faithfully,

For Thyrocare Technologies Limited,

Ramjee Dorai
Company Secretary and Compliance Officer

0Thyrocare
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"Thyrocare Technologies Limited Q4 & FY23 Earnings
Conference Call"

May 23, 2023

M
MANAGEMENT : M R. RAHUL GUHA – MD & CEO
MR. SACHIN SALVI – CFO
MR. PRATIK HIRE – HEAD (STRATEGY & IR)
MR. ADITYA SHINDE – HEAD (FINANCE , ADMIN , &
SOURCING)

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Moderator: Ladies and gentlemen, good day and welcome to the Thyrocare Technologies Limited earnings conference call.

As a reminder, all participants' lines will be in the listen -only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" then "0" on your touch-tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Pratik Hire from Thyrocare. Thank you and over to you, Mr. Hire.

Pratik Hire: A very good evening to all and thank you for joining us today for Thyrocare's earnings conference call for the 4th quarter and full year FY23. Today, we have with us Mr. Rahul Guha – MD & CEO and Mr. Sachin Salvi – CFO to share the highlights of the "Business and Financials" for the quarter. I hope you have gone through our Results Release and the Investor Presentation which now have been uploaded on the Stock Exchange websites. The transcript of this call will be available in a week's time on the company's website.

Please note that today's discussion may be forward looking in nature and must be viewed in relation to the risks pertaining to our business. After the end of this call, in case you have any further questions, please feel free to reach out to the investor relations team.

I now hand over the call to Rahul Guha to make the opening remarks.

Rahul Guha: Welcome to everyone on the call. Thank you once again taking out time from your schedules to join us this evening. For those who are joining for the first time, just a quick introduction of us on the call. My name is Rahul Guha. I am the MD and CEO of Thyrocare. It has actually been roughly a year since I joined, and I want to thank you all for the opportunity to present the Q4 and annual results for this year. I am joined by Sachin Salvi who is our CFO and well known to most people on the call. Pratik Hire who is part of our strategy team and leads investor relations and had kicked off the call is with me along with Aditya Shinde who heads the Finance, Admin, and Sourcing functions.

First, I will just talk a little bit about

the focus areas over the last few months and the years that have been. But before we begin, as with all my calls, I will always start with a quote from the Mahatma. "The future depends on what you do today ." And therefore, I wanted to give you a sense of some of the highlights of what we have been doing in the last 1 year. If you recall, when we first took over, we touched upon rebranding efforts to bring a fresh and modern perspective to the brand. That has been completed and with our new test menu of 700 tests, our brand logo actually moves away from the old imagery of the thyroid gland and anchors on two tenets. The first is a drop of blood signifying our heritage in pathology. The second element is a microscope which we wanted to use to bring out our anchoring in science and technology at all our labs. 0Thyrocare

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Quality remains the key priority area for us. It is an ongoing journey and I wanted to report that we have made good progress over the last 1 year. We have now 20 labs that are NABL accredited which includes our COVID lab. And today I am very happy to say that 85% of our sample load is processed in an NABL lab. It's important to understand this in context, this 85%. Today, only 2% of the pathology labs in the country have an NABL accreditation. Actually, what we did was we commissioned an independent study on quality through a research paper that has been published in the International Journal of Advanced Research, Ideas, and Innovations in Technology. That report actually shows that 9 out of 10 doctors trust Thyrocare reports to be

reliable and recommend their patients to get tested from Thyrocare. That is a big stamp of approval of all the work that we have been doing on the quality front.

Beyond the work on quality, we have expanded our offerings substantially in the preventive care space. As I had talked in the last quarter call, we had introduced the plus and pro series in our flagship Aarogyam. We have also launched a set of 24x7 non-fasting packages to offer round-the-clock diagnostic services on the wellness front and working closely with doctors. We have launched a new series of investigative packages under the brand name Jaanch. These packages are curated by doctors to help doctors and patients investigate specific chronic and lifestyle diseases in a much more targeted way. The way to think about it is Aarogyam is our wellness brand, but Jaanch is our

investigative or sickness brand. If you are concerned about any health concerns, then we would recommend that you use a targeted Jaanch profile. We have Jaanch profiles for hair fall, for fever, for PCOS, for thyroid, for any sickness or chronic condition, we have worked very closely with doctors to curate these packages to put the power of diagnosis in the hands of the patients and doctors and we have branded this series Jaanch and we have launched it across the country in the last few weeks.

We also continue to engage doctors on diagnostic testing and how it can better impact their patients and the treatment that they receive. In partnership with key opinion leaders, we have developed educational videos and have also started participating in conferences to share and debate our perspectives on diagnostic testing. You would have seen through many of our campaigns we have engaged with regional celebrities to create awareness about diabetes testing. We have also revamped our technology stack. We have now launched our new platform called ThyroNXT for our franchisee partners, which aims to make transacting with Thyrocare as a B2B company a frictionless experience.

On the financial front, we also distributed an interim dividend of Rs. 18 per share at the rate of 180%. We are very proud of some of the key milestones that we achieved in the last year. On the quality front, we have reduced our complaints per million samples by 35

%. Our P90 TAT is

now less than 24 hours pan India as compared to 28 hours last year. And our average TAT now is less than 14 hours. Through our pin expansion initiative, we are now active in 4,600 pins as compared to 2,600 pins in FY22. And we now have 7,400+ active franchisees as compared to 4,400 in FY22. As a result, we processed 22.3 million non-COVID samples and served 15 million patients in the year which is 39% and 32% year-on-year growth respectively in volume.

In our nuclear business, we performed 30,800 PET CT scans this year which compares to 24,000 Thyrocare

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scans last year. And when it comes to API group, it continues to account for 12% of our pathology revenue for the full financial year.

As we get into the results, I wanted to share with you a few highlights or pointers before we deep dive into the same. Our focus has always been in driving sustainable revenue growth. We have seen a healthy recovery in our non-COVID volumes year on year. This has been driven by continued focus on channel expansion through our pin expansion initiative and a focus on on-boarding new key accounts in our partnerships. As a result, we have been able to deliver a 22% year-on-year growth in our non-COVID business. And while there has been a degrowth at an overall level, that is largely the drop from COVID being out of the picture.

With that, I will hand over to Sachin to just cover the overall results.

Sachin Salvi: Good evening and thank you everyone for joining this call. I will briefly update you about the key highlights of Q4 and full year FY23 Financial Performance.

Before we get into the details, I will reiterate about ESOP program that I had mentioned in the last couple of calls and last couple of quarters. This program has been introduced at the group level to retain talent at Thyrocare. The ESOPs of parent company will vest over the next 6 years and we are recognizing the same as per IndAS and IFRS framework. As a book entry towards the share base payments and appropriately reflected in the profit & loss account as an expense and in the balance sheet as equity contribution with us from the parent company. The total value of the ESOPs granted are Rs. 45.53 crores over a period of 6 years, but one thing you must note that this is a cashless charge. It is not cash outflow. As per the IndAS framework, the options are valued at grant date as per the Black Scholes formula which is a charge to the profit & loss account over the vesting period proportionately and typically which results in hit in the year of grant and then proportionately charged over the vesting period. The breakup is included in our detailed presentation. As these are not operating expenses and do not impact our cash flow, we have normalized EBITDA to the extent and reported EBITDA in line.

Now, going into the performance :

First, I will start with the revenue from operations:

Our revenue from non-COVID operations for the year on a standalone basis has increased by about 22% Y-o-Y to Rs. 457 crores. Our COVID revenue, however, has declined by 96% Y-o-Y to Rs. 6 crores for the current year. In Q4, we have grown our non-COVID revenue by 18% and overall revenue by 4% Y-o-Y. Additionally, we have seen a strong recovery in our radiology business. We have grown revenue on a Y-o-Y basis by 47% in the year and 53% Y-o-Y in Q4.

As far as the EBITDA margin is concerned, our standalone normalized EBITDA margin stands at 31% for the quarter and 29% for the full year. Thyrocare

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With these brief highlights, I will pass it on to our MD and CEO – Rahul Guha, for the strategic updates to the investors.

Rahul Guha: Just one important point to clarify, while we have reported a consolidated normalized EBITDA of Rs. 151 crores, our free cash flow

from operations this year is Rs. 129 crores. So, our normalized EBITDA to cash conversion is quite good.

I will just take now a few minutes to recap our strategic direction and then I will open it up for Q&A. As always, I will first start with our value proposition with the customer. We will continue to remain an affordable option to all patients with good quality and on-time reports. All our efforts on our value proposition is towards ensuring low cost to the patients, assurance on quality of testing through our certifications, and engagement with doctors. We have made substantial progress on this which I updated in my initial comment and you will see reflected in the presentation. This will remain at our core and will continue to guide all that we do. Second is our strategy. We continue to maintain our strategy of being the B2B partner of choice to all front-end diagnostic services companies in India. Whether it is a small diagnostic center in a semi-urban area, a pharmacy in a metro, a small nursing home, an individual doctor, or even a leading online diagnostic platform or a health tech marketplace, we are happy to work with them to provide low-cost robust testing solutions so that they can serve their patients in the most effective manner. If they require phlebotomy, we are happy to mobilize our phlebotomy network of almost 900 company and 400 network phlebotomists to serve them better. This strategy has been working well for us. We have delivered a year-on-year value growth of 22% and a 39% volume growth in our non-COVID business.

Going forward, we have 3 key pillars of growth which is slightly different from what I have been sharing over the last few quarters. We are focusing on 3 areas. The first is our franchise business. The focus is to take our franchise business deeper into India with a focused test menu and provide our clients with a frictionless experience to transact with us and provide their customers the best testing experience that they can. We continue to focus on private as well as public partnerships. In the public space, we will focus on TB and infectious disease where we are by far one of the strongest players in these segments with large screening programs partnering with health bodies and funding agencies who participate in this segment to provide better testing and care to the patients who are suffering from these diseases. Additionally, we will continue to expand our partnership across healthcare companies, hospitals, and other health services companies and enable them to provide diagnostic testing to their customers. The third area which is new from before is we believe we have a strong and robust B2B model with a core of execution. We believe there is a lot of opportunity to take this model internationally and we will start to look at emerging markets where we can bring this model to provide affordable testing into those markets.

That, in a brief, is our mandate management. Thank you for giving us a patient hearing. I will once again end with a quote from the Mahatma. "Find purpose; the means will follow." And our Thyrocare Tests you can trust

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purpose always remains to be to provide affordable high-quality testing to the masses. With that, I will open it up for Q&A.

Moderator: We will now begin the question & answer session. Ladies and gentlemen, we will wait for a moment while the question queue assembles.

The first question is from the line of Rahul Agarwal from InCred Capital. Please go ahead.

Rahul Agarwal: I will start with your last point, Rahul. You mentioned foray into international market. Could you elaborate please why this is even needed for the company?

Rahul Guha: As we look at growth and our business model, we are always looking for new avenues for growth, Rahul, and we believe there is an opportunity there. We have done a detailed study in

some of the emerging markets in terms of the prices, in terms of our operating model, and the return that we can generate and also with a thorough assessment of the competitive scenario, we believe there is a sweet spot to disrupt some of these emerging markets in the same way the

Thyrocare has disrupted the Indian market a few years ago. We believe there is a substantial opportunity to take our model international.

Rahul Agarwal: Any countries you would want to focus on at the initial stages?

Rahul Guha: We are looking largely at emerging markets, Rahul. We have done a full assessment on Africa. We believe that could be an interesting opportunity for us. The Middle East is the second and Southeast Asia is the third that we have prioritized. Again, I want to reiterate we will follow a B2B model there where we will largely to a large extent take our lab strength infrastructure and

the ability to operate large -scale labs at a low cost and work with partners in those respective markets to develop the market.

Rahul Agarwal: Labs essentially are going to get set up there and they will service the B2B market. Is that correct? Or they will get the samples come to India and get tested?

Rahul Guha: Labs will have to be set up there. We won't be able to manage the international air freight logistics.

Rahul Agarwal: Secondly, a few discussions or question. I will put in two and then I will come back in the queue.

We generally end up discussing all B2B on Thyrocare's calls. Just on B2C, how is that moving for the business? Obviously, the own lab infra, your pin code expansion, your NAVL labs actually boost that. Isn't it? Plus, the D2C side of it, which is our own app and website, how is the traction there?

Rahul Guha: On the D2C side, just to clarify, we don't follow a customer -acquisition strategy where we invest in Google AdWords and Facebook and so on and then acquire customers into the platform. It's largely organic traffic that we look at. That business has grown well but to be honest, it's a very 0Thyrocare

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small part of our business and we haven't really pushed that part because it's very expensive to acquire customers online and then service them. That's not really our cup of tea. We prefer to work with partners who have acquired customers and then we are able to work with them to service their diagnostic testing needs and that strategy has overall worked fine. If you look at our business, as I said many times, we break our business into two parts – the franchise business and the partnerships business and then we have our D2C which is 5%. Our franchise business, as you have seen, same year last quarter was Rs. 62 crores and now is Rs. 73 crores and our partnerships was last time Rs. 31 crores and now is Rs. 38 crores which is roughly kind of the same growth rate for the company. So, I would say both engines, our offline franchise business and our largely online partnerships, are kind of firing at the same rate.

Rahul Agarwal: Overall B2C, not the D2C part but including D2C, overall B2C should be like 15% of the business – your Thyrocare denominated report?

Rahul Guha: I would say almost all our reports are Thyrocare denominated reports. As I said, we look at our business, Rahul, in two ways. We believe we are largely B2B, and we look at our business in two ways. One is our franchise network that works with pathology labs to collect samples and then process them at Thyrocare and then our partnerships which may be like PharmEasy, but we work with many of the other online players who have their own captive customers but for diagnostics actually pass on the customers to Thyrocare and we.... Except for PharmEasy, actually for almost all our partners, we release the Thyrocare reports.

Rahul Agarwal: Lastly on Aarogyam, what was the revenue share for fiscal '23 and after the realignme

nt you

have done across being across price points, the pro and plus series, what has been your experience over the last 2 -3 months?

Rahul Guha: Pro and plus was basically launched in the last quarter. I think it will take some time. Roughly, there is a set of packages that have got launched over that, they account for about 10% or 15% of the overall Aarogyam now, the new packages that were launched in the pro and plus series.

Our Aarogyam to non-Aarogyam remains at about 40:60. 40% of FY23 would be Aarogyam and about 60% would be non-Aarogyam. To be precise, it is 41 something and 58.7 something but that is you say 40:60 is the rough ratio of turnover for Aarogyam to non-Aarogyam.

Moderator: The next question is from the line of Nitin Shakhder from Green Capital Single Family Office.

Please go ahead.

Nitin Shakhder: My question is in relation to what is the exact current cash on books? And obviously related to that is what is the management viewpoint on increasing shareholder value in terms of buyback? Because, obviously the share price while the management will not comment on it, but as an investor, I feel that it is heavily undervalued. Any initiatives on that which will change the current paradigm? 0Thyrocare

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Rahul Guha: To a large extent, the cash on books is about Rs. 40 crores now after the payout of the Rs. 95 crores dividend that we paid just in the month of April. Post the payout of the Rs. 95 crores, cash on books is about Rs. 40 crores. We generate a decent amount of operating cash every month.

As I said, over the course of last year, we generated Rs. 129 crores of cash. Coming to the shareholder point, one of the things that we always followed is a pretty aggressive dividend strategy and we will continue to do that when we don't have material uses for the cash. We

haven't evaluated a share buyback at this point but we typically use dividend as the way to return money to the shareholders. The other is, in our capital allocation when we reviewed it as well, we found that actually we use a lot of operating cash to fund equipment purchases. There, we have moderated some of that capital allocation strategy where we will use debt to finance equipment and actually free up a lot of the shareholder cash that normally would get used in purchasing equipment and continue to dividend it out unless we find a material deployment area that can be remunerative from a return -on-capital point of view. The international expansion is one area where we may deploy some of that cash to generate returns but that's the overall capital allocation strategy right now.

Nitin Shakhder: I will come to the international expansion which is interesting because you are looking at Africa as No. 1 and obviously in Africa whether Uganda, Nigeria, or South Africa or even Kenya, can you just highlight a little bit on the strategy in terms of what exactly business to business (B2B) you will be looking at? Is it in terms of tie -ups or is it in terms of endemic or pandemic pathology testing or is it in terms of even exploring markets like Vietnam , Philippines, Thailand, Indonesia? Is the strategy going to be different for Southeast Asia or is it going to be very similar for Africa as well? And how much is allocated for the international expansion for the financial year, please?

Rahul Guha: As I said, there are 3 geographies – Africa, Middle East, and Southeast Asia that we have prioritized as the first wave. Just to say what do I mean by the B2B model, typically in all these markets, there are a large number of hospitals and large number of pathology labs that operate in these markets. Typically, just like in India, we go in and set up a lab of a significant scale that allows us to bring the cost down of testing of particularly high CAPEX kind of tests – hormones; one of the most famous is thyroid, but there are mu

ltiple hormones that can be leveraged as well; many of the infectious diseases, particularly when it comes to PCR -based testing and things like that, which are also endemic in these countries. So, the strategy is to go in and set up a large infrastructure and lab which can process significant volumes at a much lower cost and then work with distribution partners in those markets to gather the volume and those distribution partners are typically distributors to hospitals or distributors to pathology labs, typically people who sell vials or reagents into these labs and have a good connect and we are effectively then able to provide them one more line of business into that market. That's the outline of the strategy. For next year, we have actually allocated not a lot; we have allocated about Rs. 20 crores that will be largely deployed in the Africa space. But once we taste success in Africa, and then we will look to scale the model is when we will consider a further capital deployment.

Moderator: The next question is from the line of Aditya Khemka from Incred Asset Management. Please go ahead. OThyrocare

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Aditya Khemka: Rahul, sorry I couldn't catch that. How much capital would it need for you to enter these emerging markets next year?

Rahul Guha: We have allocated for the first wave Rs. 20 crores.

Aditya Khemka: This Rs. 20 crores would be spent across what time frame?

Rahul Guha: To be honest, I would say between the quarter 3 and quarter 4 is when we would incur most of it because that is the time that the labs would get set up.

Aditya Khemka: This Rs. 20 crores would be sufficient for how many geographies, Rahul, as in how many countries or how many labs?

Rahul Guha: Three geographies and it would be 3 labs.

Aditya Khemka: Secondly, on the government receivables that you have provisioned for, how long have these receivables been due? And we also look at this company called Krishna Diagnostics that does business from the government. We haven't seen any provisioning ever from their side. They seem to get their receivables on time. If you could slightly elaborate on what caused this to be provisioned and how long has it been overdue from the government and why is it different from what other players are going?

Rahul Guha: I cannot comment on the provisioni ng practices of other companies. I will just talk about ours. This outstanding has been due for 2 years. It is COVID testing related. I don't think we are very different from other players when it comes to the COVID receivables, particularly the specific testing agencies that we have been working with. It has taken time to clear the bill. And according to the process with our auditors, we are provisioning as appropriate and we remain hopeful as and when the funds are released that we will receive the funds, but we have been provisioning as per the guidance from our auditors. But there are 2 parts of the government business; there is pathology and there is radiology. In radiology, the receivables tend to be more under control because you control the equipment as well. If over a period of time you have not been paid, you can shut down the equipment and effectively precipitate your payment whereas in pathology and particularly in COVID where the testing was done at such a high volume in

such a short period of time, it will take time for the payments to get released.

Aditya Khemka: Could you also clarify is this state government payments or central or spread between state and central?

Rahul Guha: This is state government payment and municipal corporations.

Aditya Khemka: Okay, that clarifies. Because, under the NHM which is the National Health Mission controlled by the state government and my understanding is that the payments are fairly moving fast. Any which ways.

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Lastly, on the non-COVID business. We see that the volume is going faster than the top line revenue. Is the realization per test coming down? Are we actively offering discounts to get higher volumes in? And if so, what is your outlook for the future? Can you maintain a 20% ish kind of volume growth with out taking significant price cuts or you feel you would have to offer some sort of price cuts to get volumes north of 20% on the non-COVID side?

Rahul Guha: Actually there, Aditya, if you recall when I had first taken over, we had done a price correction in just prior to in Q4 of 2022 and so if you look at Q4 '22 to Q4 '23, actually our gross margin has improved over the year. After that Q4 '22 price correction that we took, we have not corrected prices throughout this financial year. If you compare Q4 to Q4, our gross margin has improved by a percentage point overall. I would say whatever correction we did to the operating model which was to correct the prices and all of that in Q4 of last year, since then, that has carried us forward in terms of the revenue growth that we are seeing.

Aditya Khemka: Just one more question from my side. This ESOP pool of Rs. 45 odd crores which we are writing off over the next 6 years, in the foreseeable future as in the next 2 -3 years, do we have any visibility if there would be a fresh pool again created for ESOP to incentivize more or do you think this ESOP pool will last for 10 years or 6 years or whatever? Can you give us some clarity on can there be further ESOP grants in the future?

Rahul Guha: There could be a possibility of further ESOP grants in the future. It always remains our priority to attract talent, but as of now, at least at the Board level, there has been no discussion of enhancing or increasing this, but as and when we get key talent, we may evaluate a further ESOP grant, but at this time, there isn't any plan.

Aditya Khemka: Just to clarify on this, Rahul, these ESOPs are for the API holding shares. In Thyrocare's book, there is an entry, but there is no cash impact and there is no dilution of equity happening in the books of Thyrocare.

Rahul Guha: That is correct. As I discussed also, if you look at our normalized EBITDA for the year, our normalized EBITDA was Rs. 150 crores and our reported EBITDA actually goes down all the way to Rs. 120 crores because of this ESOP cost and provision for receivables, but our free cash flow from operations for the year is Rs. 130 crores.

Aditya Khemka: A normal ESOP of Thyrocare shares would not also impact cash flow but it would impact your number of shares – diluted number of shares. So, I just wanted this to be on record that your diluted number of shares is not actually increasing because of this ESOP grants.

Rahul Guha: Because of the API ESOP grant, no, but as you know, we have a small ESOP program in Thyrocare as well, which when created was 1% of the outstanding equity. That ESOP plan continues as always.

Aditya Khemka: What is the quantum of that ESOP plan? 1% of Thyrocare's equity? 0Thyrocare

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Rahul Guha: It was about 5 lakh shares. It was granted over a period of 10 years. It roughly works out to about 50,000 to 60,000 every year.

Aditya Khemka: 50,000 to 60,000 shares every year? And out of the 10 years, how many years have lapsed and how many years are left?

Rahul Guha: Eight have lapsed. There are two more years left.

Aditya Khemka: So, this total ESOP cost of Rs. 45 odd crores holding cost

Rahul Guha: Yes, this is the API holding ESOP cost. The Thyrocare cost is not significant. We have absorbed it as part of employee benefits.

Aditya Khemka: When you say not significant, it's not north of Rs. 1 crore?

Rahul Guha: Rs. 2 crores.

Aditya Khemka: And it's included in employee benefit expenses.

Moderator: The next question is fr

om the line of Nilesh Doshi from Prospero Tree. Please go ahead.

Nilesh Doshi: Sir, I had 2 questions only. One is that you have answered a little bit on the government outstanding amounts. What is the exact amount outstanding as on the 31st March 2023?

Sachin Salvi: It is about Rs. 60 crores. Rs. 51 crores from one debtor which is NHM Maharashtra. There would be some small debtors like NHM Uttarakhand, NHM Jharkhand, and Bengaluru Municipal Corporation that would be another Rs. 7-8 crores.

Nilesh Doshi: It was around Rs. 80+ crores as on 31st March 2022. So, we received Rs. 20 crores during the year or we have provided out of this Rs. 20 crores?

Sachin Salvi: The total billing was about Rs. 80 crores. We have received about Rs. 20 crores in financial year '22. During financial year 22-23, we have not received significant amount. That's why as on 31st March 2023, the tentative outstanding from NHM on account of COVID business was roughly about Rs. 60 crores.

Nilesh Doshi: But we had the provision of around Rs. 10 crores in FY23. The amount is outstanding amount is Rs. 61 crores for FY23. It was Rs. 80+ crores in 2022. The difference between these two amounts is Rs. 20 crores. So, we must be made the provision of Rs. 20 crores if we have not received any amount from the government authority?

Sachin Salvi: The provision for the outstanding receivables is made on the basis of ECL model. It is not on the basis of whether the money has been received or not, it is based on the amount which is outstanding and for how much period the amount is outstanding of the total quantum. So, this OThyrocare Tests you can trust

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has nothing to do with how much a mount has been received, how much amount was outstanding as on 31st March '22 or as on 31st March 2023.

Nilesh Doshi: We are not receiving the amount because of any dispute or there is anything wrong billing or what has actually exactly happened?

Sachin Salvi: There is no wrong billing and all. The only thing is there was some clarification which has been sought by the regulator on account of the repose which has been uploaded on the ICMR website. We have given that clarification to the respective department and they are just quantifying by seeking confirmation from the respective MoH who are sitting in the respective districts.

Nilesh Doshi: When we are expecting to receive this amount?

Sachin Salvi: We were expecting it to receive in the last quarter, but somehow this process is getting delayed, and we have been informed that it will take another say 5 to 6 months to release this payment. So, by September 2023, we are expecting.

Nilesh Doshi: But now, are we doing any government project and for that we are receiving the regular payment?

Sachin Salvi: As of now you are asking? As of now, we are not doing any big government project as such. We are doing municipal corporation project that is BMC but that has been awarded to Krishna in the month of February. So, that has been stopped. As far as that payment is concerned, we are receiving that payment against that outstanding quite regularly. But we are not doing any substantial government business as of now.

Nilesh Doshi: What is the total receivables as on 31st March '23 including the other party and government authorities?

Sachin Salvi: The total receivables as on 31st March 2023 net of provision is about Rs. 85 crores.

Nilesh Doshi: Excluding the provision or after making the provision?

Sachin Salvi: That is true.

Nilesh Doshi: My last question is related to dividend. Sir has described the capital allocation strategy. That's fine. But we are a growing company and should not we use the fund for the infrastructure development rather than distributing the amount to the shareholders which is beneficial to the shareholders? Because, dividend is good, but I think growth is better.

Rahul Guha: A

As I said, the overall company growth is fine. Last year we grew at 22%. The important question is at this point in time, should we invest in setting up more labs in the country to deepen our

presence? At this point, we don't believe it's prudent to use cash and shareholder cash to fund OThyrocare

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equipment. We can actually fund equipment at very good rates with debt. So, there is no need to use shareholder money to fund equipment at this point in time. At this point in time, we are not

looking at any material acquisition to purchase any kinds of companies to see if we can grow

the top line and our organic growth has actually been quite healthy. As I said, we are allocating

some capital for our international expansion strategy, but we don't think it's prudent at this point to hold on to the cash or to use it to fund equipment and deprive the shareholders of their own

avenues to deploy the money. And as I said, our current strategy has been delivering healthy growth. So, we don't foresee very large CAPEX or acquisition to deploy that cash.

Nilesh Doshi: So, the current dividend rate will be maintained if the earning met the current status?

Rahul Guha: Yes, unless something changes, and we see a great opportunity where we can really accelerate the performance of the company.

Nilesh Doshi: Sir, are we offering any special discount to PharmEasy, or we are giving the same rate to other parties?

Rahul Guha: This forms part of the related -party transactions which we get approved in the AGM also. It is actually a public information. PharmEasy gets a 15% discount over our published rate but many of our other customers also get the benefit of discount basis their volumes. It is we have a standard published rate card basis volume and PharmEasy enjoys that 15% discount because of the volumes that they give to us.

Nilesh Doshi: What is the status of the pilot project which is likely to be set up at Ghatko par with some other partner?

Rahul Guha: The pilot project is operational, sir. We have just done a few scans, CT scans in particular. The full project will be operational, I would say, towards June -July, but that is a small pilot as you rightly pointed out and it's our first step in the radiology segment. Let us see how it goes. If it goes well, then we will consider investing. But at this point, it is just a pilot.

Nilesh Doshi: Will there be any conflict of interest with our nuclear business or both are in the different business?

Rahul Guha: Very different segments. Nuclear is in the PET CT business. The pilot that we are doing has CT, MRI, X-ray, and ultrasound. It is a full radiology business but does not have PET CT.

Nilesh Doshi: Last question. Under nuclear, we are operating 9 centers or 10 centers.

Sachin Salvi: We are operating 9 centers. 0Thyrocare

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Nilesh Doshi: Because, in the presentation, Baroda and Surat are provided and in the annual report Baroda was there. So, I was confused it is 9 or 10. In the annua l report, 8 centers are mentioned. So, Surat is the new one?

Sachin Salvi: Sorry, we operate 10 centers including Surat and Baroda.

Moderator: The next question is from the line of Harini Dedhia from Tamohara Investment Managers. Please go ahead.

Harini Dedhia: Just 2 quick points. One was on when I look at the partnership workload, we are in Q4 marginally lower than Q1, but our revenues are higher. Is it just because we have increased the number of tests per sample or is it because we have managed to push some price hikes on platforms?

Rahul Guha: Some of our newer partners are coming at a much better realization and that is what is driving the growth rate.

Harini Dedhia: Maybe we have seen some workload rationalization happening at the older partners which is understandable because of the push to profitability. But the newer partners, because they are not giving us the same volumes as yet, realizations are better.

Rahul Guha: Yes.

Harini Dedhia: Just broadly speaking, Sachin, if you could help us what portion of other expenses are variable and fixed? If you can just give a broad number, that would be very helpful?

Sachin Salvi: Thyrocare or nuclear?

Harini Dedhia: Thyrocare, the pathology business.

Sachin Salvi: As far as the pathology business is concerned, I think almost all the expenses are variable except for the rental cost which we are incurring. Otherwise, all the expenses are almost variable. As far as nuclear is concerned, I would say mostly all the expenses are fixed expenses and hardly anything variable because the CMC cost of the machine, the cost of the personnel which are engaged to report as well as the cost of running the center like rent and other charges lik e electricity, all these costs are fixed.

So, for pathology, most of the expenses would be variable whereas for radiology, it's the reverse – most of the expenses would be fixed.

Harini Dedhia: For the quarter, of the Rs. 27 crores odd that are other expenses, what is the rental?

Sachin Salvi: I am not having the exact number as of now. Maybe we can take that separately.

Moderator: The next question is from the line of Rahul Agarwal from InCred Capital. Please go ahead. 0Thyrocare Tests you can trust

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Rahul Agarwal: First question is on CAPEX . Rs. 41 crores was spent in fiscal '23. Could I understand where was this spent? And fiscal '24, you mentioned Rs. 20 crores is what you allocate for international expansion into second half. What would be the total CAPEX for the year including t he Indian operations?

Rahul Guha: To answer your question, a large part of the CAPEX, Rahul, of FY23 would have gone in the regional labs. We would have set up 6 regional labs through the year. Each lab roughly costs us between Rs. 5 crores to Rs. 6 crores as the full setup. Additionally, we had set up, I think, 2 nuclear centers during the year, new centers. But I would say the large part of the CAPEX, Rahul, has been in setting up of new labs.

Rahul Agarwal: For fiscal '24, Rs. 20 crores international plus domestic, how much will that be?

Rahul Guha: It would be in the range of Rs. 40 crores again because, as I had guided earlier, we are not planning too many more new labs. I think only our Goa lab and maybe a couple of satellite labs will be there. And then in nuclear, we are shifting a couple of labs. That is the main CAPEX that we are forecasting in India. If we do any material CAPEX, I would imagine it would go in the international side.

Rahul Agarwal: One question was on the international foray again. Will the P&L balance sheet see any material change in the short term? And let's say most of that actually comes in second half, will the P&L

balance sheet have some kind of OpEx impact on EBITDA level or on the balance sheet side purely from an investment perspective? How should we see that going into next year?

Rahul Guha: Difficult to say at this point, Rahul. We are still in the MoU stage the CAPEX feasibility and all of that. What I can say is that obviously as a new business, it will not deliver a 30% EBITDA from the get-go, but again, as I said, we are only allocating Rs. 20 crores across 3 geographies, it would be about Rs. 6-7 crores in each geography. If I look at the overall revenue and operating cost of those labs in those cities, I don't think it would have a very material impact in the second half of the year on a Rs. 500 odd crores base. Let's see how it plays out. But it is very early for me to be able to tell you how much of this will really hit the P&L. But all I would say is they are small investments. Actually, to enter an African market is almost as much as what we

would

have spent to set up our radiology center in Ghatkopar in partnership. Let's see how it plays out.

Rahul Agarwal: Thirdly, MCGM, the business goes off from Feb. What was the annual revenue in fiscal '23 from that business?

Rahul Guha: It would have been about Rs. 12 crores, roughly Rs. 1 crore a month.

Rahul Agarwal: And lastly, is nuclear a growth business or an annuity business for us? Obviously, fiscal '23 looks like a stellar year, but I think that's more recovery from a lot of issues we had. How should we look at that going forward? Should we expect a similar growth rate, 25% to 30%, or it stabilizes here only? 0Thyrocare Tests you can trust

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Rahul Guha: I think you should expect it to stabilize. A large part of this growth has come from the centers that were not operational because of different issues over there. To us, resolving those issues and getting those centers operational, I think going forward we would expect two more centers where we would get them operational. That is on a base of 10 centers, you can say. One might expect two more centers to get operational in the second half of this year. That is how I would look at it. But I would not anticipate this kind of growth rate coming into the next year.

Rahul Agarwal: So, essentially 10+2. Let's say second half would have like 10% volume impact purely from new centers. And is there any pricing here or pricing is largely stabilized?

Rahul Guha: I think pricing has largely stabilized. It's not easy to raise price in this market.

Rahul Agarwal: So, we should think it as an annuity business going forward. Is that correct? Like 10% to 12% of pure pure scan growth is what we should predict. Is that correct?

Rahul Guha: Yes. I would say think of it as a more organic business.

Moderator: As there are no further questions, I would now like to hand the conference over to the management for closing comments.

Rahul Guha: Thank you everyone for the questions and spending the time with us. Some of the questions, particularly it seems like our international strategy has evoked quite a bit of interest, and as we progress on that in the coming quarters, I will share more details around it. The base business, as I have shared, our franchisee business and our partnerships are going well, and we are very encouraged by the results in the last quarter as well. Let's see how the next year goes. On the quality front, as I have shared, we made a lot of progress. The validation from the doctor community also has been well received and we hope to anchor our communication around that, i.e., Thyrocare trusted by at least 9 out of 10 doctors in the country. I look forward to next year and our continued engagement. And with that, I will hand over to Pratik to close the call.

Moderator: On behalf of Thyrocare Technologies Limited, we conclude this conference. Thank you for joining us and you may now disconnect your lines. 0Thyrocare Tests you can trust

Call: Aug 2023

August 04, 2023

The National Stock Exchange of India Limited BSE Limited
Exchange Plaza, Phiroze Jeejeeboy Towers Bandra Kurla Complex, Dalal Street,
Bandra (E), Mumbai - 400 051 Mumbai- 400 001 (SYMBOL: THYROCARE) (SCRIP CODE 539871)
Dear Sir/Madam,

Sub: Transcript of post results earnings
conference call held on August 01, 2023 with
Analysts / Investors

Ref: Disclosure under Regulation 30 of the Securities and Exchange Board of India
(Listing Obligations and Disclosure Requirements) Regulations, 2015 ("SEBI Listing
Regulations")

Pursuant to Regulation 30(6) and 46(2) read with Part A of Schedule III of the SEBI Listing
Regulations, please find enclosed herewith the transcript of earnings conference call with
Analysts and Investors held on August 01, 2023.
We wish to add that the same has also been made available on the website of the Company
(www.thyrocare.com).

This is for your information and records.

Yours Faithfully,
For Thyrocare Technologies Limited,

Ramjee Dorai
Company Secretary and Compliance Officer

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"Thyrocare Technologies Limited
Earnings Conference Call"
August 01, 2023

MANAGEMENT : M R. RAHUL GUHA – MANAGING DIRECTOR AND
CHIEF EXECUTIVE OFFICER – THYROCARE
TECHNOLOGIES LIMITED
M R. PRATIK HIRE – STRATEGY TEAM – LEAD
INVESTOR RELATIONS – THYROCARE TECHNOLOGIES
LIMITED
M R. ADITYA SHINDE – VICE PRESIDENT , FINANCE –
THYROCARE TECHNOLOGIES LIMITED

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Moderator: Ladies and gentlemen, good day, and welcome to Thyrocare Technologies Limited Earnings
Conference Call. As a reminder, all participant lines will be in the listen-only mode. There will
be an opportunity for you to ask questions after the presentation concludes. Should you need
assistance during the conference call, please signal an operator by pressing star then zero on
your touchtone phone. Please note that this conference is being recorded. I now hand the
conference over to Mr. Pratik Hire from Thyrocare Technologies Limited. Thank you, and over to you, sir.
Pratik Hire: Thank you, Dorwin. Good evening to all, and thank you for joining us today for Thyrocare's
earnings conference call for the first quarter of the year FY '24. Today, we have with us Mr.

Rahul Guha, MD and CEO; and Mr. Aditya Shinde, VP Finance, to share highlights of the business and financials for the quarter. I hope you have gone through our results release and the quarterly earnings presentation, which are now uploaded on the stock exchange website. The transcript of this call will be available in a week's time on the company's website. Please note that today's discussion may be forward-looking in nature and must be viewed in relation to the risks pertaining to our business. After the end of this call, in case you have any further questions, please feel free to reach out to the Investor Relations team. I now hand over the call to Mr. Rahul Guha to make the opening remarks.

Rahul Guha: Thank you, Pratik. Welcome. Good evening and a warm warm welcome to all on the call. Thank you for making the time out on the evening time to join us. My name is Rahul Guha, I'm the MD and CEO of Thyrocare and will take this opportunity to present the Q1 results of FY '24. As Pratik said, I'm joined with Aditya Shinde, who is our VP Finance, who will take us through the numbers and Pratik, who is part of our strategy team and leads Investor Relation.

As always, before we begin, I will start with the quote from the Mahatma. The future depends on what you do today. And so, I wanted to give some of the key highlights of what we've been up to in the last quarter. I'll start off with some good news. We have received payments of INR38 crores from NHL for the testing done during the COVID period. We still have an outstanding of about INR13 crores, which has been provisioned in the book.

Another big change that we implemented this quarter is a pay-for-performance pricing structure. Earlier, our discount structure was one size fits all, but now we have moved to a slab-based pricing model, which we implemented in May of 2023. This has led to an increased synergy with our franchisee network with motivation to move up volumes and enter higher slabs. This has resulted in both volume and value growth in this quarter.

Our efforts in our franchisee business to go deeper into India with a focused test menu continues. And this quarter, we added 385 transacting franchises. We also continue to selectively expand our offerings. As I shared last quarter, we launched Jaanch packages, which is our investigative or sickness brand. Just to remind all of you, our Jaanch lineup is targeted towards lifestyle challenges or for you to better understand your health. We have solutions across this spectrum for anything you might be worried about.

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Is it fever or something more serious? Why is my hair falling, might be I have cancer through our cancer screening as well as deep investigations for common chronic diseases like diabetes, heart ailment, PCOS, amongst others. Jaanch is already doing INR70 lakhs of monthly business, and we also launched Troponin I Heart Attack Risk to assess the early risk of heart attack as part of our Jaanch series. In addition, we have revamped our Gynec portfolio and relaunched it under the name

of Her Check.

So that's on the Jaanch for packages front. We've also revamped our equipment platform. Our equipment was quite old with an average life of 12 years. We have added 24 new machines from our vendors in the last year, bringing our average life span down to 6 years. These new additions have dramatically improved our reporting accuracy and turnaround time. As I shared, we have equipment-financed these machines over a period of 5 years.

In the partnership business, unfortunately, we had a setback due to the degrowth in API business and the closure of the MCGM contract. The remainder of our partnerships business has been steady. And as we add new partners and continue to bid for tenders with the government, I think we will be able to close this gap shortly. On the international expansion side, we are on track to start operations in Africa before the end of this financial year.

As we get into the results, I wanted to share with you a few highlights or main pointers before Aditya deep dive into the same. Our franchisee business, which accounts for 2/3 of our business showed a strong revenue growth of 16% year-on-year. However, our partnerships business saw a decline. As I mentioned earlier, largely because of API and the closure of our MCGM contract.

The API Group revenues and MCGM together account for INR6 crores of the degrowth in the partnerships business. Some of the other health tech partners also faced degrowth. However, that gap was filled by onboarding new clients. And overall, the partnership business has remained steady. Our margins have expanded this quarter, primarily due to gross margin expansion because of the improved efficiency of the new machines, exit of lower-margin government business and our costs have remained controlled year-on-year with both manpower and overheads more or less flat versus last year. Our radiology business did a revenue growth of 30% year-on-year. And overall, we did a 6% year-on-year revenue growth.

With that, I'll hand over to Aditya to cover the results.

Aditya Shinde: Thanks, Rahul. Good evening, everyone. So, I'll briefly update you about the key highlights of Q1 FY '24 financial performance. But before we get into the details, I'll reiterate something

we've been telling for quite a few quarters now about the ES OP program that we've been mentioning. This program has been introduced by the API to retain talent at Thyrocare. The ESOP of parent company will invest over a period of 6 years, out of which one has already passed, so we have 5 more to go. And we are recognizing the same as per the India's IFRS as a book entry towards the share-based payment. The total value of the ESOP is close to INR45.53 crores to be prorated over a period of 6 years. Out of this INR45 crores, we already provisioned INR24 crores. So, there is only halfway to go.

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But one thing we all know that this is a cashless charge being an ESOP. It is no way affecting my cash flow. And as for Ind AS options are valued at the grant date as per the Blacks and Scholes, which is a charge to the P&L over the vesting period. The breakup of this INR45 crores year-wise is given in our quarterly presentations. As these are not operating expenses and do not impact the cash flow of the company, we have normalized EBITDA, which takes into account along with the provision for RTD. Along with this, I'd also like to say that this is not -- this doesn't dilute the Thyrocare equity in any way. So even after the 5-year period, whenever the ESOPs are vested, it will not impact the Thyrocare equity in any way. To go into the performance, our overall revenue saw a growth of 6% Y-o-Y. This is consolidated. And this has been driven by a 16% growth in our base franchise business. As far as EBITDA margin is concerned, our stand-alone EBITDA at a normalized level stands at 33% versus 29% same quarter last year. The EBITDA margins in our Radiology segment stood at 12% versus 23% same quarter last year. There has been a dip in the EBITDA margins in Radiology because of increased CMC costs and reduced gross margins. While the consolidated normalized EBITDA margin stands at 31% versus 28% same quarter last year, which is a 15% Y-o-Y growth in EBITDA margins. With these brief highlights, I'll pass it back to our MD and CEO, Mr. Rahul Guha, for the strategic updates. Thank you.

Rahul Guha: Thank you, Aditya. I'll just once again take a few minutes to recap our strategic direction, and then we'll go into Q&A. First, as always, our value proposition to the customer. We will continue to remain an affordable option to all patients with good quality and on-time reports. All of our efforts on our value proposition is towards ensuring this low cost to the patient, assurance on quality of testing to our certifications and engagements with doctors. We have made substantial progress on this, which I updated in my initial comments and is also reflected in the presentation in the chapter on raising the bar on quality. This remains at our core and will guide all that we do. Second, our strategy. We continue to remain the B2B partner of choice to all front-end diagnostic services companies in India, whether it is a small diagnostic centre in a semi-urban area, a pharmacy in the metro, a small nursing home, an individual doctor or a leading online diagnostics platform or health tech marketplace. We are happy to provide low-cost, robust testing solutions to ensure that they can serve their patients in the most effective manner. If they require phlebotomy, we are happy to mobilize our phlebotomy network of almost 900 companies and 400 networks phlebotomists to serve them better. This strategy has been working well for us with our franchise business at an all-time high. And hopefully, the headwinds in the e-commerce space will abate, and our partnerships business will be back on a strong growth trajectory. As I outlined last time in our strategy, we have 3 key pillars of growth. The first is our franchise business. The focus is to take our franchise business deeper into India. As I've shared, we are actually largely Tier 2 plus as an organization. And we will continue to go deeper into India with a focused test menu and provide our clients with a frictionless experience to transact with us and provide their customers the best testing experience that they can.

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We will continue to focus on private as well as public partnerships. In the public space, we will focus on TB, an infectious disease, where we are by far one of the strongest players in this segment. Additionally, we will continue to expand our partnerships across health care companies, hospitals and other health services companies to enable them to provide diagnostic testing to the customer. The third area is new for us. As I said, we have a strong and robust B2B model with a core of execution. We believe there is a lot of opportunity to take this model internationally, and we will start to look at emerging markets where we can bring this model to provide affordable testing into these emerging markets. As I mentioned, we are on track to have our operations in Africa start by the end of this year.

And I'm hopeful after that, we will look at newer opportunities to expand the international business. That in a brief is our mandate as management. Thank you for giving us a patient hearing. I'll once again end with the quote from the Mahatma, which I always do. Find purpose, the means will follow. And as always, our purpose remains to provide affordable, high-quality testing to the masses. With that, we'll open up for Q&A.

Moderator: Thank you very much. The first question is from the line of Rahul Agarwal from Incred Capital.

Rahul Agarwal: A few questions. Firstly, on the partnership. Is it

his the new normal? Like should we assume

INR30 crores, INR32 crores of run rate going forward as sustainable?

Rahul Guha: I think, look, I don't think the dip is -- I would say, I think INR32 crores is an anomaly because we haven't had the time to plug the government contracts since we lost the MCGM contract. So, I would say in the INR35 crores range would be the new normal.

Rahul Agarwal: Okay. I get it. Secondly, on the price hike, the presentation mentioned about some hikes taken in the quarter. Just wanted to clarify, was this like done on specific test or it was largely a mix impact?

Rahul Guha: Actually, as I mentioned around at the beginning, we moved from a base of performance structure. So earlier, we used to have a universal discount regardless of whether you were a small client or a large client. So, what we have done is move to a greater discount or a slab-based discount structure. So as a result, some of the smaller clients saw a higher price hike and the larger clients did not. So that's what you see there. So, it's not really mix, it was across the board, more determined by how large a client you were.

Rahul Agarwal: Got it. So, this -- the number should continue it as well, right, because the system has changed permanently. So, this should be retained by the company, right?

Rahul Guha: Yes.

Rahul Agarwal: Okay, got it. And just purely on MCGM, when did this contract actually close? And what was the revenue number for fiscal '23?

Rahul Guha: Okay. So, it was roughly INR1 crores a month, and it closed on 4th April.

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Rahul Agarwal: Okay. And is it worth discussing Pulse Hi Tech financials? I think it's consolidated nuclear. I know it's very small, but any -- what is the contribution and the potential ahead for this business?

Rahul Guha: See, it's only 1 centre, as I have shared in the last couple of calls, it's a pilot that we are doing in the Radiology business. It's too small to spend time on it just now.

Rahul Agarwal: Got it. And last question is on tax rates. The effective tax rate for the quarter is 31%. Any adjustments done here? Or is it like it will come back to 25% for the full year?

Rahul Guha: Aditya, do you want to take that?

Aditya Shinde: So yes, there have been a deferred tax one-off entries this time. It should normalize to 25%, 26% plus or minus 2% going ahead on a sustainable basis.

Rahul Agarwal: What was the quantum of the adjustment, please?

Aditya Shinde: So, the net tax asset has been roughly INR3.5 crores to INR4 crores taken this quarter.

Moderator: We have the next question from the line of Rahul Agarwal from Incred Capital.

Rahul Agarwal: Sorry, I thought I'll continue with the conversation, did the point you guys enter the queue. One, on the commentary, Rahul, you mentioned some of her health tech platforms apart from API also degrew, could you explain how the market actually shaping up because these are the guys who are competing pretty hard, both on B2B, B2C and the entire industry had some issues around pricing and competition. Could you just elaborate a bit what is really happening in this part of the market?

Rahul Guha: Yes. See, I would not want to name specific companies. But I would say the partnerships are -- it really depends on what is the nature of the partner. I would say the more services-driven partners have been largely stable. The more product supply-driven partners, that is where the dip is coming.

Rahul Agarwal: Okay. Got it. So, it's more like pure product aggregators have degrown. Is that correct?

Rahul Guha: Yes. I would put it that...

Rahul Agarwal: And on Africa, you want to talk a bit more as in -- obviously, it starts by year-end, but how are we going about building the network there? Like have we hired people, how are we building the network there?

Rahul Guha: So, in Africa, there is a team on the ground right now, finalizing the lab location

and placing the

equipment orders. We have a daily partner who is actually helping us with the market development part of the business. They are a very large distributor in that country. And so, they

will be helping us with the market development. We are in the process of actually hiring that sales team at this point in time.

Moderator: The next question is from the line of Kahlil Randeria from Ruliad.

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Kahlil Randeria: My question is really on the strategy of going to Africa. And is this the first of others to come? And so, let's start with that, and then I can take the second one.

Rahul Guha: Sure. I had -- in the last call, Kahlil, identified 3 regions that we would be looking at. Africa was one. Middle East was the second and Southeast Asia was the third. So, our foray in Africa, the first foray in Africa is this year. Once we are able to wrap our minds around this part of the business, then we would look to the Middle East.

Kahlil Randeria: Okay. And I would imagine you would have some sort of an advantage over here, which is why you wanted to get in here. And if so, if you could just remunerate a few of them.

Rahul Guha: Sure. So one is, actually, it is about replicating our India competitive advantage, which is low-cost, good quality testing through a centralized network at high scale, right, that allows us to bring the prices down substantially in these markets. And that is the strategy that Thyrocare has been built on. We are effectively replicating that same strategy in Africa where there is a large out-of-pocket paying population.

And so therefore, they are always looking for an affordable option. And it allows us to take the India model, which has worked very well into that market.

Kahlil Randeria: So, pricing would be similar over there or it's much higher?

Rahul Guha: Pricing is much higher currently. On average, the pricing from our market study has been almost 3x India rate. But the cost structure also tends to be higher. But we believe with the large-scale centralized lab, we should be able to go in and bring cost-to-the-patient down substantially.

Kahlil Randeria: Sure. And in terms of working capital, is there anything over here? Or would it all be direct to you? Would you have an intermediary and then you will have a working capital need to be met over there?

Rahul Guha: So, we are setting a JV operations there. So, the invoicing will happen in the JV. To a large extent, when you're running a B2B model, particularly with a focus on hospitals and servicing other diagnostic labs for their outsourced need, you require a certain amount of working capital, but it is in the 30 to 60 days range, not longer than that.

Kahlil Randeria: All right. My second part of the question is really on -- you had mentioned that there is a -- you have reset the EBITDA margin to a certain level. And you also say that you continue to remain to be a low cost, I would imagine one of the lowest cost providers of this business. How do you expect that to play out over the next 24 months, given that you had substantial time that you spend with Thyrocare already so far?

Rahul Guha: Sure. Although it's been 14 months at Thyrocare, yes, look, if you see quarter-on-quarter or year-on-year, if you compare versus quarter 1 of last year versus quarter 1 of this year, our manpower cost or our employee benefit expenses and our other expenses are more or less flat. We have not allowed any or very tiny increase in that cost structure. And therefore, with the pricing strategy that we have taken in the market and all of that, we've been able to also improve the EBITDA margin quite healthily, I would say, over the last year.

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Of course, one should not expect this kind of trend as we move into the next year. But I do believe that we will continue to remain focused sharply

on our costs and ensure that operating leverage comes through.

Kahlil Randeria: So, would you -- earlier when Mr. Velumani was running at, it was more like a 40%-odd kind of a number. Is that anywhere on the horizon? Or do you think this is more or less a reset permanently?

Rahul Guha: So, as I said, when it was a 40% EBITDA business, and I think this quarter, we've executed Thyrocare stand-alone at 33%, right? So, the gap is about 7%. That investment in EBITDA has gone into 2 areas. One is expanding our lab network. And so earlier, we had a much more consolidated lab network. I think in today's competitive environment, turnaround time of at least

24 hours is required or turnaround time, if you see in the presentation, could go as high as 48 hours. So that has been 1 investment that we have made in ensuring that we are able to bring our all-India turnaround time within 24 hours.

That results in higher rental costs and all of that as -- in our cost structure. And the second investment that we have made is largely on the quality side. Actually, very few of our labs had pathologists. Today, all our labs have pathologists overseeing the operations and are able to interact with doctors so that if they have any queries or complaints, they can address them in a timely manner.

And that cost structure is an investment that we have made. So, we've made 2 large investments.

One is in our lab network and the other is in our quality, particularly when it comes to having pathologists in each of our labs. So, I think if you were to ask me, will we go back to 40%? I think that will be a difficult journey.

However, I do see scope for still some margin expansion from where we are today.

Moderator: We have the next question from the line of Saurabh Shroff from QRC Investments.

Saurabh Shroff: My first question is on the ESOP. I'm not sure if all have covered it in the past, but any reason why the management is taking up in unlisted parent rather than the listed operating company?

Rahul Guha: That is how it was structured at the time. So, I mean, that was the parent company, and the time was to be listed very shortly. And it was felt that instead of diluting Thyrocare shareholders who, at that point, the management was quite new into this organization. It was felt that it would be best to, how do you say, create these or pool at the parent. As you know, Thyrocare has an ease of pool already in place, but it is quite limited in the number of options and the -- it was also quite restrictive in terms of the grant period and the exercise period and so on. So therefore, overall, I think the group decided to grant options at the group level. And that was a call taken at that time.

Saurabh Shroff: So, if for whatever reason, the parent does an IPO, there is still no fallback or the management to get compensated for the good work that they will do with Thyrocare in any way? I mean, essentially, the management is running the risk of not getting compensated for whatever reason, the IPO doesn't happen. Is that fair to understand?

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Rahul Guha: Yes. I would say, yes, that could be a risk overall.

Saurabh Shroff: Okay. And secondly, I guess just circling back on the slowdown in the business from the parent, you said that you expect this to normalize over time. I didn't catch the run rate that you think -- that you mentioned that we should sort of work with.

Rahul Guha: So, I had said we should normalize at about 35%. Actually, that is because we have been bidding very aggressively for a number of government contracts and some of them hopefully will come through in this quarter or the next. I anticipate that the API group level, it will remain at the level that it is, which is roughly about INR4 crores a month. I think that is the range that it will remain as it used to be about between 5% and 5.5% in its peak. But I expect -- I don't expect it t

o move below 4%, but I think it will be a while before it recovers back to 5%.

Saurabh Shroff: Okay. And on this partnership business, what is the split between government and nongovernment, is it sort of -- I know there has been a runoff on the government business, but typically, is this something that you try to sort of solve for? Because I would imagine the government business is not at the same margins, right? Or is it?

Rahul Guha: No, the government businesses at a substantially lower margin. Actually, in that -- after the COVID, business that we had done, the only government business that we had was our TB business, which continues and the MCGM contract. MCGM, as I shared, was roughly INR1 crores a month, and our TB is roughly about INR50 lakhs, I think, a month. So, you could say about INR4.5 crores in that INR32 crores is our government business -- in that INR38 crores is our government business, of which INR3 crores is completely gone.

Saurabh Shroff: Okay. Got it. All right. So that's why you are saying that it should come back to that 35% kind of a number.

Moderator: The next question is from the line of Amit Aggarwal from Leeway Investments.

Amit Aggarwal: Just a have a general question. What is the business model adopted advanced companies like U.S. or Japan, because I don't think they have a system of calling it cleaning and asking for diagnostics test. Do they buy test s over there? Or what is the future of this business model 20 years ahead or 10 years ahead?

Rahul Guha: So, I'll take up the question a little bit, Amit. I think in the U.S. and in many developed markets where the entire health system is reimbursed, right, actually, a lot of the diagnostic testing is largely controlled by the -- not -- I wouldn't say control, but largely mandated either as part of the annual health checkup of your insurance company or with the doctor, right? And that is because those are largely reimbursed economies.

In India and a few of the emerging markets that I highlighted, it is very much a self-pay market, where effectively you go and pay for your doctor visit, you pay for your insurance -- sorry, you pay for your diagnostic test, you pay for your medicines out of pocket, right? And in this kind of economy or in this kind of health care system, actually having good diagnostics plays a very critical role because the early, as they say, a stitch in time saves 9. We all know the difference between diagnosing diabetes early versus late cancer versus early versus late.

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So at least it is my view that in the emerging markets, diagnostics will continue and for as long as I can see it, play a very critical role in helping patients identify diseases early, helping

governments also manage the cost of health care with early diagnosis and then better treatment.

So, I don't think -- I do believe the business model of diagnostic testing out of pocket is, of course, largely prevalent in emerging markets as is branded generics and other health care nuances that are largely related to out-of-pocket markets. But I think in out-of-pocket markets, you will see that diagnostics plays a very critical role.

Amit Aggarwal: Because my worry is that I read in the descriptor that Google also entering into this testing. And soon, they will have an AI model which replaces all the testing in the future. Is it possible? And can I buy some kit in the future for testing myself the cluster or diabetes or something like this?

How can -- and this market could shrink, calling somebody to get a test done at home?

Rahul Guha: No. See, many of these tests still require final confirmation through a diagnostic test. So if I take the example of HbA1c, you can get HbA1c kits that are very much like the rapid kits that we use during COVID, where you can do a skin prick and get a reading of your HbA1c but in 99% or 100% of those cases, if you have elevated HbA

1c or even normal HbA1c, most people will recommend that you do a confirmatory test, right? And some of these are, I would say, more

screening rather than confirmatory technologies.

So, I think if you're a diabetic, you will need to do an HbA1c once every 6 months for the rest of your life. Yes, the first time you may be able to do a quick iPhone camera and see that you may be diabetic or at risk of diabetes. But the testing life cycle is very, very long when you're a chronic patient.

Amit Aggarwal: But because in India, we did for general testing and after 6 months or 1 year, is just a normal test over all the problem solution have in the bordereau, can this market shrink over market in shrink, is a possibility, what is the chance of that?

Rahul Guha: See, I think we are far away from a single technology like a phone-based platform, replacing all the possible health checkup -- all the possible health parameters in our health checkup. If the technology could be advanced to a point where you may be able to do screening for very specific

1 or 2 largely diabetic parameters, right, but I think we are a long way from an annual health checkup happening on the phone.

Moderator: The next question is from the line of Aditya Khemka from InCred Asset Management.

Aditya Khemka: Just a question on the partnership business again. Just help us understand a little better. We understand that it has dipped. But why would the API platform revenues dip from the previous subdebt of INR5 crores a month to INR4 crores a month now? I mean what was the problem or the challenge that, that business faced?

Rahul Guha: As I said earlier as well, I think there has been a movement towards a part to profitability as which actually most of our health tech platforms. So, I think the burn model, or the deep discount model has pulled back substantially, right? And as a result, volumes have come down. And

actually, I've seen that across, as I shared, across most of the aggregator or product-based

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marketplaces where they have pulled down substantially the discounts. And as a result, we have seen a bearing down on volumes.

Aditya Khemka: Got it. So, it's mostly volume-driven. And I can see that data in your presentation as well. Okay.

So, API, understood. What was the challenge on the B2G side? Because given the numbers that you have spoken, the decline is not only in the API business, there is a decline on the B2G business as well. So, could I understand a little better on what's happening there?

Rahul Guha: That is, as I mentioned, the MCGM tender, which used to account for about INR1 crores a month of our business, that tender when it came for renewal, the pricing that was what was there was we had quoted our best possible rate protecting our margins. And we lost the tender to go below that, what we had bid didn't make sense for us. So, we didn't pick up that tender. Also, given that we had so much government receivables in the last quarter, we were very cautious about what we would bid for and what we would go after.

I think now with a lot of the government receivables having come in and with line of sight on at least many of our payments, I think now we are more comfortable looking at this business and seeing how we can participate most strongly.

Aditya Khemka: Got it. So, excluding API and government business on the B2G channel, the numbers seem to then flatten out, right? Because you lost INR1 crores a month on the government and INR1

crores a month on API. So that INR6 crores that we can see Y-o-Y, that I understand. Now

coming to the rest of the B2G business, excluding government and API. Why was there no growth? It was Y-o-Y flat. So, I mean, even if you haven't added partners, your existing partners

would have given you a higher volume Y-o-Y, right? So, what exactly went wrong there?

Rahul Guha: Yes. So, as I mentioned, API is one of our pa

partners, which I can talk about because in a way, they are our parent. But we had a multiple of other partners whose volumes also pulled back, right? But we were able to cover it up by adding new partners and getting that business flat. As you know, Aditya also -- Q4 tends to be the best quarter for partnerships because we get into the annual health checkup and all of that. So, the fact that we were able to maintain between the non-API, non-B2G partnerships at the same level as Q4 of FY '23 was also reasonably good.

Aditya Khemka: Yes. No, I get that. Now coming to the B2C business, right? So, I see a topline growth of about 15%. So that INR6.5 crores going to INR7.4 crores but I see a workload growth of about 30%, so 1 crore samples probably going to 1.3 crore samples, which means realizations dropped there, right? I mean so in an environment where health tech companies are reducing discounts and moving to profitability, on our B2C franchise, it seems our realizations are dropping. So, I just want to understand a little better what the strategy there is?

Rahul Guha: No. Actually, that's -- I can explain it. I'll try to simplify it as much as possible. One of the things that we launched was upsell for our D2C platform. So, if you place an order on Thyrocare, typically, this level would ask you if you would like to add 1 or 2 additional tests at the time of collection, those tests get that collected in another sample. So effectively, what you're seeing there is the additional tests getting added on, which is getting reflected as higher volume.

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But if there was a INR1,000 package and you added a INR300 test at the time of collection, yes, on a per-sample basis, the realization has gone down. But you will find on a per-patient basis, our realization has gone up.

Aditya Khemka: Right. So, the right way to look at it is not really on a per sample or a workload basis, but to look at it on a per-patient basis?

Rahul Guha: Yes.

Aditya Khemka: Maybe that should be the rate we should be sharing in the back, right, in terms of per patient, but we... I understand the conundrum there. Yes, I get that. So, Rahul, given that we have done only 6% topline growth for various reasons in the first quarter, so how would you guide us to your FY '24 topline? Market doesn't seem to be growing at what we thought was 10% growth from the market. Your pincode-driven growth seems to be there because your franchise business is growing, but not at the rate that we had thought. So, what would you believe would be your guidance for FY '24 in terms of the topline?

Rahul Guha: See, there, I just -- I mean our franchise business grew 16% year-on-year, right? If you look at - and if you see our franchise business, it has even grown over Q4 of last year. So, both, if you look year-on-year, it was 16% and Q4, which typically tends to be the best quarter, we've done better than that in this quarter as well. So, I am quite bullish about the franchise business and its prospects.

I would expect to sustain the growth that we've seen in the first quarter through the year. I expect that in the partnerships, if we can get back to a 35%, 36% average, right, I would be happy of where we are. So, I think the weighted average of that would probably land you at about 12% and 14% overall.

Aditya Khemka: On 40%. And in terms of margins, I heard you say that there is still scope for improvement, but I must comment to you on the margins that we are doing, at least on an adjusted basis once we call out the soft costs. So, on the margin side, 31% for this quarter, am I right? What would you say would our full year margins look like?

Rahul Guha: I would say, in that range, Aditya, plus minus 1%.

Aditya Khemka: Got it. One last question. So, you mentioned you received some money from the government, and I believe some of this was already provisioned for. So, I don't

see a write-back of provisions in the financial statement. So can the CFO, please help me on this front? What's happening there?

Rahul Guha: I can take it, and I'll ask Aditya to add on. We are still waiting for -- as I said, there is still some payment left, right? So, the entire receivables now has been provisioned. We're just waiting for the final closure on the payments to evaluate what is the full write-back. We wanted to do it in 1 shot. So, we're just waiting. I think we'll get complete clarity at the end of this quarter, and we'll take a call then. Aditya, you have anything to add?

Aditya Shinde: Yes, that's right. So, we are fully provisioned. In fact, we are overprovisioned. But then before we write back and reverse these provisions, we would like to wait at least 2 quarters to get a full

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view of how much is the final -- full and final settlement coming from the government parties, especially the NHM project. And once that visibility is there, we'll be proceeding with the write-

back or reversals of these provisions. But no more province or...

Aditya Khemka: Aditya, is this a good quarter, we have provided another INR1.3 crores. So, is this not related to the government?

Aditya Shinde: This is partially related to the government. So typically, we provide this is an ECL model and basis our alignment with the statutory auditors. So, basis that INR1.29 crores is towards -- some part of it is towards government because we still have INR9.9 crores of government receivables.

And a part of it is also for the corporate business as we've been subscribing to. So, yes.

Aditya Khemka: Sorry, I have one more question. Can you guide us to your full year capex number for this year?

And what is our current cash or net debt position, if you can help us with that?

Rahul Guha: Aditya, you can take that.

Aditya Shinde: Sure. So, for FY '23, we did close to INR40 crores of capex. In FY '24 as well, we are expecting a...

Aditya Khemka: You are guiding for FY '24, right?

Aditya Shinde: Yes. I'm guiding for FY '24. So, FY '23, we did INR40 crores. FY '24, we should be doing close to INR45 crores, out of which, in Q1, we have already spent INR1.8 crores, right? So, balance will be spent largely towards maintenance capex, INR20 crores of INR15 crores and another INR10 crores for overhauling our IT infrastructure and large physical infrastructure plan India, another INR10 crores for the international expansion. So overall, INR45 crores is what we are budgeting this year towards capex. And out of that INR45 crores, INR10 crores has been incurred in Q1 '23.

Aditya Khemka: Right. And the current cash position?

Aditya Shinde: So currently, we are sitting on a cash of INR 80 crores, both Thyro and ESL put together.

Aditya Khemka: 8-0?

Aditya Shinde: INR80 crores, correct. That's right. And these are marked in the debt mutual funds, essentially.

Aditya Khemka: So, Rahul, I have one more because I hope there are no people waiting in the queue. On the B2C side, obviously, when we look at the other companies, which are listed diagnostic labs, they have a fairly profitable B2C business model. Could you talk to us a little about what initiatives we are taking on the D2C side?

I know it's a small baby right now, but this model definitely has potential. So, can you talk to us a bit about what you're doing? What are the initiatives they're taking here? And how would you -- what is your right to win in the D2C space?

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Rahul Guha: Yes. So, on the D2C side, to a large extent, we are learning from our partners in the B2C side of the business or the partnerships where all of them run a number of health tech platform. So, we are learning from all of them. For example, most of the growth that you've seen over the last year in our D2C has come largely organically, mostly through

remarketing, which we have effectively learned going back to our old customers, being able to mine the data and see when it

is time for them to retest.

That has worked very well for us. The other was what we call multi-patient or adding more patients to the same order. And as I talked about also what we call upsell, which is at the time of collection, asking the patient, if they would like to add any other tests and so on. We have still not reached the point, Aditya, where we think it is -- it makes sense to put, how do you say, marketing dollars behind this because the ROI is still -- the cost per lead acquisition on Google and Facebook and all of them still remains exorbitantly high.

So, we still haven't invested in the, what we would call, marketing burn model or the marketing-led model. We are trying to maximize the organic traffic that we have right now and also leveraging our franchise business, along with our online business to be able to service orders deeper and deeper in the country.

Moderator: The next question is from the line of Rahul Agarwal from Incred Capital.

Rahul Agarwal: On the NABL accreditation, are we done largely this 20 or 31 or we are looking for more accreditation please?

Rahul Guha: I think 20 out of 31 are our largest labs. I think now we are at almost 85% of our samples processed in a lab -- in an NABL lab. What you should know also is NABL is a biyearly process.

So almost every lab will go for their NABL renewal, either this year or in the next year. So that keeps us on our toes. I don't anticipate to increase this number beyond 24.

Rahul Agarwal: Got it. And last question from my side was, you mentioned we're trying to recoup the partnership business through bidding for some new government contracts. I'm assuming and please correct me if I'm wrong, that these contracts will be actually bid by the partner and not by Thyrocare.

And hence, the entire billing risk is on them and not us. Is that correct?

Rahul Guha: No. In that -- in the government business, we have to participate as the principal. So, we will be billing, and the billing risk will be with us. But we are being cautious, as I mentioned, focusing on areas where we have a right

to win and are our strengths, particularly in TB, an infectious disease. And so that's where we have actually limited the scope of our participation in the government contracts.

Rahul Agarwal: Okay. So, this is not in partnership with any part ners, this is direct Thyrocare going and bidding for new contracts, which obviously you have been trying to be very conservative on this. Is that correct?

Rahul Guha: Exactly. That's correct.

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Moderator: As there are no further questions, I would now like to hand the conference over to Mr. Pratik Hire for closing comments. Over to you, sir.

Pratik Hire: Thank you, ladies and gentlemen. On behalf of Thyrocare Technologies, that concludes this conference. We thank you all for joining us.

Moderator: Thank you, everyone, for joining us. You may now disconnect your lines.

Call: Nov 2023

November 03 2023

The National Stock Exchange of India Limited BSE Limited
Exchange Plaza, Phiroze Jeejeeboy Towers Bandra Kurla Complex, Dalal Street,
Bandra (E), Mumbai - 400 051 Mumbai- 400 001
(SYMBOL: THYROCARE) (SCRIP CODE 539871)

Dear Sir/Madam,

Sub: Transcript of post results earnings
conference call held on October 31, 2023 with
Analysts / Investors

Ref: Disclosure under Regulation 30 of the Securities and Exchange Board of India
(Listing Obligations and Disclosure Requirements) Regulations, 2015 ("SEBI Listing
Regulations")

Pursuant to Regulation 30(6) and 46(2) read with Part A of Schedule III of the SEBI Listing
Regulations, please find enclosed herewith the transcript of earnings conference call with
Analysts and Investors held on October 31, 2023.

We wish to add that the same has also been made available on the website of the Company
(www.thyrocare.com).

This is for your information and records.

Yours Faithfully,
For Thyrocare Technologies Limited,

Ramjee Dorai
Company Secretary and Compliance Officer

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"Thyrocare Technologies Limited Q2 FY '24 Earnings
Conference Call"

October 31, 2023

M
ANAGEMENT : M R. RAHUL GUHA – MANAGING DIRECTOR AND
CHIEF EXECUTIVE OFFICER , THYROCARE
TECHNOLOGIES LIMITED
MR. PRATIK HIRE – STRATEGY TEAM & LEAD
INVESTOR RELATIONS , THYROCARE TECHNOLOGIES
LIMITED
MR. ALOK JAGNANI – CFO, THYROCARE
TECHNOLOGIES LIMITED
MR. ADITYA SHINDE – VICE PRESIDENT , HEAD
(FINANCE , ADMIN , SOURCING), THYROCARE
TECHNOLOGIES LIMITED

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Moderator: Ladies and gentlemen, good day, and welcome to Thyrocare Technologies Limited Earnings Conference Call.

As a reminder, all participant lines will be in the listen-only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing '*' then '0' on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Pratik Hire from Thyrocare Technologies Limited. Thank you, and over to you.

Pratik Hire: Thanks, Yashashri. A very good evening to all and thank you for joining us today for the Thyrocare Earnings Conference Call for the 2nd Quarter of the Year FY '24.

Today, we have with us Mr. Rahul Guha – MD and CEO, and Mr. Alok Jagnani – CFO, to share highlights of the "Business and Financials" for the quarter.

I hope you have gone through our results release and the quarterly earnings presentation, which has been now uploaded on the stock exchange website. The transcript of this call will be available in a week's time on the company's website.

Please note that today's discussion may be forward-looking in nature and may be viewed in relation to the risks pertaining to our business. After the end of this call, in case if you have any further questions, please feel free to reach out to the Investor Relations team.

With this, now I hand over the call to Mr. Rahul Guha to make the opening remarks.

Rahul Guha: Thank you, Pratik. Welcome. Good evening, and welcome to all on the call. Thank you for taking the time out from your busy schedules to join us this evening.

Quick introduction of those of us on the call. My name is Rahul Guha. I am the MD and CEO of Thyrocare, and I thank you for the opportunity to present the Q2 results for FY '24. I am joined with my colleague Alok Jagnani, who has recently joined us as our CFO. Additionally, Pratik Hire, who you heard, is part of our strategy team and leads investor relations and is with me along with Aditya Shinde who heads our finance, admin, and sourcing function.

Before we begin, as with all my calls, I will always start with a quote, but this time from Nelson Mandela in recognition of our foray into Africa. It is in your hands to make a better world for all who live in it, and we believe Thyrocare can bring our business model to Africa to make affordable and good quality diagnostics available to all.

With that, I will give you some key highlights of what we have been up to in the last quarter.

Before we get into the details of the quarter, I will reiterate our pay-for-performance pricing

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structure that we implemented at the beginning of this financial year. Earlier, our discount structure was one size fits all, but now we have moved to a slab-based pricing model, which we implemented in May 2023. This has led to an increased synergy with our franchisee network with motivation to move up volumes and enter higher slabs. This has resulted in both volume and value growth in the last two quarters.

As always, we continue to selectively expand our offerings. Aarogyam, which has been a flagship brand in the preventive healthcare segment. As I have discussed in previous presentations, we have two more brands, Jaanch, which is our diagnostic or illness package offering and Her Check, which is focused on women, and largely focused in the maternity and women's health space.

Jaanch, as I said, is targeted towards lifestyle challenges for you to better understand your health.

We have solutions across the spectrum for anything you might be worried about, whether it's fever or something more serious, hair fall, cancer screenings, as well as deep investigations for common chronic diseases like diabetes, heart health a

mongst others. Jaanch is already doing 1

crores of monthly business. We have revamped our Gynec portfolio and relaunched it under the brand name Her Check, which is focused on women's health.

Our partnerships business excluding API and the government business continues to do well as we grow existing accounts and add new partners. On the government side, I am very happy to share that we have been awarded contracts for TB testing in the state of Gujarat and Assam and the work has started from this month.

On the international expansion side, we have signed the JV agreement and the company has been incorporated in Tanzania. Our plan is to be operational in the Tanzania market by the beginning of the next financial year.

As I told last quarter, we have revamped our equipment platform. Our equipment was quite old with an average age of 12 years. We have added 24 new machines from our vendors, bringing our average age down to 6 years. These new additions have dramatically improved our reporting accuracy, performance, and

turnaround time. As you will see in the financials, we have financed these machines over a period of 3 years. As we get into the results, I wanted to share with you a few highlights or pointers before we deep dive into the same. Our franchisee business showed a strong revenue growth of 20% year-on-year and while our partnerships business saw a decline because of API and the closure of our MCGM contract. Our partnership business excluding API and B2G showed a strong revenue growth of 22% year-on-year. The API business while degrown year-on-year, has now stabilized quarter-on-quarter at a quarterly run rate of 12.7 crores, which is a growth of 5% quarter-on-quarter.

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Our radiology business did a revenue growth of 20% year-on-year, which includes our new foray into full-scale radiology through Pulse Hi Tech. Overall, we did a 10% year-on-year revenue growth on a consolidated basis.

With that, I will hand it over to Alok to take us through the results.

Alok Jagnani: Thank you, Rahul. Hi, all. Good evening. Thanks for this opportunity. I am very excited and thrilled to present my first financial highlights of Thyrocare. I will briefly update you about the key highlights of Q2 FY '24 financial performance.

Before we get into the details, I will reiterate about the ESOPs program that we have been mentioning in the last few quarters. This program has been introduced at group level to retain talent at Thyrocare. These ESOPs of parent company will vest over a period of six years and we are recognizing the same as per IND AS IFRS as a book entry toward our share-based payment and appropriately reflecting profit and loss account as an expense and balance sheet as an equity contribution from parents.

The total value of ESOPs granted is 45.53 crores over a period of 6-year period. But one thing we must note is that this is a cashless transaction, and there is no cash outflow going to happen. As per the Ind AS, the options are valued at a grand period as per Black-Scholes formula, which is charged to P&L over the vesting period proportionately and typically it results in a hit in the year of the grant. Then proportionate charge over the resting period. The breakup is included in the presentations.

As these are not operating expenses, do not impact the cash flow of the company, we have normalized EBITDA, which takes into account along with the provision of bad and doubtful debts.

Now, to go into the performance, our overall revenue saw a growth of 10% year-on-year driven by 20% growth of franchise business and around 22% growth in partnership business excluding API and B2G.

Our employee benefit expenses have increased by 1.3 crores versus last quarter mainly on account of actuarial valuation impact on leave encasement and gratuity which may be a timing difference and expected to be actualized in a subsequent quarter based on the actual leave availed by our employees during the festive and holiday periods which are in Q2, Q3.

Our other expenses overhead have increased by 4.8 crores versus last quarter on account of increased spend on business promotion expenses, loss on sale of end-of-life assets and onetime legal fees and some other IT expenses.

As far as the EBITDA margin is concerned, our standalone normalized EBITDA margin remains flat at 30% compared to the same quarter last year. EBITDA margin in NHL stood at 6% versus

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17% Y-o-Y mainly due to the age machines coming out of the CMC period and increased transportation cost due to the increased volume of FDG and plasma sales. Our consolidated normal EBITDA margin remained stable at 29% same quarter last year.

With this brief highlight, I will pass it on to my MD, our MD, CEO, Mr. Rahul Guha for the strategic update. Thank you.

Rahul Guha: Thank you, Alok. Briefly, I would like to take a few minutes to recap to you our strategic direction, and then I will open it up for Q&A.

First, I will reiterate our value proposition to the customer. We will continue to remain an affordable option for all patients with good quality and on-time reports. All our efforts on our value proposition are towards ensuring low cost to the patient, assurance on quality of testing through our certifications, and deep engagement with doctors. We have made substantial progress on this, which I updated in my initial comments and is reflected in the presentation. This will remain at our core and will guide all that we will do.

Second, our strategy. We hope to become the B2B partner of choice to all front-end healthcare services companies in India, whether it is a small diagnostic center in a semi-urban area, a pharmacy in a metro, a small nursing home, an individual doctor or a leading online diagnostic platform or health tech marketplace.

We are happy to provide low-cost, robust testing solutions to ensure that they can serve their patients in the most effective manner. If they require phlebotomy, we are happy to mobilize our phlebotomy network of almost 900 companies and 400 networks phlebotomists to serve them better. This strategy has been working well for us with our franchise business at an all-time high again. And hopefully, the headwinds in the e-commerce space will abate, and our partnerships business will be on a strong growth trajectory once again.

We remain true to our strategy. We have three key pillars of growth which I have shared in the last few quarters. I will reiterate once again. We are focusing on three areas. The first is our franchise business. The focus is to take our franchise business deeper into India with a focused test menu and provide our clients with a frictionless experience to transact with us and provide their customers the best testing experience that they can.

The second area is where we will focus on our partnership. We continue to focus on private as well as public partnerships. In the public space, TB, an infectious disease, which is our focus where we are by far one of the strongest players in this segment where we have won tenders now in Gujarat and Assam.

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Additionally, we will continue to expand our partnerships across health care companies, hospitals, and other health services companies and enable them to provide diagnostic testing to their customers.

The third area is new for us. We believe we have a strong and robust B2B model with a core of execution. We are ready to take this model forward with our first entry in Tanzania. The opportunity is tremendous for us to be able to launch affordable tests. I happy to share that the JV is signed and Thyrocare Tanzania has been incorporated.

That in a brief is our mandate as management. Thank y

ou so much for giving us a patient hearing.

As always, I will end once again with a quote from the Mahatma. Find purpose. The means will follow. And our purpose remains to provide high quality, affordable testing to the masses. With that, I will open it up for Q&A.

Moderator: Thank you very much. We will now begin the question-and-answer session. We have our first question from the line of Rahul Agarwal from Incred Capital. Please go ahead.

Rahul Agarwal: Sir, first question was on the TB testing contract. Could you please explain whatever is possible on the Gujarat and Assam contracts in terms of the revenue size, the tenure, and how does the business grow forward? That's the first question.

Alok Jagnani: I think it's early days for the government business. These two contracts will total to about 4.5 crores. So, it's the first set of wins that we have won after. I think now we will, as we gain confidence in the execution of that, I think we will go after more and more contracts.

Rahul Agarwal: This is 4.5 crores annual, right?

Alok Jagnani: Yes.

Rahul Agarwal: And for what is the tenure here.

Alok Jagnani: It's for one year.

Rahul Agarwal: Secondly, on the working capital side, what I can see is as of September we are down about 38 days of net sales, which is good. I just wanted to know what should be the sustainable level.

Once we get whatever outstanding payments we have with the government on a sustainable basis, our net working capital cycle should stabilize at what level, sir?

Rahul Guha: So, firstly, I think we have received most of our government payment, and overall, I will let Alok take this. Alok, why don't you take it?

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Alok Jagnani: Thank you, Rahul. So, on the trade receivable in the working capital piece, Rahul, we are expecting that on an average trade receivable are going to be around 90 to 120 days outstanding because it's going to have a B2G outstanding and all, and it requires a longer period of payment. In the case of our partnership business, we are expecting that it's going to be as per the terms and condition of business agreement which is between 75 to 90 days. So, on an average, it's going to be 3 months working capital is going to be on the credit side. In B2B business, most of the things are on prepaid model. So, there is no working capital blocking is required there.

Rahul Guha: Rahul, I think you are saying, I would expect from here, I think one could expect another 20% reduction as the remaining outstanding gets cleared.

Rahul Agarwal: So, on a net basis, 30 days should be like a reasonable number long term, 30 days of sale?

Rahul Guha: Yes, I think 30 days of sales working capital should be more or less what you should expect.

Rahul Agarwal: And third question, then I will come back in the queue, was on CAPEX, 47 crores for first half. This number I have taken from the cash flow. Could you explain the breakdown please?

Rahul Guha: Alok, you want to take that? Capital. His question is 47 crores of CAPEX in the first half. What is the breakup of that?

Alok Jagnani: So, Rahul, from where you have got the number of 47 crores? So, as per our working, in H1, we have incurred around 16 crores of CAPEX, which include plant and machinery worth of Rs. 11 crores, IT and other software spends which we have done Rs. 2.5 crores, and in office and lab furniture, we have spent around Rs. 2.5 crores.

Rahul Agarwal: Maybe I have got the figure wrong. I will come back in the queue.

Moderator: Thank you. We have our next question from the line of Rahul Mulani from Aditya Birla Finance. Please go ahead.

Rahul Mulani: My question is, there is a penalty of 2 crores from NMMC. So, what is the exact situation and whether we are going to appeal or not?

Alok Jagnani: So, we have received a demand notice from LBT last week and we have updated the exchange portal. We are evaluating

our options for appeal and further course of action, and we will update accordingly soon.

Rahul Mulani: And the second question is like now you have entered the African continent. So, any future plans like other geographical expansion?

Rahul Guha: Yes, as I had highlighted in my earlier presentation, we have shortlisted three main kind of geographies. One was Africa. Second was the Middle East, and third is Southeast Asia. Tanzania

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is our first step into Africa, but our plans are to at least expand into the East Africa Nations as our first entry into Africa. We are evaluating options to enter the Middle East. I think Southeast Asia is on the back burner at this point in time until we sought out both Africa and Middle East.

Moderator: Thank you. We have our next question from the line of Megh Shah from Prospero Tree. Please go ahead.

Megh Shah: I just have some simple questions. On an overall basis, our GP margin looks good. So, what percentage of the operating expenses are fixed in this? And in absolute terms, when will the operating leverage be generated? Like, what should be the revenue should be above this number for operating leverage should be generated after that? So, what is that absolute number?

Rahul Guha: Can I ask you to repeat your first question because your voice broke a little bit at that point?

Megh Shah: What is the percentage of fixed operating expense like a fixed percentage of other expenses?

Rahul Guha: So, see, our employee, if you look at our total expenses, we were at about 60 odd crores.

Employee benefits remain by and large fixed, and I would say in the other expenses, you can take a ratio of 50:50. So, I would say, of the 60 crores total expenses, about 40 crores would be fixed and 20 crores would be largely linked to revenue to a broad extent.

So, if you are looking for operating leverage at this point, I think it will kick. I have been always guiding that we will kind of maintain our EBIT DA margin in the 30s range because as we grow the business and invest in growth, as I said both the franchise business and our partnerships business have grown at (+20%) and our D2C or our Direct-to-Consumer business also has grown at 19%, right? All of this has required investment in business promotion and visibility and the like. So, we will continue to invest back any operating leverage we get into business promotion to fuel the growth.

So, for this financial year, I would not expect much operating leverage. I have been saying that we will be in the 28% to 30% range for our normalized EBITDA. I am hopeful if we are able to sustain the growth into next year, you will start to see operating leverage kick in. It's too early to comment. Maybe towards Quarter 4 Results, I will start to give guidance for next year, but I am hopeful that next year we will start to see operating leverage.

Megh Shah: So, anything about 30% of EBITDA margin would be invested in marketing? That's what you are saying.

Rahul Guha: Yes.

Megh Shah: That's pretty clear. My next question is, so for the radiology business, up to what level of revenue would the operating costs not increase? Like, right now, the revenue has been quite flattish for

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some time of 10 crores this quarter as well, and the operating costs are almost 70%. So, what is the level of revenue where the operating costs will not increase?

Rahul Guha: See, the operating cost actually in our nuclear business is by and large fixed. The main categories of operating cost in our nuclear business if you look at it is about 8 crores. Most of that is actually CMC cost, rent cost for the locations. CMC is the maintenance of our PET/CT machines. Then we also have rent at those locations plus the staff at those locations.

So, I think a large amount of our cost in NHL is fixed. So, I would s

ay of that 8 crores, almost 4

crores would be fixed. The rest is commission that we pay to our centers where we have franchises, right. So, that is linked to the revenue. My sense is actually we should be at a healthy profitable level once we cross 12 crores a quarter.

Megh Shah: So, when will that happen? Because it has been quite flattish since quite a long time. So, when will the business grow because it is not able to grow as of now?

Rahul Guha: No, actually, that's not true. If I look at, see, last year, we had grown on a very high base, right? So, if you look at our last year, full-year consolidated growth rate for nuclear, I think it was 40 plus percent on a revenue basis. And this year, yes, it looks like about 10% year-on-year growth, but it's off a very strong base of last year.

And what has happened is not all our centers are still operational. So, as we get one or two more centers operational, you will start to see the growth rate come back in this business. I can't give you specific guidance on when we will hit 12, 12.5. I am hopeful that by the time we enter the next financial year, we should be there.

Megh Shah: So, by '25, FY '25?

Rahul Guha: Yes, I would say, you can take almost by, I would anticipate by the first half of '25, we should get there.

Megh Shah: And in the other expense, you have mentioned that there were some onetime expenses of legal fees. So, what was that quantum?

Rahul Guha: So, the legal, there are two onetime expenses in other expenses of the tune of 1.5 crores. 1 crore is because we had very old equipment, which we sold at scrap value. There was a residual value attached to that which we charged to the P&L. So, that was 1 crore, and the legal expenses of 50 lakhs which is largely linked to our international outstanding.

Megh Shah: So, is it recurring?

Rahul Guha: No.

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Alok Jagnani: It's a one time.

Rahul Guha: One-time.

Moderator: Thank you. We have our next question from the line of Ankeet Pandya from Incred Asset Management. Please go ahead.

Ankeet Pandya: Firstly on the provision for receivables. So, from the March quarter to from in the first half, we have seen a reduction in receivables from 85 crores to almost 55 crores, but yet on the sequential basis, the provision for receivable is yet there. So, any particular reason we are still charging for that, accounting for that?

Rahul Guha: See, we still continue to have government receivables which are long outstanding, and as per our provisioning policy, we are following the policy strictly which is beyond a certain point of outstanding, on a conservative basis, we provide it in the P&L. So, until we finish all the government receivable, I think that provision is there. It's, of course, substantially reduced to, let's say, what it was in Q4 of last year. But the tail end of our government money is still pending. So, as per our own policy, we provide for it in advance. However, we are still hopeful that money will come because these things take time.

Ankeet Pandya: And from last quarter, I think you mentioned that around 38 crores have been received, 35 or 38 crores. So, what will be the balance amount pending from the government?

Rahul Guha: The total balance account pending from the government is about 10 crores.

Ankeet Pandya: 10 crores, okay. Lastly, on the CAPEX fund. So, just wanted some clarity. So, on the cash flow, it's mentioned that around 46.9 crores CAPEX has been done, and we have in the quarter, Q1 quarter, you had guided that around 40 crores CAPEX for FY '24. So, just wanted to your sense on what CAPEX can we assume for FY '24?

Rahul Guha: Your question was what did you spend the 46 crores off?

Ankeet Pandya: On the CAPEX?

Rahul Guha: What are you planning to spend additional CAPEX?

Ankeet Pandya: Yes, 46 crores of CAPEX have been spent in first half '24. And in

previous, in Q1 call, you had

mentioned that for the whole year, our CAPEX will be around 40 crores. So, just wanted some clarity on the CAPEX front?

Rahul Guha: No problem. I will give you at least a view on what we are planning to spend our CAPEX on, and then I will ask Alok to reconcile for you the first half. So, look, first half, actually, a lot of

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our CAPEX went towards plant and equipment and some of the office refurbishment and all of that. I will ask Alok to take you through the details.

In H2, actually, majority of our CAPEX investment will be in the maintenance CAPEX area.

We expect to spend about five more crores on some of the older machines in the lab and also

overhauling our IT architect infrastructure by another 5 crores. And then I think we will have our international expansion in Tanzania where the lab is getting set up, which I think we will incur another 10 crores.

So, in the second half of the year, I would anticipate about 20 crores of CAPEX to happen. In the first half, I will ask Alok to take you through what has the 46 crores break up more or less.

Alok Jagnani: So, in the cash flow, like, you have seen that around 46 crores of CAPEX investment was happened, and mainly on account of, like Rahul said, that 24, around 25 plus machines which we have replaced over a period of last 6 months which required a CAPEX and around 16, 17 crores of CAPEX investment has happened on account of that. We have invested on lab infrastructure facilities and expansions on efficiencies. Around 16 crores of further investment has happened on account of that. That is around 33 crores of total investment. Rest is a capital advance for other business plan which has been given.

Moderator: Thank you. We have our next question from the line of Girish Bakhru from Orbimed. Please go ahead.

Girish Bakhru: Just want to check, so these new packages, Jaanch and Her Check, is there a major overlap with Aarogyam? How are they exactly different?

Rahul Guha: No, they are very, firstly, nice to see you, Girish on the call, and just to give you at least the overall strategy, see, we have a routine test and semi-specialized tests. Aarogyam is largely an annual checkup, preventive wellness brand, right? So, it is focused for people who would like to do an annual health checkup. The problem with annual health checkup is it is only once a year. Jaanch is really focused on the therapeutic side, right? So, it is, think of it as a consumer facing therapeutic brand. So, for example, if you have a fever and you are worried about whether you have malaria, right? Then you would do Jaanch Fever. If you are, let's say, having hair fall, and you feel that there is something wrong with your body for hair fall, so you would do Jaanch Hair fall.

Similarly, we have Jaanch Diabetes, Jaanch Healthy Heart. So, if you are worried about heart attack risk, then you would use J Healthy Heart that gives you a 360-degree picture of your heart health. Right?

So, Jaanch is very therapy specific and largely focused on lifestyle and chronic diseases. We have Jaanch Cancer. We have Jaanch Pollution and many other kinds of segments. Jaanch STD. Thyrocare Technologies Limited

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So, it's really, the way to think about it is Aarogyam is your annual health checkup for a full scan of your body. Jaanch is really therapeutic focused, if you are worried about the detrimental effects of any lifestyle disease or chronic disease that is there in your body.

Her Check is a pure women's health package. It is largely focused for gynecologists. It has two main legs. Maternity. So, all the maternity tests double marker, triple marker, NIPT, everything that goes in the section of maternity all the way to newborn screening and so on is under the umbrella of Her Check. And the other area is infertility, which is really ab

out doing investigations to understand the reasons for infertility, and that covers the whole gamut of infertility as well. Her Check is largely focused on gynecologists to treat women.

So, that's how you should think about the three brands. Aarogyam is the annual health checkup wellness brand. Jaanch is the therapeutic chronic disease focused brand where if you are worried about something, you do a Jaanch. And Her Check is largely for gynecs to help them manage women's health.

Girish Bakhru: That's a very detailed description. So, when we look at these new packages, would it be fair to assume there are certain tests which you have, let's say, maybe introduced or they were in the panel before, but they were not getting scaled up? So, is there a higher mix of specialization expected with these packages increasing in the overall contribution?

Rahul Guha: Yes. So, just to give you a sense, our average Aarogyam package to the B2B channel, right, which is where we sell, or actually, let me not comment on my B2B rates. Our average Aarogyam package typically operates at a 1,400, 1,500 MRP in the market. Our average Jaanch package actually operates at closer to 2,000, and so does Her Check. So, I think these are two strategies also to move the mix upwards.

Girish Bakhru: And what would be your, let's say, target to revenue for these packages put together, let's say, fiscal '25-'26? Any ballpark number?

Rahul Guha: See, Her Check is actually still too new. So, we are still promoting and marketing it at this point in time. So, it's too early to comment on Her Check. Jaanch has beaten all our expectations. I think within three months of launch, it has already touched 1 crore a month, right. I am hopeful by the end of the year we should be able to double it. So, I would say between 25, 30 crores is what you can take as Jaanch as our target for next year.

Girish Bakhru: And just one more on the expansion strategy. You talked about, of course, international and B2G now coming with these contracts. What about the hospital channel that you had mentioned in

the past that maybe small hospitals less than 100 beds could be a potential target. Are they still accessible?

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Rahul Guha: Yes, we have been working on the hospital space, I think, now for about four, five months. To be honest, I mean, it is not scaling as per the expectation because it does take a long time to convince a hospital to hand over their lab to us. At this point of time, it's not material. We only have two or three hospitals, but we are learning the business with the help of those.

Moderator: Thank you. We have our next question from the line of Amit Agarwal from Nirvay Investment. Please go ahead.

Amit Agarwal: My question is regarding the term loan we have taken for the business. Thyrocare has always been a very cash rich company. So, what is the need to take this term loan? And for how long this loan has been taken. And do you think that this will be unit practice in future also? And regarding the capital expenditure of 47 crores, this company has been always left as capital-intensive business. So, what is the change in the future business plan? Because I think cash is becoming debt now?

Rahul Guha: Maybe first I will just share a bit of philosophy, and then I will ask Alok to cover the details. See, I think one is to use equity to fund equipment. In my mind yes, it is debt, but I mean, it is not bad debt. It comes at quite a reasonable cost and helps us overall expand the business and be more prudent about our capital allocation.

So, as a philosophy, we didn't feel it makes a lot of sense to deploy equity capital to fund equipment, right? And so, therefore, we looked at loans, and we said, how best can we leverage the debt market rather than deploying equity capital? That improves the return for our shareholders and everybody and the amount of

equity that remains with us, we can deploy it to fund much better avenues of growth and hopefully grow the business faster.

So, with philosophy in mind, we decided to take the loan to fund our equipment. I thought it was quite an attractive value proposition. So, I will let Alok take you through the details.

Alok Jagnani: I think almost Rahul has covered, and we all know that debt is lesser costly as compared to equity come at a higher cost, and we have taken around 30 crores of loan in our financial books which you can see all against the plant and machinery which is going to have a tenure of three years, and it's a very reasonable best rates what we have taken. And this gives us the leverage to expand our business and cash in our hand to further expand and opportunities in CAPEX and business expansion piece, which were like other expansions we are doing.

Amit Agarwal: So, you mentioned we have taken almost 16 crores for capital advances. So, could you explain that item more? Because 246 crores for capital expenditure have been a huge amount, and we have always been very less capital-intensive business? Because I think there is a change of model of business right after the change of management.

Rahul Guha: So, wait, first, let me take again the first question around the change of business model. Look, as I said before, our average age of equipment was 12 years. Okay. It becomes very difficult to

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run reliable testing on equipment that is so old. We had to replace that equipment to ensure that we could have much better accuracy, throughput, and performance. So, that was a need of the hour for the business to ensure that we have the right equipment so that we can give our customers the right and most reliable testing. So, that decision to replace the equipment was not a change of business model. It was the need of the hour to ensure that we improve our accuracy and reliability in the lab.

Now, how we chose to fund it, that I will let Alok take over.

Alok Jagnani: So, the funding piece we have already explained. So, the total spend of CAPEX and investment what we have done till in H1 is Rs. 46 crores what my friend, Incred friend has also asked. So, we have spent on the machine expansions and replacement of existing old machine with the new machines and around 24 machines have been replaced which resulted spend of around Rs. 16 crores, Rs. 17 crores.

Our labs also require some infrastructure and improvements, and we have gone for the infrastructure improvement and expansions. That leads to a CAPEX advance of around 16 crores, which are going to be capitalized over a period of the next three to six months' time once the entire expansions are going to complete.

We have done a LLP investments, which is going to have around Rs. 6 crores in the last 6 months what we have done, and other IT and other CAPEX investment which has happened to around Rs. 63 crores, which leads to a total 46 crores of CAPEX spends which we have done, and depending upon the best leverage what is available, we evaluate whether we go with the equity or cash available or we will evaluate with the debts which is available at the best market rate, and then we take the decisions for the interest of best to the interest of our shareholders.

Amit Agarwal: My question is also that is this amount going to reduce in future or is this amount going to stay as it is in this financial year? And what is the investment required for Tanzania project?

Rahul Guha: So, that I have already shared. As I said in H2, we anticipate to spend another 5 crores to replace the machines and another 5 crores for our physical infrastructure and IT. So, that is 10 crores, and our international expansion, which is primarily Tanzania is another 10 crores.

Amit Agarwal: What about in the next financial year?

Rahul Guha: In the next financial year, I would think again, de

pending on the maintenance CAPEX and all, I

will be able to give you a more concrete guidance towards the latter part of the year. But if you force me for a number, I would say it would be in the 50 crores range.

Amit Agarwal: 50 crores?

Rahul Guha: Yes.

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Amit Agarwal: For the whole, yes?

Rahul Guha: Remember, I have a lot of old aged equipment even in the NHL business for which we will have to replace those equipments over time.

Amit Agarwal: So, are we taking some more term loan or we can manage these investments?

Rahul Guha: So, currently we are not forcing on the more term loans, but it's a like open opportunity depending upon the business CAPEX investments what we are going to happen, based on the best available options, we will evaluate.

Currently, we have like 30 crores of debt in our books which is having a tenure of three years against the assets. There is no bank guarantee, no collateral security attached to that. In subsequent CAPEX investments, maybe we can go, but as of now we have enough cash and immediately in the next three months to six months' time, we are not planning to go for further debt funding.

And again, I wanted to once again just reiterate our balance sheet is very strong. Our cash flow is also very strong. Because we are a healthcare company, we get preferential rates as healthcare equipment finance. I don't see anything wrong in taking debt to finance healthcare equipment for our expansion. In fact, I feel using shareholder money to finance equipment is perhaps less prudent.

Amit Agarwal: No, I understand that point, but I just wanted to evaluate how much dividend we will be getting in future, like earlier two years back we used to get 25%. Now we are reduced to 18%. I think we don't foresee an increase in dividend in future I suppose.

Rahul Guha: No, I would not anticipate, expect an increase in dividends in the future.

Moderator: Thank you. We have our next question from the line of Aneesh Deora from Nomura. Please go ahead.

Aneesh Deora: Firstly, I just needed clarification. I was looking at the PDF slide nine, where we have shown the breakup of the pathology revenues quarter-on-quarter. So, in the footnote it has been written that Q1 FY '24 and Q2 FY '24 refer to non-COVID numbers, and oh, sorry, all numbers except Q1 and Q2 of FY '24 refer to non-COVID numbers. So, I just wanted to understand if any COVID revenues are there in these two quarters and if you can quantify the same?

Rahul Guha: Yes, just give me one second. See, these numbers largely are non-COVID numbers. the COVID numbers are almost negligible at this point in time. So, we didn't see any reason to really bifurcate it at this point. Aditya can share. Aditya, what is our COVID number for this quarter?

I think it's less than 1 crores, right?

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Aditya Shinde: It is 38 lakhs for Quarter 2.

Rahul Guha: So, yes, our COVID number, Aneesh, is 38 lakhs for Quarter 2. So, we didn't bother bifurcating it at this point.

Aneesh Deora: And can I just know the COVID number of Q2 of FY '23 in that case?

Rahul Guha: 2 crores roughly.

Aneesh Deora: I see. Secondly, I just wanted your comments around the pricing in the industry currently. Have you like taken any price increases? And what are the quantum of growth that we are seeing through price? Just wanted to understand the dissection between volumes and price growths that is currently there.

Rahul Guha: See, for us we are taking price up through mix. We are not taking prices up through price hikes.

So, our price hikes have been marginal I think to the range of about 4%, 5%. The rest we have actually been driving through mix which is to larger and larger Aarogyam packages, Jaanch the newer test menu, more movement towards vitamins and all of that. So, I would say, if you see we grew about 2

0% by revenue in our franchise business, I think about 8% by volume. So, roughly about 12% is price, which is about 4%, 5% has come from just price increase and the remaining has come largely from mix.

Aneesh Deora: And the price increases have been taken on like the entire portfolio or only on some specified portfolio of tests?

Rahul Guha: So, we have taken the price increases across the board. There are just a few losses leader kind of tests that we have which kind of brings the customer through the door. Over there, we have chosen not to raise the price.

Moderator: Thank you. We have our next question from the line of Rahul Agarwal from Incred Capital. Please go ahead.

Rahul Agarwal: I think the CAPEX question got answered. One question on the nuclear business. How should this business grow? And what are the sustainable EBITDA margins for this business?

Rahul Guha: See, the business, as I said one, we are faced with a few headwinds in nuclear to be quite candid.

As I said, the machines are very old. So, the cost to maintain those machines is just going up and up, which is what you see in the other expenses over there. As a result, we have been steadily also increasing prices in line with that to cover the cost, but that naturally has an impact on the volumes as well.

I would say, if I were to guide you, a normal EBITDA for this business given the state of the machines that we have and the kind of OPEX that goes into maintaining and running these
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machines, I would say in the between 10% and 15% range is where you should look at NHL on a steady state. We are doing a lot to fix the business, but I would say, don't anticipate anything more than between 10 and 15. If we touch 15, I will say, we would have done a fantastic job with the kind of infrastructure that we have.

Rahul Agarwal: And a related question as you also mention on volume, I think just wanted to know the sustainable growth here. The revenue growth in the first half I think is 10%. What was the volume growth here? Like, if you could share the number of scans for first half, that will help.

Rahul Guha: For first half versus?

Rahul Agarwal: Y-o-Y volume growth for nuclear?

Rahul Guha: The total scan volume will be about 10% is what I think, which is where we have landed year-on-year in revenue also.

Rahul Agarwal: But you mentioned price hike. So, I just wanted to clarify that the growth is driven by...

Rahul Guha: No, the price hikes we started now because we started in September and actually now going into October, November. The reason we started to do the price hikes is when the CMC cost started to hit us. And as you know, it's not easy to take price up in this business. So, it takes a little bit of time to ensure it lands.

Rahul Agarwal: Thanks for the clarification. Another thing was we, obviously, lot of PharmEasy shareholders have got their forms for rights issue. My sense is legally that process is going through. For us from a business perspective, API partnership, would you expect that business to revive back on the growth path? Any color could you provide on this?

Rahul Guha: So, I am hopeful, I think it has already been publicly declared. So, I can quote it that the rights issue has been oversubscribed. So, I would hope that now that some of the funding issues at PharmEasy are resolved, that they would also go back on a full growth trajectory.

As you know, our entire API business is just cross-selling on the diagnostics platform, on the PharmEasy medicine platform. So, as that medicine platform goes back on its path to recovery, our cross-sell business obviously grows lock in step with that. So, I am very hopeful that with this funding issues behind us the third engine of Thyrocare will now start to fire.

Our first two engines are firing quite well. The franchise business, as you have seen, Rahul, has grown at 20%. Our

partnerships business excluding API and government has also grown at 22%.

In fact, surprisingly, even our D2C or our direct business has grown at 19%.

The last engine that we have is the API cross sell where I am hopeful that would also come back on its original growth trajectory.

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Rahul Agarwal: And Tanzania overall investment is going to be 20 crores. 10 is done. 10 in second half. Is that correct?

Rahul Guha: No, no, we have not done any investment till now. 10 will happen in the second half. That's all we will plan to invest in.

Rahul Agarwal: And lastly, just one clarification. I think you answered partially, but I wanted to clarify. The provision the line reads in your presentation essentially provision for receivables taken for foreign accounts after netting of reversal from B2G business. Could you just explain this please?

Rahul Guha: So, as you know we had provided quite heavily for the NHM business, which we received the

money. So, there was a provision that was released from the P&L. However, as per our normal policy, our receivables from Thyrocare Gulf have been pending for a significant amount of time, and we have now gone into litigation. So, therefore, our advisers have asked us to or our auditors have insisted that we provide for that. So, these two have netted each other off, and it's landed at about that 1.36 crores seen in the P&L.

Rahul Agarwal: So, the Thyrocare Gulf receivable is higher than one, and whatever releases happened from NHM is also higher than one, right? And hence the net number is 1.5. Is that correct?

Rahul Guha: Yes, it's I think about five and six.

Alok Jagnani: Correct. So, around Rs. 6 crores of Thyro Gulf which is netted up against the NHM and few other small receivables.

Moderator: Thank you. We have our next question from the line of Ashu tosh Parashar from Mirabilis Investment Trust. Please go ahead.

Ashutosh Parashar: I just had a couple of questions. So, first regarding, so we had initiated this pin code expansion like four, five quarters back. So, I wanted to check whatever new areas that we have entered into in these last four, five quarters, how have those been performing? And if we were to sort of index it to a mature area, so, currently, what level would these areas be operating at? So, that is the first question.

Rahul Guha: So, if I could repeat your question so I have understood it, you are asking what is the status of the pin code expansion and where do you think it will reach?

Ashutosh Parashar: And how has those areas been performing for us? The new areas basically.

Rahul Guha: So, look, we continue to expand in the pin codes through our pin code expansion team. We actually now have started to track it from number of active franchises. So, we closed last year, I report it on an annual basis. I don't report it quarter-on-quarter.

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But last year, if you remember, we had ended at about 7,488 active franchises, and we continue to add franchises at a run rate of between 100 and 135 per month depending on the month. So, I would say, in July, August, September, net-net we would have added about 400 new franchisees.

I think we will peak out at about 10,000.

Of course, it's like same store sales growth in a retail business, right? The new franchisees take a little time to scale up. So, obviously, in the first year, they open rate at about 25% of the full potential of the base, and then it takes them about three years to reach the full potential. So, it takes a little bit of time to scale up, but we are running at a healthy run rate over there.

Ashutosh Parashar: And secondly, on the Aarogyam portfolio, so, currently, what is the total contribution of Aarogyam portfolio to total sales?

The second one is on the Aarogyam portfolio. So, what is the contribution currently

to our total sales?

Rahul Guha: Aarogyam remains at 36% of our turnover which is the same as what it was last year.

Ashutosh Parashar: And the Pro and Plus range that we have launched a couple of quarters back, so is it contributing meaningfully? Would you be able to quantify the contribution?

Rahul Guha: See, we have now a full lineup. It is Aarogyam Pro and Plus. It is all part of the Aarogyam. They are just, how do I describe it? A completion. At that point, our Aarogyam average order value used to be about Rs. 1,200. Over time, it is moving up in that way, but we had also at the same time launched Aarogyam Mini and Aarogyam Basic, which were at the entry level. So, at a mixed level, I would say, we are kind of at the same place, but I would say our franchise business has grown at 20% year-on-year and Aarogyam remains at 36% of our overall business. So, the Aarogyam business continues to grow at a strong clip of about 20%, most of that driven by our investments in Plus and Pro because we haven't done anything else with Aarogyam.

Ashutosh Parashar: That would be all.

Moderator: Thank you. We have our next question from the line of Siddhesh from ICICI. Please go ahead.

Siddhesh: So, my question was regarding this depreciation policy. So, if we look at plants and diagnostic equipments, we are depreciating that over 13 years. But you mentioned that some of the old assets that we replaced, those were having an average year of 12 years. So, is there a need to revisit this depreciation policy of 13 years?

Rahul Guha: I will let Alok take that.

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Alok Jagnani: We will go through the policy and come back on that, but the current policy what we are having of 13 years asset life, which we are taking for our accounting purpose, remain valid. But point taken. We will evaluate, but on average, our machines have life cycles of around 13 to 15 years.

Rahul Guha: So, just to clarify also, I just wanted to say the average age of our equipments were 12 years,

right, before we bought the new equipments. That means, of course, that many of our equipments were well beyond 13, 14, even 15 years. So, it's those equipments that we have replaced. It doesn't mean that we are not using equipment, that is, let's say, 8, 9, 10, 11 years old.

Moderator: Thank you. We have our next question from the line of Megh Shah from Prospero Tree. Please go ahead.

Megh Shah: I just wanted to confirm one thing. As on 31st March 2023, the total outstanding net of impairment was 83 crores, of which 72 crores were outstanding from government, and right now the total amount outstanding from government is 10 crores. Is my understanding correct?

Rahul Guha: Yes.

Megh Shah: So, in the Quarter 1, we received 38 crores, and in 2nd Quarter we received 24 crores?

Alok Jagnani: So, what you said is like 85 crores of total outstanding was there in March. Against that, the current outstanding which is showing is 55 crores. The moment is of 30 crores, correct?

Megh Shah: I actually want to focus on the government part.

Alok Jagnani: So, on the government part, the outstanding what was we have received around 41 crores of amount we have received from NHM, and few other government collections also happened. In addition to that, other business partners where the outstanding was there, we collected around Rs. 20 crores, which resulted in deductions of trade receivable. In addition to that, subsequently, in 6 months' time, fresh filling has happened and resulted to add on to the data receivable piece.

Megh Shah: So, in March, the total outstanding from government was 72 crores according to the figures you have mentioned in the annual report.

Alok Jagnani: No, so in March, the total outstanding with government was around 35 crores.

Megh Shah: As on March 2023?

Alok Jagnani: 35 plus 15. Just 35 -- it's around 50 crores net of provision.

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Megh Shah: 50 crores net of provision.

Alok Jagnani: So, in March '23, the total outstanding from government net of provision was 50 crores. Now after in September, net of provision, the total outstanding from B2G business is around 10 crores.

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Rahul Guha: Let me try to make an attempt once again to receive. So, net of provision, so without provision, right? Our March outstanding was 71 crores. We received 45 crores. We have provided for 16 crores until now, which leads us to a net balance of after provision of 10 crores.

Megh Shah: And all these are outstanding figures, right?

Alok Jagnani: Sorry.

Megh Shah: All these are outstanding from government or total outstanding? These figures?

Rahul Guha: No, no, only government outstanding.

Megh Shah: Only government.

Alok Jagnani: We have done B2G outstanding only.

Megh Shah: Just a follow-up question for the CAPEX which you mentioned of 47 crores. So, I just wanted to understand where is that amount reflected in the balance sheet?

Alok Jagnani: So, that is you can see in the fixed assets addition and deletion, both addition and replacement have both has happened. So, it will be seen as a net of once you are going to receive a detailed financial in the month of March detailed financial which is going to come with annual financials, you will find where the additions and deletions can be seen.

Megh Shah: So, the net amount is the same, right.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to Mr. Rahul Guha for closing comments. Please go ahead, sir.

Rahul Guha: Thank you everyone for spending the time with us this evening. I think this is the longest question-and-answer session I have seen since the time I joined. It has been now I think about 15 months or 16 months ago.

As always we continue to remain focused on our strategy, which is to be the most affordable, good quality diagnostic testing partner for anyone in the healthcare business, and we continue to execute on that strategy. We have been investing in improving our quality, improving our reach, and ensuring our turnaround time is as close to the best in class. We have made substantial progress on all of this as I have been reporting over the previous year, and I thank you all for your support in this journey.

With that, I will hand over to Pratik for closing comments, and then we can close.

Pratik Hire: Thank you, ladies and gentlemen. On behalf of Thyrocare Technologies, that concludes this conference. We thank you all for joining us.

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Moderator: Thank you.

Pratik Hire: Over to you, Yashashri.

Moderator: Thank you. On behalf of Thyrocare Technologies Limited, we conclude this conference. Thank you for joining us, and you may now disconnect your lines.

Call: Feb 2024

February 05, 2024

The National Stock Exchange of India Limited BSE Limited
Exchange Plaza, Phiroze Jeejeeboy Towers
Bandra Kurla Complex, Dalal Street, Bandra (E), Mumbai - 400 051 Mumbai- 400 001
(SYMBOL: THYROCARE) (SCRIP CODE 539871)

Dear Sir/Madam,

Sub: Transcript of post results earnings c onference call held on February 01, 2024 with
Analysts / Investors

Ref: Disclosure under Regulation 30 of the Securities and Exchange Board of India
(Listing Obligations and Disclosure Requir ements) Regulations, 2015 ("SEBI Listing
Regulations")

Pursuant to Regulation 30 and 46(2) read with Part A of Schedule III of the SEBI Listing Regulations, please find enclosed
herewith the transcript of earnings conference call with
Analysts and Investors held on February 01, 2024 for financial results of the Company for the
quarter and nine months ended on December 31, 2023.

We wish to add that the same has also been made available on the website of the Company
<https://investor.thyrocare.com/financials/quarterly-financial-results/>

This is for your information and records

Yours Faithfully,
For Thyrocare Technologies Limited,

Ramjee Dorai
Company Secretary and Compliance Officer

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"Thyrocare Technologies Limited
Earnings Conference Call "
February 01, 2024

MANAGEMENT : MR. RAHUL GUHA – MANAGING DIRECTOR AND
CHIEF EXECUTIVE OFFICER – THYROCARE
TECHNOLOGIES LIMITED
MR. ALOK KUMAR JAGNANI – CHIEF FINANCIAL
OFFICER - THYROCARE TECHNOLOGIES LIMITED
MR. PRATIK HIRE – STRATEGY TEAM – LEAD
INVESTOR RELATIONS – THYROCARE TECHNOLOGIES
LIMITED

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Moderator: Ladies and gentlemen, good day and welcome to the Thyrocare Technologies Limited Earnings
Conference Call. As a reminder, all participant lines will be in the listen -only mode. And there
will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the
conference call, please signal an operator by pressing star then zero on
your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Pratik Hire , from Thyrocare Technologies Limited. Thank
you and over to you, sir.

Pratik Hire: Thanks, Rio. A very good evening to all and thank you for joining us today for Thyrocare's earnings conference
call for the third quarter of the year FY'24.

Today, we have with us Mr. Rahul Guha, MD and CEO of Thyrocare; and Mr. Alok Jagnani of

Thyrocare, along with other key members of the senior management on this call to share highlights of the business and financials for the quarter. I hope you have gone through our results release and the quarterly earnings presentation, which has now been uploaded on the stock exchange website.

The transcript of this call will be available in a week's time on the company's website. Please note that today's discussion may be forward-looking in nature and must be viewed in relation to the risks pertaining to our business. After the end of this call, in case you have any further questions, please feel free to reach out to the Investor Relations team.

I now hand over the call to Mr. Rahul Guha to make the opening remarks.

Rahul Guha: Thanks, Pratik. Welcome -- good evening and welcome to everyone on the call. Thank you for taking out the time from your busy schedules to join us this evening. Just a quick introduction to us on the call. My name is Rahul Guha and I'm the MD and CEO of Thyrocare and thank you for the opportunity to present the Q3's results for FY'24. I'm joined with my colleague, Alok Kumar Jagnani, who is our CFO; and Pratik, who handles our Strategy and Investor Relations.

As I did in the last call, I will start with a quote from Mr. Nelson Mandela in recognition of foray into Africa. "It is in your hands to make a better world for all live who in it." And we believe Thyrocare can bring our business model to Africa to make a affordable and good quality

diagnostics available to all. I'll give you some of the key highlights of what we've been up to this quarter.

Before we get into the details of this quarter, I'll reiterate the pay-for-performance pricing structure that we implemented at the beginning of this financial year. Earlier, our pricing structure was one-size-fits-all but now we have moved to a slab-based pricing model, which we implemented in May 2023. This has led to an increased energy with our franchisee network with motivation to move up volumes and enter higher slabs.

It will result in a movement towards larger franchisees and enable much greater reach from our large partners. As always, we continue to selectively expand our offerings. Aarogyam has been our flagship brand in the preventive healthcare segment. And now we have 2 more brands, JAANCH and Her Check.

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JAANCH is targeted towards lifestyle challenges or for you to better understand your health.

We have solutions across the spectrum from anything you might be worried about, whether it is fever or something more serious, hair fall, cancer screening as well as deep investigations for common chronic diseases like diabetes, heart health, amongst others. JAANCH already doing INR1 crore of monthly business. We've also revamped our gynac portfolio and relaunched it under the brand name of Her Check and it focuses on women's health.

I'm very pleased to announce that we have partnered with TestEasy to intro

duce genomics testing

in our test menu. These tests will be targeted towards hospitals and marks our entrance into the genomic space. We are taking a very focused approach here and it is largely genomics in the diagnosis space and not consumer genomics. Specifically, some of the tests are launched to use genomics in diseases such as tuberculosis, fungal infections, bacterial infections and others.

Partnerships business, excluding API and B2G did phenomenally well in the quarter as we onboarded new clients in health tech segments and continue to grow our existing accounts. To bolster this success, I'm happy to share that we have acquired Think Health to strengthen our offering for the insurance segment with the additional capability of ECG at home. This will allow us to give our insurance partners a one-stop solution for blood and ECG testing and will further deepen our presence in the premedical checkup, pre-policy medical checkup and the annual health checkup market. We will soon be the company that can offer health checks with ECG at home and I'm sure that will enhance our Aarogyam brand. On the B2G side, we continue to execute TB projects in the state of Gujarat, Assam and Maharashtra. On the international side, our lab setup in Tanzania is going on full swing and we are on track to start the operations before the end of this financial year.

As I've been telling you in the last quarters, we have revamped our equipment platform. Our equipment was quite old with an average age of 12 years. We've added 24 new machines from our vendors, bringing our average age down to 6 years. These new additions have dramatically improved our reporting accuracy and turnaround time. As we get into the results, I wanted to share with you a few highlights and pointers before we deep dive into the same.

Our franchisee business showed a revenue growth of 11%. Growth rate in franchisee business has slowed down, largely because of churn in the small franchisees. This churn was expected after we implemented volume-based pricing. Our big franchisees have grown at 25%, while the small franchisees have degrown. In terms of mix, contributions from big franchisees has increased from 80% last year to 90% this year.

Majority of the small franchisees have churned and the base is stabilized now. Going forward,

the franchisee business should return back with high teens growth rate. Our partnerships business, excluding API and B2G showed a strong revenue growth of 33% year-on-year. On overall level, our partnerships business has remained flat year-on-year on account of decline in API and closure of the MCGM contract.

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Overall, the pathology business has grown by 8% year-on-year, excluding materials. Radiology business, including Pulse Hitech, did a revenue growth of 9% year-on-year. However, at a consolidated level, we did a 5% year-on-year growth, largely because of the headwinds in API, the government business and some of the smaller material sales business. With that, I will hand over to Alok to cover the results.

Alok Jagnani: Thank you, Rahul. A warm welcome to each of you. I will briefly update you about the key highlights of Q3 FY'24 financial performance. Before we get into the detail, I will reiterate about the ESOP program that we have been mentioning in the last few quarters. This program has been introduced at group level to keep talent at Thyrocare. These ESOPs of parent's company will vest over a period of 6 years and we are recognizing the same as per the NDA as a book entry share-based payment reflected in P&L as expense and balance sheet as a equity contribution from parents.

The value of ESOPs granted is INR45.53 crores over a period of 6 years. But one thing we must note that this is a cashless charge and there is no cash outflow. As per Ind AS option are valued at grant date as for the Black-Scholes

formula, which is charged to P&L over the vesting period proportionately.

Typically, its result in a hit in a year of grant, then proportionally charge over a period of vesting period. The breakup is included in the presentation. As these charges are noncash transaction -- noncash operating expenses and do not affect the cash outflow of the company, we have normalized EBITDA, which takes into account along with the provision of bad and doubtful debts. As Rahul mentioned, revenue of the quarter stood at INR 123 crores standalone and INR135 crores at consolidated level, a year-on-year improvement of 5%, aided by better partnership, revenue growth in -- excluding B2G and API revenue, by 30% and franchisee by 11%.

Our pathology revenue, excluding material, has grown by 8% year-on-year. Our gross margin has improved by 3.3 percentage points, which is stable in quarter-on-quarter, mainly driven by price increase, volume growth and better negotiations. Our employee expenses has increased year-on-year on account of annual increment and decreased quarter-on-quarter due to the actuarial valuations. The same has been clarified in the previous quarter presentations that actuarial valuation is a timing difference. Our standalone normal EBITDA margin stayed flat 28% year-on-year, while quarter-on-quarter dipped by 2% on account of dip in revenue due to festivity and seasonality, whereas overhead remained similar.

EBITDA margin at radiology stood at 8% versus 19% year-on-year, mainly on account of aged machines coming out of the CMC period and change of revenue mix mainly moving to FDG/PSMA. Quarter-on-quarter EBITDA has improved due to the price revision in few centers.

Our consolidated financial normalized EBITDA margin is at 26%, lower by 1% as compared to the same quarter previous year.

With this, I will hand over the call to Rahul for strategic update. Thank you.

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Rahul Guha: Thank you, Alok. Just to reiterate our overall growth, if you exclude API and B2G, as a company is about 15%. With that, I'll just take a few minutes to recap to you our strategic direction, then I'll open it up for Q&A. First, I will reiterate our value proposition to the customer. We will continue to remain an affordable option to all patients with good quality and on-time reports. All our efforts on our value proposition is towards ensuring low cost to the patient; assurance on quality of testing through our certifications and engagement with doctors.

We have made substantial progress on this, which I updated in my initial comments, is reflected in our presentation and this continues to remain at our core and will guide all that we do. Second, our strategy. We hope to become the B2B partner of choice to all front-end diagnostic services companies in India, whether it is a small diagnostic center in a semi-urban area, a pharmacy in a metro, a small nursing home, an individual doctor, or a leading online diagnostic platform or health tech marketplace. We are happy to provide low cost, robust testing solutions to ensure that they can serve their patients in the most effective manner. If they require phlebotomy, we are happy to mobilize our

phlebotomy network of almost 900 company and 400 network phlebotomists to serve them better. With the addition of Think Health, we will be able to serve our B2B partners with the solutions for ECG at home as well. With the addition of TestEasy, we will be able to now help them add genomic sequencing or genome sequencing for their customers too. Further to reiterate, as I've shared in the last few quarters, we have 3 key pillars of growth. The first is our franchise business. The focus is to take our franchise business deeper into India with a focused test menu and provide our clients with a frictionless experience to transact with us and provide their customers the best test

ing experience they can. We continue to focus on private as well as public partnerships. In the public space TB and infectious disease, where we are, by far, one of the strongest players in these segments. We have won tenders in Gujarat and Asam and have been working with Maharashtra. Additionally, we will continue to expand our partnerships a cross health care companies, hospitals and other health services companies to enable them to provide diagnostic testing to their customers.

The third area is new for us. We believe we have a strong and robust B2B model with a core of execution. We are ready to take this model forward with our first entry in Tanzania. The opportunity is tremendous for us to be able to launch affordable tests. That in brief is our mandate as management. Thank you so much for giving us a patient hearing. As always, I will end once again with a quote from the Mahatma, "find purpose, the means will follow" and our purpose remains to provide affordable, high-quality testing to everyone.

With that, I'll open up for Q&A.

Moderator: Thank you very much. The first question is from the line of Mr. Rahul Agarwal from In Cred Equity. Please go ahead.

Rahul Agarwal: Firstly, congratulations Rahul for the additional role at API. Just to start with the questions. I think first on the Think Health acquisition, I wanted to know, the buying of this asset, I think Thyrocare Technologies Limited
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that the benefit will come through insurance clients but I couldn't really figure out what do we get in return? So essentially, does this -- thoughts, like is there was an entry barrier, which is getting solved by this acquisition? And what is this asset a II about? Would you just help me to understand that please.

Rahul Guha: Sure. So see, in insurance, one of the big gaps that Thyrocare had is, when you have a pre-policy medical checkup ECG is actually an integral component of that, right? Since we were largely pathology only, we could not service those orders fully. So it was -- many of the aggregators would work directly with the insurance companies where they were able to give both of these solutions to an insurance company through a single window.

Of course, there are not significant barriers to entry. We could have built this ourselves. But we felt that the Think Health team brought to the table 2 things. One is the full understanding of how the insurance segment works and they have built the appropriate technology and operations to be able to service that demand, right, which if we had done on our own would have taken us upwards of 1 year.

The second is they have 100 phlebotomists on the ground who have been trained in administering ECG at home, right? And they are equipped with ECG, mobile ECG equipment and that was a ready base that we were immediately able to integrate into -- or we will be immediately able to integrate into our phlebotomy network and be able to service ECG at home orders for all our insurance clients. So the real rationale behind the acquisition was it's a new capability, which would have taken us a long time to develop on our own and we were able to get it quite quickly into our system.

Rahul Agarwal: Perfect. Got it. Clearly understand what's happening. Secondly, on the sample volumes, I understand there was churn in franchisees and seasonally, this is a big quarter. Any other reason for sample volumes to decline Y-o-Y?

Rahul Guha: No, it's largely the churn. As I said, we have categories going from Bronze all the way to Diamond. So Bronze being where we have the highest rate in the market and Diamond where we offer franchisees a significant discount because of the volumes. So as I highlighted in the initial comments, our mix of large clients, which I would call Diamond to Silver was about 80-20. So 80% Diamond to Silver and then 20% Bronze. That has actually now moved to 90% Diamond

and Silver and less than 10% or roughly about 10% Bronze.

This we were expecting would happen as we moved into that price tierization. What happens is, over time, these clients, actually the Bronze kind of category will get attached to some of our Diamond franchisees and will come back but we were expecting this to happen. We had timed it in a way that it happens in this quarter because this quarter is the quarter where anyway you see a significant dip because

of the season and all of that. So we have timed it in a way that this would come roughly 6 months after we launched the pricing strategy and that's kind of how it has played out.

It's reached the bottom. So the churn in Bronze also over the last 3 months has more or less been stable. There's not that much more churn that is happening over there. So as we get into the peak

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season, which is the entire fourth quarter or Feb, March more specifically, actually, we enter the peak season with a much stronger franchisee network. Each of them transact much more with Thyrocare and, therefore, have the ability to invest and expand their business as we enter into the festive season. So I think most of this will get sorted out over the next 2 quarters.

Rahul Agarwal: But in my understanding, if the Silver and Diamond go up and Bronze goes down, it basically means that the volumes go up and realizations go down. Isn't that correct?

Rahul Guha: Yes. But while the churn is happening, the net volumes remain the same. Once the churn stops, which is what I said over the last couple of months, 3 months exact -- to be precise. We've kind of reached the bottom of where the churn would be. So now we will see the growth going forward. If you're referring to our overall pathology workload, right, so the franchisee workload has been more or less stable. It's not gone down at all. Our partnerships workload has kind of been in line with revenue. So if we grew about 33% our partnership revenue, our workload has grown, I think, about 25% roughly.

The big dip in overall workload has come because of API and B2G, where we've lost about 4.5 lakhs of workload on a year-on-year basis. So MCGM was a very large workload account. From a margin and EBITDA point of view, it was very narrow. So actually, there's very little impact on the EBITDA of that workload going. And API, of course, over the last year has degrown.

Rahul Agarwal: Got it. And last question from my side on radiology. Obviously, there's a declining trend in margins because of cost increases. Just wanted to understand, are we headed to a cash loss here or this declining trend will stop next quarter?

Rahul Guha: No. Actually, last quarter also was quite bad but we've been able to arrest the decline, right? So last quarter, I think we landed at about INR 70 lakhs EBITDA. This quarter, we land at about INR 80 lakhs. The bulk of the cost increases have actually got -- come into the base. So if you see the biggest hit that has happened in NHL is our other expenses has gone up by roughly about INR 60 lakhs, INR 70 lakhs.

That is because the machines are very old and the CMC or maintenance costs have gone up. But now that's in the base, right? So I don't see this business going into a cash loss. There is also a significant amount of depreciation there, right? So it's about INR 1.5 crores of depreciation that sits there. So I think from a cash position, we are reasonably comfortable.

Moderator: The next question is from the line of Aditya Khemka from InCred PMS.

Aditya Khemka : Rahul, so since the management changed at Thyrocare, we have rationalized 3 streams of revenue. That's the API business, B2G, I understand it's not intentional rationalization but it's happening nevertheless and now the low ticket or the smaller franchisees. Going forward, is there any other part of the business which we see will get negatively

impacted due to the

initiatives that we are taking? Or do you think in terms of the rationalization of the less productive streams of revenue, we're doing one.

Rahul Guha: I think largely -- you have pointed the right thing. I think API has bottomed out. Government we had exited. And I think this entire Bronze category cleanup that we were planning is done.

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Broadly, I think -- actually, if you look over the last 2 quarters, the growth has been quite robust of all of these base businesses. This was -- this franchisee pricing cleanup was the last one that we had to do. But I think broadly that's it.

Aditya Khemka : Got it. And you said that the larger franchisees were earlier 80% of the revenues, now they are 90%. So the balance 10% Bronze franchisees, I think, will have, is there a chance that they will also eventually drop out over the next 2, 3 quarters? Or have you got an indication from them that they are okay with the pricing and they would actually scale up the business to move upwards on a redrawn strategy?

Rahul Guha: Yes. So as I said in the call, we've been studying this quite closely. If you look at it over the last, I would say, 4 to 5 months, that number has been stable. So if they had to -- so they churned in the initial part. So during July, August, September is where we saw the maximum amount of churn, right, just after we launched the scheme. Over the last 4 months, it's been stable. Whereas if you look at the large franchisees, the large franchisees have actually grown by almost 25%.

So that base has actually done quite well. This cleanup was expected, we were saying because our cost to serve over there is quite high. So if we were charging the same price for a large franchisee versus a small franchisee, it was actually margin dilutive to some of those clients at the old rate that we were serving them. So we had to do this cleanup.

Aditya Khemka : Understood. On the margin profile side, Rahul, so 28%, if I'm reading it correctly of ESOP charges and some one-offs that we have reported this quarter. Where do you see the margins, adjusted over the ESOP charges, where do you see the margin projected once the Bronze sites move out because the previous participant asked a question that if Bronze guys move out, basically margins could probably get compromised because these guys will pay lesser as you take more from them. So how do you see the margin projected over the next 1 or 2 years for the console business?

Rahul Guha: See, I have been always guiding to between 29% and 30% on the EBITDA side. I mean this quarter is -- any way normally this quarter is muted from a margin point of view. But I think we'll cover it up in the fourth quarter. So I think on a 9 months basis, they're still on track. And I think for the full year, we'll be on track to be in that range of 29% to 30%.

Aditya Khemka : This is from the fourth quarter, you're saying or from the net fiscal year?

Rahul Guha: No, no. From the fourth quarter itself. The fourth quarter tends to be very good from a revenue point.

Aditya Khemka : Yes, that's right. It's the best quarter seasonally. Okay. Last question and a more strategic top-down question. Now that API PharmEasy doesn't have much business with us as it is and they've been unable to raise capital to sort of deleverage themselves, what do you see is the benefit of being part of the API Group? Why would they still hold on to Thyrocare as a subsidiary? And how does this benefit Thyrocare to be a subsidiary of API?

Rahul Guha: Yes. See, for us, right now, these are related parties but all transactions are done on arm's length basis. For us, PharmEasy still accounts for about INR 12 crores, INR 13 crores of revenue every Thyrocare Technologies Limited

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quarter. So they are our largest client or customer by far. So from that point of

view, they're very important to Thyrocare.

Even from the API Group side, the diagnostics business is one of the most -- actually the most profitable business on API as well. And so, therefore, there will be intent to scale that business and see how much we can grow it because if you look at it between medicines, OTC products and diagnostics, diagnostics, even at an EBITDA level at API Group is the most profitable. So there is definitely intent from that point or API's point of view also to scale the business.

Aditya Khemka : Right. And is this being a part of the API Group a hindrance for you guys to approach other med tech platforms to get B2B business from them or are they fairly open to you guys despite you being a [inaudible 0:28:06] competitor?

Rahul Guha: We haven't faced that challenge at all, Aditya. We work with almost every health tech company in the country. It has never really affected Thyrocare. We do -- even on PharmEasy, we sell a

Thyrocare. So if you see -- go on the PharmEasy app, you'll see it's powered by Thyrocare. And if you go on any of the other apps, I don't want to name names but if you go on many of the other apps, almost every other app, you will find Thyrocare there as well.

Aditya Khemka : Right. Sorry, I have one more. On the African entity, so how much have we already invested? And you said it will get operational from the fourth quarter. So are you already sort of incurring expenses on the African business on your P&L? Or are you capitalizing those businesses as for the operating expenses?

Rahul Guha: So overall, we have a commitment. As I had said, we would basically invest about INR 10 crores overall in Africa. So far, we have funded the African entity with roughly about \$375,000. So that's the level of investment we've put. The partner also will put, match us at every stage of the investment. The work is underway. The lab will get operationalized this month. So we will be starting operations in this quarter. All the equipment has been bought. The lab has been set up.

I mean the final touches are going on at this point in time. But I think we are on track to launch this month. And so the business will be operational, I would say, in this quarter.

Aditya Khemka : So you are not currently incurring various -- as in -- when I say currently, I mean in the December quarter. So in the December you did not have any operating expenses from the African entity...

Alok Jagnani: So there is no operating expenses as the pre-operation expenses are incurring and all is going to be capitalized once we are going to start the operation, which is going to happen at the end of quarter, what Rahul has said.

Moderator: Thank you. The next question is from the line of Shubh from RatnaTraya Capital. Please go ahead.

Shubh: So I wanted to get some more clarity on the API front. How do you see it going further as a percentage of revenue? And second, what is the difference in pricing given to API and to other

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Rahul Guha: Sure. See, in growth, I -- as the funding comes in and the investment plans get drawn up, right, I would expect API to grow between -- in the teens, I would say, not the super normal growth that we saw earlier. So I would be happy if that business grew at about 15%, maybe between 15% and 20% is where I would be happy if that business grew. On the second question, the pricing is declared as part of our related party transactions.

I mentioned earlier, they're the largest customer that we have by far. So we have a standardized rate across all our B2B partners and we have a slab-wise discount from there, depending on the size of the customer. At this point in time, PharmEasy enjoys a 15% discount from the B2B rate or basically our market rate, which is in line with the slab-based discounts that we have basically put in place.

Shubh:

Understood. Understood. And second, earlier, you had taken some efforts and experiments with changing the realizations. How do you see it going forward, like sales and realizations versus change in volumes? What is your view on that?

Rahul Guha: See, we normally, I think, pretty consistently do volume growth of roughly about 10%. I expect that to continue and maybe a 3% to 4% from price. So that's kind of where I see us land.

Moderator: The next question is from the line of [Meg Shah], who is an individual investor.

Meg Shah : And first of all, congratulation to Rahul ji for the appointment as President, operation of API, in addition to the MD of Thyrocare. But sir, my question is that will you be able to serve both the company efficiently?

Rahul Guha: See, the -- as it has been declared, I continue to remain the Managing Director and CEO of Thyrocare. I am spending time at API to coordinate the synergies across the group. So individual business leaders in API run their individual businesses. I'm there to ensure synergies are captured across the group. So -- and for that, I'm spending time as needed but my primary focus remains Thyrocare and ensuring that Thyrocare continues to grow and deliver the margins expected.

Meg Shah : Yes. Because the many investors are wishing that you remain the MD and you look after the Thyrocare business more than the API business. It's our wish. That's it, my concern. And sir, the second question is how much capex the company Thyrocare has incurred, particularly during the quarter 3 and the first 9 months of the year on at pathlab and radiology? And when the benefit of this capex will be seen in the profitability?

Rahul Guha: Sure, sure. I'll let Alok take that question. But Meg, first, thank you for the vote of confidence and be rest assured that my primary responsibility continues to ensure that Thyrocare is on track. With that, Alok, you can take the capex .

Alok Jagnani: So on the total capex , what we spend over the period of 9 months is around INR 46 crores, of which INR 6 crores pertains to previous year, which cash outflow has happened in the current financial year. As mentioned in the brief presentations also, we have spent -- we have replaced our 24 machines on which we have spent around INR 16 crores.

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Around INR 24 crores we have spent on lab infra and interior upgradations because all the labs are old and require lots of upgradations and internal works, so that has happened. Around INR 6 crores we have spent on the equity and IT leased line upgradations, IT capex and all. So in total, we have spent around INR 46 crores in the last 9 months, that include INR 9 crores we spent on Pulse radiology business also.

And on the benefit side, the second question is the -- how is going to benefit us? The capex spend has happened mainly because, mainly on account of -- as the machine was -- is between 9 to 12 years and require a replacement. So on the quality front, we have spent all this capex , so that we will be more focused on quality, so that reports are more accurate and correct. So that's resulted in converting revenue and price from consumer sites -- franchisee sites. On radiology business, we have spent INR 9 crores and that we have through Pulse investment and which is coming in radiology investments. We are expecting over a period that's going to convert into a more beneficial -- more EBITDA for investors.

Rahul Guha: And Mr. Shah...

Meg Shah : My question is that whenever there was some CapEx that OpEx is reduced, their capacity is increased, quality is increased. And ultimately, because of this, the profitability increase. When the -- in real term -- in the financial terms, the profitability will increase.

Rahul Guha: So sir, there are 2 parts to this. One is, what we invested in capex last year. If you see our gross margin also, most of that is, comes

from efficiency in our cost of operations. So if you look at the P&L also, our gross margin last year was about 68%. This year, it is 71%. Of course, some of that has come from price hikes but also from the efficiency from the equipment. So one part of that is, we are already seeing the efficiency in the gross margin, right?

The second part, which is, wherein when we have invested in quality, over time that results in more and more confidence on our reports and more and more customers coming and repeating with us. That I think will come over the next couple of years, the confidence from the equipment and the -- but the benefits have already come in the gross margins.

Meg Shah : Okay. Sir, in this quarter, there is a revenue growth. And I think it is mainly due to the value growth, not the volume growth. The volume has reported some degrowth. So when the company will be on a sustainable volume growth path?

Rahul Guha: As I said, the volume decline has largely been because of the exit. We exited from the government business. And we -- our parent PharmEasy's business has degrown. The rest of the business is growing quite well. But if you remove these 2 effects, API and B2G, rest of the business has grown at 15% year -on-year. And as these reach the bottom or have reached the bottom, I expect the business also to get back on a strong growth trajectory.

Meg Shah : Okay. Okay. Suppose we don't get any business from the government or the very less business from the API, then this -- can we consider this is a base and now on what the company will report the volume growth?

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Rahul Guha: Yes, yes, very much. In fact, if you look over the last 2 quarters, we were tracking quite well on the volume and revenue growth. This quarter, as I said, we had put some pricing strategy in place, which we were expecting the smaller companies to -- smaller technology partners to act upon because we had [inaudible 0:39:46] prices there, which was expected. But as Aditya had also asked, right, I think for all the business we have tried to exit, I think we have now reached the bottom. So going forward, I think we should be back on a good growth track.

Meg Shah : Sir, earlier, we were making the good profit from the radiology business [inaudible 0:40:10] radiology business has reported the, I think the operating loss or net loss. So when this business will achieve breakeven or what is the reason for making them loss?

Rahul Guha: The radiology business, as I said, sir, last quarter was in a loss. This year, it has come back into profit. And the reason it had gone into a loss was largely because of the equipment being very old and having a lot of breakdowns in the business. We have been working very hard to [inaudible 0:40:47] of that and bring the business back on track. As I said, from a cash flow basis, the radiology business is quite fine. What hurts us is effectively the equipment life and the breakdowns and there's still a substantial depreciation charge that hits us.

Moderator: We'll move to the next question. The next question is from [Aditya] from Securities Investment Management Company. Please go ahead.

Aditya : Sir, we break up our revenue into 3 parts that is franchisee, partnerships and B2C. Sir, if you can just help us understand the difference between all these 3 categories in terms of scope of work of Thyrocare for the customers, pricing or any different parameter?

Rahul Guha: Sure. It's also explained in the presentation towards the end but I'll just give you a brief summary. Our franchisee business are offline partners. So think of it as Thyrocare branded pathology lab or pathology collection centers or even independent pathology centers who outsource their testing to Thyrocare. So that is largely our franchisee business. So it's an entirely offline business. where we partner with roughly now, I think, ab

out 7,500 of these offline partners.

Our partnership business is working with many of the large insurance players, large pharma companies, large health tech players, of which one is PharmEasy, where we not only support them with the testing but we also support them with the blood collection, right? So we have a fleet, as I mentioned, 900 company phlebotomists and 400 franchisees phlebotomists who go to the house and service the orders.

So like for PharmEasy, PharmEasy places an order on Thyrocare, Thyrocare technician will go to house, collect the blood sample, bring it to a Thyrocare lab and then Thyrocare will release the result to PharmEasy. So we have similar arrangements with almost all the health tech players. And we also work with many large corporates, insurance, as well as pharma companies to help do the same. So that's the scope of partnerships business. The B2C business is the business that we get from thyrocare.com, the website, ThyroApp, which is our app.

Aditya : Got it. And sir, these individual franchisees, are they only give the volumes to Thyrocare or they can give the volumes to other players as well?

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Rahul Guha: So as I said, the Thyrocare -branded franchisees work exclusively with Thyrocare. But the non-branded franchisees work with multiple partners.

Aditya : And currently, what will be the proportion of PoCs?

Rahul Guha: So from a count point, roughly about 800 Thyrocare branded. The remaining are Thyrocare unbranded. From a business point of view, I think it will be -- about 30% from a business point of view will be Thyrocare branded and 70% will be Thyrocare unbranded.

Aditya : Okay. Okay. And sir, in terms of geographic expansion, we had set up lot of labs when you took over the company. So have we [inaudible 0:44:33] capex expansion will be completed or do you -

- you still feel that we have to open more labs and deepen our geographical presence?

Rahul Guha: See, I think by and large, the capex expansion in India is completed. We may do a few selective expansions where we feel there is a good opportunity. And our presence is low. But by and large, we are quite happy we have about, I think, 50 labs across the country. If you drop a pin anywhere in India, there will be a Thyrocare within 200 kilometers. So by and large, we are quite happy with the expansion. I don't think we'll be expanding too many at this point in time. But we will look at Africa, as I mentioned.

Aditya : Got it. Sir, just one last question. We had a partnership revenue of total INR 35 crores and INR 23 crores was excluding API and B2G. So this balance INR 12 crores is purely from PharmEasy?

Or we have some government revenue as well?

Rahul Guha: Government business will be about INR 1.5 crores. The vast majority will be PharmEasy.

Moderator: The next question is from the line of Rahul Agarwal: from Incred Equities.

Rahul Agarwal: Just 1 question on the international business. How much time will it take for this business to reach your internal revenue expectations at Tanzania?

Rahul Guha: So I would say 2025, FY 2025. Our expectation from Tanzania is quite small. I would anticipate in the INR 2 crores to INR 3 crores range. That's it. But then as we scale up in the next year and the year after is when we expect it to be substantial.

Rahul Agarwal: Okay. And related question was, so let's say, annually you reach INR 2 crores, INR 3 crores. When do we start investing for other locations? Does it start in fiscal '25 at all? Or do you want to stabilize Tanzania first, experience that and get -- and then maybe FY '26, you start investing for other locations?

Rahul Guha: Yes, yes. Africa, we will not do the lab in 2025. We will only look after we have stabilized and we have fully understood the market.

Rahul Agarwal: Okay. Get it. So the next year's capital expenditure plan, would you have a number

to that, fiscal

'25 apart from...

Rahul Guha: I'll ask Alok to take it up.

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Alok Jagnani: So approx, in this quarter, we are going to spend around INR [6] crores approximately, out of which maybe INR 1.5 crores is going to be on account of machines replacement and INR 3.5 crores is going to be for miscellaneous. That's the projections we have.

Rahul Guha: He is asking for next year.

Alok Jagnani: Next year, we'll come back on the guidance.

Rahul Guha: It's not going to be substantial, Rahul but we'll come back with a specific number. I would anticipate it to be around the INR 30 crores range.

Rahul Agarwal: Okay. Get it. And fourth quarter, Alok said was INR 5 crores, right?

Alok Jagnani: Yes, approximately INR 5 crores we are projecting, we're going to spend. INR 5 crores.

Moderator: We will take that as the last question. I would now like to hand the conference over to Mr. Rahul Guha for closing comments.

Rahul Guha: Thank you, everyone, for joining us and spending time with us and for all the questions. I hope I've been able to answer them to your satisfaction. As always, we continue to remain focused on our strategy, which is to be the most affordable, good quality, diagnostic test partner for everyone in the health care business and we continue to execute on that strategy. We have been investing in improving our quality, improving our reach and ensuring our turnaround time is as close to best-in-class and we made substantial progress on all of this.

I thank you for all your support on this journey and with that, I'll hand over to Pratik to close the call.

Pratik Hire: Thank you, ladies and gentlemen. On behalf of Thyrocare Technologies, that concludes this conference. We thank you all for joining this evening. Over to you, Rio.

Moderator: Thank you very much. With that, we conclude the conference. Thank you for joining us. You may now disconnect your lines.

Call: May 2024

May 17 , 2024

The National Stock Exchange of India Limited BSE Limited
Exchange Plaza, Phiroze Jeejeeboy Towers
Bandra Kurla Complex, Dalal Street,
Bandra (E), Mumbai - 400 051 Mumbai - 400 001
(SYMBOL: THYROCARE) (SCRIP CODE 539871)

Dear Sir/Madam,

Sub: Transcript of post results earnings conference call held on May 14, 2024 with
Analysts / Investors

Ref: Disclosure under Regulation 30 of the Securities and Exchange Board of India
(Listing Obligations and Disclosure Requirements) Regulations, 2015 ("SEBI Listing
Regulations")

Pursuant to Regulation 30 and 46(2) read with Part A of Schedule III of the SEBI Listing
Regulations, please find enclosed herewith the transcript of earnings conference call with
Analysts and Investors held on May 14 , 2024 for financial results of the Company for the
quarter and financial year ended on March 31, 2024.

We wish to add that the same has also been made available on the website of the Company
<https://investor.thyrocare.com/financials/quarterly-financial-results/>

This is for your information and records
Yours Faithfully,
For Thyrocare Technologies Limited,

Ramjee Dorai
Company Secretary and Compliance Officer

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"Thyrocare Technologies Limited Q4 FY24 Earnings
Conference Call"

May 14, 2024

M
ANAGEMENT : M R. RAHUL GUHA – MD & CEO, THYROCARE
TECHNOLOGIES LIMITED
MR. ALOK JAGNANI - CFO, THYROCARE
TECHNOLOGIES LIMITED
MR. NITIN CHUGH – CHIEF COMMERCIAL OFFICER ,
THYROCARE TECHNOLOGIES LIMITED
MR. PRATIK HIRE – STRATEGY TEAM, INVESTOR
RELATIONS , THYROCARE TECHNOLOGIES LIMITED

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May 14, 2024

Moderator: Ladies and gentlemen, good day and welcome to Thyrocare Technologies Limited Earnings Conference Call.

As a reminder, all participant lines will be in the listen only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing "*" then "0" on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Pratik Hire from Thyrocare Technologies Limited. Thank you and over to you, sir.

Pratik Hire: Thank you, Neerav. A very good evening to all and thank you for joining us today for Thyrocare's Earnings Conference Call for Q4 and Annual Results for FY24.

Today, we have with us Mr. Rahul Guha - MD and CEO; Mr. Alok Jagnani - CFO and Mr. Nitin Chugh - Chief Commercial Officer along with other key members of Senior Management on this call to share highlights of the business and financials for the quarter and the annual results.

I hope you have gone through our Results Release and the Quarterly Earnings Presentation, which has now been uploaded on the Stock Exchange website. The transcript of this call will be available in a week's time on the company's website. Please note that today's discussion may be forward-looking in nature and must be viewed in relation to the risk pertaining to our business. After the end of this call, in case if you have any further questions, please feel free to reach out to the Investor Relations team. I now hand over the call to Mr. Rahul Guha to make the opening remarks.

Rahul Guha: Thanks, Pratik. Good evening and welcome to all on the call. Thank you for taking time out from your busy schedules to join us this evening.

I will do a quick introduction to us on the call. My name is Rahul Guha. I am the MD and CEO of Thyrocare and thank you for the opportunity to present the Q4 and annual results for FY24. I am joined with my colleague, Alok Jagnani, who is our CFO and Nitin Chugh, who is our Chief Commercial Officer along with Pratik Hire who is a part of our strategy team and leads investor relations.

As I did in the last call, I will start with a quote from Nelson Mandela in recognition of our foray into Africa, "It is in your hands to make a better world for all who live in it" and we believe Thyrocare can bring our business model to Africa to make affordable and good quality diagnostics available to all. I will give you some of the key highlights of what we have been up to in the last quarter.

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Before we get into the details of this quarter, I will reiterate the pay-for-performance pricing structure that we implemented at the beginning of this financial year. Earlier, our discount structure was one-size-fits-all, but now we have moved to a slab-based pricing model which we implemented in May 2023. This has led to an increased energy with our franchise network with motivation to move up volumes and enter higher slabs. It continues to result in the movement towards larger franchisees and enables much greater reach for our larger partners. Quality remains the key priority for us. It is an ongoing journey and we are proud to announce that 25 out of our 30 labs are now NABL accredited. Currently, 94% of our total sample load is processed in NABL labs. It is significant achievement considering only 2% of pathology labs nationwide hold this accreditation. To validate our commitment to quality, we conducted an independent study published in the International Journal of Advanced Research, Ideas, Innovations and Technology. The findings revealed that 9 out of 10 doctors trust Thyrocare report and confidently recommend our services to their patients. This endorsement underscores the dedication and effort we have invested in maintaining high

quality standards. On the quality front, as you would see in the presentation, we have reduced our complaints per million samples by 30%. A P90 TAT is now less than 20 hours Pan India, as compared to 24 hours last year. Beyond the work on quality, we continue to selectively expand our offering. Aarogyam has been a flagship brand in a preventive healthcare segment and now we have two more brands, JAANCH and Her Check. JAANCH, as I have said before, is targeted toward lifestyle challenges or for you to better understand your health. We have solutions across the spectrum for anything you might be worried about. Whether it is fever or something more serious, why is my hair falling? Cancer screening as well as deep investigations for common chronic diseases like diabetes, heart health, amongst others. In its first year since launch with a very limited marketing budget, JAANCH has done Rs. 12 crores. We have also revamped our Gynac portfolio and relaunched it under the brand name of Her Check and it focuses on Women's Health. In the six months since launch, we have touched roughly Rs. 6 crores in Her Check. I'm happy to share that our pathology revenue growth has surpassed pre-COVID levels. From FY17 to FY20, our non-COVID revenue CAGR growth of 13%, however, in the period from FY21 to 24, our non-COVID revenue CAGR has soared to 18%. This remarkable achievement speaks volumes about our strategic business expansion, investments in quality enhancement and organizational

restructuring effort. Active franchise base has now more than doubled over the last three years. On the financial front, the company enjoys a good cash position and we have proposed to distribute a final dividend of Rs. 18 per share. We are also very proud of some of the key milestones that we achieved in the last year. Now, we have 7900 active franchisees. As a result, we processed 22 million samples and served 15 million patients in the year, which is 8% and 6% year-on-year growth respectively in volume terms for our core business. Total tests conducted for FY24 is 147 million. In our nuclear business, we performed 31,500 PET CT scans this year which compares to 30,800 last year. Our partnerships business, which excludes API and the government did phenomenally well in the quarter and grew as we onboarded new clients

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in health techs, insurance and other segments and we have continued to grow the existing accounts.

Also, I am happy to share we have completed the acquisition of Think Health to strengthen our offering for the insurance segment with the additional capability of ECG at home. This allows us to give our insurance partners a one stop solution for blood and ECG testing and further deepen our presence in the pre-policy medical checkup and annual health checkup market. And now, we are the company who offers health checks with ECG at home, and I am sure that will enhance our Aarogyam brand.

On the B2G side, we continue to remain focused on technologies where we have expertise. And we continue to execute TB projects in the government, state of Gujarat and Maharashtra. On the international expansion side, our lab is now set up in Tanzania and we are going out in the market in full swing. I am proud to say in April we processed our first sample in Tanzania. Also, as I have been telling you in the last quarter, we have revamped our equipment platform. Our equipment was very old, with an average age of 12 years. We have added 30 plus new machines during the financial year, resulting in the average machine age coming down to now six years. These new additions have dramatically improved our reporting accuracy and turnaround time. I will now hand over to my colleague, Nitin, to cover the highlights for the quarter and the annual business performance.

Nitin Chugh: Thank you, Rahul. A warm welcome to each one of you. I will briefly update you about the highlights of Q4

and annual business performance. Our franchisee business for the year showed a revenue growth of 14% year-on-year and in Q4 our franchisee business showed a revenue growth of 13%. Reiterating what we had mentioned last quarter, growth rate in franchise business has slowed down because of the churn in the small franchisee segment. This churn was expected after we implemented our volume-based pricing. Our big franchisees have grown at 25%, while the small has degrown by 39%.

In terms of mix, contribution from big franchisees has increased from 81% to 90%. Majority of the small franchisees have been churned out and the base is stabilizing now. Going forward, the franchisee business should return back to its high teens growth rate. This is evident from the improvement we see in franchisee growth of 13% year-on-year this quarter as compared to 11% year-on-year of last quarter. Our partnership business, which excludes API and B2G for the year showed a phenomenal growth of 23%, whereas in Q4 it showed a tremendous growth of 40%.

On an overall level, our partnership business for the year has remained stable on the account of decline in API and closure of MCGM contract. Overall, the pathology business excluding materials and others for the year has grown by 10% year-on-year and in Q4 it is increased by 16% year-on-year. Radiology business, including Pulse Hitech for the year did a strong revenue growth of 18% year-on-year. Overall, at a consolidated level, we did 9% year-on-year revenue growth for the year and Q4, we did 14% year-on-year revenue growth. Balance, I will hand over to my colleague Alok to cover the financial results.

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Alok Jagnani: Thank you, Nitin, and a warm welcome to everyone joining us today. Before diving into the specifics, I would like to touch upon our ESOP program, which we have discussed in previous quarter. This initiative, implemented at the group level, aimed to retain talent within the Thyrocare. This ESOP issued by the parent company will vest over the period of 6 years. From an accounting perspective, we recognize these ESOPs in accordance with IndAS standard as a book entry towards share based payment. They are reflected as an expense in the profit and loss account as an equity contribution from the parent in the balance sheet.

The total value of ESOPs amounts to Rs. 45.53 crore over a period of 6 years. It is crucial to note that this represents a cashless charge, not a cash outflow. According to IndAS, options are reviewed at the grant date using the Black-Scholes formula, resulting in a charge to P&L over the vesting period. Typically, this leads to an impact in the year of the grant followed by

proportionate charge over the vesting period. Further details are available in the presentation. As these expenses are non-cash operating expenses and do not affect the company cash outflow, we calculate normalized EBITDA which accounts for these expenses along with the provision for bad and doubtful debts.

Now, moving on to our Financial Performance:

Revenue for the year stood at Rs. 524 crores standalone and Rs. 572 crores consolidated, making an annual year-on-year improvement of 9% at consolidated level. This growth was supposed by better partnership revenue growth, excluding B2G and API of 23%, franchisee growth by 14%. Pathology revenue excluding material and others increase by 10% year-on-year. In Q4, revenue increased by 14% year-on-year, primarily driven by strong partnership revenue growth excluding B2G and API by 40% and franchisee business grew by 13%. Pathology revenue excluding material and other grew by 16% year-on-year.

Our margin for the year improved by 2% point driven by price increase, volume growth and better negotiation, employee expenses incurred increase year-on-year due to the increments addition to the growth team and quality personnel to meet and reveal deployment. Standalone nor

malized EBITDA margins for the year improved by 1%, primarily due to the improvement in gross margin. EBITDA margin in radiology NHL stood 9% versus 18% year-on-year, mainly due to the increased operational cost and change in revenue mix between scanning and FDG sales.

At a consolidated level, our normalized EBITDA at 28%, which is 1% lower than last year, primarily due to the increase in business promotion, marketing spends and new business expansions. We reported a consolidated normalized EBITDA of Rs. 162 crore for the year, directly reflecting to our cash flow operation, which amounts to Rs. 168 crore.

With that, now I pass follower to Rahul for our strategic updates.

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Rahul Guha: Thank you, Alok. Briefly, I would like to take a few minutes to recap to you our strategic direction and then I will open it up for Q&A.

First, I will reiterate our value proposition to the customer. We will continue to remain an affordable option to all patients with good quality and on time reports. All our efforts on our value proposition is towards ensuring low cost to the patient, assurance on quality of testing through our certifications and engagements with doctors. We have made substantial progress on this, which I updated in my initial comments and is reflected in the presentation. This will remain at our core and will guide all that we do.

Second, our strategy, we continue to maintain our strategy of being the B2B partner of choice to all front-end diagnostic services companies in India, whether it is a small diagnostic center in a semi-urban area, a pharmacy in a metro, a small nursing home and individual doctor or a leading online diagnostic platform or health tech marketplace. We are happy to work with them to provide low-cost robust testing solutions so that they can serve their patients in the most effective manner. If they require phlebotomy, we are happy to mobilize our phlebotomy network of almost 900 companies and 400 network phlebotomists to serve them better. This strategy has been working well for us with both our franchise and partnerships business posting strong growth. We continue our commitment to add more value added services for our partners and with the acquisition of Think Health we have entered into ECG at Home Services which will strengthen our offering to our insurance partners. Further, to reiterate, as I have shared in the last few quarters, we have 3 pillars of growth. The first is our franchise business. The focus is to take our franchise business deeper into India with the focused test menu and provide our clients with a frictionless experience to transact with us and provide their customers the best testing experience that they can. We continue to focus on private as well as public partnerships. In the public space, we will focus on TB and infectious disease where we are by far one of the strongest players in these segments with large screening programs partnering with health bodies and funding agencies who participate in this segment.

Additionally, we will continue to expand our partnerships across healthcare companies, hospitals and other health services companies and enable them to provide diagnostic testing to their customers. The third area is relatively new for us. We believe we have a strong and robust B2B model with the core of execution. We are ready to take this model forward with our first entry in Tanzania. The opportunity is significant for us to be able to launch affordable tests in the country of Tanzania. That in a brief is our mandate as management. Thank you so much for giving us a patient hearing. I will once again end with a quote from the Mahatma, "Find purpose.

The means will follow" and our purpose remains to provide affordable, high-quality testing to the masses.

With that, we will open it up for Q&A.

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erator: Thank you very much. We will now begin the question-and-answer session. The first question is from the line of Aditya Khemka from InCred PMS. Please go ahead.

Aditya Khemka: Rahul, first question on the volume growth, did I hear you right, volume growth year-over-year for the quarter was 6%, is that right?

Rahul Guha: For the total business.

Aditya Khemka: I think you were mentioning about the franchisee business, the core business. So, for the core business, I think you said volume growth was close to 6%. I just couldn't understand whether it was for the year or for the quarter. That is the clarification I need?

Rahul Guha: So, just to clarify, it was for the year. Quarter-on-quarter, it is 10%. I am talking about sample growth, patient growth will be slight lower than that, but our year-on-year, financial year volume growth for the core business is 8% and quarter-on-quarter 10%.

Aditya Khemka: Volume growth is 8% for the financial year?

Rahul Guha: Yes.

Aditya Khemka: And the second question, Rahul, on the non-COVID business, slide 10 on your presentation deck, so non-COVID business you are showing 18% CAGR, FY21 to 24. If I look at the number of franchisees, that has also gone up from 3000 to 8000 and I know that this non-COVID revenue would have two parts, the B2B franchisee part being one and the other being your other partnerships with government and healthcare aggregators etc. My question is that if your franchisees have doubled to like 8000 and I think in a comment one of your team members mentioned that now 90% of the revenue comes from large franchisees. So, out of this 8000 franchisees, would like approximately 1000 qualifies large franchisees and would 7000 be roughly smaller franchisees or the marginal franchisees or what would be the split between the 8000?

Rahul Guha: So, roughly without going into, so first let me just take the overall COVID. I think our franchisee business from the low COVID base recovered very quickly. And then as we added more and more franchisees we continued to see the uplift in our growth profile. As I updated last quarter, we had a little bit of a road bump because we saw a lot of churn in the smaller franchisee, but now the larger franchisees have picked up and things are more or less okay. In terms of count, if you look at the roughly 7900 franchisees, about 3500 are what we categorize as Diamond to Silver and about 4500 would be bronze and others, so it is a rough I think, 45-55 kind of split between large and small.

Aditya Khemka: The revenue ratio?

Rahul Guha: The revenue ratio would be roughly 65:35.

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Aditya Khemka: 65 for the large and 35 for the small?

Rahul Guha: Yes.

Aditya Khemka: So, now when I look at FY24, if my understanding is correct, you had almost three businesses which were declining in FY24. One was your B2G, the other was your smaller franchisee business and the third was pharmacy business, right. Now, on all these three plants as of 4Q, have all three businesses stop declining or is there still some way to go on either of these three business to decline?

Rahul Guha: The small franchise business had stopped declining as I had shared last time by the time we had come into October, November, so we had bottomed out there and therefore you see the accelerating growth coming out in Q4. So, I think the small franchisee base has bottomed out.

The government business has bottomed out because it is almost practically 0 now, except for the EV contracts that we are doing very selectively. So, I would say that has bottomed out, but we are not entering into many of these government contracts, so at 0, it won't give you a growth jump over there. On the pharmacy side, actually we have seen some recovery in Q4, but Q4 as you know is the big season for packages and annual health checkups, so I think going into the next quarter, we will have to see how

it pans out, right, but at least Q4 we saw a decent recovery on the API side as well.

Aditya Khemka: But could you tell us if pharmacy now offers like deep discounts like earlier used to do to acquire customers to sell Thyrocare packages or are they now also on a unit economic level making money when they sell Thyrocare packages?

Rahul Guha: The diagnostics business at pharmacy is now profitable at the proper transfer price as approved by the shareholders as part of the related party agreement. So, effectively now, Thyrocare, of course earns a decent EBITDA anyway, which was never dilutive, but the pharmacy business also this year has turned profitable, the pharmacy diagnostics too.

Aditya Khemka: So, then when we look at year-over-year for FY25 over FY24, there should be no reason why

pharmacy should not at least even flat or grow from the days that they are formed in FY24 for Thyrocare, I am saying?

Rahul Guha: Logically, yes, but it is difficult to speak on behalf of our customer, but logically what you are saying, if this is the profitable pool for pharmacy as well, I think all intents are there to grow, but if you go back to the old customer acquisition and discounting, then of course, the profitability gets hit. But yes, logically what you are saying, there is no reason to believe that it will degrow from next year.

Moderator: Thank you. Next question is from the line of Prakash Kapadia from Spark PMS. Please go ahead.

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Prakash Kapadia: Couple of questions from my end, so how much of our franchise revenue is from the Thyrocare brand in FY24 and how much is the revenue from local pathology lab centers or B2B tie-ups? And in this business the right to and remains more geographic expansion or is it increasing throughput per franchisee or getting more B2B business if you could give some thoughts on this?

Rahul Guha: Yes. So, roughly, about 40% of our topline is Thyrocare branded, right and the remaining 60% is B2B tie-ups. Also, ideally I should count all my Aarogyam packages as Thyrocare branded regardless of where they are sold because they are branded as Aarogyam. But for a minute I am keeping that aside, roughly 40% of our revenue comes from centers which have a Thyrocare board and a Thyrocare Livery 27.06 and all of that. In all the instances, most of our franchisees would share a Thyrocare report. So, that is definitely out there. The right to win, see largely I think throughput per franchisee is only so much. It takes time to scale up, right and a lot of it is how much share of wallet you are able to capture. They typically have one or two local partners who they work with as well, and then Thyrocare as a second partner and so on. So, your share of wallet is always in a highly competitive space, right, where you are constantly in a price battles where Re. 1-Rs. 2 and then someone can move the volumes to another B2B partner. I

think the right to win really comes from leveraging the Thyrocare brand, our image now and our investments in quality and going deeper and deeper into India where frankly speaking, the competitive side is not the large, branded labs, but actually the local labs which don't have NABL, don't have the quality processes that we have in terms of cold chain, don't have the checks and balances in terms of every report has a QR code, all of that right. And so therefore people in Tier 2 towns at pretty much the same price of a local lab is getting a high-quality report with all the backing of Thyrocare, so I firmly believe our right to win is there.

Prakash Kapadia: On the partnership business, would I understand there are two facets to that business also. So, during the non-KPI non-B2G, what will drive this business from here on and how are we differentiating there? And I think in the B2G business, you said it has bottomed out. So, any risk of further write offs on receivables because we

have seen some provision which we have done now also, so in these two segments, if you could give some color?

Rahul Guha: On the first one on partnerships, I think we have many kinds of partnerships, right. I think the health tech partnerships, we have gone quite deep in that. There is not that much more growth we will get from there, but two large unexplored avenues for us has been the hospital segment and the insurance segment, right, both of which are very large customers who buy diagnostics, hospitals of course where we go in and set up the lab or work with them for their outsourced business as a partner. And then the insurance players, almost every policy that is issued as a pre-policy medical checkup and also they offer many of their policy holders a free annual health checkup. So, it is a very large market, which has been largely untapped by Thyrocare. So, we will continue to drive the franchise growth by exploring these avenues. And as I mentioned, with the acquisition of Think Health, we now have ECG at home as a capability so that only enhances our right to win in this segment. You must remember we are the only national player that has a company owned phlebotomist fleet. What that means is if you are a partner with Thyrocare with

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a single API integration, I can get you activated with diagnostics in 300 cities overnight. So, if you are an insurance player with one single API integration you can service all your policy holders with annual health checkup and all your PPMC screening in 300 cities through a single window. No other company offers this to their partners.

Prakash Kapadia: What I was trying to understand out was, a lot of these hospitals are focusing lot more on diagnostic and have become pretty aggressive. So, is there an outsourcing arrangement which you think will drive the hospital segment? Or some specialized tests which the hospitals would not want to give lower volumes?

Rahul Guha: No, it is largely outsourced. I think specialized test for us as Thyrocare is not our cup of tea. So,

I would say largely outsourced would be the focus. Many of the equipment in the hospital is very low throughput and therefore very high cost to operate. So, many times hospitals find that even though they have a lab in their hospital, the Thyrocare billing price to them is sometimes lower than their reagent cost because of our economies of scale and the throughput of our machines. So, they keep some of the basic tests in the hospital and then outsource everything else.

Prakash Kapadia: I understood the hospital and the insurance part. The second part you were saying?

Rahul Guha: Yes. Your second question was on the B2G business, right?

Prakash Kapadia: Yes.

Rahul Guha: See, there firstly, if you recall, almost two years ago when I first took over, we had a government outstanding of close to, I think Rs. 80 crores, I think almost all that money has either been received or provisioned. So, I think though government outstandings as a sword hanging on our neck is more or less behind us. I don't anticipate any more further significant provisions on the government business. That being said, we have also realized that we need to be focused when we do the government business and we have to again focus on our right to win. We are the strongest in TB testing and infectious disease testing where we have the full range of technologies starting from the very basic sputum tests, all the way to whole genomic sequencing for TB. So, we have decided to focus on these areas where our strengths are there. And work with the government to partner with them on their screening program. And we feel that is the niche that we will go after. The government also is taking both these topics, TB and infectious disease very seriously and has put a lot of money behind it. So, we believe there is a reasonable opportunity there, but on the genera

I tests, we will not rush in.

Prakash Kapadia: And lastly, one data keeping question from my side, can you share the geographical breakup of our pathology business, say between North-South, East, West? What I am trying to understand are there specific areas where we are strong? Are we a Pan India player, are we maybe North and South focused, if you can give some idea that would be helpful?

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Nitin Chugh: So, see, generally our business, we are very strong in the Eastern region and the Western region.

North, our share of business is very less and our third in that ranking from the Southern region.

Moderator: Thank you. Next question is from the line of Shubh Shah from Ratna Traya Capital. Please go ahead.

Shubh Shah: Sir, can you please break down the 9% pathology growth in terms of volume, in terms of price and in terms of mix and also the mid teen or high teen growth guidance that we have for next year, similar breakdown for that?

Alok Jagnani: So, we have seen 9% revenue growth in the financial year, FY24 versus FY23 and the revenue, if you see the split, our franchisee revenue has grown by around 31% whereas our partnership revenue has gone by around 23%, this is excluding API and B2G. We are expecting in FY25, the mid teen growth is degrowth that is between 15% and 18% growth is expected.

Rahul Guha: And see overall sample growth for the year would be, I think, roughly about 2%, but this should not be considered that then all our growth has come from price. Actually, what you should consider is we had a very high-volume contract with MCGM which is the Mumbai BMC which was at a very low revenue per sample. That contract ended last year, so actually the volumes came out of that, but our franchise and partnerships business covered up for that. So, it is actually the mix of business shifting that has given us the revenue growth, not a price increase.

Shubh Shah: Second, can you explain the provisions, what are the provisions for this year?

Alok Jagnani: So, provision for receivables, you are talking about here. So, in the current financial year, we have provided around Rs. 8.7 crores of provisions against the receivable that is mainly on account of two accounts. One is on the government accounts; we have provided around Rs. 2.5 crores of provisions for the receivables which is doubtful and another provisions has been taken against the international business as well. We have taken around Rs. 6 crores provision. And still that amount we are litigating, and things are going to be all stable, then we will see, but mainly this Rs. 8.7 crore provision pertains to B2G business where we have provided and Rs. 6 crores pertains to international gulf base.

Shubh Shah: And also, are you planning to take some additional marketing or advertising cost?

Rahul Guha: No. So, the marketing and business promotion cost, what is current year model is going to remain the same for the next financial. No additional.

Shubh Shah: And this financial letter, those costs mostly pertain to?

Rahul Guha: This cost means in the current financial year, the cost increase in the other expenses mainly pertain to business promotions, marketing spends on Her Check, JAANCH and other branding what we have done. And that reflect in our revenue also, the revenue growth what we have seen.

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Moderator: Thank you. The next question is from the line of Jainam Shah, Individual Investor. Please go ahead.

Jainam Shah: So, I wanted to ask that the interim dividend of Rs. 18 was declared in April and the dividend of Rs. 18 that you declared today, are those both dividends same or different as in the total dividend is 18+18 or just Rs. 18?

Rahul Guha: So, interim dividend has been declared last year that pertained to the financial year FY23, not pertain to the current financial year and subse

quently the amount of interim dividend was the

final dividend has been approved in AGM. Now, coming to the current financial year, we have proposed the Rs. 18 dividend subject to approval of shareholders in AGM. There is no linkage on that.

Jainam Shah: And also, I just wanted to know about the growth strategy in FY25 guideline for the FY25?

Nitin Chugh: See the growth strategy is that we will divide it into two parts, right. One is your core franchise business. I think the core strategy will be like Rahul said, to go deeper into the country, right, expand our network, expand in cities and in geographies where we are not very strong today.

And secondly on the partnership side, the main focus will be on getting bigger accounts on board, specifically focusing on our insurance play with the help of Y2H, I think we can make a lot of inroads in the insurance business with having both pathology and ECG. So, these two areas will be our core focus.

Jainam Shah: And also can you tell me about margin guidelines, EBITDA margin guideline?

Alok Jagnani: So, margins are going to be more or less same sort we are expecting in the next financial year also. It is going to be between 25% and 50%.

Rahul Guha: See, margins will remain flat. You must remember we are investing in a few areas, right. We are investing in Tanzania where currently the lab is set up and the revenue will take some time to scale up. We are also investing in Think Health and scaling it up, so there will be some costs

that we will incur to scale up these businesses. The four businesses will of course give us operating leverage, but that operating leverage we will reinvest in some of these new initiatives.

Moderator: Thank you. Next question is from the line of Neeraj from Prospero Tree Financial. Please go ahead.

Neeraj: Actually, our promoters are still pledged and recently we did a fund raise so why the promoters are not pledging the shares actually it gives a very bad impression that promoters shares are pledged and recently we have done a fund raise as well so why the promoters are not unpledged the shares?

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Rahul Guha: See, I am the MD and CEO of Thyrocare. I am not the promoter, so it will be difficult for me to comment on this question.

Moderator: Thank you. Next question is from the line of Abdul Kader Puranwala from ICICI Securities. Please go ahead.

Abdulkader Puranwala: Just on this promotion cost what you guys incurred in this particular quarter and for the full year, are you calling about that number or what you spend this quarter and for the full year?

Rahul Guha: It will be there in the detailed financials, but Alok you can give some rough numbers on that.

See, I will just give you a background of what these marketing costs are. These are largely promotion costs that we do to promote JAANCH and many of the other new packages. So, they are of three nature. One is some kind of schemes and gift that we use to our franchisees to promote the new brand. The second is we invest on a few celebrity and doctor promotions where they help us talk about the new products or specific packages that have been released for the festival. And the last is, we do incur some cost with our large franchisees to effectively have a set of franchise meets at the end of the year to discuss strategy and get their inputs on how we can grow the business. So, the marketing cost is largely in these three buckets and that is where we are investing. The exact number, maybe Alok can get back to you.

Alok Jagnani: So, the incremental numbers approximately is going to be between Rs. 8 and Rs. 10 crores we have spent on multiple projects including (Inaudible) 45.01, including promoting JAANCH, the Hitech branding and multiple other marketing.

Abdulkader Puranwala: So, this Rs. 8 to Rs. 10 crores used for the full year fiscal FY24, right?

Rahul Guha: Yes, for FY24.

Abdulkade

r Puranwala: So, about the 25%-27% EBITDA margin guidance for next year, so this 25%-27%, we are talking about the reported EBITDA or the normalized EBITDA?

Rahul Guha: Our normalized EBITDA overall as a company is roughly 28%, we expect to hold it in that range. The reported EBITDA is 24%. That will actually go up because the ESOP charges as Alok has shared in the beginning of the call starts to substantially go down, so the number we look at internally as management is the normalized EBITDA. We expect to hold it between the 28% and 29% range.

Moderator: Thank you very much. Ladies and gentlemen, we will take that as the last question. I will now hand the conference over to Mr. Rahul Guha for closing comments.

Rahul Guha: Thank you everyone for joining us and spending the time with us this evening. As always, we continue to remain focused on our strategy, which is to be the most affordable, good quality diagnostic testing partner for anyone in the healthcare business and we continue to execute on

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this strategy. We have been investing in improving our quality, improving our reach and ensuring our turnaround time is as close to best in class and we have made substantial progress on all of this. I thank you all for your support in this journey. With that, I will hand over to Pratik for closing comments.

Pratik Hire: Thank you all. On behalf of Thyrocare Technologies that concludes this conference. We thank you all for joining us. Over to you, Neerav.

Moderator: Thank you very much. On behalf of Thyrocare Technologies Limited, that concludes this conference. Thank you for joining us and you may now disconnect your lines. Thank you.

Call: Jul 2024

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Exchange Plaza,

Bandra Kurla Complex,

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(SYMBOL: T HYROCARE)

Dear Sir/Madam,

Sub: T

ranscript of post results earnings conference call held on July 23, 2024 with Analysts

/ Investors .

Ref

: Disclosure under Regulation 30 of the Securities and Exchange Board of India

(Listing Obligations and Disclosure Requirements) Regulations, 2015 ("Listing

Regulations")

Pur

suant to Regulation 30(6) and 46(2) read with Part A of Schedule III of the Listing

Regulations, please find enclosed herewith the transcript of the earnings c onference call with

Analysts and Investors held on July 23, 2024.

The same has also been made available on the website of the Company (www.thyrocare.com).

Thi

s is for your information and records.

Yours Faithfully,

For Thyrocare Technologies Limited,

Ramjee Dorai

Company Secretary and Compliance Officer

Encl: A /a 0Thyrocare

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"Thyrocare Technologies Limited

Q1 FY '2 5 Earnings Conference Call "

July 23, 2024

MANAGEMENT : MR. RAHUL GUHA – MANAGING DIRECTOR AND

CHIEF EXECUTIVE OFFICER – THYROCARE

TECHNOLOGIES LIMITED

MR. ALOK KUMAR JAGNANI – CHIEF FINANCIAL

OFFICER – THYROCARE TECHNOLOGIES LIMITED

MR. NITIN CHUGH – CHIEF COMMERCIAL OFFICER –

THYROCARE TECHNOLOGIES LIMITED

MR. KAPIL GUPTA – MANAGER , STRATEGY AND

INVESTOR RELATIONS – THYROCARE TECHNOLOGIES

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Moderator: Ladies and gentlemen, good day, and welcome to Thyrocare Technologies Limited Earnings Conference Call. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Kapil Gupta from Thyrocare Technologies Limited. Thank you, and over to you.

Kapil Gupta : Yes. So thank you, Yashashree. A very good evening to all and thank you for joining us today for the Thyrocare Earnings Conference Call for the first quarter FY' 25. Today, we have with us Mr. Rahul Guha, MD and CEO of Thyrocare; Mr. Alok Jagnani, CFO of Thyrocare; and

Mr. Nitin Chugh, Chief Commercial Officer of Thyrocare, along with other key members of the senior management on this call, to share highlights of the business and financials for the quarter.

I hope you have gone through our results and the quarterly Earning Presentation, which has now been uploaded on the Stock Exchange website. The transcript of this call will be available in a week's time on the company's website.

Please note that today's discussion may be forward-looking in nature and must be viewed in relation to the risk pertaining to our business. After the end of this call, in case of any further questions, please feel free to reach out to the Investor Relations team.

I now hand over the call to Mr. Rahul Guha to make the opening remarks.

Rahul Guha: Thanks, Kapil. Good evening, and welcome to all on the call. Thank you for making time out from your busy schedules to join us this evening. Just a quick introduction to us on the call.

My name is Rahul Guha and I'm the MD and CEO of Thyrocare and thank you for the opportunity to present the Q1 results of FY '25. I'm joined with my colleague, Alok Kumar Jagnani , who is our CFO; Nitin Chugh, who is our

Chief Commercial Officer; and Kapil, who did the introductions.

As in all my calls, I will start with the quote from Nelson Mandela in recognition of our foray into Africa. It is in your hands to make a better world for all who live in it. And we believe Thyrocare can bring our business model to Africa to make affordable and good quality diagnostics to all.

I'll give you some of the key highlights of what we've been up to in the last quarter. Before we get into the details of this quarter, I'll reiterate the pay -for-performance pricing structure that we implemented last year. Last year, our discount structure was one-size-fits -all, but now we have moved to a slab- based pricing model, which we implemented in May 2023, and it's now been a year and has been well accepted by our franchisee network. This has led to an increased energy with our franchisee network with motivation to move up volumes and enter higher slabs. It will result in a movement towards larger franchisees and enable much greater reach from our large partners.

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Quality remains a priority for us. It is an ongoing journey, and we are proud to announce that 25 out of our 30 NABL labs are now NABL accredited. Currently, 94% of our total sample load is processed in NABL Labs, a significant achievement considering only 2% of pathology labs nationwide hold this accreditation.

To validate our commitment to quality, we conducted an independent study, published in the International Journal of Advanced Research Ideas and Innovations and Technology. The findings revealed that 9 out of 10 doctors trust Thyrocare reports and confidently recommend our services to their patients. This endorsement underscores the dedication and effort we've invested in maintaining high-quality standards.

On the quality front, this quarter, we have reduced our complaints per million samples by 70% against last year. We hosted an advisory board meeting in June 2024, with the panel of esteemed doctors to gain insights on enhancing our quality milestones. Doctors witnessed our cutting -edge technologies and stringent protocols, reinforcing our commitment to diagnostic excellence.

Beyond the work on quality, we continue to selectively expand our offering. Aarogyam has been our flagship brand in the preventive health care segment. And now we have two more brands, Jaanch and Her Check. Jaanch, as I have said before, is targeted towards lifestyle challenges for you to better understand your health. We have solutions across the spectrum for anything you might be worried about. Is it fever or something more serious? Why is my hair falling? Cancer screening, as well as deep investigations for common chronic diseases like diabetes, heart health, amongst others.

We are doing well on Jaanch, which has grown 25% year -on-year. We are very proud of some of the key milestones that we have achieved in this quarter. We have 8,100 active franchisees. As a result, we processed 6 million samples and served 4 million patients in this quarter, which is 13% and 9% year -on-year growth, respectively, in volume.

Total tests conducted during this quarter was around 41 million, which grew by 13% year -on-year. I'm happy to share that we have entered into a business transfer agreement on 2nd July to acquire Polo Labs Private Limited. Polo Labs is a pathology diagnostic company based out of Punjab with a wide presence in Punjab, Haryana and Himachal Pradesh. This will allow us to expand our footprint in North India, where we have been weak. This transaction is expected to close by the end of this month.

Partnerships business did phenomenally well in the quarter and grew as we onboarded new clients in Health Tech segments and continue to grow our existing accounts. Also, with the acquisition of Think Health last year, it has strengthened our offering for the insurance

segment with the additional capability of ECG at home. This allows us to give our insurance partners a one -stop solution for blood and ECG testing and further deepen our presence in the pre policy medical checkup and annual health checkup market.

And now we are the company which offers health checks with ECG at home. And I'm sure that will enhance our Aarogyam brand. On the B2G side, we continue to execute TB projects in the OThyrocare

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state of Gujarat and Maharashtra. On the international expansion side, our lab is now set up in Tanzania, and we are going out in the market in full swing.

With that, I will hand over to my colleague, Nitin, to cover the highlights for the quarterly performance.

Nitin Chugh: Thank you, Rahul. A warm welcome to each one of you. I will briefly update you about the business performance highlights for the first quarter of FY '25. Overall, at a consolidated level, we did a 16% year -on-year revenue growth this quarter, which was primarily driven by our Pathology business. Our Franchise business showed a revenue growth of 11% year -on-year.

We have started focusing towards opening up smaller labs in partnerships and focusing more on franchise store fronts, which will lead to a higher processing capabilities and ultimately leading to higher franchise business growth in coming quarters.

Our Partnership business, like Rahul said, grew by 29% year -on-year. And if we exclude API and B2G, it showed a phenomenal growth of 41%. Radiology business, including Pulse Hitech did a strong revenue growth of 15% year -on-year balance.

With that, I will hand over to my colleague, Alok to cover the financial results.

Alok Jagnani : Thank you, Nitin. A warm welcome to everyone joining us today. Before diving into the specifics, I would like to touch upon the ESOP program, which we have discussed in previous quarters. This initiative implemented at group level aim to retain the talent within Thyrocare.

These ESOPs issued by the parent company will best accept the policy.

From an accounting perspective, they are reflected as an expense in the profit and loss account and as an equity contribution from the parent in the balance sheet. It's crucial to note that, this represents a cashless charge and does not affect company's cash outflow. Accordingly, we calculate normalized EBITDA, which accounts for these expenses. Further details are available in presentation.

Now moving to the financial performance. Revenue for the quarter stood at INR 144 crores stand -alone and INR 157 crores consolidated, making a year -on-year improvement of 16% at consolidated levels. This growth was supported by strong partnership revenue, grew by 29%, franchisee growth of 12%. Total Pathology revenue increased by 16% year -on-year and Radiology revenue increased by 15%.

Our stand -alone gross margin for the quarter stood at 70%, which reduced by 1 percentage point, primarily driven by increase of material cost and foreign exchange fluctuations.

Employee expenses increased year -on-year due to annual increments, addition to the growth picking and new business acquisitions. Stand-alone nonlife EBITDA margin for the quarter stood at 31%, which is lower by 1 percentage point, primarily due to the decrease in gross margin and increase in marketing and overhead costs.

EBITDA margin in Radiology stood at 8% versus 11% -- 12% year -on-year, mainly due to the increase in operational costs and change in revenue mix between scan and FDG sales. At a consolidated level, gross margin was 71% and normalized EBITDA was 29%. Reported OThyrocare

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EBITDA for the company grew by 21% year -on-year. PAT showed a growth -- strong growth of 35% year -on-year.

Now with this, I will pass the call to Rahul for strategy coverage.

Rahul

Guha: Thank you, Alok. Briefly, I would like to take a few minutes to recap to you our strategic direction, and then we'll open it up for Q&A. First, I will reiterate our value proposition to the customer. We will continue to remain an affordable option to all patients with good quality and on-time reports. All our efforts on our value proposition is towards ensuring low cost to the patient, assurance on quality of testing through our certifications and engagement with doctors. We have made substantial progress on this, which I updated in my initial comments and is reflected in the presentation. This will remain at our core and will continue to guide all that we do. Second, our strategy. We continue to maintain our strategy of being the B2B partner of choice to all front -end diagnostic services companies in India, whether it's a small diagnostic center in a semi -urban area, a pharmacy in a metro, a small nursing home, an individual doctor or a leading online diagnostic platform .

We are happy to work with them to provide low -cost, robust testing solutions so that they can serve their patients in the most effective manner. If they require phlebotomy, we are happy to mobilize our phlebotomy network of almost 1,300 phlebotomists to serve them better. We remain dedicated to expanding our business. And with the acquisition of Polo Labs, we plan to significantly increase our presence in North India.

Additionally, to further boost our Partnerships business, the acquisition of Think Health allows us to offer ECG at home services, further enhancing our value to our insurance partners. This strategy has been working well for us, with both our Franchise and Partnerships business posting strong growth. In order to provide affordable and superior quality services to our customers, we are committed to 3 pillars of growth.

The first is our customer success. The focus is to ensure that our customers receive accurate, timely and affordable diagnostic services. We emphasize on quality control, robust data management and customer support to meet and exceed client expectations. Through

streamlined processes and advanced technology, we aim to enhance the overall customer experience and satisfaction. We invest in technology to help our franchise partners market themselves better and win in the market.

The second is network expansion. We aim to take our Franchise business deeper into India, with a focused test menu and provide our clients with a frictionless experience to transact with us. At the same time, expanding our growing private and public partnerships. In the public space, we focus on TB, an infectious disease, where we are by far one of the strongest players in these segments with large screening programs, partnering with health bodies and funding agency.

Additionally, we will continue to expand our partnerships across health care companies, hospitals and other health services companies. The third area where we are now focusing is to enhance our menu expansion. We're introducing a diverse range of new tests and health OThyrocare Tests you can trust

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packages to our partners, which includes adding more specialized and high demand tests to cater to a broader customer base. We aim to actively promote these high -value offerings to ensure they're well integrated into our partner services.

For example, we have now launched histopathology in-house in our Delhi lab and offer histopathology services now to our customers, provided in- house by Thyrocare. Additionally, by leveraging targeted marketing strategies, we will push for increased adoption of premium health packages, maximizing value for both our partners and end customers. That in a brief is our mandate as management.

Thank you so much for giving us a patient hearing. I'll once again end with the quote from the Mahatma, Find purpose, the means will follow. A

nd our purpose remains to provide

affordable, high- quality testing to the masses. With that, we'll open up for Q&A.

Moderator: Thank you very much. We will now begin the question-and -answer session . We have our first question from the line of Shubh from RatnaTraya Capital . Please go ahead.

Shubh: Sir, I have two questions for now. First is more on the accounting side. What is a quarterly cadence of depreciation, it was around INR 13 crores in the last two quarters, this quarter depreciation has reduced?

Rahul Guha: Okay? And your second question?

Shubh: Yes. And the second question is on the partnership front. Again, what would be like INR 40 crores, INR42 crores the normal cadence of partnership revenues on a quarterly basis?

Rahul Guha: Sorry, I didn't understand the second question. You said INR 42 crores of partnerships? Can you please repeat your question?

Shubh: Yes, sir. So this quarter, we have considerably higher growth in partnerships revenue. What -- how do you see it going forward?

Rahul Guha: Okay. Understood. So maybe I'll answer the first -- second question, and then I'll hand over to

Alok to answer the depreciation question. On the partnerships, and I'll let Nitin add, at the high level, a lot of the partnerships got added in the second half of last year. So if you look at our jump, we actually saw a big jump between Q3 and Q4, and that jump continues to sustain.

The second part is API is now back on the growth path. So that is also giving a boost to our partnership business, and we expect this trajectory to continue. Nitin, do you want to add anything?

Nitin Chugh: No, no. So we have some strong pipeline on the partnership front, and we see that this business will continue to grow strong to a delta to even our Franchise business. So this will grow our upwards of high teen growth in the coming quarters as well on the back of good revenues -- upgrowing revenues from API, from also Corporate and Insurance business also we see a lot of potential.

Shubh: Understood. Just a follow-up on this, if you can share the contribution of API this quarter? 0Thyrocare

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Rahul Guha: API has done roughly 8% growth. I think it's INR13 crores of revenue is what it would have done. On the depreciation piece, you ask questions. So answering to that, if you see compared to previous quarter, we have an increase in -- we have dip in INR 1.6 crores in depreciation cost because in Quarter 4, we have taken onetime depreciation cost of delayed depreciation has happened for Pulse entity. That is normalized. So if we exclude that INR 1 crore, it's going to be normalized depreciation.

Shubh: And it was around 13 quarters in Quarter 3 of FY '24 as well. So it's the same reason?

Rahul Guha: No. So in Quarter 3, we have done lots of capitalizations, if you remember, there are lots of infrastructure developments and all has happened and all the capitalization happened in September, October. And because of that, the 3 months capitalization, it has come in for

September July -August in the month of Quarter 3 for Thyrocare. INR 46 crores of capitalization we have done during that period of time.

Shubh: So going forward, INR11 crores to INR 12 crores per quarter is...

Rahul Guha: It's going to be between INR 11 crores to INR 12 crores, subject to further business expansion, what's going to come in pipeline.

Moderator: We have a next question from the line of Prakash Kapadia from Spark PMS. Please go ahead.

Prakash Kapadia: If I look at Pathology revenues, they are up around 16%. So last year, we had a low base. So -- has that helped? And if I were to look at Pathology revenues, they're flat sequentially. So how do we read this? Is there a seasonality in Pathology revenues?

Rahul Guha: Yes. Maybe I'll take both questions. I'll take the second one first and then the -- see, the business

is seasonal. Actually, if you go in the past, sequential between Q4 tends to be the best quarter in our business. And actually, normally, we see a mild dip in Q1. So a sequential growth is actually quite welcomed from that point of view because as you might be knowing, in Feb and March, a lot of people do the annual health checkup because of the tax benefit that you get against that.

So we do see Q4 tends to be quite strong and Q2 also tends to be very strong because we get into the fever season and the monsoon and all of that. Q1 is normally quite a dull quarter.

Actually, we normally see a mild dip in Q1, which we haven't seen.

Now coming to the second question, year-on-year, actually, if you look at our non-COVID business, right, that has been growing at a CAGR of, I think, roughly 18%, 19%. So I wouldn't say last year was a low base from a non-COVID side. Every year, our non-COVID business has been growing between the high teens and in the 20% range. So I would not contend that it's a low base. So growing on top of that at 16% is actually quite good in my opinion.

Prakash Kapadia: Okay. Understood. And if I look at the Franchisee revenues, I think they are up around 11%.

Last quarter, you did mention the contribution of larger franchisees has been increasing. So could you give us some sense on the growth and contribution from larger franchisees, what are the factors which could drive Pathology revenues from here on? Obviously, you mentioned 0Thyrocare

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about the Partnership revenues, which have grown quite healthy. So would Franchisee and Partnership be in this range? What are we looking at for further growth for Pathology?

Rahul Guha: Yes. So, let me answer the first question, and then I'll come to the second question. So if you look, we bifurcate our 8,100 roughly franchisees into large and small; roughly about 3,500 are large today and about 4,500 are small. If I go back last year, the equivalent would be roughly

2,900 and 4,600. So we've added actually roughly about 600 large franchisees year -on-year, right? Whereas our small franchisees have stayed more or less stable. So the mix of large to small has moved quite substantially over the year, and that is what is actually driving our growth.

And on the second question in terms of -- we don't -- we try to avoid giving guidance. I will say, though, that our Franchisee business will be range bound in the low double digits to mid-teens. And I expect our Partnership business to grow faster than that.

We may not be able to maintain the 29% that you are seeing. But I definitely feel our Partnership business will grow faster than our Franchisee business. And our Franchisee business is picking up. So I expect it will remain in that, I would say, low double digit to mid-teens with Partnerships growing faster.

Prakash Kapadia : And this Partnership -- Sorry, you were saying something.

Rahul Guha: No, no, I was asking Nitin, if you would like to add but you can go to your next question.

Prakash Kapadia : Yes, the Partnership business, you mentioned will grow slightly faster. So is it some of the new products or customized products, which certain partners are looking at or certain geographies where there is some new launches. So what is going to drive that?

Rahul Guha: Understood. There, I'll have to take you a little bit into the history, okay? If you go back in the past, Partnerships was never a large business for us as Thyrocare. And over the last couple of years, a lot of health tech players have come in, where we had not tapped into that potential.

The other is actually multiple insurance players control a large amount of volume because they all do pre policy medical checkup and they also give an annual health checkup to the policyholders. So there's significant volume that sits with the insurance companies where we have not been pr

esent, right?

And so therefore, just focus on these 2 segments, Health Tech players or health player -- players who are in the health services space, which includes hospitals, clinics, many of these larger chains, plus focus on the Insurance segment is what has been yielding us dividends. We have been actually -- we were almost practically hardly anything in these segments. And with putting in place the Partnership team and dedicated key account managers and all of that is actually what you're seeing yielding the dividends now.

Moderator: We have our next question from the line of Bino Pathiparampil from Elara Capital . 0Thyrocare Tests you can trust

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Bino Pathiparampil: Just a further clarification on this EBITDA margin that has come off by 100 basis points Y -o- Y. I believe you attributed a bit to the increase in material costs. Is that the only reason? And what is the outlook going forward? Is it going to be a bit less than what you earlier expected?

Rahul Guha: Alok, I'll let you take that.

Alok Jagnani : So this is -- for EBITDA margin, there is a dip of 1% percent but. ..

Management : Gross margin.

Alok Jagnani : So I've given EBITDA margin, 1% dip has come. If you compare year -on-year, mainly come from material cost that is one of the reasons. And some cost has increased on account of other expenses also like volume -- because of the volume increase, other costs has gone up by 13%, in terms of phlebo and LME cost has gone up.

Some marketing spends, what we have to start spending from last Q2 onwards, has in the same line or some increase has seen there? And employee cost has also gone up on account of increments and business expansion. So these are the major 3 reasons; if we consolidate all then

1% margin dip has come in EBITDA. And for the subsequent quarters, we are expecting it's going to be between 25% to 30% that is the target which we are looking for.

Bino Pathiparampil: Sorry, did I hear between 29% and 30%?

Alok Jagnani : Between 25% to 30%, it's going to be between 27% to 30%.

Moderator: We have our next question from the line of Laxmi Narayanan from Tunga Investments .

Laxmi Narayanan : I have three questions. First is the stock options, which are being allocated. This is all Thyrocare's share, right? That's my first question. Second question is that if you look at the employee count, if you look at it on a long-term basis, let's say 2019, we had around close to

1,200. I think now the employee base is around 1,700 or so or even more. Just want to understand how do you look at this equation between employee cost to the overall revenues?

Is it that the employee cost as a percentage will change over a period of time? That's the second question. And the third question is related to the total capital outlay for non-Indian operation. How much is the total plan? And how much we have done so far? Three questions.

Rahul Guha: Okay. On the first one, these are ESOPs granted from the parent. So these are actually API ESOPs that are granted to Thyrocare employees and recorded in the P&L. So there is actually no cash impact and no share dilution at a Thyrocare level. It's mostly at the parent level. Does

that explain that?

Laxmi Narayanan: So how it looks at the parent's balance sheet? I'm just trying -- curious to understand how...

Rahul Guha: The same is reflected in our P&L as a charge and whereas in the balance sheet, it's contribution from parents as soon as in our balance sheet.

Management: And in the parent balance sheet, it is basically share-based payment to the subsidiary. OThyrocare

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Laxmi Narayanan: Got it. And why not Thyrocare shares itself?

Rahul Guha: The pool that, when we wanted to create a sufficiently large pool, the pool available at Thyrocare was not large enough to app

ropriately incentivize the leadership team here. And if

you go for additional pool, then it is go for dilution of sales, which you don't want at this point of time as an impact.

So that was your first question. Second is employee count. See, the employee count largely is because if you go back to 2019, I think we had 10 labs. Today, we have 30 labs and that is

largely the increase in employee cost, running 10 labs versus 30 labs. And the second is we

have put a lot of investment in feet on street and outbound calling. So that is the other.

But if I break up that journey from 1,200 to 1,900, I would say almost 80% of that will be

largely because of the number of labs that we have opened and the remaining will be in

business development and all of that.

Laxmi Narayanan: Got it. And when do you think the utilization of those will go up so that we have an operating

leverage? Is it like are we 10%, 20% utilized of the incremental head count? What is the

normalized employee cost or something which we can actually expect?

Rahul Guha: Look, our labs -- actually, we are now reasonably efficient from a lab point of view. I think our

overall utilization will be in the 60s, which is a good comfortable position to be because we have -- this business is seasonal

plus seasonal in the week as well. So the peak versus trough is

quite substantial, both at weekly level as well as monthly and quarterly levels.

So I think 60% to 70% utilization is a good place to be. If you see over the last couple of years, last year to this year, we

haven't added much headcount, at least in lab operations. So you

should start to see the operating leverage come in, especially now that we are growing at 16%

plus, then you get -- if you go back to previous years, at an overall revenue level, it was --

while we were growing on non-COVID basis, at the overall revenue level, we were not

growing.

Laxmi Narayanan: Got it. So we can take around 200 employees per year, it would be an increase to support a 15% growth in the top line?

Rahul Guha: I think as a percentage, you should roughly expect the employee cost to track 0.75 of the revenue. If your revenue is growing 16%, you can anticipate employee cost to grow 12%.

Laxmi Narayanan: Got it. Okay. Fair enough. Yes. And the third is regarding African.

Rahul Guha: So Africa, I think we have spent INR8 crores. We spent INR 8 crores. And as of now, we have

no plan. Further plan is going to come depending upon how the outcomes are going to come.

Alok Jagnani : Yes. So , at this point, our capital outlay in Tanzania is actually not even INR 8 crores because

half is INR 4 crores from us and INR 4 crores from the partner.

Rahul Guha: So yes, it's actually a total of INR 4 crores. OThyrocare

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Management: Provision of INR 4 crores upon further expansion.

Laxmi Narayanan: Just one last question from a rent point of view, what is the annual rent expenses you have for all the 30 labs approximately?

Rahul Guha: I don't think I have that number on hand. Maybe Alok will look at it and come back. Maybe we

go to the next question, I'll interrupt the call at some point and give you that number. Rent is

around INR60 lakh to INR70 lakh, we are spending monthly. This is for all 30 labs.

Moderator: Should we take the next question, sir?

Rahul Guha: Yes, please.

Moderator: We have a question from Niraj Vijay Kamtekar from ProsperoTree.

Niraj Kamtekar : Sir, my question is regarding the subsidiary company. That is the Radiology business. Sir, we

have a meaningful investment in the Radiology business but the contribution is very less or

hardly there is any contribution at the bottom line level. When this business will really contribute to the balancing up to the bottom line?

Because only the Pathology business is performing, Radiology business is not performing. We have increas

ed the number of centers from 9 to 10, but there is a very hardly there is a revenue improvement and the profit is very negligible. So my question is -- first question is regarding the Radiology business improvement.

Rahul Guha: So again, I'll take you to the history before I answer that question. NHL used to be a loss - making entity. At least now it is no longer loss -making and not negative in the P&L. That has taken us about a journey of almost 2 years to turn around this business. Now that it is profitable, yes, I agree with you. It is not generating a lot of profit.

Primary reason of that is because the maintenance cost of the equipment has gone up substantially because the equipment is very, very old. So we would have to invest substantially in this business, if we are to expect this to come into a meaningful contribution overall. As it is

currently, I don't expect the business to do -- I mean we can take the business up a little bit, INR1 crore EBITDA may become INR 2 crores. But in the INR 45 crores, INR50 crores that we are targeting, it will not be meaningful.

But if we are going to make this a meaningful business, we are going to have to invest substantially. And for that, we are making the plan now that at least the business is not loss - making, we are now more confident to evaluate investments there, but the investments will be substantial. So therefore, we have not taken that decision at this point.

I would not bank very much on the Radiology business becoming a substantial contributor of the profits.

Niraj Kamtekar : Because my question is the when the -- originally, the PharmEasy has taken over the business of the Thyrocare, there was a substantial investment earlier also. Now we are putting more money into the Radiology business, which is not generating the desirable IRR from that 0Thyrocare

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business. why we are interesting on the Radiology business and why we are not concentrating only on the Pathology business?

Rahul Guha: No, no. So let me clarify. We have not taken any decision to invest in the Radiology business. We are currently happy with where it is just now. It is not loss -making. It adds and it is not cash flow negative. So with that, we are happy with where the business is now. All our focus is on the Pathology business, which hopefully you are also seeing in the figures. We will continue to maintain the Radiology business. But rest assured, not a lot of management bandwidth is going behind the Radiology business.

Niraj Kamtekar : So now onwards, there will be no major capex for the Radiology business. Is it correct?

Rahul Guha: It is not anticipated. If we have it, we will, of course, first consult with the Board and make a thorough plan. But as of now, there are no plans to substantially invest capex in the Radiology business.

Niraj Kamtekar : Okay. And sir, my second question is regarding the cash flow. Though the company is distributing the very good amount of the dividend. But when we declared and we paid the dividend, we have to encash some investment which we made in the mutual fund or somewhere else. So is there a -- can we curtail the dividend and maintain the investment for the future possibility of investment, other investment in the business investment? Or will we maintain such high dividend every year?

So because in the first half, mostly we divested some investment and paid the dividend out of that money. So can we have a minimum amount of dividend and keep the investment for the business purpose, is it possible?

Rahul Guha: That is always possible. But right now, we are generating a healthy amount of cash flow. Even in the first quarter, our cash generation will be, I think, close to INR 40 crores. So we generate INR150 crores, INR175 crores in that range of cash every year. We don't require that much cash. We are almost negative working

capital business to some extent. So our cash requirements even to grow the business are not substantial. Then we feel it's better to reward our shareholders with a good dividend rather than keeping the cash in our treasury.

Niraj Kamtekar : But, then can we...

Moderator: Mr. Kamtekar, I request you to join back the queue, please.

Niraj Kamtekar : But my question is not over, madam. I will join in the queue, but my question is not over.

Rahul Guha: Please take this question, no problem. Please continue, please ask question, sir.

Moderator: Sir, I believe his line got disconnected. I'll take the next question. This is from the line of Aditya from Securities Investment Management .

Aditya: Sir, firstly, how do you define a large and a small franchisee. So is there any regular target set for a franchisee to become large or small? 0Thyrocare

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Rahul Guha: Yes. So at Thyrocare billing levels, a large franchisee is, I think, INR 50,000 plus and small franchisees is someone who does less than INR 50,000 at Thyrocare billing, which just to give you context, while the thyroid MRP is in the range of INR 300 to INR 350, our billing to the franchisee is roughly about INR 60, INR 60 to INR 70 you can take it. So when I say it is at Thyrocare billing, it is a multiple at the franchisee level, but that's the definition. More than INR 50,000 less than INR 50,000.

Aditya: Understood, sir. This Franchisee business, who would be our major competitors? Would it be the larger listed players only or there are smaller players as well?

Rahul Guha: It's -- there are competition. I'll let Nitin take this question.

Nitin Chugh: So see, the competition is also very, very area specific. The competition comes from both sides. Large established players like Metropolis and Lal and in certain areas, there are a lot of local players as well, which are very, very region -specific.

Aditya: But if I have to understand the kind of volumes which the larger players would be doing, the cost -- the cost advantage, which we would have, those cost advantage the smaller player shouldn't have. So ideally the larger players should keep gaining share in this market, right?

Rahul Guha: Yes. I think both in terms of share and profitability. If you study the larger players versus the smaller players, you will see both from a growth rate point of view as well as from a profitability point of view, the larger players, which are listed stand out. And if you study any of the unlisted, you will not see similar growth or profitability in many of those businesses.

Aditya: Understood, sir. And sir, now this Partnership business, excluding API and government, we have been seeing very good growth. So is this growth coming from new partners or there has been increasing contribution from existing partnerships?

Rahul Guha: I'll let Nitin answer.

Nitin Chugh: So it's a mixture of both. Like Rahul said, see a couple of years back, the partnership business was negligible. And like if you see last year also, we added major partnerships during the quarter 1 and quarter 2 of last year, which are giving us the desired results this year as well. So it's on both sides, we are -- while we continue our efforts on adding new partners, but on the other side, for players like corporates and other insurance partners, we try to get in the more wallet share from there, either the PPMC business, a Prepolicy Medical health Checkup or annual health checkup business as well.

Rahul Guha: And just to underscore one point, with Thyrocare, actually, we are the best person for partnerships, whether you're an insurance player, a health tech player, or, let's say, a health clinic or a hospital chain because we are -- actually we have a complete company -owned phlebotomy fleet, and we have a full tech backend that has been built.

So today, with one single API busin

ess -- API integration, Thyrocare can enable you in 300 cities in the country. And that actually, as a proposition is leading us to win in the franchisee network -- sorry, in the Partnerships business because if you are a very large insurer, actually, Thyrocare Tests you can trust

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with one single API integration, you can start your entire prepolicy medical checkup and annual health checkup, across 300 cities with just one partner Thyrocare.

Aditya: So sir, is this business granular or the share of top 5, 10 partnerships would be contributing 70% to 80% of revenue?

Rahul Guha: Actually, there is a long tail in this business. But you're right, the top 5 will account for 50%, 60% of the revenue.

Aditya: Understood. And sir, if I look at the realization per test in Partnership business. So it has been constantly declining. So it was around INR 345 in December '22 quarters and now it is around INR 303. So what is the reason for the same?

Rahul Guha: Actually, that is realization per sample across the Partnerships business. What happens is we have been focusing a lot on upsell or, let's say, add-on tests at the patient's residence. So what happens is as tests get add on at the time of collection, an additional sample is used, but the realization of that sample is lower.

So that mix is what you're seeing playing out in the revenue per sample. So I mean, it's just adding on -- customers adding on tests that they want to add on during the time of collection, which typically, if our average collection will be about INR 1,200, they're adding another INR 200 to INR 400 of test. So that is what is -- but that is an incremental revenue that comes at no additional cost. So that flows straight to the bottom line. But that is the effect you are seeing in the revenue per sample.

Aditya: Understood, sir. And sir, is there a difference in margins between our Franchisee business and our Partnership business at a gross or EBITDA level?

Rahul Guha: I'll let Nitin take it.

Nitin Chugh: No, so it's not like that. So even if our partnership business is increasing at, say, range of 29%, the EBITDA and the gross margin levels, they are fairly similar. So it will not dent our EBITDA margin. I think that's the question that if the Partnership business is growing at a faster rate than partnership, will there be an impact on EBITDA, then the answer is no.

Aditya: Understood. And sir, you mentioned that we have invested around INR 8 crores in Tanzania.

So if you could just provide a brief outlook. How do you see this business shaping up in the next 3 to 4 years? So can this business become 7% to 8% of revenue? If you can provide an outlook on Africa business, that would be helpful.

Rahul Guha: Just to clarify, it is INR 4 crores from our side and INR 4 crores from our partner side. The total investment is INR 8 crores, but it's a 50-50 JV. So we've only put INR 4 crores. It's too early to say. It's our first foray into the market. We are understanding a new geography, it's the first time in a long time that we have done an international foray. 0Thyrocare

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The last was almost a decade ago in the Gulf, but now in Africa. So I think it's too early for me to give you a forecast of where I think Tanzania will go. Maybe by the end of this year is when I think I will have full clarity on how much we can scale that business.

Moderator: We have a next question from the line of Aniket Kulkarni from BMSPL Capital . Please go ahead.

Aniket Kulkarni : Sir, can you give some brief commentary on how competitive intensity has been in the market? And what is the pricing situation which we are witnessing right now? Just the brief commentary will be fine.

Rahul Guha: This is Nitin's favorite subject. So I will let him take it.

Nitin Chugh: I didn't

get the full question. One is...

Rahul Guha: Competition and pricing...

Aniket Kulkarni : Competition and pricing, yes.

Rahul Guha: If competition increases...

Nitin Chugh: The competition, I feel it's slowly, I think cooling down. I think initially, the impact post-COVID was definitely a lot of competition coming in and decreasing prices and trying to win the market. But as the non -COVID business has stabilized over the last couple of years, we are

now seeing some cling -on effects of the pricing decrease that the competition was doing. This has also led to -- we're also stabilizing our prices. Like we said, we last year went live with us lab based pricing, which led to some goodness on our side, and we now don't see that much intensity and price pressure from competition now.

Moderator: Thank you. Ladies and gentlemen, we'll take that as the last question for today. I now hand over the call to Mr. Rahul Guha for closing comments. Over to you, sir.

Rahul Guha: All right. Thank you for all the questions and the engaging discussion. I look forward to seeing you in the next quarterly presentation. And have a fantastic evening, and I wish you a good day.

Moderator: Thank you, members of the management team. On behalf of Thyrocare Technologies Limited, we conclude this conference. Thank you for joining us, and you may now disconnect your

lines. 0Thyrocare

Tests you can trust

Call: Oct 2024

October 28, 2024 To,
The Secretary, The Secretary,
Listing Department, Listing Department,
National Stock Exchange of India Limited BSE Limited
Exchange Plaza, 5
th Floor, Phiroze Jeejeeboy Towers
C/1 G Block, Bandra Kurla Complex, Dalal Street,
Bandra (E), Mumbai - 400 051 Mumbai- 400 001 (Scrip Code: THYROCARE) (Scrip Code: 539871)

Dear Sir/Madam,

Sub: Transcript of post results earnings conference call held on October 23, 2024 with Analysts / Investors. Ref: Disclosure under Regulation 30 of the Securities and Exchange Board of India (Listing Obligations and Disclosure Requirements) Regulations, 2015 ("Listing Regulations") Pursuant to Regulation 30(6) and 46(2) read with Part A of Schedule III of the Listing Regulations, please find enclosed herewith the transcript of the earnings conference call with Analysts and Investors held on October 23, 2024. The same has also been made available on the website of the Company (www.thyrocare.com). This is for your information and records. Yours Faithfully,
For Thyrocare Technologies Limited,

Ramjee Dorai
Company Secretary and Compliance Officer

Encl: A/a

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"Thyrocare Technologies Limited
Q2 FY '25 Earnings Conference Call "
October 23, 2024

MANAGEMENT : MR. RAHUL GUHA – MANAGING DIRECTOR AND
CHIEF EXECUTIVE OFFICER – THYROCARE
TECHNOLOGIES LIMITED
MR. ALOK KUMAR JAGNANI – CHIEF FINANCIAL
OFFICER – THYROCARE TECHNOLOGIES LIMITED
MR. NITIN CHUGH – CHIEF COMMERCIAL OFFICER –
THYROCARE TECHNOLOGIES LIMITED
MR. KAPIL GUPTA – STRATEGY AND INVESTOR
RELATIONS – THYROCARE TECHNOLOGIES LIMITED

Thyrocare Technologies Limited
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Moderator: Ladies and gentlemen, good day, and welcome to Thyrocare Technologies Limited Earnings Conference Call. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Kapil Gupta from Thyrocare Technologies Limited. Thank you, and over to you, Mr. Kapil.
Kapil Gupta: Thank you, Rutuja. A very good evening to all and thank you for joining us today for the Thyrocare Earnings Conference Call for the second quarter of FY '25. Today, we have with us Mr. Rahul Guha, MD and CEO of Thyrocare; Mr. Alok Jagnani, CFO of Thyrocare; and Mr. Nitin Chugh, Chief Commercial Officer of Thyrocare, along with other key members of the senior management on this call to share highlights of the business and financials for the quarter. I hope you have gone through our quarterly results press release and earnings presentation which has now been uploaded on the Stock Exchange website. The transcript of this call will be available in a week's time on the company's website. Please note that today's discussion may be forward-looking in nature and must be viewed in relation to the risks pertaining to

our business. After the end of this call, in case you have any further questions, please feel free to reach out to the Investor Relations team. I now hand over the call to Mr. Rahul Guha to make the opening remarks.

Rahul Guha: Thank you, Kapil. Good evening, and welcome to all on the call. Thank you for taking out time from your busy schedules to join us this evening. Just a quick introduction to us on the call. My name is Rahul Guha, and I'm the MD and CEO of Thyrocare, and thank you for the opportunity to present the Q2 results of FY '25. I'm joined with my colleague Alok Kumar Jagnani, who is our CFO; and Nitin Chugh, who is our Chief Commercial Officer; along with Kapil Gupta, who is part of our Strategy and Investor Relations team.

As in all my calls, I will start with the quote from Nelson Mandela in recognition of our foray into Africa. It is in your hands to make a better world for all who live in it. And we believe Thyrocare can bring our business model to Africa to make affordable and good quality diagnostics available to all.

I'll give you some of the key highlights of what we've been up to in the last quarter. Before we get into the details of the quarter, I'll reiterate the pay-for-performance pricing structure that we implemented last year. Last year, our discount structure was one size fits all, but now we have moved to a slab-based pricing model, which we implemented in May 2023. It's been 1 year plus since that implementation, and this has led to an increased energy with our franchisee network with motivation to move up volumes and enter higher slabs. It will also result in a movement towards larger franchisees and enable much greater reach from our large partners. Quality continues to be our top priority and we view it as an ongoing journey. We are proud to say that currently, 28 out of our 32 labs are now NABL accredited. Currently, 96% of our total sample load is processed in NABL Labs, a significant achievement considering only 2% of Thyrocare Technologies Limited
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pathology labs nationwide hold this accreditation. To validate our commitment to quality, we conducted an independent study published in the International Journal of Advanced Research, Ideas and Innovations in Technology. The findings revealed that 9 out of 10 doctors trust Thyrocare reports and confidently recommend our services to their patients. This endorsement underscores the

dedication and effort we've invested in maintaining high-quality standards.

Further, in September 2024, we were recognized and rewarded by the College of American Pathologists, CAP for short, for maintaining excellence in high-quality laboratory care for 15-plus years. CAP is the international gold standard accreditation that represents the top-tier quality in pathology and laboratory medicine. We hosted our third Advisory Board Meeting in August 2024 with the panel of esteemed doctors to gain insights on enhancing our quality milestones. Doctors witnessed our cutting-edge technologies and stringent protocols, reinforcing our commitment to diagnostic excellence.

I'm also very proud to say just this week, we launched the largest HbA1c study across the country where we have studied 20 lakh HbA1c results to publish the largest HbA1c study in the country. This quarter, we have also reduced our complaints per million samples by 58% as compared to last year.

Beyond the work on quality, we continue to selectively expand our offerings. Aarogyam remains our flagship brand in the preventive healthcare segment, but now we have two more brands,

Jaanch and Her Check. Jaanch, as I've said before, is targeted towards lifestyle challenges or for you to better understand your health. We have solutions across the spectrum for anything you might be worried about. Is it fever or something more serious? Why is my hair falling? Cancer screening as well as deep investigations, common chronic diseases like diabetes, heart health amongst others.

We are very proud of some of the key milestones that we achieved in this quarter. On the revenue front, we have achieved a record high of INR 177.4 crores this quarter, surpassing even our peak during the COVID period. This remarkable achievement is a testament to our strategic business expansions, investments in quality improvement and our unwavering customer-centric approach. We have 8,400 active franchisees. As a result, we processed 6.7 million samples and served 4.4 million patients in this quarter, which is 15% and 9% year-on-year growth, respectively, in volume. Total tests conducted during this quarter was around 44 million, which grew by 15% year-on-year. I'm happy to share that we have completed the acquisition of Polo Labs on 29 July. Polo Labs is a pathology diagnostic company based out of Punjab with a wide presence in Punjab, Haryana and Himachal. This allows us to expand our footprint in North India. Additionally, I'm delighted to share that we signed a business transfer agreement to acquire the clinical diagnostics business of Vimta Labs on 30th August. Vimta's Clinical Diagnostics division with its established presence in Telangana and Andhra Pradesh offers us an excellent opportunity to further strengthen our presence in South India. This transaction was successfully closed on 11th

October.

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Partnership's business did phenomenally well in the quarter and grew as we onboarded new clients in Health tech segments and continue to grow our existing accounts. Also, with the acquisition of Think Health, it has strengthened our offering for the insurance segment with the additional capability of ECG at home. This allows us to give our insurance partners a one-stop solution for blood and ECG testing and further deepen our presence in the pre-policy medical checkup and annual health checkup business. And we are now the company who offers health checks with ECG at home, and I'm sure that will enhance our Aarogyam brand.

On the B2G side, we continue to execute TB projects in the state of Gujarat and Maharashtra. In Tanzania, since going live in March 2024 and processing our first sample in April, we have successfully partnered with over 50 health care facilities in Dar es Salaam. Our mission is to continuously collaborate with major hospitals ensuring they have a

ccess to comprehensive diagnostic services. With our state-of-the-art laboratory equipped with world-class machines and infrastructure, we're poised to make a significant impact on health care in the region.

With that, I will now hand over to Nitin to cover the highlights for the quarterly business performance.

Nitin Chugh: Thank you, Rahul. A warm welcome to each of you. First, I would like to start with our pillars of growth which have been contributing strongly to our consistent performance. The first one is customer success. The focus is to ensure accurate, timely and affordable diagnostic services through quality control, robust data management and customer support. With advanced technology and streamline processes, we aim to enhance customer satisfaction. Launching of live reports for our franchise base and our D2C customers is a testament of our continuous work towards customer success.

The second pillar is network expansion. We are deepening our presence across India by going deep into the country with our franchise network and through our acquisitions as well. Also on the partnership side, we are expanding our footprint going deep into the PPMC in insurance business. The third pillar is our test menu expansion. We are introducing a wider range of specialized tests and health packages for our partners using targeted marketing to drive adoption and increase value for both partners and our customers.

Now I will briefly update you about the business performance highlights for the second quarter of FY '25. Overall, at a consolidated level, we did 20% year-on-year revenue growth this quarter, primarily driven by our pathology business. Our franchise business showed a revenue growth of 14% year-on-year. We have started focusing towards opening up smaller labs in partnership along with franchise and storefronts, which shall lead to higher processing capabilities and ultimately leading to a higher franchise business growth in coming quarters.

Our partnership business grew by 33% year-on-year, and if we exclude API, it showed a tremendous growth of 40% year-on-year. Radiology business, including Pulse Hitech, did a strong revenue growth of 21% year-on-year. With that, I will hand over to my colleague, Alok, to cover the results.

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Alok Jagnani: Thank you, Nitin, and a warm welcome to everyone joining us today. I'm pleased to announce that we have achieved the highest ever consolidated revenue in a quarter with robust growth across all key business parameters. I would like to highlight that the pathology diagnostic industry is growing in the early mid-teens, whereas we had consistently delivered mid-teen to high-teen growth over the past few quarters. This highlights our sustained strong performance and reflects the strength of our leadership and strategy.

For the quarter, revenue stood at INR163 crores stand-alone and INR 177 crores consolidated, reflecting a 20% year-on-year growth. Of this, 19% was coming from organic and the remaining 1% attributed to inorganic expansions, what we have done recently. This growth was driven by 33% rise in the partnership revenue, whereas 14% revenue is coming from franchisee business. Total pathology revenue grew by 20% year-on-year, while radiology business saw a 21% year-on-year increase. Our stand-alone gross margin for the quarter was 71% up by 95 basis points, primarily due to the improved negotiation, favourable revenue mix and better infrastructure utilization.

Employee expenses increased year-on-year, driven mainly because of annual increments and head count increase from the new business acquisitions. The stand-alone normalized EBITDA margin for the quarter stood at 31%, an increase of 186 basis points, primarily due to the margin improvement and operating leverage. EBITDA margin in radiology NHL declined, largely due to the onetime profit

essional charges and other costs.

At a consolidated level, gross margin stood at 71%, while the normalized EBITDA margin was 29%. Reported EBITDA grew by 28% year -on-year, PAT grew by 29% year -on-year despite some margin pressures from the recent acquisition and geographical expansions.

In terms of cash flow, we have done a great job, and we generated around INR 89 crores from the operations during H1 FY '25, which shows around 28% increase over H1 FY '24. This demonstrates improved operational efficiency and stronger cash flow generation, reflecting our strategic initiatives to drive growth and enhance profitability.

Now I will hand over the call to Rahul for a strategic update.

Rahul Guha: Thank you, all. Briefly, I would like to take a few minutes to recap to you a strategic direction, and then I will open it up for Q&A. First, I will reiterate our value proposition to the customer.

We will continue to remain an affordable option to all patients with good quality and on-time reports. All our efforts on our value proposition is towards ensuring low cost to the patient, assurance on quality of testing through our certifications and engagement with doctors. We have made substantial progress on this, which I updated in my initial comments, and is reflected in the presentation. This will remain at our core and will continue to guide all that we will do.

Second, our strategy. We continue to maintain our strategy of being the B2B partner of choice to all front-end diagnostic services companies in India, whether it is a small diagnostic center in a semi -urban area, a pharmacy in a metro, a small nursing home, an individual doctor or a leading online diagnostics platform or Healthtech Marketplace. We are happy to work with them to provide low -cost, robust testing solutions so that they can serve their patients in the most

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effective manner. If they require phlebotomy, we are happy to mobilize our phlebotomy network of almost 1,500 phlebotomists, including our network partners, to serve them better. This strategy has been working well for us as is reflected in the really strong growth in our partnerships business.

We remain dedicated to expanding our business, and with the acquisition of Polo Labs and Vimta Clinical Diagnostics, we plan to significantly increase our presence where we had gaps, particularly in North India and in particular, Punjab, and in South India across AP and Telangana. Additionally, to further boost our partnerships business, the acquisition of Think Health allows us to offer ECG at home services, further enhancing our value to our insurance partners.

That, in a brief, is our mandate as management. Thank you so much for giving a patient hearing.

I will once again end with the quote from the Mahatma, Find purpose, the means will follow, and our purpose remains to provide affordable, high-quality testing to the masses. With that, we will open up for Q&A.

Moderator: Thank you very much. We will now begin the question-and -answer session. The first question is from the line of Prakash Kapadia from Spark PMS. Please go ahead.

Prakash Kapadia: A couple of questions from my end. Currently, what would be the revenue contribution from packages to the total pathology revenues? And if I look at the presentation, the active franchises further increased. So how much can it grow from here on, and partnership revenues has seen a 34% growth. So is it low base or what is happening on the partnership side of pathology? These were my questions.

Rahul Guha: Got it. So, I'll let Nitin take the packages question. I'll address your other question. See, we are -- the second question, which is how much -- with 8,500 franchisees? How much more can you grow? You must recall, we are a B2B partner of choice. India, depending on who you ask, but the rough estimate is there are about 1.5 la

kh pathology labs. We work with 8,500. So, we are

only scratching the surface when it comes to our penetration into that base. So, I think there, there is ample opportunity to continue to grow.

On your other question on partnerships, is it a low base? Actually, partnerships now is almost 33% of our business. So it's not a low base anymore. But as I said in the beginning, being a B2B partner, it allows us to service a lot of health care companies and enable them to offer diagnostics. Just to give you an example, if you are a hospital and you do diagnostics in the hospital. When it comes to home collection, it becomes very difficult for a small hospital to offer home collection to their patients, right?

Because once they walk out of the hospital, it becomes difficult. And for those situations, they turn to Thyrocare. Similarly, if you are a Health tech platform, largest being PharmEasy, but we work with every single major Health tech platform. It becomes difficult for them to maintain a lab network, phlebotomy network for what is not exactly their core business, right? And so therefore, they offload or outsource that part of their business to us.

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And we're seeing a lot of traction on that front, Prakash. I'll let Nitin answer your question on the packages.

Nitin Chugh: And just to add on to this, even at a franchise level, if I compare Q2 of last year versus this year, we have added almost 2,500 franchises, right, to the transacting base. And we feel that, like Rahul said, this is just scratching the surface, and we feel that over 1,500 franchisees can be added the next year as well, 1,500, 2,000 franchisees.

Now talking about packages, more than 30% of our revenue -- a little more than 30% of our revenue comes from packages and rest is all stand-alone tests. And when I say packages, I include all our brands, Aarogyam, Her Check, etcetera, and Jaanch.

Prakash Kapadia: Okay. And just one last thing, if I may. In terms of the pathology revenues over the long run, in case there is a mix change between franchisee and partnership, does that change the overall margin profile of the company or no? And is there any broad direction which we are working to have a mix or grow franchisee at a certain level or partnership at a certain level?

Rahul Guha: Maybe I'll take that. The Partnerships business does come at a lower margin than the Franchise business, marginally, not a big difference, but yes, it comes at a lower margin. But our aspiration, and I have always been saying this, I expect Franchise business to grow in the early double digits to mid-teens, and we have been consistently performing on that front. And I expect our Partnerships business to outperform our Franchise business, and that is what is panning out.

Moderator: The next question is from the line of Nilesh Sharma from Anantnath Skycon Private Limited . Please go ahead.

Nilesh Sharma: My question is regarding the shares pledged by parent company. So is there any clarity how we can see in future the pledge will be removed?

Rahul Guha: See, we, as management, are operating Thyrocare Technologies Limited. We are not privy to all the discussions on the pledge front. So, it's difficult to comment or give any guidance on this.

Moderator: The next question is from the line of Amey Chalke from Jio Financial .

Amey Chalke: So, first question I have is on the franchise sales, which is around 68%, shown in the PPT. Is it possible to give a breakup in terms of how much would be roughly coming from mom-and-pop local labs and nursing home and hospital?

Rahul Guha: For the Franchise business?

Amey Chalke: Yes.

Rahul Guha: So I would say -- I'll let Nitin take that.

Nitin Chugh: Yes. So see, generally, because our end customer is the franchisee, right? So, we only categorize our franchisees into, say, large and small franchisee so the e

nd customer for a franchisee, like you rightly said, can be a mom-and-pop laboratory or a direct consumer or a hospital wherever

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they are picking a sample from. We generally, at our end, only categorize those as large franchisees or small franchisees. But we have a rough estimate in mind, but...

Rahul Guha: But just to give you a broad -- I mean, broadly, I would say 90% of our franchisees are pathology labs. They in turn may be servicing hospitals, but direct hospital as franchisees, actually, that's not a very large number. They get served -- because there's a fair amount of credit and all of that, and we work in our franchisee model as a prepaid company. So therefore, our franchisees service those hospitals. Direct hospitals would be a very small proportion of our revenue.

Amey Chalke: Got it. And one more question. On the volume growth side, is it possible to give a breakup like the franchise growth is around 14%. So, is it possible to give the price and volume breakup here?

Rahul Guha: Yes, yes, it's there in the presentation, but Nitin can clarify it.

Nitin Chugh: Yes, around 9.5%, 10% has come from volume growth out of the 14%.

Moderator: The next question is from the line of Bino Pathiparampil from Elara Capital. Please go ahead.

Bino Pathiparampil: A quick question on the number of labs added. This Polo which you have acquired, does it come with the lab or so?

Rahul Guha: So, Polo comes with, I think, 3 or 4 labs, which we have not included in our lab count. It's a good call out. But our total -- of about 8,500, we have not included the Polo count as yet because it's too early.

Bino Pathiparampil: Okay. So, I mean, is it just this quarter thing, next quarter onwards, I assume you will add that to the number of labs?

Rahul Guha: Yes.

Bino Pathiparampil: Okay. And you have added 2 labs on your own in the quarter. So going ahead, what would be the plan to add more labs? Would you be organically adding a couple of them a year or so?

Rahul Guha: Yes, I would say -- as I guided, maybe a year ago. Today, there is a Thyrocare lab anywhere in India within -- if you drop a pin anywhere in India, roughly in 200 kilometers. There are still

some white spaces that we don't occupy. Those -- you can expect at best 3 or 4 labs a year.

Bino Pathiparampil: Understood. And finally, on the imaging business, this 21% growth that we have seen in the quarter, is that sort of a sustainable number? Do you plan to invest more there, accelerate it? Or is it just a base effect and it will gradually come off a little bit?

Rahul Guha: See, I have always been saying you should expect -- I will be happy if we do mid-teen growth with stable EBITDA. I don't think we are in a position at this point to change the guidance.

Moderator: The next question is from the line of Aditya Khemka from Incred PMS. Please go ahead.

Aditya Khemka: Rahul, on the EBITDA margin guidance, just a little perplexed. In your opening remarks, you made a comment that you guys have been able to maintain EBITDA despite the operating

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deleverage of opening new franchisees, expanding your reach. So natural corollary would be that once your new franchisees become productive enough, breakeven and then start producing profits, your EBITDA margin should expand from here. So care to clarify on that?

Rahul Guha: Yes, but we'll continue. This expansion, Aditya, is a never-ending journey, right? So as the current franchisees stabilize and come back to normal EBITDA, we will continue to add new franchisees and I don't expect that journey to stop anytime soon. And we are also, as you know, on the lookout for more acquisitions. So therefore, what I've always been saying is, any operating leverage coming out of the business, we will invest back into growth, and I continue

to maintain that.

Aditya Khemka: Understood. That's helpful. Secondly, I saw a 4% increase in realization per test in your deck. Again, can you sort of explain if this 4% realization per test is a function of the mix or is it the pricing that we have used to produce this?

Rahul Guha: It's mix. We haven't raised prices, so it's only mix.

Aditya Khemka: It's all mix. Okay. On that side then, Rahul, if I look at Thyrocare historically, one of the strengths or weaknesses of Thyrocare was a narrow test menu, which obviously limited your ability to attract customers, but at the same time, it gave you operating leverage because you were able to use the same analyzers a lot more than other labs are able to do. So when you expand test menu and you offer new more esoteric tests, wouldn't that sort of impact asset utilization adversely?

Rahul Guha: Yes and no. We are very cautious about the menu that we expand. We try to -- without getting into the technicalities, right? If you have an analyser that can do, let's say, 20 tests. Currently, we do 10, right? We know there is a demand for 5 more, but it does not require any additional CapEx. It does introduce complexity in the number of reagents that you store and the number of controls that you have to run and all of that. But from a capex point of view, there's hardly any additional investment.

So our expansion of test menu is what more can we do with the technologies that we have rather than entering into new technologies. The exception is histopathology, where we felt there is a definitely large market there, which we have been unable to tap in to, right? And we invested in that technology.

Also, with the acquisition of Vimta, they had a much larger test menu than us. And we were able to see areas which were white spaces, which we hadn't tapped into, right, where there is a significant demand, but we hadn't gone into that test menu. But I think with the exception of histopathology, we haven't expanded our technology platform. Our technology platforms remain the same. Within the technology platforms, we have expanded the menu.

Aditya Khemka: Got it. And actually, my last question was on Vimta itself. So you have recently closed the transaction. Could you give us a flavor of what it's looking like? Or is it too early to say?

Rahul Guha: It's quite early, but I'll let Alok give you some snapshot.

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Alok Jagnani: So, Aditya. So we have recently completed the Vimta deal BTA on the 11th of October only. And in the presentation also, we have given some highlights that what the turnover of business, what they had. Around INR 30 crores of top line, what they are carrying. And they have a more or less very close to breakeven or some minor negative EBITDA margins, what they are currently managing. We are very hopeful that now it's going to be in our portfolio, we are going to change and turn around the same over a period of next 9 to 12 months' time and going to bring back to the Thyrocare parameters and margins what we are looking for, normally what -- so that's what we are expecting.

Vimta was having around 7, 8 labs, but all the labs we have not taken over. We have a consolidated a few labs. A few labs, we have taken over and continue to do what they are having.

And over a period of time, we are going to review that what can be consolidated and what can be expanded depending upon the volumes and other things. So next 6 months, let's wait and watch how Vimta businesses are moving towards.

Moderator: The next question is from the line of Nilesh Sharma from Anantnath Skycon Private Limited . Please go ahead.

Nilesh Sharma: Sir, will you please guide us any improvement from the front of insurance company tie-up that you were talking in your last con call, that you're planning to tie up with the insurance companies for a big corporate tie-up where we can

increase our regular checkup. So is there any significant development on that front?

Rahul Guha: Sure. I'll let Nitin take that question.

Nitin Chugh: Yes. So see, our focus continues to be on the insurance business, both on the annual health checkup as well as the PPMC. Yes, we have made some strides because I cannot obviously name the people that we are working with and name the companies. But yes, there are a few clients that we have added on and we are adding regularly every quarter.

Rahul Guha: Just to clarify also, the July, August, September period tends to be a low period for insurance. You will actually see the pickup as we come into November, December and then, of course, the peak is Jan, Feb, March.

Nilesh Sharma: Okay, sir. Sir, in continuation of this answer, roughly how much percentage of revenue we can capture from this segment?

Rahul Guha: Difficult to give you an estimate at this point in time. It's still early, but I would say our insurance segment roughly in partnerships is how much?

Nitin Chugh: Okay. Under partnerships, you say...

Rahul Guha: It's about -- 30% to 40% of our partnership business is insurance. And that, of course, is a very low base compared to the opportunity in the insurance space.

Nitin Chugh: Addressable market is very high. Yes. We have hardly touched anything on that.

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Rahul Guha: But difficult to give you an estimate. I think as we come into the March quarter is when I will be able to have a clearer view on how large this segment can be for us.

Nilesh Sharma: Okay. Sir, our focus in this area is because of the GST council meeting, which is going to happen in October, where the GST decision will be taken on health insurance segment. So, is there any positive outcome that we can also expect from that GST meeting that's indirectly favourable for our business as well?

Rahul Guha: Alok?

Alok Jagnani: So, I think it's going to be early to comment on that, but valid point, may be possible that like currently, we are not getting any standard credit on the procurements, what we do. But once the GST is going to come, then maybe we are going to get a cost benefit on the inputs, what we are going to have. But let's see if some amendments come, then what will happen?

Another impact is going to be that like currently, customers are not paying GST. Once GST is going to be cost to them, then the price how it's going to be covered up in the price or we are -- customers are ready to pay additional GST, that we have to see in the market also, how market responds to that?

Rahul Guha: So, I think this question -- so just to answer for us, at least our internal view is that GST in diagnostics will only give a boost to the organized segment where we are present. On your question on the GST Council meeting in insurance segment, we haven't studied the outcomes of that. So difficult to comment at this point.

Moderator: As this was the last question, I would now like to hand the conference over to Mr. Rahul Guha for closing comments.

Rahul Guha: Thank you, everyone, for joining us and spending the time with us this evening. As always, we continue to remain focused on our strategy, which is to be the most affordable, good quality diagnostic testing partner for anyone in the health care business, and we continue to execute on that strategy. We have been investing in improving our quality, improving our reach and ensuring our turnaround time is as close to best-in-class. We've made substantial progress on all of this, and I thank you all for your support in this journey. With that, we can close the call. Thank you, everyone.

Moderator: Thank you. On behalf of Thyrocare Technologies Limited, we conclude this conference. Thank you for joining us, and you may now disconnect your lines.

Call: Jan 2025

January 27, 2025

To, National Stock Exchange of India Limited BSE Limited
Exchange Plaza Phiroze Jeejeeboy Towers Bandra Kurla Complex, Dalal Street,
Bandra (E), Mumbai - 400 051 Mumbai- 400 001 (SYMBOL: THYROCARE) (SCRIP CODE 539871)

Sub: Transcript of post resu lts earnings conference call held on January 23, 2025 with
Analysts / Investors.

Ref: Disclosure under Regulation 30 of th e Securities and Exch ange Board of India
(Listing Obligations and Disclosure Re quirements) Regulations, 2015 ("Listing
Regulations")

Dear Sir/Madam,

Pursuant to Regulation 30(6) read with Part A of Schedule III of the Listing Regulations,
please find enclosed herewith the transcript of the earnings conference call with Analysts and Investors held on January 23,
2025.

The same has also been made available on the website of the Company (www.thyrocare.com).

This is for your information and records.

For Thyrocare Technologies Limited

Ramjee Dorai
Company Secretary and Compliance Officer
Encl: A/a

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"Thyrocare Technologies Limited
Q3 FY '2 5 Earnings Conference Call"
January 23, 2025

MANAGEMENT : MR. RAHUL GUHA – MANAGING DIRECTOR AND
CHIEF EXECUTIVE OFFICER – THYROCARE
TECHNOLOGIES LIMITED
MR. ALOK KUMAR JAGNANI – CHIEF FINANCIAL
OFFICER – THYROCARE TECHNOLOGIES LIMITED
MR. NITIN CHUGH – CHIEF COMMERCIAL OFFICER –
THYROCARE TECHNOLOGIES LIMITED
MR. KAPIL GUPTA – STRATEGY AND INVESTOR
RELATIONS – THYROCARE TECHNOLOGIES LIMITED

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Moderator: Ladies and gentlemen, good day, and welcome to the Thyrocare Technologies Limited Earnings
Conference Call. As a reminder, all participant lines will be in the listen -only mode. There will
be an opportunity for you to ask questions after the presentation concludes. Should you need
assistance during this conference, please signal an operator by pressing star and then zero on
your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Kapil Gupta from Thyrocare Technologies Limited.

Thank you, and over to you, sir.

Kapil Gupta: Thank you, Darwin. A very good evening to all and thank you for joining us today for the
Thyrocare Earnings Conference Call for the Third Quarter of FY '25. Today, we have with us
Mr. Rahul Guha, MD, and CEO of Thyrocare; Mr. Alok Kumar Jagnani, CFO of Thyrocare; and
Mr. Nitin Chugh, Chief Commercial Officer of Thyrocare, along with other key members of the senior management on this call
to share highlights of the business and financials for the quart er.

I hope you have gone through our results release and the quarterly earnings presentation, which has now been uploaded on the stock exchange website. The transcript of this call will be available in a week's time on the company's website. Please note that today's discussion may be forward-looking in nature and must be viewed in relation to the risks pertaining to our business. After the end of this call, in case if you have any further questions, please feel free to reach out to the Investor Relations team.

I now hand over the call to Mr. Rahul Guha to make the opening remarks.

Rahul Guha: Thank you, Kapil. Good evening, and welcome to all on the call. Thank you for taking out time from your busy schedules to join us this evening. Just a quick introduction to us on the call. My name is Rahul Guha; I am the MD and CEO of Thyrocare and thank you for the opportunity to present the Q3 results of FY '25. I'm joined by my colleague, Alok Kumar Jagnani, who is our CFO; Nitin Chugh, who is our Chief Commercial Officer; Kapil Gupta, who is part of our Strategy and Investor Relations team.

As in all my calls, I will start with a quote from Nelson Mandela in recognition of our foray into Africa. "It is in your hands to make a better world for all who live in it." And we believe Thyrocare can bring our business model to Africa to make affordable and good quality diagnostics available to all.

Before we get into the details of this quarter, I'll reiterate the pay-for-performance pricing structure that we implemented in 2023. Earlier, our discount was one size fits all, but now we have moved to a slab-based pricing model, which we implemented last year. It's been close to 1.5 years since that implementation, and this has led to an increased energy with our franchisee network with motivation to move up volumes and enter higher slabs. It will result in a movement towards larger franchisees and enable much greater reach from our large partners.

Quality remains our highest priority, and we see it as a continuous journey of improvement and innovation. We are immensely proud to share that now we are India's first and only 100% NABL accredited national laboratory chain. This prestigious milestone reflects our unwavering

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commitment to delivering world-class diagnostic services, and it is the result of relentless dedication to quality excellence.

Achieving NABL accreditation across all our labs has been made possible through the implementation of robust quality management systems, investment in cutting-edge technology and equipment, rigorous training programs for our staff, and consistent participation in proficiency testing.

Considering that only 2% of pathology labs nationwide hold this accreditation

, this achievement

is not just a significant milestone for us, but also a testament to our leadership in redefining diagnostic standards across India. Further, to validate our commitment to quality, we conducted an independent study published in the International Journal of Advance Research, Ideas, and Innovations in Technology.

The findings revealed that 9 out of 10 doctors trust Thyrocare reports and confidently recommend our services to their patients. This recognition highlights the dedication and hard

work that we have consistently invested in upholding the highest standards of quality.

Also, recently, we were recognized and rewarded by the College of American Pathologists, CAP, in short, for maintaining excellence in high-quality laboratory care for 15-plus years. CAP is an international gold standard accreditation that represents the top-tier quality in pathology and laboratory medicine.

We regularly host advisory board meetings with a panel of esteemed doctors to gain their insights on enhancing our quality milestones. Doctors witness our cutting-edge technologies and stringent protocols, reinforcing our commitment to diagnostic excellence.

I'm also very proud to say that recently, we launched the largest HbA1c study across the country, wherein we have studied 20 lakh HbA1c results to publish the largest insights on HbA1c in the country. While maintaining the highest quality standards, on an average, we released the report within 3.65 hours of samples reaching the lab.

This rapid turnaround is made possible by our robust operational processes, advanced automation systems, and streamlined workflow. By combining precision with efficiency, we ensure timely and accurate diagnostics, which empowers patients and health care providers to make informed decisions swiftly.

Beyond the work on quality, we continue to selectively expand our offering. Aarogyam has been our flagship brand in the preventive health care segment, and now we have Jaanch as our disease-specific packages. Jaanch, as I have said before, is targeted towards lifestyle challenges or for you to better understand your health.

We have solutions across the spectrum for anything you might be worried about. Is it fever or

something more serious? Why is my hair falling? Is pollution affecting my health, cancer screening, as well as deep investigations for common chronic diseases like diabetes and heart health.

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We are very proud of some of the key milestones we have achieved in this quarter. We have 9,100 active franchisees. As a result, we processed 5.9 million samples and served 3.9 million patients in this quarter, which is 15% and 13% year-on-year growth, respectively, in volume. Total tests conducted during this quarter was around 39 million, which grew by 14% year-on-year.

We are regularly taking strategic initiatives to further expand our footprint. In July 2024, we acquired Polo Labs, which is a pathology diagnostic company based out of Punjab, with a presence in Punjab, Haryana, and Himachal Pradesh. This allows us to expand our footprint in North India.

We also acquired the clinical diagnostics business of Vimta Labs in October 2024. Vimta's Clinical Diagnostics division, with its established presence in Telangana and Andhra Pradesh, offers us an excellent opportunity to further strengthen our presence in South India.

Partnerships business did phenomenally well in the quarter, as you have seen in past quarters as well, and grew as we onboarded new clients in health tech segments and continue to grow the existing accounts. Also, with the acquisition of Think Health last year, it has strengthened our offering for the insurance segment with the additional capability of ECG at Home.

Now we are covering ECG at Home services in 1,000-plus pin codes with a dedicated fleet of 125 ECG Phlebos. This allows us to give

our insurance partners a one-stop solution for blood and ECG testing and further deepens our presence in the pre-policy medical checkup and annual health check-up market.

And now we are the company who offers health checks with ECG at Home, and I'm sure that will enhance our Aarogya brand. In Tanzania, since going live in March 2024, and processing our first sample in April, we have successfully partnered with over 100 health care facilities in Dar es Salaam.

Our mission is to continuously collaborate with major hospitals, ensuring they have access to comprehensive diagnostic services. With our state-of-the-art laboratory equipped with world-class machines and infrastructure, we are poised to make a significant impact on health care in that region.

With that, I will now hand over to my colleague, Nitin, to cover the highlights for the quarterly business performance.

Nitin Chugh: Thank you, Rahul. A warm welcome to each one of you. First, I would like to start with our pillars of growth, which have been contributing strongly to our consistent performance. The first is customer success. The focus is to ensure accurate, timely and affordable diagnostic services through quality control, robust data management and customer support.

With advanced technology and streamlined processes, we aim to enhance customer satisfaction. Just to give you an example, we have now started showing the turnaround time for reporting as a guidance to our franchise network, which is further helping our franchisees on predictability of reports delivery.

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The second pillar is network expansion. We are deepening our presence across India by going deep into the country with our franchise network and through our acquisitions as well. Also, on the partnership side, we are expanding our footprint by going deep into the pre-policy medical checkup and annual health checkup in the insurance business.

The third area where we are now focusing is to enhance our menu expansion and visibility. We are introducing a wider range of specialty tests and health packages for our partners using targeted marketing to drive adoption and increase value for both partners and our customers.

Again, an example of this is introduction of Fetal Medicine Foundation backed certified pregnancy markers and starting histopathology and cytology in-house in our labs is a testament of our focus on expanding our menu with highest quality standards.

Now I will briefly update you about the business performance highlights for the third quarter of FY '25. Overall, at a consolidated level, we did a 23% year-on-year revenue growth this quarter, primarily driven by our pathology business. Our franchise business showed a revenue growth of 24% year-on-year. This includes 5% of inorganic growth coming from our recent acquisitions

of Polo and Vimta, and 19% organic growth, and this has come on the back of focus on customer success, menu expansion and network expansion.

Our partnership business grew by 23% year-on-year, wherein our API PharmEasy Diagnostic business also showed a growth of 19% year-on-year. Radiology business, including Pulse Hitech, did a strong revenue growth of 13% year-on-year.

With that, I will hand over to my colleague, Alok, to cover the results.

Alok Jagnani: Thank you, Nitin, and a warm welcome to everyone joining us today. Before we dive into the specifics, I would like to emphasize that while the pathology diagnostic industry is growing at an early to mid-teen rate, we have consistently achieved mid-teen to high-teen growth over the past few quarters. This sustained outperformance reflects the strength of our leadership and our ability to capitalize the on-market opportunity.

Before diving into the financials, I would like to revisit our ESOP program, which has been focused in previous quarters. This initiative introduc

ed at a group level is designed to retain the

talent at Thyrocare. The ESOP issued at our parent company will vest according to the policy.

From an accounting standpoint, these are recognized as an expense in the profit and loss account and as an equity contribution from the parent in the balance sheet.

It is important to emphasize that this is a noncash charge and does not impact the company cash flow. Accordingly, we calculate a normalized EBITDA to account these expenses. Further details can be found in the accompanying presentations uploaded.

This year, as the Thyrocare ESOP program is coming to an end, the group has expanded the API ESOP pool to the senior management at Thyrocare. For the quarter, revenue stood at INR 153 crores stand-alone, and at a consolidated level, INR 166 crores, reflecting a 23% year-on-year growth.

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This growth was driven by strong 24% rise in franchisee revenue and a 23% increase in partnership revenue. Total pathology revenue grew by 24% year-on-year, while the radiology revenue saw an increase of 13% year-on-year. On the total 23% revenue growth, 19% has driven from organic, while the remaining 4% resulted from the inorganic expansion.

Think Health and Polo business have largely stabilized and now fully integrated with the Thyro ecosystem. Meanwhile, the acquisition of Vimta Clinical Diagnostics is currently in the transition phase with operations expected to stabilize and consolidate within the next 3 to 6 months' time.

Our stand-alone gross margin for the quarter was 72%, up by 181 basis points, primarily due to onetime audit note and basis volume targets with our vendors -- correction, onetime credit note and basis volume target from our vendors. Employee expenses increased year-on-year, driven by annual increment and headcount growth from new business acquisition.

The stand-alone normalized EBITDA margin for the quarter stood at 31%, an increase of 375 basis points, primarily due to margin improvement and operating leverage. EBITDA margin in radiology business, NHL, has improved as compared to the last year due to growth in revenue and efficiency in other expenses.

At a consolidated level, gross margin stood at 73%, while the normalized margin -- EBITDA margin was 30%. Our normalized EBITDA grew by 42% year-on-year, while reported EBITDA increased by 32% year-on-year and PAT increased by 30% year-on-year. This performance was achieved despite margin pressure from recent acquisition, geographic expansion, high noncash ESOP costs, and increased depreciation expenses due to the change in accounting estimate.

Now I will hand over the call to Rahul for strategic updates.

Rahul Guha: Thank you, Alok. Briefly, I would like to take a few minutes to recap to you our strategic direction, and then I will open it up for Q&A. First, I will reiterate our value proposition to the customer. We will continue to remain an affordable option to all patients with good quality and on-time reports.

All our efforts on our value proposition is towards ensuring low cost to the patient, assurance on quality of testing through our certifications and engagement with doctors. We have made tremendous progress on this, which I've updated in my initial comments, and is reflected in detail in the presentation. This will remain at our core and will continue to guide all that we do.

Second, our strategy. We continue to maintain our strategy of being the B2B partner of choice to all front-end diagnostic services companies in India, whether it is a small diagnostic center in a semi-urban area, a pharmacy in a metro, a small nursing home, an individual doctor, or a leading online diagnostics platform, a health tech marketplace.

We are happy to work with them to provide low-cost, robust testing solutions, so that they can serve their patients in the most effective manner. I

f they require phlebotomy or ECG at Home,
we are happy to mobilize our network of almost 1,500 phlebotomists, including our network
partners, to serve them better.
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We remain dedicated to expanding our business. And with the acquisition of Polo Labs and
Vimta Clinical Diagnostics, we plan to significantly increase our presence in North and South
India, respectively. Additionally, to further boost our partnerships business, the acquisition of
Think Health allows us to offer ECG at Home services, further enhancing our value to our
insurance partners.

This strategy has been working well for us with both our franchise and partnerships businesses
posting strong growth. That in a brief is our mandate as management. Thank you so much for
giving us a patient hearing. I will once again end with a quote from the Mahatma. "Find purpose,
the means will follow." And our purpose remains to provide affordable, high-quality testing to
the masses.

With that, we'll open up for Q&A.

Moderator: Thank you very much. We will now begin the question-and-answer session. We have the first
question from the line of Prakash Kapadia from Spark PMS. Please go ahead.

Prakash Kapadia: A couple of questions from my end. Generally, we've seen Q4 is a strong quarter for us. So, what
kind of growth are we looking at on a sequential basis? And also, if you could comment on the
range of health packages we have? And what is the contribution to our annual sales maybe last
year and currently in the 9 months? That's my first question.

Secondly, on the franchisee network, can you share some color in terms of contribution for 9 months in terms of silver,
bronze? How is the strategy of focusing on larger franchisees? How
is the throughput increasing through them? And what kind of franchisee addition is possible
from here on, say, next year and beyond?

And lastly, there have been media reports suggesting PharmEasy is contemplating a reverse merger with Thyrocare. So, any
comments, any discussions? So, these were my 3 questions.

Rahul Guha: Sure. Prakash, thank you for the questions. And see, the momentum for the business, as you can
see, is quite strong, right? If I look at YTD also, we have kind of landed at around 20%, whereas
in the beginning part of the year, I was indicating mid-teens is what I would be happy with.

Of course, Q4 last year was very good for us also, right? So, when you look at it on a year-on-
year basis, it is off a very good quarter. And yes, while definitely Q4 will be much better than
Q3, to give you a specific growth number at this point in time will be difficult. But I will be
happy if we are able to continue the YTD momentum into the fourth quarter as well, right? That
is as much as I can kind of speculate at this point.

With regard to package mix, see, we have 2 main brands. We have Aarogyam and Jaanch, right?
Aarogyam roughly accounts for about, I think, last, about 35% of our total sales, right? And
Jaanch would be about, I would say, 3% to 5%, in that range. Jaanch has doubled over the
previous year. Aarogyam continues to kind of track our normal growth.

So as a mix, it is growing, Jaanch, but it is off a small base, because the brand is a little over a
year old. So, it will take some time to mature. But the growth rate is very encouraging, right? So
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that is that. Now when it comes to the mix between franchisees, right? So, I guess you are quite
pry to our categorization of diamond. So, diamond to silver, Prakash, has grown -- at a volume
level, it would have grown at about 25%, and bronze and others would have grown at 9% on a
volume level.

Prakash Kapadia: Okay. So, it's in line with -- the strategy is working?

Rahul Guha: Yes, very much in line with what we were planning. And as that mix shifts also, we ge
t much

better retention, much better average revenue per franchisee, because everyone is incentivized
to move up the ladder, right? So that goes very well for us. On the pharmacy point, see, firstly,
the media reports are idle speculation.

It's very difficult for me to comment on articles that are speculative in nature. What I can tell
you is at the Thyrocare Board, none of this has been discussed or contemplated. So, it seems like
this is just media speculation at this point in time.

Prakash Kapadia: Understood. And if you could comment on the range of packages. You obviously mentioned the
contribution of Aarogyam and Jaanch. So, the range of packages which we have at Aarogyam
or Jaanch. Jaanch obviously would be much smaller in terms of pricing because it's bucketing
into different segments. So, what kind of a range of package, say, starting from INR2,000 to

INR7,000, or Jaanch would be INR 500. Some color on the pricing or the range?

Rahul Guha: Sure. Maybe I'll let Nitin also chip in. But just to give you a sense, Aarogyam actually starts from Aarogyam A to Aarogyam XL. Aarogyam A is priced at INR 1,000. Aarogyam XL is closer to INR10,000, right? So, starting from INR 1,000 to INR 10,000, we have the full range of packages.

And the number of parameters start at about 40 and go to almost 200 plus in our largest thing. I think Aarogyam XL has 139 or 145 if I remember. So, we have the full range. That being said, the most popular package is Aarogyam C, which is in the INR 2,000 to INR 2,500 range. Nitin, you want to add anything?

Nitin Chugh: Yes, that's absolutely correct. And if you talk about Jaanch also, see, Jaanch also covers various types of therapy areas. So basis the type of therapy area and what test it includes, it also starts, the basic test of Jaanch for a certain therapy starts about INR1,000-odd range -- INR800 to INR1,000-odd range, and they also go up to like INR 4,000, INR5,000 kind of range.

Prakash Kapadia: Okay. That's a broader range. Understood.

Moderator: We have the next question from the line of Ashutosh Parashar from Mirabilis Investment Trust . Please go ahead.

Ashutosh Parashar: Congrats on another good quarter. So, a couple of questions. First is on the Vimta Labs acquisition. So, if you could just share how long has it been consolidated in the current quarter's number? And run rate wise, what is the current quarterly run rate you are expecting it will generate in the coming quarters, and maybe outlook on the next year after all the consolidation happens? So first is on that.

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And second is on -- so we have been focusing on partnering with a lot of insurance players and med tech players on the pre-policy health checkup and annual insurance health checkup plans. So, if you could give us some numbers on number of partners that we are currently working on in insurance space and the med tech space? And how much is it contributing in the entire partnership revenue pool?

So, these are the first couple of questions. And then lastly, on the new ESOP pool that we are creating, what is the pool size and how will it be amortized in the P&L over the next few years? So those will be the questions.

Rahul Guha: Sure. So, I'll break it up. I'll let Alok take the Vimta question, and I will let Nitin take the insurance question. So Alok, if you would like to cover when we acquired Vimta, how much does it sit in this quarter? And what do we think will be the run rate over the next few quarters?

Alok Jagnani: Yes. Thank you. So, Vimta, we had acquired mid-October, and it's only 2.5 months has gone. Vimta is in a stabilization phase, and it will take another 3 to 6 months' time to consolidate and set up with the entire Thyrocare ecosystem. In terms of revenue, around INR 4 crores, INR 4.5 crores has been added on account of Vimta in the Q3 as inorganic growth, which

we have seen

highlighted also that around 4% of the total revenue contribution has come from inorganic, in which INR4.5 crores is contributed by Vimta.

When we had acquired Vimta, Vimta had a turnover run rate of INR 2 crores approximately with a mix of B2B and B2C business. And we are trying to retain the maximum what we can retain the business, because there's a lot of mix, B2B and B2C. We are primarily a B2B player and also focusing what B2C -- with various means of strategy what we are going to do. And the lab consolidation and other activities are in progress. So, in the next 6 months, the entire consolidation, the transitions are going to be completed.

Rahul Guha: So, I would say, if you're looking for a forecast, you should not look at more than between INR 3 crores and INR 4 crores a quarter at least for the next 4 quarters until we stabilize Vimta. And then we will see from there whether we grow under the Vimta brand or we use that network to expand the Thyrocare brand in AP and Telangana, and then therefore, where the revenue gets reported.

But at a steady state, at least for the next 2 quarters, I can definitely say don't expect more than INR3 crores to INR4 crores a quarter from Vimta. Nitin, do you want to take the insurance question?

Nitin Chugh: Yes. So, insurance business, we have taken some steps towards increasing that, especially in the quarter 3. We are now working with -- I cannot obviously name the partners right now, but we're working with more than 5 aggregators or TBAs, which in turn service about 10-odd insurance partners.

And this business, especially on the PP MC side has been steadily growing and contributes to almost 10% of the partnerships business. This is excluding API PharmEasy. The remaining partnerships business, more than 10%, around 12%, 13% is contributed by insurance.

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Rahul Guha: He was also asking like how many insurance...

Nitin Chugh: I just told that we work with health aggregators and partners. There are about 5 to 6 that we are currently working, which in turn service about 10, 12 insurance partners.

Rahul Guha: On the ESOP pool question, there was an old ESOP pool of about INR 43 crores that was at the -- actually, first, let me clarify. These are API Holdings' ESOPs, not Thyrocare ESOPs, so there will be no dilution of Thyrocare shareholding as a result of these ESOPs. These are ESOPs granted by the parent to employees of the subsidiary.

The second point is there is a noncash charge, which Alok already explained once. It's routed via the P&L, but it has no cash impact on Thyrocare. The old ESOP pool, which was at the pre - IPO price of API, was about INR 43 crores. And the new ESOP pool is about INR 47 crores, which is at the revised price.

So, there are 2 things that have happened. One is, of course, the old ESOPs have already been fully charged into the P&L. But given the prices at the API side, to retain the talent, the API Group felt it was prudent to expand the pool. So actually, there was a very limited pool in Thyroc are that was getting API ESOPs.

Now it's a much broader set of employees who are getting these ESOPs. The ESOPs are being charged over the P&L over a period of 6 years, and that is the broad vesting schedule as well.

So, it's there to ensure that the talent remains for the long term to create value for the business.

I hope I've answered all your questions.

Ashutosh Parashar: Yes. Just one follow -up. So earlier pool, we have charged fully to the P&L, and the new, INR 45 crores, roughly, pool will be charged over the next 6 years?

Rahul Guha: Yes. You would see in the P&L roughly about INR 7 crores charge in this quarter. That, of course, comes down as the calculation pans out, but you'll see it in the P&L.

Alok Jagnani: So, in the initial years, like the vesting period is 5 years and 6

years, depending upon the vestings

what grant has offered. And the initial, first year, second year, and third year, the charge to the P&Ls are going to be on higher side, whereas over a period of 4 years , 5 years, we will find that the charges are comparatively much low.

Ashutosh Parashar: Got it. Just one last follow -up on the Vimta Labs acquisition. So, during the time of acquisition, I guess, in our filings, we had mentioned that it had Q1 revenues of around INR 7 crores. So, anything that we had closed down from the acquisition that has led to this reduced run rate from INR7 crores to roughly INR 3 crores to INR 4 crores?

Rahul Guha: Yes. There were some businesses that were being done for clinical trials in Vimta, which we did not want to continue. And then there were some geographies, while we had acquired Vimta to deepen in the south of India, but Vimta had also labs in UP, Odisha, where the Thyrocare network is already very strong. So, we didn't see any sense in duplicating that network. So that business, we moved into Thyrocare. And whatever remaining Vimta business is largely focused on South India.

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Moderator: The next question is from the line of Bino Pathiparampil from Elara Capital . Please go ahead.

Bino Pathiparampil: Congrats on a good set of growth numbers. Just a couple of clarifications. The 38 labs count that you have given, does that include all the Vimta, Polo, all these acquisitions that you have done?

Rahul Guha: Sorry, you said 38 lakhs ?

Bino Pathiparampil: 38 labs, count of lab.

Rahul Guha: Yes, that includes Vimta and Polo also.

Bino Pathiparampil: Okay. Great. Second, on the ESOP cost continuing. So, this quarter was INR 7.2 crores. So, for the next 3 more quarters, should we expect a similar number, INR 7 crores roughly a quarter?

Rahul Guha: Roughly. It will go down marginally, because the original INR 3.1 crores will come down, right?

So, to that extent, you will see some benefit, but it will be in this range.

Alok Jagnani: In addition, if additional pools are going to be offered -- added, then the charge will be higher.

Otherwise, it will be more or less flat.

Bino Pathiparampil: Okay. So, is it going to be an ongoing process, the addition of pools? Or was it a onetime exercise?

Rahul Guha: No. Largely, it was a onetime exercise. I don't anticipate too many more pools being created at this point in time.

Bino Pathiparampil: Okay. Understood. Second question on depreciation. In your notes, you have said that you have reassessed the life of some of your machines. And...

Moderator: Ladies and gentlemen, the current participant seems to have dropped from the queue. We have the next participant, Mr. Abdulkader Puranwala, with the next question. Please go ahead, sir.

Abdulkader Puranwala: Just on this ESOP charge again. So, the INR 7.7 crores, I mean, if you could help us understand how -- what would be that number in FY '26 and '27? I understand there might be a certain drop, but if you could help us quantify that?

Alok Jagnani: So here, like Rahul mentioned that the pool is -- new pool of INR 47 crores has been added in this ESOP API, and which is going to be a vesting period of 5 years. So, in this quarter, the charge has come INR 7.7 crores. And in subsequent quarters, we will see more or less -- at least next 2, 3 quarters, we'll see the same charge or slightly lower charge.

And then the impact of second year and third year is going to be 31%, then 20%, and then 14% and all, which you see the last, because it has a present value impact, pricing impact, and timing gap, because first year...

Rahul Guha: What we'll do is next quarter, we'll include the charge schedule, so that you can see it. We'll show it for the full year, and we'll give you a schedule of how it will be charged in the P&L over Thyrocare Technologies Limited
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the next 2, 3 years. But as Alok said, of the total pool of INR 47 crores, typically 30% is charged in the first year, 20% in the second year, I think 15% in the third year, and then it goes...

Alok Jagnani: That's normally that.

Abdulkader Puranwala: Got it, sir. Yes, that would be helpful if you can add that to the presentation. Sir, second is on the depreciation cost, where I understand there was some asset-related adjustment. So just wanted to understand, first of all, at a consol level, the INR 17 crores of depreciation cost, does that include any one-off? Or this is the quarterly run rate to what we can see in quarters ahead as well?

Rahul Guha: I'll let Alok take this.

Alok Jagnani: Yes. So, there is a one-off here. So, we have revised our assets' life expectancy. And based on the revised expectancy of assets' life, the depreciation hit has come, and hit has taken on prospectively -- onetime hit has come in the depreciation because of the revised estimates of depreciation is INR 4.75 crores.

The main call what we have taken like we were having the old assets, which we are depreciating at a life of 12 years to 14 years, which we have revised to 7 years. Because of the change in life expectancy, the depreciation impact has come.

Rahul Guha: I guess the question is, this INR 4.7 crores, do you expect it next quarter?

Alok Jagnani: No, it's a onetime. Yes.

Rahul Guha: So, to answer your question, Abdulkader, there is a one-off in there, and that's largely due to the revision in policy.

Abdulkader Puranwala: Okay. So INR 4.75 crores will be the one-off this quarter. Understood. And sir, on your operations in Tanzania, heard in your opening remarks about 100 health care units you have been tied up with. Any color on what is the revenue potential you see coming from Tanzania, what we are doing and what sort of margins you can make there?

Rahul Guha: I'll let Nitin take that. As I said, look, we processed our first sample in April, right? So, we have started to get initial samples from hospitals. And in Tanzania, it's largely polyclinics that work with us. But they are trying us out right now. So, it's not a, how do you say, very large business at this point. I think in Q3, it would be about INR 35 lakhs of top line. But I'll let Nitin give some more color on this.

Nitin Chugh: Yes. So, see, it started in Q1 of FY '25, right? And quarter 2 was like INR 10 lakhs a quarter, and then we moved to INR 30 lakhs. It's still at a very nascent stage. So, any kind of guidance is very difficult to be given at this point. It's just that we are on the ground working and tying up with more and more partners.

We have crossed 100 partners in the Dar es Salaam region, which, like Rahul said, are trying us out. So, it's a matter of time where we slowly not only just increase our partnership base, but also increase wallet share from existing partners once they have started trying our services.

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Abdulkader Puranwala: Got it. And sir, just final one from my end. In terms of your capex program. So earlier you were talking about some refurbishment capex for the radiology business. So, have we completed that? And how do we see the capex spending, say, for FY '26 and '27, if you could give us some color there?

Rahul Guha: Sure. So firstly, on the radiology business, we have not incurred a large amount of capex, nor is there any plan to significantly invest in capex in the radiology business. The machines are old but still running. And at this point, we will look at refurbished options should machines come to

end of life, but we are not going to put in a lot of fresh capex into this business. When it comes to our total capex that we have spent in this year, I think up to Q3, we've spent about INR15 crores. And I think over the entire year, you can assume maybe another

INR10

crores odd. So that is broadly -- which is the new labs and that I had indicated in the beginning. But I think that's about it.

Moderator: We have the next question from the line of Shivam Saxena from ICICI Bank. Please go ahead.

Shivam Saxena: Just having a question on -- is there any impact of -- recently, India has banned import of refurbished machines. So, any impact do you see on your operations? On your course?

Rahul Guha: We stopped the policy of buying refurbished machines, I would say, almost 2 years ago. All the machines that we buy today are brand new top-of-the-line equipment. So, we haven't been impacted in any way by this.

Shivam Saxena: Okay. And how do you see pricing environment going forward in this space?

Rahul Guha: The pricing environment, as I've been saying over the last 3, 4 quarters, has softened. I'm not seeing as much price competition as we did, let's say, last year at the same time. A lot of rationality has set in. And so, we are now mostly competing on service and quality rather than price.

And this is a sector which, to some extent, deep discounting doesn't work, right? And I think many players who try to take away share from this business by doing deep discounting have realized that one, you don't get the volume; two, your cost structure remains unsustainable. So, I'm seeing a fair amount of rationality in pricing right now.

I'm not saying -- India's prices still remain the lowest in the world. But at least within India, definitely some easing up on price competition we have seen over the last 2, 3 quarters and hopefully will continue for some time.

Moderator: We have the next question from the line of Aditya from Securities Investment Management. Please go ahead.

Aditya: I just needed one clarification. So, since all the ESOPs have been done at API level, so ideally, there shouldn't be any dilution at Thyrocare. But when I look at Q-o-Q, our number of shares have increased. So, what has led to that?

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Rahul Guha: Yes. So one is, let me confirm, there is no dilution in Thyrocare. I will say it repeatedly. However, as I have mentioned, there is a Thyrocare ESOP program that is coming to the end, right? So, the share increase that you're seeing in the BENPOS and in the shareholding reporting is basically the shares under the Thyrocare ESOP program that were declared granted and vested by Thyrocare employees, and now they hold the shares in their hands.

Moderator: The next question is from the line of Ashutosh Parashar from Mirabilis Investment Trust. Please go ahead.

Ashutosh Parashar: So just another question on Vimta was what is the kind of profitability profile that it is currently working on? And after ESOP, if we look at the margin of around 25.2%-odd that we reported during the quarter, do we expect it to increase from the next quarter? Or will we have some kind of impact from the acquisition?

Rahul Guha: Sorry, your question was not very clear, Ashutosh. Repeat, if you don't mind.

Ashutosh Parashar: Yes, sure. So, the question was on the Vimta Labs profitability. Since the revenue run rate has somewhat reduced, so is it profitable for us at the EBITDA level at the moment? And the current quarter's margin after ESOP that we reported around 25.2% or 25.3%-odd. So, do we see this margin at the current level for the next couple of quarters by the time it gets consolidated fully and then the revenue run rate increases? Or should we expect it to normalize to 26%, 27% from the next quarter?

Rahul Guha: So, the Vimta profits are, I mean, around -- sorry, EBITDA loss at Vimta is about INR1.4 crores for the quarter, which is baked into the financials. There are no ESOPs in Vimta. So, keeping that aside, if I look at the next quarter, we definitely expect those losses to come down. Will they become 0? Probably not. But will they be at this

level? Also no, right? Our goal is at least to get it to 0 towards the back 0.5 year. But definitely, it will not be at this level.

Moderator: Thank you. Ladies and gentlemen, we have no further questions. I would now like to hand the conference over to Mr. Rahul Guha for closing comments. Over to you, sir.

Rahul Guha: Thank you, everyone, for joining us and spending time with us this evening. I want to go back to the original question on the reverse merger speculation that has been in the media. Once again, I would like to say, at the Thyrocare Board, this has not been discussed. And therefore, this remains largely speculative in nature.

I would like to remind all Thyrocare shareholders that should anything like this come up, it

would require an affirmation from the Thyrocare shareholders and would require a majority of the minority. That being said, it remains, at this point, media speculation.

However, the company performance of Thyrocare is on a fantastic growth trajectory with both revenue much above the industry growth rate, and the profit growth also coming through despite all the different hits that we've had to take that Alok shared. And we continue to remain focused on maintaining this kind of growth trajectory and improving profits as we go forward.

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As always, we continue to remain focused on our strategy, which is to be the most affordable, good quality diagnostic testing partner for anyone in the health care business, and we continue to execute on that strategy. We have been investing in improving our quality, improving our reach, and ensuring our turnaround time is as close to best-in-class.

And we've made substantial progress on all of this, and that is what is driving the results that you see. I thank you all for your support in this journey, and I wish you all a good evening.

Thank you.

Moderator: Thank you. On behalf of Thyrocare Technologies Limited, that concludes this conference.

Thank you all for joining us. You may now disconnect your lines.

Call: Apr 2025

April 29, 2025

To, National Stock Exchange of India Limited BSE Limited
Exchange Plaza Phiroze Jeejeebhoy Towers Bandra Kurla Complex, Dalal Street,
Bandra (E), Mumbai - 400 051 Mumbai- 400 001 (SYMBOL: THYROCARE) (SCRIP CODE 539871)

Sub : Transcript of post results earnings conference call held on April 23, 2025
with Analysts / Investors.
Ref : Disclosure under Regulation 30 of the Securities and Exchange Board of
India (Listing Obligations and Disclosure Requirements) Regulations, 2015
("Listing Regulations")

Dear Sir/Madam,
Pursuant to Regulation 30(6) read with Part A of Schedule III of the Listing Regulations,
please find enclosed herewith the transcript of the earnings conference call with Analysts and Investors held on April 23,
2025.
The same has also been made available on the website of the Company
<https://investor.thyrocare.com/>
This is for your information and records.
Yours faithfully
For Thyrocare Technologies Limited

Brijesh Kumar
Company Secretary and Compliance Officer
Encl: A/a

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Q4 & Annual Results FY '25 Earnings Conference
Call"
April 23, 2025

M
MANAGEMENT : MR. RAHUL GUHA – MANAGING DIRECTOR AND
CHIEF EXECUTIVE OFFICER – THYROCARE
TECHNOLOGIES LIMITED
MR. ALOK KUMAR JAGNANI – CHIEF FINANCIAL
OFFICER – THYROCARE TECHNOLOGIES LIMITED
MR. NITIN CHUGH – CHIEF COMMERCIAL OFFICER –
THYROCARE TECHNOLOGIES LIMITED
MR. KAPIL GUPTA – STRATEGY AND INVESTOR
RELATIONS – THYROCARE TECHNOLOGIES LIMITED

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Moderator: Ladies and gentlemen, good day and welcome to the Thyrocare Technologies Limited Earnings
Conference Call. As a reminder, all participant lines will be in the listen-only mode and there
will be an opportunity for you to ask questions after the presentation concludes. Should you need

assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone.

I now hand the conference over to Mr. Kapil Gupta from Thyrocare Technologies Limited.

Thank you and over to you.

Kapil Gupta: Thank you, Yasha shri. A very good evening to all and thank you for joining us today for the Thyrocare Earnings Conference Call for Q4 and annual results of FY '25.

Today, we have with us Mr. Rahul Guha, MD and CEO of Thyrocare; Mr. Alok Kumar Jagnani, CFO of Thyrocare; and Mr. Nitin Chugh, Chief Commercial Officer of Thyrocare along with other key members of the senior management on this call to share highlights of the business and financials for the quarter and the annual results.

I hope you have gone through our results release, the quarterly earnings presentation and press release which has now been uploaded on the Stock Exchange website. The transcript of this call will be available in a week's time on the company's website. Please note that today's discussion may be forward-looking in nature and must be viewed in relation to the risks pertaining to our business. After the end of this call, in case you have any further questions, please feel free to reach out to the Investor Relations team.

I now hand over the call to Mr. Rahul Guha to make the opening remarks.

Rahul Guha: Thanks, Kapil. Good evening and welcome to all on the call. Just a quick introduction to us on the call. My name is Rahul Guha, and I am the MD and CEO of Thyrocare. And thank you for the opportunity to present the Q4 and annual results of FY '25. I am joined with my colleagues, Alok Kumar Jagnani, who is our CFO; Nitin Chugh, who is our Chief Commercial Officer; and Kapil Gupta, who is part of our Strategy and Investor Relations team.

As in all my calls, I will start with a quote from Nelson Mandela in recognition of our foray into Africa. "It is in your hands to make a better world for all who live in it." And we believe Thyrocare can bring our business model to Africa to make affordable and good quality diagnostics available to all.

Before we get into the details of the quarter, I'll reiterate the pay-for-performance pricing structure that we implemented in 2023. Earlier, our discount structure was one size fits all, but

now we have moved to a slab-based pricing model, which we implemented in May of 2023. It's been close to 2 years since that implementation, and this has led to an increased energy within our franchise network, with motivation to move up volumes and enter higher slabs.

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It will result in a movement towards larger franchisees and enable much greater reach from our large partners. In the quarterly presentation, you will see how that strategy is panning out in terms of the new partner addition and the rate at which they are scaling year-on-year.

Quality remains our highest priority, and we see it as a continuous journey of improvement and innovation. We are immensely proud to share that we are India's first and only 100% NABL accredited national laboratory chain. This prestigious milestone reflects our unwavering commitment to delivering world-class diagnostic services, and it is the result of relentless dedication to quality excellence.

Achieving NABL accreditation across all our labs has been made possible through the implementation of robust quality management systems, investments in cutting-edge technology and equipment, rigorous training programs for our staff and consistent participation

in

proficiency testing. Considering that only 2% of pathology labs nationwide hold this accreditation, this achievement is not just a significant milestone for us, but also a testament to our leadership in redefining diagnostic standards across India.

Further, to validate our commitment to quality, we conducted an independent study published in the International Journal of Advanced Research, Ideas and Innovations in Technology. The findings revealed that 9 out of 10 doctors trust Thyrocare reports and confidently recommend our services to their patients. This recognition highlights the dedication and hard work we have consistently invested in upholding the highest standards of quality.

Also, we were recognized and rewarded by the College of American Pathologists, CAP in short, for maintaining excellence in high quality laboratory care for 15 plus years. CAP is an international gold standard accreditation that represents the top-tier quality in pathology and laboratory medicine.

We regularly host Advisory Board Meetings with a panel of esteemed doctors to gain their insights on enhancing our quality milestones. Doctors witness our cutting-edge technologies, stringent protocols, which reinforces our commitment to diagnostic excellence.

I am proud to share under Thyrocare Anusandhan, our research wing, we conducted and published India's largest HbA1c study, analyzing 20 lakh results to provide deep insights into diabetic trends nationwide. We also studied nearly 1 lakh dengue cases, revealing a shift in transmission patterns in 2024, marked by early onset and regional surges, highlighting the need

for timely and broader diagnostics beyond just platelet counts.

Additionally, our cervical health study, based on 15,000 plus samples collected via preventive CAMS and screening, reinforced the value of regular Pap smear and LBCC tests in detecting early cancer signs and other cervical health issues.

While maintaining the highest quality standards, on an average during FY '25, we released the report within 3.43 hours of the samples reaching the lab. This rapid turnaround is made possible by our robust operational processes, advanced automation systems and streamlined workflows. By combining precision with efficiency, we ensure timely and accurate diagnostics, empowering patients and healthcare providers to make informed decisions swiftly.

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Beyond the work on quality, we continue to selectively expand our offering. Aarogyam has been our flagship brand in the preventive healthcare segment, and we have 2 more brands, Jaanch and Her Check. Jaanch, as I have said before, is targeted towards lifestyle challenges or for you to better understand your health. We have solutions across the spectrum for anything you might be worried about. Is it fever or something more serious? Why is my hair falling? Cancer screening, as well as deep investigations for common chronic diseases like diabetes, heart health, and of course, thyroid.

We are very proud of some of the key milestones we've achieved in FY '25. We now have more than 11,000 plus active franchisees, and as a result, we processed 167.9 million tests, which grew by 14% year-on-year. It's important to contextualize this volume. If you add the volume of our competitors from their published reports, you will find that our test volume of 167.9 million is more than all the competitors combined. We served 16.7 million patients in the year, which increased by 11% year-on-year.

We are regularly taking strategic initiatives to further expand our footprint. During FY '25, we acquired Polo Labs in July 2024, which is based out of Punjab with a wide presence in Punjab, Haryana and Himachal Pradesh. This allows us to expand our footprint in North India.

We also acquired the Clinical Diagnostics business of Vimta Labs in October 2024. Vimta's Clinical Diagnostics division,

with its established presence in Telangana and Andhra Pradesh, offers us an excellent opportunity to further strengthen our presence in South India. This is in line with the pockets of white spaces strategy where we were looking for acquisitions in regions and I am very happy to say that both of these complement and will enhance our presence in areas where we have been vacant.

Partnerships business did phenomenally well this year and grew as we onboarded new clients in health tech segments and continued to grow the existing accounts. Also, with the acquisition of Think Health last year, it has strengthened our offering for the insurance segment with the additional capability of ECG at Home.

Now we are covering ECG at home services in 1,000 plus pin codes, with a dedicated fleet of 170 ECG phlebotomists. This allows us to give our insurance partners a one-stop solution for blood and ECG testing and further deepen our presence in the pre-policy medical checkup and annual health checkup market.

On the B2G side, we continue to execute TB projects in the state of Gujarat and Maharashtra. In Tanzania, since going live in March 2024 and processing our first sample in April, we have successfully partnered with over 150 healthcare facilities in Dar es Salaam. Our mission is to continuously collaborate with major hospitals, ensuring they have access to comprehensive diagnostic services. With our state-of-the-art laboratory equipped with world-class machines and infrastructure, we are poised to make a significant impact on healthcare in that region.

With that, I will now hand over to my colleague, Nitin, to cover the highlights for the quarter and annual business performance.

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Nitin Chugh: Thank you, Rahul. A warm welcome to everyone. First, I would like to start with our pillars of growth, which have been contributing strongly to our consistent performance. The first is customer success. The focus is to ensure accurately, timely and affordable diagnostic services through quality control, robust data management and customer support. With advanced technology and streamlined processes, we aim to enhance customer satisfaction.

Launching live reports for our franchisee base as well as our B2C customers is a testament of our continuous work towards customer success. We've also added things like reminder service, turnaround time visibility, etcetera, to our franchise partners to help them build a stronger

business for themselves.

The second pillar is network expansion. We are deepening our presence across India by going deep into the country with our franchise network and through our acquisitions as well. We have moved to a transacting base of franchise of upwards of 11,000, and at the same time last year we closed this number at 9,400. Also on the partnership side, we are expanding our footprint going deep into the pre-policy medical checkup and annual health checkup in the insurance business.

The third area where we are now focusing is to enhance our menu expansion and visibility. We are introducing a wider range of specialized tests and health packages for our partners and using targeted marketing to drive adoption and increase value for both our partners and our customers. We have launched histopathology in-house, FMS certified markers and many other new tests and profiles to our menu.

Now I will briefly update you about the Q4 and annual business performance of FY '25. Overall, at a consolidated level, this year we did a 20% year-on-year revenue growth and this quarter we delivered 21% year-on-year revenue growth, primarily driven by our Pathology business. This includes 2% of revenue contribution coming from the inorganic growth contributed by Polo, Vimta and Think Health.

Our franchise business for the year showed a revenue growth of 18% year-on-year and in Q4 our franchise business showed

a revenue growth of 22% year-on-year. We have started focusing towards opening a smaller lab in partnerships along with the franchisee and storefronts, which shall lead to higher processing capabilities and ultimately leading to a higher franchise business growth in coming quarters.

Further, revenue per retained franchise has been consistently growing for the franchise added post FY '22. This has been possible because of our slab-based pricing model, improved quality, strengthening our relationship with doctors and channel partners and our test menu expansion. Our partnership business for the year grew by 27% year-on-year, whereas in this quarter, it showed a tremendous growth of 24%. If we exclude API, this year, our partnership business grew by 36% and this quarter, it grew by 40% year-on-year. Our API Pharmacy Diagnostics business, this grew by 11%. Radiology business including Pulse Hitech did a strong revenue growth of 14% year-on-year this year.

With that, I will hand over to my colleague Alok to cover the financial results.

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Alok Jagnani: Thank you, Nitin. A warm welcome to everyone joining us today. I want to begin by highlighting that while the pathology diagnostic industry is growing at an early to mid-teen rate, Thyrocare has consistently delivering mid-teen to high-teen growth over recent quarters. This sustained outperformance underscored the strength of our leadership and our ability to seize the market opportunity.

Before moving to the financials, let me briefly revisit the ESOP program, which has been a key focus in recent quarters. As the Thyrocare ESOP program concludes this year, we have expanded the API ESOP tool to include our senior management at Thyrocare. This group level initiative aimed to retain the critical talent with ESOP issued by our parent company and vesting as per policy.

From the accounting perspective, these ESOPs are recognized as an expense in the profit and loss account and as an equity contribution from the parents in the balance sheet. It is important to note that this is a non-cash charge and does not affect our cash outflow. To provide greater clarity, we report normalized EBITDA excluding these non-cash expenses and also included an annexure in our quarterly earnings presentation to further explain the accounting statement.

Now moving to the financial update. Firstly, full year performance update. For the year, stand-alone revenue reached INR633 crores and the consolidated revenue stood at INR687 crores, reflecting a robust 20% year-on-year growth. This was driven by 18% increase in franchisee revenue and a 27% rise in our partnership business. Total Pathology revenue has grown by 21%, while Radiology business has increased by 14% year-on-year.

Coming to the quarterly performance, Q4 performance, revenue increased by 21% year-on-year supported by 22% increase in franchisee business, 24% increase in partnership revenue. Pathology revenue in Q4 grew by 23% year-on-year, whereas Radiology revenue grew by 17% year-on-year. On the total revenue growth, 2% was attributable to inorganic growth.

Our Think Health business, Polo, has stabilized and now fully integrated to the Thyrocare ecosystem. The acquisition of Vimta is currently in transitions and expected to completely integrate to Thyrocare system in next 3 months' time.

Stand-alone gross margin for the quarter stood at 73%, up by 426 basis points, primarily due to the better negotiation and favorable product mix. Employee expenses increased year-on-year, reflecting because of annual

increment, volume growth and new business acquisition and expansion.

The stand-alone normalized EBITDA margin for the quarter was 36%, an increase of 1,026 basis points. The improvement was driven by in gross margin and the operating leverage. High EBITDA margin in Radiology NHL compared to the last year d

ue to the revenue growth and

mix changes. NHL revenue mix moved to our FDG, which is normally called isotope sales, which generated higher EBITDA margins.

At the consolidated level, Q4 gross margin stood at 74% and the normalized EBITDA margin was 35%, whereas full year FY '25 gross margin 72% and the normalized EBITDA margin stood at 31%. In absolute terms, FY '25 normalized EBITDA reported is INR210 crores and the PAT

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excluding exceptional deferred tax provision reversal was INR101 crores, representing our year - on-year increase of 37% and 45%, respectively.

This is the first time wherein both normalized EBITDA and PAT crossed the benchmark of INR200 crores and INR100 crores, respectively. This strong performance was achieved despite margin pressures from recent acquisition and geographic expansions what we have done in the financial year FY '25.

In addition to that, we want to update that the Board of Directors has recommended a final dividend of INR21 per equity share for the year ended 31st March 2025, subject to the approval of shareholders in AGM.

With that, I will now hand over to Rahul for a strategic update.

Rahul Guha: Thank you, Alok. Briefly, I would like to take a few minutes to recap to you our strategic direction and then I will open it up for Q&A. First, I will reiterate our value proposition to the customer. We will continue to remain an affordable option to all patients with good quality and on-time reports.

All our efforts on our value proposition is towards ensuring low cost to the patient, assurance on quality of testing through our certifications and engagement with doctors. We have made substantial progress on this, which I updated in my initial comments and is reflected in the presentation. This will remain at our core and will guide all that we will do.

Second, our strategy. We continue to maintain our strategy of being the B2B partner of choice to all front -end diagnostic services companies in India, whether it's a small diagnostic center in a rural area, a pharmacy in a metro, a small nursing home, an individual doctor, or a leading online diagnostics platform or health tech marketplace. We are happy to work with them to provide low cost, robust testing solutions so that they can serve their patients in the most effective manner.

If they require phlebotomy, we are happy to mobilize our phlebotomy network of almost 1,900 phlebotomists, including our network partners to serve them better. We remain dedicated to expanding our business. And with the acquisition of Polo Labs and Vimta Clinical Diagnostics, we plan to significantly increase our presence in North and South India, respectively.

Additionally, to further boost our partnerships business, the acquisition of Think Health allows us to offer ECG at home services, further enhancing our value to our insurance partners. This strategy has been working well for us with both our franchise and partnerships businesses posting strong growth.

That in a brief is our mandate as management. Thank you so much for giving us a patient hearing. I will once again end with a quote from the Mahatma. "Find purpose, the means will follow."

And our purpose remains to provide affordable, high-quality testing to the masses.

With that, we will open up for Q&A.

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Moderator: Thank you very much. We'll take our first question from the line of Raman Kv from Sequent Investments.

Raman Kv: Sir, I just have 2 to 3 questions. So one is, in this quarter, we paid an excess tax of INR11 crores, like the 54% is the tax rate. Can you give me a clarification on why did we pay the excess tax in this quarter?

Rahul Guha: Yes. I'll defer this question to Alok, who can explain it. Alok, the question is, you have paid almost 54% in tax. What is...

Alok Jagnani: So thanks

, Raman, for these questions. And this is not the excess tax what we have paid in past.

This is a deferred tax provision what we have made in 2019, 2020 when the investment of NHL

impairment has been done. And at that point of time, the management was on the belief and that -- and if -- every year when we are reviewing the NHL investments and impairment, we are carrying that provision on account that the impairment is going to be actual.

And whenever we are -- if we are going to sell that investment, the impairment loss -- against the impairment loss, the deferment tax benefits are going to come. But now NHL business is doing good. If the business come on track, generating positive EBITDA and the revenue growth is also 10% plus revenue growth year-on-year, they are delivering.

Considering that provision for deferred tax carrying is not wise decision and management has taken that call that the deferred tax assets provisions taken against the impairment of asset investments, what we have done is not supposed to be reversed and we have taken that call and that hit has come in the P&L as an exception item, deferred tax reversal INR11.2 crores. If we exclude that in our P&L performance, PAT has been grown by 88%.

Raman Kv: Now I understood it. And my doubt was with respect to the margins. This is probably in the past 2, 3 years, this quarter has recorded highest level margins, EBITDA margin. So what was the primary reason for the expansion of the EBITDA margin?

Rahul Guha: I'll take that, Alok. So if I break up the overall year margin, we are at roughly about 32% in our Pathology business. And of course, for this quarter, it is 35%. What happens is you would see a gross margin uplift in this quarter. That is because of our increased volumes this year, we were given quite a few year-end discounts from our vendors, right? So that is what you see in the gross margin uplift.

And the other is, of course, we are getting operating leverage. If you just go with 20% growth, our expenses don't grow at that rate. So about 2% we have got from operating leverage. And the remaining is -- actually last year, we had a significant provisioning for receivables, which is not there this year.

So if I draw a bridge from the 25% odd to 35% or the 10% EBITDA improvement, 4% is from gross margin, largely because of year end credit notes for increased volumes, 2% is operating leverage and 4% is because of reduced provisioning of doubtful debts because we've been quite diligent in ensuring we collect our money and our outstanding is within range.

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Raman Kv: And my last question is with respect to the FY '26 guidance. So can you give us some guidance with respect to revenue growth and volume growth?

Rahul Guha: See, we normally don't give guidance, but I can give you a broad range. See, at 20% growth, we are already sitting on a very high base with the industry growing only at 10%, 11%. You must recognize growing at 20% means we have taken a substantial share from the unorganized players as well as a little bit from the organized players. They will come back with price reductions and all of that.

So I would be cautious and say that mid-teens growth is what I have always been guiding towards. We have, of course, over delivered in this year. But you must recognize it's on a very high base. So I think we'll be confident in growth continuing into the next year. And I think if we can keep the full year margin at this level, we will be happy.

Of course, we may get some benefit of increased volumes and additional, let's say, discounts when we go into the next year, but it's difficult to commit at this point in time. So I would say, while not a specific guidance, I would say, we are would be happy with mid-teens growth with stable margins where they are.

Raman Kv: Mid-teens volume growth or revenue growth?

Rahul Guha: Revenue

growth. Actually, our -- see, the -- while it may appear that a lot of our growth has come from price because the volume growth is about 14%, while the revenue growth is about 20%. But actually, it's not because we have raised prices. It is just our tiered pricing structure. It's a bit complicated to explain in a call, but we have a slab wise pricing structure. So as people move into different slabs and whoever joins us at a low -- at a high slab, they typically see a higher price and only when they grow their volumes does their price come down.

So you're seeing a little bit of that effect. It's not like we raised the price -- base price itself. If you look at our thyroid price, it hasn't changed in the last 10 years. So I would say you should expect volume growth and price growth more or less to be at similar levels. We don't anticipate any material increase in price next year.

Raman Kv: Okay. Thank you so much.

Moderator: Thank you. We'll take our next question from the line of Prakash Kapadia from Spark PMS.

Please go ahead.

Prakash Kapadia: So 2, 3 questions from my end. So one is you did explain, Rahul, about the pricing and margin drivers which have happened this year. So if I were to further break it down, pathology segment, is 90% contribution, saw revenue growth of 22%. Even on a sequential basis, we've seen a huge

margin expansion. Year-on-year, obviously, it is very large. Even on a sequential basis, it is up from 32% to I think 36.7%. And in the franchisee network, we have seen a 22% revenue growth. So what leads to this? Is it high value test? Is it mix change? If you could explain that a bit, that will help up the revenue on the margin side.

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Yes, I will repeat my question. So Rahul, I was trying to understand the revenue and margin piece a bit better. So, the question was related to the both the segments. If I were to look at franchisee network, there has been a huge realization change. So is it high end test? Is it more package test? What has happened on the franchisee network? If you could comment on that? And pathology segment, we've seen a huge margin change on a sequential basis also from 32% to 36.7%. So what kind of a margin and a mix would you expect next year as we move forward? That's the question on the margin and the revenue side. Secondly, from a competitive landscape, have things been better off, which is leading to some of this margin expansion? I did hear you are talking about operating leverage because of higher growth. So any sense on the ground competitive intensity? Lastly, any update on the PharmEasy merger, which media was speculating few months ago?

Rahul Guha: Sure. So let me take the questions one by one. I'll just give you a little bit of a highlight and then I'll ask Nitin to cover little bit more in detail.

Prakash Kapadia: Sure.

Rahul Guha: As I said, one is you must recognize the base of franchisees. We have added a quite substantial over the last 3 years. That is one which is the revenue uplift. Now when you come to the realization, it's largely actually coming from one the mix of the partner. So what happens is if you are a small partner, you get a higher rate and if you are a large partner, you get a lower rate. And we've been adding more and more smaller partners that actually improves the realization. But more importantly, actually, we are really worked on enhancing the mix. So and Nitin will talk a little bit more in detail. But thyroid used to be our dominant test. And we were mostly a routine player, but we have launched technologies like coagulation, histopathology and many, many more technologies into our network. We used to have a test menu of 300 tests. Today we have a test menu of close to 1,000 tests. So that is also helping on the realization.

Prakash Kapadia: It's a combination Rahul, it's a combination of specialized test co

tribution increasing and the mix from the franchisee which is leading to this higher realization.

Rahul Guha: Exactly.

Prakash Kapadia: And if you could share a number on an annual basis, what would be specialized as contribution versus routine? That would help?

Nitin Chugh: See, generally, we qualify -- we generally categorize our tests as basically preventive full body Aarogyam versus non-Aarogyam. Our Aarogyam revenue share is almost 35% on. They used to be almost 40%. So while Aarogyam business has individually also grown upwards of 20%, our contribution from our non Aarogyam which includes a single test, some profile and specialized test has continuously been increasing and thus giving us some leverage on the margin front.

Rahul Guha: Okay. Then coming to your second question on the margin improvement, both -- and I'll ask Alok also to chip in a little bit. But on a sequential basis, if you see the gross margin has improved

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by about 1%. We were anyway last quarter at about 73% GM, we are at 74%. So 1% is what I said the year-end volume discounts that we've been able to realize from our partners. But the other is our employee cost is on the lower side this year, this quarter relative to the last quarter and we are getting the benefit of operating leverage on our other expenses.

Quarter-on-quarter, our revenue grew 13%, but our other expenses grew only 6%. So that's broadly what explains the improvement in margin from Q3 to Q4, which is what you're saying.

Alok Jagnani: Many people have [inaudible 35:46] provision for receivable has also reduced.

Prakash Kapadia: You said directionally, if we maintain this 33%, 34% on an annual basis EBITDA margin, we should be happy from a next year perspective?

Rahul Guha: No, I was saying 31% if I can manage on an annualized basis FY '26. This is a very seasonal business Prakash. So Q3 is bad, Q1 is bad.

Prakash Kapadia: Yes, Q4 is heavy.

Rahul Guha: So, I don't want to build expectations on -- yes, while in the second half of the year, we have been quite healthy on the margin. We are also investing in specialized tests, newer labs, all of that. So and as I have always been saying, any operating leverage I get, I invest back in growth.

Prakash Kapadia: Understood. And if you could, Rahul, comment on that PharmEasy thing, it'll be helpful. Any progress, any update or status quo?

Rahul Guha: I don't think -- see, these were just media speculation. As I said, there was no discussion in the Thyrocare Board. I don't think there has been any material discussion on this in the PharmEasy side as well. I think we can leave it as media speculation. If there was any real need to it, I think it has been now 4 months, 5 months since that something would have happened by now.

Prakash Kapadia: Right. Understood. I will join back the queue if I have more questions. Thank you. All the best.

Moderator: Thank you. We'll take our next question from the line of Chirag from Keynote Capital. Please go ahead.

Chirag: Yes. Thank you for the opportunity. So first of all, I would like to know that this time the sample data was not mentioned in investor presentation. Could you give me the annual number of samples?

Rahul Guha: The quarterly number of samples is about 66 lakhs. So it's a year-on-year growth of about 15%, which is in line with patient growth. The total annual volume, I mean, it will be about INR66 lakhs into 4 roughly, so 240. The number is 25.3 million.

Chirag: Sir, my second question is when I was trying to understand that as you have mentioned that the reason for growth has to do with addition of more smaller partners, do you mean that -- from this do you mean that the franchise partner you are talking about?

Rahul Guha: Yes.

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Chirag: So the number of franchises

that we have at almost 16 percentage, it is clearly visible that, that is driving the volume. I wanted to understand the second part of the business, which is the partnership business, which includes the online platform or the PPP, Public Private Partnership. That growth has also had been significant. Could you give some color what was the reason behind that growth in this part of the business?

Rahul Guha: Sure. I'll again give you just the highlight and then let's actually health tech is a significant part of this business. But also now, our penetration into corporate and insurance is actually doing very well for us. Nitin, you can add a little bit.

Nitin Chugh: Yes. Like, Rahul said, without actually naming the partners, health tech has been the most significant in terms of revenue for us.

Moderator: I'm sorry, sir. Sorry to interrupt. Sir, you're sounding a little distant. Can you come closer to the microphone, please?

Nitin Chugh: Okay. So, see, if I -- yes, is it better or no?

Moderator: Yes, sir. Please go ahead.

Nitin Chugh: Okay. So see, health -- so we generally categorize our businesses like health tech, corporate business, insurance and there is another section which is our online campaigners and other partners. All four lines have actually grown upwards of 25% almost. With health tech and corporate leading the charge. So these are all either on APIs or using some sort of platform with us. And they can be from any segment be it aggregate -- health aggregators or online platforms like PharmEasy and others.

It can be your all your insurance players which can include annual health checkups as well as PPMC businesses and many other such partners whosoever has a another front end or a corporate directly diagnostic business and we just plug in our services from the back end.

Chirag: Okay. Fair enough, sir. Sir, as you just mentioned that, that you'd like to give the revenue side ...

Moderator: I'm sorry to interrupt, Chirag. Can you use your handset mode, please?

Chirag: Am I audible now?

Moderator: Yes. Please go ahead.

Chirag: Sir, as you mentioned that you'd like to give the bifurcation of revenue based on Aarogyam and non-Aarogyam. Is it possible for you to give the test menu also, which is currently 1,000? Could you diversify into both what number of test menus are in Aarogyam and what is in non-Aarogyam?

Rahul Guha: No, it's actually not the right way to classify. Aarogyam is a combination of many tests and then we sell the test individually. So, there is actually a fair bit of overlap of what is in an Aarogyam and what is in non-Aarogyam. It's just that when someone books an Aarogyam package, it is Thyrocare Technologies Limited

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booked as a package. Whereas you may book a CBC HbA1c or individual test and then that gets categorized as non-Aarogyam.

So there's not that much difference in what is in an Aarogyam versus what is in non - Aarogyam. So difficult to give you that breakdown, but our Aarogyam is, I think roughly 35% of our business today. Nitin Chugh: Which is basically, somebody booking a full body check up...

Chirag: If I'm correct, this Aarogyam is wellness - - Preventive wellness, right?

Nitin Chugh: Yes. You can classify Aarogyam as wellness preventive full body checkup. But see, there are lot of patients who know they are suffering from two, three different things and thus they feel instead of spending an x amount, I can just spend maybe 20%, 30%, 40% higher and get a full body done, like an annual thing. So Aarogyam is, yes, wellness. Non - Aarogyam is generally very specific.

Chirag: Fair enough. Sir, my next question is related to the dividend that we have distributed. Almost entire -- the profit that we have distributed as dividend. Is there any requirement of capex or shall I expect that dividend policy is around -- is going to be around 100% of the profit?

Ra

Rahul Guha: I'll ask Alok to give in detail, but I think from an operating cash flow generation, we are actually in a very comfortable place where we are able to cover. You have to remember, we are a zero debt company. We have significant cash returns. In FY '25, we would have generated operating cash flow of INR191 crores which is more than sufficient our capex is roughly about INR40 crores, INR50 crores a year, not more than that.

So -- and then the remaining whatever we keep a little bit of reserves, but the rest we distribute as dividend. Whenever we have a material acquisition, then we evaluate it. But we have chosen the path of a more string of pearls acquisition rather than one big acquisition. So we are typically cherry picking smaller acquisitions in territories where we are weak. And so the need to generate a lot of cash or keep aside a lot of cash for acquisition is not so much.

Alok Jagnani: Just adding down here. So we are keeping around INR80 plus crores of cash reserves in for the next financial year in the contingency. And continuing with the growth trends and all, we are expecting that the same level of cash flows are going to be generated and going to meet the requirements and also we are going to be available ready for the capex investment, some acquisition plans, what we are going to do in the coming financial year, we are able to do with the cash flow generation and the results what we are going to carry, plus the dividends what we are offering is going to continue.

Rahul Guha: Yes. And I must also caution on the acquisition side. We have found when we are doing these acquisitions, it takes up a lot of management bandwidth regardless of the size of the acquisition, whether you acquire a small company or a big company, the amount of time and effort of the management is quite substantial, right? So we are being very choosy about deploying our capital into acquisition.

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Our organic business is doing very well with 22% growth as we have seen in the presentation, inorganic added only 2%. So, I think we will keep the focus on franchise addition, partner addition and be very selective about acquisitions. And as Alok said, even with the dividend that the Board has recommended, we still have sufficient cash reserves after our depreciation, everything to manage them.

Chirag: Fair enough, sir. Sir, could you give some color on what kind of franchisee growth are you expecting in the next few years' time span as you specify that focus would be on adding number of franchisee and partners? So I'm not focusing more about the number of sales figures, but if you have some kind of criteria, self-set goals, how much growth you would take over there? Just a ballpark number.

Rahul Guha: Actually, I'll ask Nitin to take it.

Nitin Chugh: Yes.

Rahul Guha: We have our own target. So, we can share them.

Nitin Chugh: Yes. So, see, we are planning to add upwards of 1,500 new franchise partners to our ecosystem. That's the range that we are looking forward with some investment this year that we are doing in focusing more and more on network expansion. Right? There is no specific criteria as such.

Obviously, there is a certain benchmark that we feel that this much should be the business to get them into the ecosystem. Or if it is very small, how can we add the -- that franchise through our another super franchise which are basically a large franchise.

So it's a mix of both. But yes, generally wherever we see business, we are just going and meeting them, calling them etcetera and giving our value propositions to convert them, bring them to Thyrocare.

Chirag: Fair enough. Sir, any color on the extension of test menu that you have under which is currently 1,000, any guidance that you're going to make it 2,000 in the coming 5 year time span? Any color on that?

Rahul Guha: Let me take that. See, actually

a better start is to look at it by technology rather than by test. I think now we are present in almost every single technology relative to our peers, except allergy and genomics, which are two areas that we are exploring for this year. Then once you have the technology, actually once you see the demand, it's very easy to, let's say, go from 1,000 to 1,500, right? And expand it, but it really you need to be able to be confident that you are going to see the demand.

But there's actually very little incremental cost for us to go from 1,000 to 1,500. But technologies that we are going to invest in are allergy and genomics in this year.

Chirag: Fair enough. And sir, the last question from my side. I wanted to understand the reason for company to issue the parent companies as ESOP and not Thyrocare. Any particular reason for this? And what kind of revenue contribution does Thyrocare has in its parents overall revenue?

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Rahul Guha: So, API accounts for about 10%, 11% of Thyrocare's revenue. We are the exclusive partner to PharmEasy Labs. So, all tests that are done in PharmEasy Labs are processed at Thyrocare. So to that extent, the exposure of Thyrocare is about 10%. The group overall has a standard ESOP policy for all group employees. And so that -- those ESOPs are given to all employees, many of us in this room as well. In a way, it is because the employees are incentivized overall at a group level and to maintain parity across the different group entity. I think that is why it's done. And also, I think we didn't want to really dilute the ESOPs of -- sorry dilute the equity of the Thyrocare shareholder. Right? By then giving ESOPs at Thyrocare, the ESOP tool was created in API that was available for all group entities, not just Thyrocare. And so in that way, the API group has been in a way generous to incentivize the management teams across all group entities with ESOPs as a group entity rather than diluting in the subsidiary entity.

Moderator: We'll take our next question from the line of Yogesh Soni from InCred Equity.

Yogesh Soni: Congratulations on the good set of performance. My first question is a follow-up on the previous participant's question on acquisition. So sir, have we set aside any amount for acquisitions for FY '26? And another question is on the Nueclear performance. So given the kind of 20% margin that Nueclear has done in Q4, how do we see the margin for FY '26? Do we expect the margin performance to sustain? That's it from my side.

Rahul Guha: Let me take the first question. Have we set aside funds for acquisition? Not a large amount, I would say. We are -- as I have said earlier, we had very focused geographies, Gujarat, Rajasthan, the Northeast, where we are looking where our presence is weak for acquisitions. I think we should be able to cover those geographies in the range of about INR15 crores to INR20 crores.

So I don't anticipate more than that being deployed in acquisitions in this financial year.

Coming to your question on Nueclear, I think Nueclear has reached a sustainable level from a gross margin point of view. We've taken substantial price movement in Nueclear. And I'm hopeful it will sustain, but I think let's wait and watch for 1 or 2 more quarters before we can say that this is the new base level for Nueclear.

Yogesh Soni: Sir, just one more question. I missed out on the Aarogyam contribution. If you could help me with Aarogyam contribution in Q4 and for the whole FY '25

Rahul Guha: Nitin, you want to take that?

Nitin Chugh: See, Aarogyam contribution is fairly similarly distributed across quarters, especially Q4, because -- so it is 35% like I said in my previous answer as well. But it's generally the same, not only in Q4 it increases by 1% or 2% on the back of a lot of full body testing that happens in Q4.

Rahul Guha: And I'll just take this opportunity

to talk about Jaanch. Jaanch has actually grown 25% year-on-year and is doing very well in our portfolio as well.

Nitin Chugh: Just on a lighter note, if you're there, we miss your colleague Aditya Khemka ...

Moderator: We'll take our next question from the line of Palak Shah from Entrust Family Office.

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Palak Shah: Nitin, you mentioned that the value volume gap that you're seeing today is predominantly because you have added a lot of new franchisees, which should be the bottom grade of the franchisee slabs, right? But as these guys grow in size, does that effectively say that the value volume gap can actually go negative given that this year was a massive addition from 9,400 to 11,100 in a single year. For subsequent years you can have that reversal happening. Is it a possibility?

Rahul Guha: Yes. I understood your question -- what we are saying is you are adding smaller franchisees, so they come at bronze category, right? And as they move up to platinum, their price realization will go down. So this positive gap that you are seeing, will it become negative as they mature?

Nitin Chugh: So the -- from our retention cohort also, if we see, yes, even if we add smaller franchisees, right, they generally end up doing their 2.5x revenue in the second year when they join us and then a 3.5x revenue to their first year revenue. So yes, they keep growing and the price realization becomes a little lesser.

But then see, there is almost more than 150,000 collection centers of sorts which are like endpoints where we can get business from. And if we are today sitting at only 11,000, we see there is a large headroom still available to get more and more new franchisees added. And with our current run rate of almost 1,500 to 2,000 odd franchisees, that's our target range if we can add. I think we are still good for next 3 years to 5 years.

Rahul Guha: Yes. So -- or to put it differently, if we were to stop franchise addition, then yes, your cases would be true. But we have no plans to stop franchise addition.

Palak Shah: Got it. And secondly, on the imaging services, imaging services profitability, what drove this delta? Because the sudden sharp delta from last quarter loss of 2 quarters to -- almost a 2-quarter profit at PBT level this quarter. So any specific interventions that we have done to change this and how sustainable are they?

Rahul Guha: Yes. As I told you, if I do the bridge year-on-year, 4% has come from gross margin, 2% has come from operating leverage, 4% has come from provision of doubtful debt. If you look at it at a sequential level, the overall leverage has been again about, I think, 4%, 5%. That has mostly come from operating leverage.

Q3 is always a bad quarter for this industry, but your costs are the same, right, because we have our manpower costs and our operating expenses, which are by and large fixed. So as you go into Q4, which is the growth quarter in this industry, and particularly for us, you get the operating leverage benefit.

So to your point on sustainability, as I said, I think you should take the full year EBITDA as the base, which is about 31% normalized before ease of cost, and we are targeting to maintain in that range.

Palak Shah: Sorry, my question was regarding the imaging services PBT profit that you have reported this quarter, not the consol number?

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Rahul Guha: Okay. Nuclear, I maybe let Alok take that. But I do believe that in Nuclear, the revenue is kind of stabilized where it is. Cost, the main intervention to your question is we have taken price up. And that is slowing down to the bottom-line.

Alok Jagnani: So we are expecting that next year also top-line and the cost lines are going to be remain in the same trend. And marginal growth is going to be there. Up in single digit which

at I am expecting.

Some price increase and other things which we are going to happen, then we are able to get the benefit of that.

Other 2 new locations for which we are exploring and if that is going to be added of the existing machine, what we have, then that's going to be add on to the -- add up on to the top-line and deliver [inaudible 57:43].

Palak Shah: So effectively, we are looking to invest more capital on the Nuclear side, right, going forward?

Rahul Guha: No. We will maintain the centers that are there. We will continue to optimize the cost. But you must remember to set up a new center is a INR10 crore investment. At this point, we are not contemplating filling up new centers and Nuclear.

Palak Shah: Great, thank you so much for your answer. And congratulations on a very good set of numbers. All the best.

Moderator: Thank you. We'll take our next question from the line of Sumit Sarda from Compound Everyday Capital. Please go ahead.

Sumit Sarda: Just two questions. One, receivables are up from around INR40 crores, INR50 crores sequentially or year-on-year to INR73 crores. So any color on those? And secondly, any update on Tanzania about revenue margins and any plans for growth?

Rahul Guha: Sure. I'll let Alok explain the receivables point. Alok you can...

Alok Jagnani: So on the receivable, if you see that there is an increase of around INR30 crores has come. So, the -- it has 2 part. One is the business has grown, partnership business where the credit we normally offer. If the business will grow at a rate of 35% to 40% partnership business, then the data on account of that is also going to happen and around INR12 crores to INR15 crores of data have gone on that term.

Similarly, like API is a similar tech partner. [inaudible 59:28] is like a Fintech partner what other partners are. And earlier we are offering lesser credit period to them. But as a competition arm length price assessment system that we have to offer the same credit period what we are offering to our other tech partners and the partnership business -- partners.

The thing that the 2 months additional credit period has been offered. Earlier it was 1 month.

Now it is increased to 3 months, that's what we are offering to other partners also, 2 to 3 months. Resulting around INR16 crore s to INR 18 crore s of top-line data's increase. With stuff together, it's a INR 30 crore s latest moment has happened.

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Rahul Guha: And on Tanzania, to be honest, it's still very, it's been less than a year since we actually started to operate the lab. But I'll give -- ask Nitin to give a little bit of flavor of what's going on there.

Nitin Chugh: Yes. See, Tanzania like, Rahul said is in its very nascent stages. Right? Because even when the lab got started, then we went out of the market for the first time explaining what Thyrocare is, going and reaching our clients with our pricing et cetera. And then people slowly started testing us with 1 or 2 samples here and there to foresee our serviceability et c.

So, if you see Q2 versus Q3 versus Q4 in this financial year, it's like we have grown more than 100% on in these quarters. So today we are already at a run rate of doing almost 60 lakhs a quarter, which is then scheduled to grow almost 100% in the next quarter. That's how we've been going.

But, yes, we are still cautious on which partners we are adding and how we are growing our business. But, yes, we are already now working with almost 150 partners there. And everyone is starting slow and then slowly moving their wallet share to us.

Rahul Guha: Yes. We've doubled every quarter. Last 3 quarters, we've doubled to reach 60 lakhs, but it's such a small number. Right now, it's not worth talking about.

Sumit Sarda: Thank you. That's all for my side, all the best.

Moderator: Thank you. We'll take our last question from the

line of Chirag from Keynote Capitals. Please go ahead.

Chirag: Yes, thank you for the follow-up. Sir, I wanted to know what will be the effective tax rate going forward?

Rahul Guha: I'll let Alok answer that. What will be the effective tax rate going forward?

Alok Jagnani: It's going to be around 28% to 29%.

Chirag: 20% to 29%?

Alok Jagnani: 28% to 29%.

Chirag: 28% to 29%. Okay. That is it from my side. Thank you.

Moderator: Thank you. Ladies and gentlemen, as there are no further questions, I now hand the conference over to Mr. Rahul Guha for closing comments. Over to you, sir.

Rahul Guha: Thank you, everyone, for joining us and spending time with us this evening. As always, we continue to remain focused on our strategy, which is to be the most affordable, good quality diagnostic testing partner for anyone in the healthcare business, and we continue to execute on that strategy.

We have been investing in improving our quality, improving our reach and ensuring our turnaround time is as close to best-in-class. We've made substantial progress on all of this, and

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that is what is driving the results that you see. I thank you all for your support in this journey, and I wish you all a good evening. Thank you.

Moderator: Thank you, members of the management team. On behalf of Thyrocare Technologies Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Note:

"Inaudible" refers to not audible or incomprehensible words.

Call: Jul 2025

July 28, 2025

To, National Stock Exchange of India Limited BSE Limited
Exchange Plaza Phiroze Jeejeeboy Towers Bandra Kurla Complex, Dalal Street,
Bandra (E), Mumbai - 400 051 Mumbai- 400 001
(SYMBOL: THYROCARE) (SCRIP CODE 539871)

Sub : Transcript of post results earnings conference call held on July 23, 2025, with
Analysts / Investors.

Ref : Disclosure under Regulation 30 of the Securities and Exchange Board of
India (Listing Obligations and Disclosure Requirements) Regulations, 2015
("Listing Regulations")

Dear Sir/Madam,

Pursuant to Regulation 30(6) read with Part A of Schedule III of the Listing Regulations,
please find enclosed herewith the transcript of the earnings conference call with Analysts and
Investors held on July 23, 2025.

The same has also been made available on the website of the Company
<https://investor.thyrocare.com/>

This is for your information and records.

For Thyrocare Technologies Limited

Brijesh Jha
Company Secretary and Compliance Officer
Encl: A/a

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"Thyrocare Technologies Limited Q1 FY'26 Earnings
Conference Call"

July 23, 2025

M
MANAGEMENT : MR. RAHUL GUHA - MANAGING DIRECTOR AND CHIEF
EXECUTIVE OFFICER , THYROCARE TECHNOLOGIES
LIMITED
MR. ALOK KUMAR JAGNANI - CHIEF FINANCIAL
OFFICER , THYROCARE TECHNOLOGIES LIMITED
MR. NITIN CHUGH - CHIEF COMMERCIAL OFFICER ,
THYROCARE TECHNOLOGIES LIMITED
MR. PREET JOSHI - STRATEGY AND INVESTOR
RELATIONS , THYROCARE TECHNOLOGIES LIMITED

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Moderator : Ladies and gentlemen, good day and welcome to the Thyrocare Technologies Limited Q1 FY '26
Earnings Conference Call.

As a reminder, all participants' lines will be in the listen-only mode and there will be an
opportunity for you to ask questions after the presentation concludes. Should you need assistance
during the conference call, please signal an operator by pressing '*' then '0' on your touchtone
phone.

I now hand the conference over to Mr. Preet Joshi from Thyrocare Technologies Limited. Thank
you and over to you, Mr. Preet Joshi.

Preet Joshi: Thank you, Zico, for the introduction. A very good evening to all and thank you for joining us today for the Thyrocare Earnings Conference Call for the Quarter 1 FY '26.

Today we have with us Mr. Rahul Guha – MD and CEO of Thyrocare, Mr. Alok Kumar Jagnani – CFO of Thyrocare, Mr. Nitin Chugh – Chief Commercial Officer along with the other key members, on this call to share the highlights of 'Business' and 'Financials' for the Quarter Results. I hope you have gone through the results, the quarterly Earnings Presentation and the Press Release, which has now been uploaded on the Stock Exchange Website. The transcript of this call will be available in a week's time on the Company's website.

Please note that today's discussion may be forward-looking in nature and must be viewed in relation to the risks pertaining to our business. After the end of this call, in case you have any further questions, please feel free to reach out to the investor relations team.

I would now like to hand over the call to Mr. Rahul Guha and make the opening remarks.

Rahul Guha: Thanks, Preet. Sorry, my throat is quite bad today. So, I hope I am audible. But good evening and welcome to all on the call. Thank you for taking out the time from your busy schedules to join us this evening.

Just a quick introduction to us on the call. My name is Rahul Guha, and I am the MD and CEO of Thyrocare and thank you for the opportunity to present the Q1 results for FY '26. I am joined with my colleague Alok Kumar Jagnani, who is our CFO, Nitin Chugh, who is our Chief Commercial Officer and Preet Joshi, who is part of our Strategy and Investor Relations team. In addition to that, I am pleased to welcome Mr. Vikram Gupta, who is joining us today and will be taking charge as Thyrocare CFO. Alok has been elevated to API Group CFO, but will continue to remain involved in Thyrocare as a Director on the Board. So, many congratulations to Vikram and of course, looking forward to continuing association with Alok.

As in all my calls, I will start with a quote from Nelson Mandela in recognition of our foray into Africa – "It is in your hands to make a better world for all who live in it". And we believe Thyrocare Technologies Limited

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Thyrocare can bring our business model to Africa to make affordable and good quality diagnostics available to all.

Before we get into the details of the quarter, it has been two years since we implemented the pay-for-performance structure, which has led to an increased energy within our franchisee network with motivation for them to move up volumes and enter higher slab. I am very happy to report that our franchisee base is at its highest ever with a steady growth in the franchisee base over the last two years. Encouraged by the success, we are now expanding our field and central teams to accelerate the franchisee addition journey and would be happy to update on the progress in the quarters to come. Quality remains our highest priority and we see it as a continuous journey of improvement and innovation.

We are immensely proud to share that last year we achieved the milestone of becoming India's first and only 100% NABL accredited national laboratory chain, a prestigious milestone that reflects our unwavering commitment

to world-class diagnostic services and a relentless focus on quality and excellence. Achieving NABL accreditation across all our labs has been made possible through the implementation of robust quality management systems, investment in cutting-edge technology and equipment, rigorous training programs for our staff and consistent participation and proficiency testing. Considering that only 2% of pathology labs nationwide hold this accreditation, this achievement is not just a significant milestone for us but also as a testament to our leadership in redefining diagnostic standards across India. But the quality journey doesn't stop there. We have now set a target of six sigma-level quality processes in Thyrocare and we track our complaints per million with a goal to bring it below 3.4 complaints per million. I am very happy to say we have already made tremendous progress on this dimension with our complaints per million at 4 versus 6.4 last year when we began tracking this and we are confident we will very soon reach 3.4 making us the first lab chain to achieve six sigma processing and we believe we will soon be best in class on this dimension not just in diagnostics but in healthcare.

To communicate our journey on quality we regularly host advisory board meetings with a panel of esteemed doctors to gain their insights on enhancing our quality milestone. In the current quarter, we have hosted three meetings where more than 125 doctors attended. Doctors witnessed our cutting-edge technologies and stringent protocols reinforcing our commitment to diagnostic excellence.

With Thyrocare Anusandhan, I am proud to share that we conducted and published India's largest HbA1c study providing deep insights into diabetic trends nationwide. More importantly, we launched our fever study which is our Jaanch fever panels and the timing is quite appropriate given the fever season is upon us. We found that one out of three fevers are not just simple upper respiratory tract infections as is commonly believed but are more complicated infections such as

dengue, typhoid, malaria and influenza underpinning the need to test and treat fevers judiciously. In the past quarters, we have added multiple advanced technologies including histopathology, a new equipment platform in HPLC and our first foray into Coagulation. We have also launched Thyrocare Technologies Limited
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BioFire, one of the most advanced PCR platforms. While maintaining the highest quality standards, we continue to improve our turnaround time. On an average during Q1 FY '26, we released the report within 3.35 hours of samples reaching the lab. This rapid turnaround is made possible by our robust operational processes, advanced automation systems and streamlined workflows. By combining precision with efficiency, we ensure timely and accurate diagnostics empowering patients and healthcare providers to make informed decisions swiftly.

I am proud to say we have now fully integrated the Polo and Vimta network and are live with ECG at home across our network for which we had acquired Think Health. Our lab network stands at 40 today across Thyrocare India, Tanzania, Po lo, Vimta and our partner labs. Beyond the work on quality and operations, we continue to selectively expand our offering. Aarogyam has been our flagship brand and we have two brands Jaanch and Her Check which were launched recently. Jaanch is targeted towards lifestyle challenges for you to better understand your health. We have solutions across the spectrum for anything you might be worried about. Is it fever or something more serious? Why is my hair falling? Cancer screening as well as deep investigations for common chronic diseases like diabetes, heart health amongst others. Jaanch has grown 17% year-on-year and is soon becoming a strong pillar of our lifestyle offer. We are very proud of some of the key milestones that we achieved in

Q1 '26. For Q1'26, we have more than 9,500 active quarterly franchises. As a result, we processed 46.9 million tests which grew by 15% year-on-year. We served 4.6 million patients in the year which increased by 12% year-on-year. Please note for reference, active franchisee count was 9,413 in Q4 FY '25 and 8,145 in Q1 FY '25 that is the same period last year.

We are regularly taking strategic initiatives to further expand our footprint. During Q1'26, we expanded our geographical footprint by opening new partner labs in Roorkee, Kashmir and started processing at a new regional lab in Bhagalpur. Partnerships business did phenomenally well as we on boarded new clients in health check segment and continued to grow our existing accounts.

In Tanzania, since going live in March 2024 and processing our first test in April, we have successfully partnered with over 192 healthcare facilities in Dar es Salaam till date. Our mission is to continuously collaborate with major hospitals ensuring they have access to comprehensive diagnostic services.

With that, I will now hand over to my colleague, Nitin, to cover the highlights for the quarter and annual business performance.

Nitin Chugh: Thank you, Rahul. A warm welcome to each of you. First, I would like to start with our pillars of growth which have been contributing strongly to our performance, our consistent performance.

The first pillar is "Customer Success":

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The focus is on ensuring accurate, timely and affordable diagnostic services through quality control, robust data management and effective customer support. With advanced technology and streamlined processes, we aim to enhance customer satisfaction. Launching live reports for our franchisee base and our D2C customers is a testament of our continuous work towards customer success. We have also added customer success features like turnaround time visibility, reminder service, microsites of our partners to help all our partners to grow their business.

The second pillar of network is "Network Expansion":

We are deepening our presence across India by going deep into the country with our franchise network and through our acquisitions as well.

Also, on the partnership side, we are expanding our footprint by going deep into the pre-policy medical checkup and annual health checkup in the insurance business. We now have more than 9,500 plus active quarterly transacting franchisees in Q1 of FY '26 versus 8,145 active franchisees in Q1 of last year FY '25.

The third pillar is "Menu Expansion":

The third pillar menu expansion where we are introducing a wider range of specialized tests and health packages for our partners using targeted marketing to drive adoption and increase value for both partners and customers. For example, launching coagulation, Hb1c graph reports in most of our lab locations. We also launched highly specialized BioFire panels in this quarter.

Now, I will briefly update you about the “Business Performance ” of Q1 of FY '26:

Overall, at a consolidated level, this quarter we did 23% year -on-year revenue growth primarily driven by our pathology business with a growth of 25% year-on-year. This includes 2% revenue contribution coming from inorganic growth contributed by Polo, Vimta, and Think Health . Our franchise business for the quarter showed a revenue growth of 20% year-on-year in Q1 of FY '26.

We have started focusing towards opening up smaller labs in partnerships along with franchises and other storefronts which will lead to higher processing capabilities and ultimately leading to a higher franchise business in coming quarters. Further, revenue for retained franchisees has been consistently growing for the franchisees added post FY '22. This has been possible because of our slab-based pricing model, improved quality, strengthening our relationship with doctors and channel partners, and t

est menu expansion.

Our partnerships business in the quarter showed a tremendous growth of 36%. If we exclude API, this quarter our partnership grew by 29% year-on-year. Our API Pharmacy Diagnostic business this year grew by 52% year-on-year. Radiology business including Pulse Hitech did a strong revenue growth of 6% year-on-year this quarter.

With that, I will hand over to my colleague , Alok , to cover the “Financial Results ”.

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Alok Kumar Jagnani: Thank you , Nitin and a warm welcome to everyone joining us today.

First to start, I am pleased to welcome Mr. Vikram Gupta who joins us today in Thyroc are as our new Chief Financial Officer.

Now moving to the Q1 FY '26 highlights :

The pathology diagnostic industry is currently growing at an early to mid-teen rate. In comparison, Thyroc are has consistently delivered mid -team to high -team growth over recent quarters, underscoring the strength of our leadership and our ability to capi talize on market opportunities.

Before going to the financials, let's first cover up the “ESOP Program Overview ”:

Before we discuss the financials, let me briefly touch upon the employee stock options plan. As the Thyroc are ESOP program concluded this year and with only a limited pool remaining for employees and senior management, our parent Company has decided to expand the API ESOP pool to include Thyroc are ESOP management. This group-level initiative is aimed to retain our most critical talent. ESOP will be issued by our parent Company and will vest in accordance with the established policy. From the accounting standpoint, these ESOPs are recognized as an expense in the profit account and as an equity contribution from the parent in the balance sheet. It is important to note that this is a non-cash charge and does not have any cash outflow. To enhance clarity, we report normalized EBITDA by excluding these non- cash charges and have included an annexure in our quarterly earnings presentation to further explain the accounting statement.

Now moving to Q1 FY '26 “Financial Performance”:

Standalone revenue for the quarter reached Rs. 179 crores and consolidated revenue stood at Rs. 193 crores, reflecting a year-on- year revenue growth of 23%. This growth was primarily driven by 20% increase in franchisee revenue, 36% rise in partnership revenue. Total standalone revenue of pathology grew by 25% year-on-year, whereas our radiology revenue has also gone up by 6% year-on -year. Of the total growth of 23%, organic growth is 21% and rest is attributable to inorganic growth.

Now with the recent acquisitions what we have done in FY '25, all are stable and fully integrated with the Thyro care ecosystems. Standalone gross margin 71% stood up by 48 basis points year-on-year, mainly due to the more favorable negotiation and product mix. Employee expenses up year-on-year, mainly on account of annual increments partially offset by actuarial gains.

Standalone normalized EBITDA margin 35% as improved by 354 basis points, largely benefiting from the better gross margins and operating leverage generated from the business.

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At the consolidated level, gross margin reported 71%, normalized EBITDA margin 33%. Q1 FY'26 normalized EBITDA in absolute value reported Rs. 63.35 crores, profit after tax reported Rs. 38.06 crores. Both normalized EBITDA and P AT are up by 42% and 62% year-on-year respectively. Notably, this performance was achieved despite margin pressures from our recent acquisit ions and geographic expansions.

With that, now I hand over to Rahul for the strategic updates.

Rahul Guha: Thank you, Alok and a warm welcome to you on the Thyro care board.

Alok Kumar Jagnani: Thank you, Rahul.

Rahul Guha: Briefly, I would like to take a few minutes to recap to you our strategic direction and then I will open it up for Q&A.

First, I will reiterate our value proposition to the customer. We will continue to remain an affordable option to all patients with good quality and on-time reports. All our efforts on our value proposition is towards ensuring low cost to the patient, assurance on quality of our testing through our certifications and processes and engagement with doctors. We have made substantial progress on this and I have updated this in my initial comments and is reflected in the presentation. This value proposition remains at our core and will continue to guide all that we do. Second, our strategy. We continue to maintain our strategy of being the B2B partner of choice to all front-end healthcare services companies in India, whether it is a small diagnostic center in a semi-urban area, a pharmacy in a metro, a small nursing home, an individual doctor or a leading online diagnostics platform or health tech marketplace. We are happy to work with them to provide low cost, robust testing solutions so that they can serve their patients in the most effective manner. If they require phlebotomy, we are happy to mobilize our phlebotomy network of almost 1,900 phlebotomists today to serve them better. We remain dedicated to expanding our business and with the acquisition of Polo Lab and Wimta Clinical Diagnostics, we plan to significantly increase our presence in North and South India respectively. Additionally, to further boost our partnership business, the acquisition of Think Health has allowed us to offer ECG at home services where we are now having a pan-India presence. This strategy has been working well for us with both our franchisees and partnerships businesses posting strong growth. That in a brief is our mandate as management.

Thank you for giving us a patient hearing. As always, I will once again end with a quote from the Mahatma. Find purpose, the means will follow and our purpose remains to provide affordable, high quality testing to the mass.

With that, I will open it up for Q&A.

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Moderator: Thank you very much. We will now begin the question and answer session. The first question is from the line of Prakash Kapadia from Kapadia Financial Services. Please go ahead.

Prakash Kapadia: Yes, thanks for the opportunity. Couple of questions from my end, Rahul. Q1 generally is a soft quarter for the industry in general. So, what are we doing right for the 23% growth which we've achieved? Any specific insights you could share which has led to this growth, be it the franchisee contribution in terms of gold, silver, larger, smaller, or any specific region which we were not there that has contributed? So, that will give some color on the revenue growth and sustainability for the remaining part of the year. I think from what I remember, employee count was around 1,700. What is it as of now? And franchisee addition, is it fair to assume from this base around 800-1,000 franchisees could be added during there? Those are my questions. Thank you.

Rahul Guha: Sure, Prakash. You're right. Q1 is typically softer than Q4 historically. This Q1 for us has been better than Q4, which is the first time, I think in a long time this has happened. And if you take out COVID, probably it is the first time ever. I think what is working for us, as Nitin also talked about in his commentary, is the number of franchisees that we've been able to add. Between last year and this year, I think the number is about 1,400-1,500 franchisees. And we are getting the compounding effect of the previous year's base also. So, if you have seen our disclosures, our annual disclosures, we showed how a franchisee added in '22, doubles in '23 and then continues to grow in '24. So, a large number of additions that have happened last year have actually given us a big boost this year. Plus, the overall franchisee count going up. It's not a

very complicated

business. You keep your customers happy and you add more customers. I think we are doing both quite well. And the results are showing up in the growth numbers. To your second question, the employee count was 1,700 last year. We were at about 1,900. So, not that much. And that is after integrating Polo, Vimta, Think Health, plus expanding three new labs over the years. So, I think it's reasonably under control. I think your last question was...

Prakash Kapadia: How many franchises will we add?

Rahul Guha: I think you can assume, we are typically operating at 100 a month. I think you should assume that that is the rate we will continue at. 1,200 to 1,500 is the net addition.

Prakash Kapadia: Over the next 3 quarters?

Rahul Guha: Yes.

Prakash Kapadia: And any color, Rahul, on any specific region or geography which has led to this growth or is it spread across the country from the revenue contribution?

Rahul Guha: It's plus minus a little bit, but I think the growth is secular across the country. We have not focused in any way in a particular region or something. We are a Pan-India player with deep presence across the country. I think some regions grow slightly faster, some regions grow slightly slower, but it's not like a big swing. So, I can say it's secular growth across the country.

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Prakash Kapadia: Okay. Thanks, sir. I will join back if I have more questions.

Moderator: Thank you. Our next question comes from the line of Raman K V with Sequent Investments. Please go ahead.

Raman KV: Sir, I just wanted to understand that during the quarter, revenue per test has increased as well as revenue per patient has increased, on a year -on-year basis. So, was there any price hike during the quarter?

Rahul Guha: So, I will let Nitin give you more details, but we typically don't take price hikes. Most of the revenue per test will largely come from mix, but I will let Nitin give you a bit more color of it.

Nitin Chugh: So, there was a very minor price hike but not in a routine test because there were a lot new tests that were launched. So, the price hike color is more or less in the range of 1.5% to 2% at max. The rest of it has majorly come from A, expanding the new cat alog and opening up newer, higher-priced items for all our partners, as well as working very, very well on mix change and helping our partners sell something which is a category above than what they usually sell. So, for example, instead of just thyroid profile, maybe sell somebody a Jaanch thyroid or just selling HbA1c, sell them a Jaanch Diabetes Basic, etc. So, a lot of work has gone on mix change, helping them understand and know what to sell next apart from what they're selling. So, that has also contributed to higher revenue per test.

Raman KV: Okay, sir. Sir, also one of the questions, basically it 's a follow -up on what the previous participants asked. Just wanted to understand how many franchise e are there with us and how many franchises did we add during the process? Can you give any figures?

Nitin Chugh: So, basically, like we said, last year, same quarter, we had about 8,100 active franchises in that quarter working with us. Now, that number is around 9,500 odd. So, there is a healthy addition, net addition of almost 1,400 odd franchises. And, sir, this year also, we are on track to, hopefully, we will be on track to do more than 1,200 odd franchise additions, 1,200 franchis es by the end of this year. That is our plan.

Raman KV: And my last question is with respect to the franchisee only. Sir, how long does it take for a franchisee to become a matured franchisee and contribut e significantly towards the topline and bottom line?

Nitin Chugh: See, we have a slightly different model also. So, we work on two models. Lesser on the fact that we have newer partners coming and opening up a franchise from s

cratch, which would require

at least 12 months for a franchisee to break even. So, our model is more focused on having partners on board who are already working in the diagnostic space, who already have a collection center of sorts, maybe say by the name of Nitin Diagnostic or an Alom Diagnostic, who is probably giving to multiple pa rtners today and distributing their business. Our business model is to add those guys and bring them into the Thyrocare ecosystem with higher wallet share, Thyrocare Technologies Limited

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hopefully converting them into a fully-fledged Thyrocare franchise, putting up a Thyrocare board. So, that is our go-to -market strategy with which I think we are able to add these many and not relying on finding 1,200 more new entrepreneurs to start a franchise from scratch.

Raman KV: So, this way, how much time it will take for them to break even?

Nitin Chugh: So, see, because more than 95% of our franchisees have existing centers, so they are already, more or less, they have established their businesses. It 's just about we try to get as much share as possible from their current business and take more wallet shares. So, most of these franchises have been well -established in, say, last 3 years, 5 years, even some are 20 years old, but very well-established busi nesses. So, hardly we see any churn because of business going down. Maybe some wallet share goes down here and there, but they are mostly profitable or breaking even.

Raman KV: Okay, sir. And, sir, are we sticking to the revenue guidance of mid-teens and volume guidance of mid- teens ?

Rahul Guha: Yes. See, it 's too early in the year to revise the guidance. As I said, while Q1 has been very encouraging and the growth has been very strong, if you look last year, H2 was very strong for us. So, I think it 's very early in the year to be revising guidance at this point. We will probably take a call after we see H1. But yes , the first quarter has been very promising. But as I said, the backup from last year was very good as well.

Raman KV: Okay, sir. Thank you.

Moderator: Thank you. The next question comes from the line of Lokesh Manik from Val lum Capital. Please go ahead.

Lokesh Manik : Hi. Good evening to the team. Rahul, my question was on the relationship with vendors in terms

of supply. Are we following a rental reagent model where it is pay as you go? Or do we take the equipment on our books in the pathology business and then we purchase the consumable? How is that supply model played out?

Rahul Guha: I will let the master of this take this question. Alok, you can.

Alok Kumar Jagnani: Thank you, Rahul. So, answering to you, we are mostly going to move in CAPEX model as we procure machine and capitalize in our books. We are not working in RR model. Earlier, prior to 2022-2023, we were mostly in RR model, which we have moved to CAPEX model.

Rahul Guha: And Lokesh, there's no free lunch. When someone gives you on reagent rental, they are typically loading their capital costs. We are a completely debt-free Company with a high operating cash flow. Our cost is significantly lower than the loading that they put on the rental model. So, we

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find it better to finance our own equipment than take financing at a very high rate in the reagent rental model.

Lokesh Manik: Understood. My second question was, a part of our strategy in 2022 was to leverage these synergies with the API group through Retailio and Aknamed. Now, the franchising growth that we are seeing, is it through the Retailio network, which has 2.8 lakh some pharmacies? Are we tapping that or this is independent of that, that we are expanding on the franchise?

Rahul Guha: No, this is completely independent of that. There is a small base of Retailio pharmacy in that transact with us. But we have not seen much success on that front.

This franchisee addition is

basically collection centers and pathology labs, which is independent of Retailio. However, I will take that opportunity to highlight one other point. A large part of synergies was the cross-

sell of diagnostics on the PharmEasy platform, which as you know, over the last few years has been muted. But now, I am very happy to say that the PharmEasy business has come back on the growth track. Last quarter, it was very strong. This quarter also, 30%-40% growth is starting to come back from the PharmEasy side of the business. So, that is very encouraging to see as well.

Lokesh Manik: Great. And one clarification from my end. In the annual report, it was mentioned that the central processing lab has a capacity of 1,30,000 samples per day. And this is for both the CPLs or this is only one CPL?

Rahul Guha: This would be across both CPLs. So, we have now one central processing lab in Mumbai and one in Delhi. This would be across both.

Lokesh Manik: Okay. And when you are introducing packages, new packages like Jaanch or Her Check or any more down the pipeline, what is the base volume that you look at that would allow you to introduce these packages or the potential volume in the market that you want to see before you introduce these packages?

Rahul Guha: See, for us, we are not launching any specific test, that is new to Jaanch. It is a consolidation of that already exists but are more targeted towards the diabetic or more targeted towards the thyroid patient. So, for us, we don't incur any incremental cost when we create Jaanch. Even if you are diabetic, people just wait till and do a full body checkup. But there are many subsequent consequences of uncontrolled diabetes that you should be monitoring that is not there, typically in a full body checkup, but is there as part of the Thyrocare menu. Right? So, we have kind of consolidated it a little bit to help a diabetic better manage his health, the thyroid patient better manage his health and so on. So, from that, we don't really look at it from a, how many test volume do I have to get? The way we work is the prevalence of the disease. So, there are many diabetics, there are many thyroid patients, and many heart patients who require specialized packages for themselves. And from that, we work backwards.

Lokesh Manik: Understood. That is it from my side. Thank you so much. I will come back in the queue.

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Moderator: Thank you. The next question comes from Raman KV from Sequent Investments. Please go ahead.

Raman KV: Hello, sir. Thank you for allowing me to ask one more question. Sir, I just wanted to understand the partnership or revenue business model. How does the revenue flow and how does the profit is accounted for? The entire business model I want to understand with respect to the partnership business.

Nitin Chugh: So, there are multiple aspects to the partnership model, what we include in our partnership business. One is the very straightforward direct to consumer business in this, which is basically somebody coming directly to us, Thyrocare website. There it is very simple, like any other D2C platform, we take the price, what we show to the customer, give some discount, etc. and that is what is accounted for. That is the highest profit margin business, because the consumer is directly coming to our platform. Then there are something called as these health checks and

corporate players, which almost contribute 50% of our partnership business. These health check players have generally a very customized design packages also for themselves while they have the normal menu in place. We generally have a transfer price set up with these people upfront, where they know if they have designed the package for a particular customer or a particular insurance Company or anyone in the corporate space, we have a particular

transfer price set, they sell it to their customer at whatever price, I will charge them on a transfer price set. It is the same model that how we work with PharmEasy, where there is a transfer price, they handle their customer acquisition, their marketing, what price. They decide what price they sell, what discount they give. End of the day, I just recognize my transfer price revenue. And this is majority of the model on which we work on our partnerships. With some smaller partners, we also have a mission led model, which are direct servicing agents, but again, that is a smaller part, but more or less it's on a fixed transfer price.

Raman KV: So, there are two, within the partnership model, there are two different models. One is your own side of a lab. Another one is where you tie up with a Company like, for example, you say PharmEasy, and for example, if a bundle A of test is offered at 5,000 in your lab, PharmEasy might price it at 4,500, you will recognize 5,000 as your revenue in your books. Am I right?

Rahul Guha: That is correct. So, if I give you an example, let's say there is a package on one of our partners for Rs. 2,000. The partner will collect Rs. 2,000 from the customer. Thyro care will invoice its cost, right? And there are two kinds of costs. One is the lab processing cost. And the other is the partner opts for home collection, and then we charge them for the phlebotomy cost as well as the transportation cost, right? So, roughly, if you look at a Rs. 2,000 package, from a lab processing point, we will get, I think about 700-800 and another 300-400 for collection and transportation, on which we make actually pretty similar EBITDA at a Company level. There's very little, I mean, the partnership comes marginally lower EBITDA than the overall Company EBITDA.

Raman KV: So, this is more or less like a B2B model, right?

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Rahul Guha: Very much. B2B2C.

Raman KV: B2B2C. So, with respect to that, can you give the split between B2C and the second one, B2B2C?

Rahul Guha: Can you give the what?

Raman KV: For example, with respect to the partnership model revenue, how much are you earning from, in terms of percentage, how much are you directly earning from B2C, wherein that customer directly comes to your own lab, versus how much you own from this B2B2C, business to business to consumer, wherein you are tied up with a business like a farming.

Rahul Guha: And as I said, the partnerships business comes at more or less the Company EBITDA. So, if our Company EBITDA at a normalized level is roughly between 30 and 35, depending on which quarter you look at, let's say this quarter is 35, then the partnerships business will be in that 33-34 range, right? Our B2C business is actually quite profitable, because we realize the entire Rs. 2000 at more or less the same, what I said, the 700 plus 400, right? So, to that extent, our direct to consumer business is quite profitable, but it's a small part of it.

Raman KV: It's only 5% odd of our entire revenue, 6%. Okay. Thank you, sir.

Moderator: Thank you. The next participant comes from the line of Girish Bakhru from OrbiMed. Please go ahead.

Girish Bakhru: Just a question on test menu. You guys said it is expanding. How much is the test menu now?

Rahul Guha: Sorry? How much is the?

Nitin Chugh: Last part of the question has been missed.

Girish Bakhru: What is the test menu? Is it above 1,000 now?

Rahul Guha: Roughly about 1,000, Girish.

Girish Bakhru: And when you are adding these new areas of histo pathology, immunoassay, I mean, I just wanted directionally, are you also aiming to go much higher than current level? Or is it something that it will be around this number only? Because I mean, of course, B2C players are above 2000. But histopathology, I think in the previous discussion, you said can it be 1,000 tests, right?

Rahul Guha: Yes. So, see, we, from a technology level, we will be comparable. I think we have almost every technology under Thyro care. But we don't activate every test in technology because we want to see a minimum volume before we activate the test. So, I would say our 1,000 while if you compare against the 2,000 and 3,000 appear low, but from a technology level, it's almost like to like.

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Girish Bakhru: Okay. And when you actually look at this number of tests, revenue per test, I mean, it seemed a good jump. Can you tell what is driving besides this, let's say some mixed change. Is there anything that we should look into it? And if you could give a number for this, what was the number, let's say 2-3 years ago?

Rahul Guha: Revenue per test two years ago was 33.8, which is 37.8. It's there in the disclosure, Girish. As Nitin articulated earlier, most of that 1% or 2% will be straight price high. The rest is mostly mix.

Girish Bakhru: Okay. I just wanted to know, let's say when you're adding these new tests, histo or immuno, would realization be higher? Like, how should we look at it? What would be the differential between let's say normal pathology, whether it's, I mean, I know all this is bundled, but is there a significant differential when you're adding new tests?

Rahul Guha: Yes. So, if you take revenue per test at 37, histopathology will be closer to 250. So, it's whatever, almost factor 10.

Moderator: Thank you. The next question comes from the line of Yogesh Soni from InCred. Please go ahead.

Yogesh Soni: Hi. Good evening. Thanks for the opportunity and congratulations on the good set of numbers.

My first question is, I just want to have a clarification on the active franchise network. In last quarter's presentation, the active franchise was mentioned around 11,250 whereas for this quarter, it is mentioned as 9,550. If you can just help me with all this disconnect.

Nitin Chugh: See last year, we reported the full year. So, that was annual transacting. And now we have moved to a quarterly reporting. So, the difference is what you saw 11,000 was over the full year.

Whereas if you see the quarter number, it was, right now it is almost 9,500 versus an 8,100 last quarter.

Yogesh Soni: What was in Q4?

Nitin Chugh: Q4 it was 9,400 which is 9,500 now.

Rahul Guha: He is asking for 11,000.

Nitin Chugh: So, in FY '25, overall transacting franchisee was 11,000 plus what we have reported. For the quarter Q4 number was around 9,400. And this quarter we have 9,500.

Yogesh Soni: Okay, understood. One question on again on the franchisee side, if you could help me understand, I mean, what is the churn rate of the franchisees?

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Nitin Chugh: Right. So, see with the net addition of almost 1,400, there is almost 1,000 odd franchisees which have gone out from the last year Q1 where then we added almost 2,400 odd and thus we landed almost 1,400.

Yogesh Soni: So, net addition remains to be 400 or I mean, that is how one should read?

Nitin Chugh: Net addition of 1,400. Total acquired was in the range of 2,400 where 1,000 odd went away and thus remaining 1,400 as such net addition.

Rahul Guha: So, just to clarify, we report only the net addition, not the churn number. So, what you see in the reporting is only the net addition minus churn. But as Nitin said, we add much more than this, but we have a significant churn. Typically, it happens in the quarter itself. So, we know what is the net addition as a result.

Yogesh Soni: Okay. And one last question. I wanted to understand on our sales force. So, I mean, what kind of efforts are we making or how the efforts are being made to add 100 franchises every month? I mean, we do not see this number for other companies, whether it is lab additions or service networks. So,

would want to understand on this bit.

Nitin Chugh: Yes. So, from an effort perspective, see, there are a couple of things. I don't want to really get into the details of exact strategy, but yes, there is a centralized team, which works on lead management and then there is an on-field team, which goes and meets the clients and converts the clients. So, they work very closely with each other to manage the lead flow, manage the lead conversion funnel, etc. And then there is a conversion team, which makes sure that the onboarding is very smooth of the newly converted franchises coming into the Thyrocare ecosystem with training, etc. So, that is the entire strategy. But yes, we have people on ground also doing this activity.

Moderator: Thank you. We move to the next question from Saket Kapoor from Kapoor & Company. Please go ahead.

Saket Kapoor: Sir, firstly, can you give the breakup or the cost of material consumed? What are the key constituents? And what is the cost of setting up one franchisee in terms of what kind of deposits are required or if you could just explain what are the key requirements for you to enroll for a franchisee to enroll under Thyrocare?

Rahul Guha: The cost of materials is mostly reagent, almost entirely reagent, which is used during the processing of the test. I hope you are asking this question because you want to become a

franchisee. And Nitin, you can explain what are the requirements to become a franchisee.

Nitin Chugh: See, to open a collection center is very, very straightforward. You approximately need a 200 square feet area, where you just need some things in place, which is a table chair, a working condition where a phlebotomist can sit and get a patient, come in and give the blood sample

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collection, obviously, a bathroom, etc., for other kinds of samples. And there are certain very simple conditions on how the board should look, what is the size of the board, what kind of mirrors you have to put, etc. It typically requires a 3 to 4 lakh kind of an investment from an infrastructure point of view, plus some basic deposit, obviously, if you really want to use a Thyrocare brand name.

Saket Kapoor: Sir, what is that deposit part? I mean, is it the refundable? I mean, out of the total, the franchisee model, how much then the deposit money we carry in our balance sheet?

Nitin Chugh: See, it is very, very less. And only certain franchisees only give us because only 10% of our franchisees have a board. But it is given back in form of wallet balance, which gets utilized in the business itself. So, nothing gets carried in our books as such. Because we typically give them back in form of, some in form of material, but mostly in wallet, so that it gets used in their business.

Saket Kapoor: And lastly, sir, for the reagents part, how are we sourcing the same? Are we dependent from domestically or are we importing the reagents? Who are the key suppliers for us?

Rahul Guha: I do not want to share the supplier names and all of that. But I would say a vast majority of reagent which is true across India is imported and we are no different from the industry.

Moderator: Thank you. Our next question comes from the line of Lokesh Manik with Val lum Capital. Please go ahead.

Lokesh Manik: Thank you again, team. A couple of questions. One is in the earlier calls, how you mentioned that we would be sealing the EBITDA margins at 30% and anything over that, we reinvest in the business. Now, clearly the operating leverage that is kicking in is much higher than even your plans, I assume, for reinvestments. Or if those plans take time, then do you see EBITDA margins at the earlier levels of 37%, 40%?

Rahul Guha: I would say, I will continue to hold to the guidance. I would say in Q1, we did not have that many opportunities to reinvest into the business. But we will continue to

see how we can

accelerate the growth. I am particularly keen to see if we can evaluate now seeing the success of Vimta and Polo and the full integration and the team's capability to be able to acquire and integrate in very short periods of time. I would like to deploy capital to see if we can expand the business much faster. So, yes, we didn't have too many acquisition opportunities in this quarter.

But I would say we are continuing to be on the lookout. So, please be calibrated in your expectations on operating leverage, because we are continuing to seek out opportunities for growth.

Lokesh Manik: No, I completely understand that. I am coming from more from a steady state perspective, because even your expansion in opportunities is more of OPEX driven rather than heavily CAPEX driven. So, these are temporary in nature, since operations stabilize. And in a steady

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state, then you go back to 35% -40% leverage. I mean, it's just the structure of your business, if I am not wrong.

Rahul Guha: But see, we are a management that doesn't stay in one place. There's no such steady state for us. We are continuously expanding and continuing to grow. So, when the steady state will come, who knows? For example, we were the first lab chain to become 100% NABL and then immediately we said, let's become Six Sigma. So, I don't think there is anything as a steady state in our business.

Lokesh Manik: Fair enough. My second question was just clarification of few cost items. So, there is healthcare service charges, business promotion and sale incentives, and legal and professional services. So, just the nature of these three expenses, if you can share, it would be great.

Rahul Guha: That details are not readily available immediately on here, but we can share with you separately.

Lokesh Manik: I will follow up with you on that. Thank you so much.

Moderator: Thank you. Our next question comes from the line of Aditya Chhe da with In Cred Asset Management. Please go ahead.

Aditya Chheda: Good evening. Congratulations on a good set of numbers. My question is on the inorganic growth reported. It has contributed around 2% to the growth. Whether there is any element of cannibalization by Thyrocare into the other acquired businesses or the run rate, because it's the

same from Q4 to Q1, is there going to be any element of growth from the inorganic acquisitions that we have done? And second, earlier in the call, you mentioned that second half was very good for us last year. That implies that the base will get slightly stiffer for further growth. So,

whether you see the current momentum in franchisee growth, etc., allowing you to deliver a similar growth that we have been delivering, if you have any comments on that? These are my questions.

Rahul Guha: So, on the first question, I would say there has been, see there were parts of the business that we or businesses that we acquired that we didn't want to continue. It has nothing to do with cannibalization. So, there were certain geographies where we felt it didn't make sense or there were lines of businesses, like for example, in Vimta Clinical Diagnostics, there was a research testing that they were doing, which we felt we didn't want to continue. So, to that extent, we pruned the business down. But once we took over the business and pruned it down, since then both businesses have delivered good growth. You have to remember, we acquire businesses only in geographies where we have very little presence. So, there is very little risk of cannibalization when we do the acquisition. And in fact, I would say the Polo region of Punjab, Himachal, and Uttarakhand, which is where we did the acquisition is our strongest growth year on year for the year. So, I think that has worked out well. Coming to your second question, which was H2. See, as I told you, I think it's too early

to revise guidance. I have been saying, we will all be happy with mid-teens growth, which is faster than the industry and volume led. That being said, Nitin Thyrocare Technologies Limited
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has done an excellent job on the franchise acquisitions and additions over the last year. It remains to be seen. As you rightly pointed out, H2 was very good. So, the base is very strong. Let's see how it pans out. But I definitely feel the franchise addition and that momentum is very, very encouraging.

Aditya Chheda: Got it, sir. Thank you and all the best.

Moderator: Thank you. The next question comes from the line of Anshul from Emkay Global. Please go ahead.

Anshul: Hi, thank you for the opportunity. My first question is on the partnership and franchise business. Is there any structural difference between these two businesses except one being online and other being offline? Is it exclusivity or any difference in tariffs?

Rahul Guha: No, I don't think there's any structural difference. They serve different segments of customers. One is an online customer. The other is a customer who likes to walk into a store and get their diagnostic test done.

Nitin Chugh: The only majorly what is different is in our partnership segment, there is a lot of sample collection also that is done by Thyrocare through its own phlebotomy fleet. So, basically, we also have very good tech APIs with one single integration, any partner can open their diagnostic Company or business in 300 plus cities, etc. where we can do the right from collection to processing to reporting, everything is done by us. Whereas on the franchise side, you are aware, these collection centers manage their own collection. We manage the logistics of sample coming from their center to our lab and then processing. That is majorly the structural difference.

Anshul: Got it. Very clear. There is no term of exclusivity with any of our partnership customers, right? They are free to use us as well as any other diagnostic chain as a vendor/partner?

Nitin Chugh: Yes and no. It depends. For example, in PharmEasy case, it is 100% exclusive partnership with Thyrocare. And with some other, couple of other smaller partners also, yes, we do have these kind of agreements because sometimes it is better for them to deal with one partner who is giving you the maximum reach that a Company can give. So, from an industry perspective, when you take large players, Thyrocare has the largest reach in terms of Phlebo network and logistics network. And thus, with one single API, like I said, you can open up your diagnostic business in 300 cities. So, some companies prefer having one partner to deal in terms of pricing and everything and integrating the APIs and building their own white-label solution on top of it. Right? So, that way, there we become exclusive. But yes, there are other partners, corporate partners or aggregators or insurers who use multiple players.

Anshul: Thanks. This is useful. My second question is on the number of patients. Are they all unique patients or are these footfalls?

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Rahul Guha: Footfalls, yes.

Moderator: Thank you. Our next question comes from the line of Saket Kapoor from Kapoor & Company. Please go ahead.

Saket Kapoor: Yes, sir. Thank you for the opportunity again. Sir, when we look at the business model and the

menu list, are we comparable with players like Suraksha Diagnostics or is there some overlap there or how should we look into the modeling part?

Rahul Guha: Suraksha is slightly different because Suraksha has both pathology and radiology. We are only pathology. So, that is one important difference. If you compare against only pathology, which is Dr. Lal, Metropolis, ourselves, Agilus, I will say, as I said earlier in the call, from a test menu point of view, we will be 1,000 ve

rsus 2,000 to 3,000 approximately. But from a technology point of view, we have every single technology in our labs.

Moderator: Thank you. Ladies and gentlemen, that was the last question for the day. I now hand the conference over to Mr. Rahul Guha for closing comments.

Rahul Guha: Thank you everyone for joining us and spending time with us this evening. I am sorry we ran out of time and could not take all questions. But it was good to have all of you and engage on all these questions. As always, we continue to remain focused on our strategy, which is to be the most affordable, good quality diagnostic testing partner for anyone in the healthcare business.

And we continue to execute on that strategy. We have been investing in improving our quality, improving our reach, and ensuring our turnaround time is as close to best in class. We've made substantial progress on all of this, and that is what is driving the results that you see. I thank you all for your support in this journey, and I wish you all a good evening. Thank you.

Moderator: Thank you. On behalf of Thyroc are Technologies Limited that concludes this conference. Thank you for joining us and you may now disconnect your lines.

Call: Oct 2025

October 17, 2025

The National Stock Exchange of India Limited BSE L td.

Exchange Plaza Phiroze Jeejeeboy Towers

Bandera Kurla Complex, Dalal Street,

Ban dra (E), Mumbai - 400 051 Mumbai - 400 001

(SYMBOL: THYROCARE) (SCRIP CODE: 539871)

Sub: Transcript of post results for the earnings conference call held on October 14, 2025, with Analysts / Investors.

Ref : Disclosure under Regulation 30 of the Securities and Exchange Board of India (Listing Obligations and Disclosure Requirements) Regulations, 2015 ("Listing Regulations")

Dear Sir/Madam,

Pursuant to Regulation 30(6) read with Part A of Schedule III of the Listing Regulations, please find enclosed herewith the transcript of the earnings conference call with Analysts and Investors held on October 14, 2025.

The same is also being made available on the website of the Company

<https://investor.thyrocare.com/> .

This is for your information and records.

Yours Faithfully,

For Thyrocare Technologies Limited,

Brijesh Kumar

Company Secretary and Compliance Officer

Encl. as above

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"Thyrocare Technologies Limited

Q2 FY '2 6 Earnings Conference Call"

October 14, 2025

MANAGEMENT : MR. RAHUL GUHA – MANAGING DIRECTOR AND

CHIEF EXECUTIVE OFFICER – THYROCARE

TECHNOLOGIES LIMITED

MR. VIKRAM GUPTA – CHIEF FINANCIAL OFFICER –

THYROCARE TECHNOLOGIES LIMITED

MR. PREET JOSHI – STRATEGY MANAGER –

THYROCARE TECHNOLOGIES LIMITED

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Moderator: Ladies and gentlemen, good day, and welcome to the Thyrocare Quarter 2 FY 2026 Earnings Call hosted by T hyrocare Technologies Limited. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference, please signal an operator by pressing star and then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Preet Joshi from Thyrocare. Thank you, and over to you, Mr. Joshi.

Preet Joshi: Thank you, Dorwin. A very good evening to all, and thank you for joining us today for the Thyrocare Earnings Conference Call for the Second Quarter of FY 2026. Today, we have with us Mr. Rahul Guha, MD and CEO of Thyrocare; Mr. Vikram Gupta, CFO of Thyrocare, along with other key members of the senior management on this call to share the highlights of the business and financials for the quarter results.

I hope you have gone through our results release, the quarterly earnings presentation and the press release, which has now been uploaded on the stock exchange website. The transcript of this call will be available in a week's time on the company's website.

Please note that today's discussion may be forward-looking in nature and must be viewed in relation to the risk pertaining to our business. After the end of this call, in case you have any further questions, please feel free to reach out to the Investor Relations team.

I now hand over the call to Mr. Rahul Guha to make the opening remarks. Over to you, Rahul.

Rahul Guha: Thank you, Preet. Good evening, and welcome to all on the call. Thank you for taking out time from your busy schedules to join us this evening. Just a quick introduction to us on the call. My name is Rahul Guha, and I'm the MD and CEO of Thyrocare, and thank you for the opportunity to present the Q2 results for FY '26. I'm joined with my colleague, Vikram Gupta, who is our CFO; and Preet Joshi, who is a part of our Strategy and Investor Relations team, along with other key members of the senior management.

To commemorate the 25th year of Thyrocare since its incorporation and in line with the forthcoming auspicious celebrations of Diwali, it gives me great pleasure to announce the following. Bonus shares. The Board of Directors have approved issuance of bonus shares in the proportion of 2:1. That is 2 bonus equity shares of INR10 each for every 1 fully paid-up equity share held as on the record date. This is subject to statutory and regulatory approvals as applicable. The bonus issue underscores our consistent performance over the years. It's important to note, our non-COVID growth over the last 4 years has been 19% CAGR. And this reflects our confidence in the company's future growth strategy and reaffirms our commitment to reward our shareholders and investors. It also aims to enhance liquidity in the stock and expand retail participation.

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Along with the bonus issue, we are very pleased to announce an interim dividend. Board of Directors has declared an interim dividend of INR7 per equity share of the face value of INR10 each pre-bonus issuance. Record date for the interim dividend payout will be considered 24th October 2025. With these 2 announcements, we wish our shareholders a very wonderful and happy Diwali.

Now coming to the business. As in all my calls, I will start with a quote from Nelson Mandela in recognition of our foray into Africa. It is in your hands to make a better world for all who live in it. We believe Thyrocare can bring our business model to Africa to make affordable and good quality diagnostics accessible to all.

Before we get into the details of this quarter, it

has now been 2 years since we implemented the

pay-for-performance structure, which has led to renewed energy and motivation within our franchise network to move up volumes and enter higher slabs. I'm happy to report that our franchisee base is at its highest ever, with steady growth over the last 2 years.

Encouraged by the success, we have expanded both field and central teams to accelerate franchisee addition. This has enabled us to go deeper into India and open highly transacting stores at a much faster pace.

Our new activation strategy has been delivering encouraging results by engaging local partners and improving conversion efficiency. We are now seeing consistent growth in sample volumes and new customer additions. We continue to expand deeper into Tier 3 cities and beyond, where we see strong long-term potential. To support this, we have made strategic investments in logistics and network capabilities, ensuring faster turnaround and better service reach.

As of Q2 FY '26, we have 10,100 plus active quarterly franchisees compared to 8,446 in the same quarter last year, and 9,413 in the last quarter. Our pace of franchise expansion has picked up meaningfully, supported by renewed partner interest, strong brand equity and our tech-driven support model.

Quality as a core focus remains our highest priority. It's a continuous journey of improvement and innovation. Last year, we became India's first and only 100% NABL -accredited national laboratory chain. A prestigious milestone reflecting our commitment to world-class diagnostics and excellence.

Achieving NABL accreditation across all labs was possible through robust quality management systems, advanced technology, rigorous training and proficiency testing. Given that only 2% of pathology labs in India hold this accreditation, this achievement underscores our leadership in redefining diagnostic standards.

But that is not enough. We have now set a target of achieving Six Sigma level quality, tracking complaints per million with a goal to bring it down below 3.4 per million. We've made tremendous progress improving from 11.8% in Q2 FY '25 last year to 3.8% this quarter, positioning us to soon become the first lab chain to achieve Six Sigma standards in diagnostics.

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On innovation and research, we conducted and published India's largest HbA1c study, analyzing 20 lakh results to provide deep insights into national diabetes trends. We also launched our PIVA study based on data from our Jaanch fever panel, which revealed that 1 in 3 fevers are more complex infection, such as dengue, typhoid, malaria and influenza, reinforcing the need for timely testing and treatment. In the last few quarters, we've added multiple advanced technologies, including histopathology, a new HPLC platform, coagulation and the BioFire PCR platform.

On operational efficiency, while maintaining the highest quality standards, we continue to improve turnaround time during Q2 FY '26, we released reports within 3.52 hours on average after samples reached the lab, made possible by robust processes, automation and streamlined workflows. We have fully integrated the Polo and Vimta networks and gone live with ECG -at-home services via SyncHealth. Our lab network now stands at 37 labs in India and 1 in Tanzania. We are also expanding our test menu and offering. Our flagship brand, Aarogyam, continues to lead the preventive healthcare segment, growing at 19% year-on-year, complemented by Jaanch, which caters to lifestyle and chronic health needs, and has grown 31% year-on-year and is becoming a strong pillar of our lifestyle offering.

On the business and market insights, fever-related volumes were muted this season, actually down 26% versus the same quarter last year, reflecting tremendous work by various government agencies to control viral vector. But our core business remains s

table, demonstrating resilience and customer trust. Our partnership's business performed exceptionally well, onboarding new Healthtech clients and growing existing accounts.

On the B2G side, we continue to execute TB projects in Gujarat and Maharashtra. In Tanzania, our business, though nascent, grew 30% quarter-on-quarter and is expected to double revenues this year and achieve operating breakeven in the 18 to 24 months.

Thus, our three pillars of growth: customer success, network expansion and menu expansion continue to be the key drivers of our consistent performance, reinforcing our focus on operational excellence, franchisee growth and continuous innovation to deliver long-term value.

Now I will briefly update you about the business performance in Q2 FY '26. We processed 53.3 million tests, up 21% year-on-year and served 5 million patients, a 12% year-on-year increase. Overall, at a consolidated level, this quarter, we did a 22% year-on-year revenue growth primarily driven by our Pathology business with a growth of 24% year-on-year.

Our franchisee business for the quarter showed a revenue growth of 20% year-on-year in Q2 FY '26. Revenue per retained franchisee has been consistently growing for the franchisees added post FY '22. This has been possible because of our slab-based pricing model, improved quality and strengthening of our relationship with doctors and channel partners, along with test menu expansion.

Our partnerships business in the quarter showed a tremendous growth of 35% and our API PharmEasy Diagnostics business this quarter grew by 46% year-on-year. Radiology businesses,

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active centers did a modest revenue growth of 3% year-on-year this quarter but improved their profitability dramatically.

With that, I will hand over to my colleague, Vikram, to cover the financial results.

Vikram Gupta: Thank you, Rahul, and welcome to everyone joining us today. I am pleased to report that we delivered another quarter of strong financial performance driven by consistent revenue growth and continued improvement in operational efficiency. This reinforces our commitment to long-term value creation for our shareholders and investors.

Thyrocare has been consistently delivering high teen to early 20% growth over recent quarters and outperformed our pathology diagnostic industry, which is growing at an early to mid-teen rates. In reality, this performance can be seen consistently over the last 2 years but was not affecting because of COVID and API degrowth. Now that COVID is out of base and API has come back on the growth track, the overall numbers are reflective of this upstream.

This sustained performance reflects not only the strength of our business model, but also our

ability to execute in a dynamic and competitive environment. Before we discuss the financials, let me briefly touch upon our employee stock option trends.

As the Thyrocare ESOP program concludes, and we have only limited Thyrocare shares remaining, our parent company has decided to expand the API ESOP pool to include Thyrocare senior management. This group-level initiative is aimed at retaining critical talent at Thyrocare level. ESOP will be issued as per the -- of parent company, and we invest in accordance with the established policy.

From an accounting standpoint, these ESOPs are recognized as an expense in the profit and loss account and as an equity contribution from the payment in the balance sheet. It is important to note that this is a non-cash charge and does not impact our cash flow.

To enhance clarity, we report normalized EBITDA by excluding these non-cash expenses also and have included an annexure in our quarterly earnings presentation to further explain the accounting treatment.

Now let me take you through some -- a few of the financial highlights of the current quarter. On the revenue

side, standalone revenue for the quarter, first time close to INR200 crores. This is the highest ever revenue we have done on a standalone basis at Thyrocare, which is a year-on-year growth of 24%, driven by a 20% growth in franchisee business and 35% growth in partnership business.

Radiology active sector business grew marginally, but this is in line with our focus on that business to drive profitable growth, enhance margin and optimize overheads to improve the overall performance. Consolidated revenue is highest ever at INR217 crores, which is a robust year-on-year growth of 22%.

On margin and profitability, standalone gross margin is at 71.6%, up by 100 basis points Y-o-Y. This is mainly due to operational efficiencies, favorable product mix and negotiations.

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Standalone normalized EBITDA margin is at 36%, which is an improvement of 470 basis points versus last year. This is largely benefiting from better gross margin, operating leverage and bad debt recovery.

At the consolidated level, consolidated gross margin stood at 72%, with normalized EBITDA margin at 34.8%. Consolidated number -- normalized EBITDA grew by 49% by PAT saw a Y-o-Y growth of 82%. Earnings per share for Q2 FY '26 came in at INR9.05, which is again, 82% up from the same period last year, which was at INR4.99.

On the balance sheet, we continue to be very strong and resilient. The company is debt free and on a consolidated basis, holds a net cash and cash equivalents, including short-term mutual funds, of INR190 crores plus.

In cash flow, in H1 '26, we generated INR127 crore cash flow from operating activities. This is higher by INR38 crores versus last year, which is a year-on-year growth of 43% over H1 FY '25. So this was on the financial performance. Lastly, as covered by Rahul, in line with strong financial performance and Silver Jubilee celebration of Thyrocare, the Board of Directors has approved a bonus issue in the ratio of 2:1. This is subject to statutory and regulatory approvals

as applicable. The record date will be communicated shortly to our Stock Exchange filings.

It is important to note that this bonus issue will be made entirely from our free reserves and will not impact the company cash position. Further, Board of Directors have declared an interim dividend of INR7 per equity share. The record date of the interim dividend is 24th October 2025.

As covered above, the company is having sufficient cash and cash equivalent results to -- for paying dividend as well as to invest into businesses expansion.

With that, now I hand over to Rahul for the strategic updates.

Rahul Guha: Thank you, Vikram. Briefly, I would like to take a few minutes to recap to you a strategic direction, and then I will open it up for Q&A. First, I will reiterate our value proposition to the customer.

We will continue to remain an affordable option to all patients with good quality and on-time reports. All our efforts on our value proposition is towards ensuring low cost to the patient, assurance on quality of testing through our certifications and engagement with doctors. We have made substantial progress on this, which I updated in my initial comments and is reflected in the presentation. This will remain at our core and will continue to guide all that we will do.

Second, our strategy. We continue to maintain our strategy of being the B2B partner of choice to all front-end diagnostic services companies in India, whether it is a small diagnostic center in a semi-urban area, a pharmacy in a metro, a small nursing home, an individual doctor or a leading online diagnostic platform or Healthtech marketplace.

We are happy to work with them to provide low-cost, robust testing solutions so that they can serve their patients in the most effective manner. If they require phlebotomy, we are happy to

mobilize our company-owned

phlebotomy network of almost 1,900 phlebotomists, including our network partners, to serve them better.

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We remain dedicated to expanding our business and with the expansion of Polo Labs and Vimta Clinical Diagnostics, we plan to significantly increase our presence in North and South India, respectively. Additionally, to further boost our partnerships business, the acquisition of SyncHealth has allowed us to scale ECG -At-Home services and enter the high-growth pre-policy medical checkup business, further enhancing our value to our insurance partners. This strategy has been working well for us with both our franchisee and partnerships businesses posting strong growth.

That, in a brief, is our mandate as management. Thank you for giving us a patient hearing. I will once again end with a quote from the Mahatma, "Find purpose, the means will follow". And our purpose remains to provide affordable, high-quality testing to the masses.

With that, we'll open it up for Q&A.

Moderator: Thank you very much. We will now begin the question-and-answer session. The first question comes from the line of Siddhant K, an individual investor. Please go ahead.

Siddhant K: Thank you for the opportunity. Sir, my question is like basically I've just started following Thyrocare. So I just wanted some clarity on the business model. So when we say -- talk about the franchise model, we are talking about the collection centers, which is under our own brand, right? But, hello, am I audible?

Moderator: Sir, you're audible, you may proceed.

Rahul Guha: Can you finish your whole question, then I will answer.

Siddhant K: Yes, yes, sure, sir. So when we talk about the franchise model, we are talking about the collection center, which are under our own brand. So sir, just wanted to understand, do we send the phlebotomist -- the phlebotomist is under our payroll or on the franchise balance sheet?

And my second question is, sir, what will be the revenue split between our brand versus non-brand. Basically, the third-party hospitals or the diagnostic centers, which are not under the Thyrocare brand?

Rahul Guha: Understood. So firstly, just to explain the business model, our franchisees can be a hospital, a nursing home, a standalone pathology lab or a collection center, right? So all four of these could -- any one of these could be our franchisees. Some of them are branded, some of them are unbranded, right?

The rough split is about, I would say, of the 10,000 franchisees, probably around 1,000 would be fully Thyro branded -- Thyrocare branded, which means that they have a Thyrocare board, Thyrocare delivery and all of that and the remaining will be unbranded or not Thyrocare branded, maybe a hospital, maybe a nursing home, maybe a standalone doctor, maybe a collection center or even a standalone pathology lab. So that's the rough mix of the footprint.

In all cases, the report is a Thyrocare report, whether they are branded or unbranded. At the end of the day, the report that is handed over is a Thyrocare report. Now coming to the split, while Thyrocare Technologies Limited

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1,000 would be branded, 1,000 out of the 10,000, that 1,000 will account for about 40% of our revenue. So that's the revenue split, while I give you the count split.

We have a separate phlebotomy fleet that is not touched -- included in our franchisee. This is what I call the Thyrocare phlebotomy fleet. This is -- in my opening comments, I mentioned 1,900 phlebotomists that work with us. These are largely to service our partnership business. We are the only company, at least in my understanding, that has a dedicated company fleet that services corporate, partnerships, healthtech orders. And that allows us to differentiate because we don't route these orders to the franchise network. We execute

it with our own phlebotomy fleet.

Siddhant K: Right, sir. Yes, sir, just one more question. Sir, when we talk about the 40% revenue, which is coming from those 1,000 centers, so then is it safe to assume that those 40% revenue is actually B2C revenue only and not a B2B revenue?

Rahul Guha: See, we don't differentiate that way. For us, all reports are a Thyrocare report. So in that way, a Thyrocare branded report is reaching the patient's hand. So we look at it that way.

Siddhant K: Okay. But is there some element of B2B in those 40% also?

Rahul Guha: There would be, but not a large component.

Siddhant K : Thank you so much, sir. I will get back in the queue.

Moderator: Thank you. Our next question comes from the line of Bhavya Nahar from Tamohara Investment Managers. Please go ahead.

Bhavya Nahar : Sir, a few quarters ago, we spoke of our franchise network being revamped with the new tiered incentive structure, and we streamlined our entire franchise network down to 7,000 franchisees.

And today, we are at 10,000 franchisees. So the growth in revenue from franchise business seems slowly driven by growth in number of partners. So with no throughput growth, are we worried of the reverse phenomenon happening?

Rahul Guha: Actually, that's not true. Actually, if you see our overall growth has largely been driven by what we call the diamond to silver category partner, our large partners. So if you look at our mix, we have -- roughly, we had said about 10,000. It's half and half, half diamond to silver and half bronze and below, right? That's how we bifurcate the list.

Actually, most of the growth has come from the diamond to silver, both from a count as well as revenue, right? So I would say we've been able to add a lot of large partners and they have grown very well with us.

Bhavya Nahar : And also, the gross margin increase, as the incentives we roll out are lower, so will that also be the reason behind the gross margin increase or... ?

Rahul Guha: No, a large -- I'll let Vikram comment a little bit, but it is my understanding that the gross margin has not come because of price, but for better negotiation. But Vikram, you can give more detail.

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Vikram Gupta: So as an affordable player, actually, we don't believe in increasing the prices. So most of the gross margin improvement has come from operational efficiencies, which includes, let's say, reduction in repeat rates, reduction in wastages.

Second, part of the gross margin improvement is procurement savings because of our volumes, we are able to negotiate harder with our vendors. And the third is the favorable product mix. So all these three factors have contributed for the improvement in gross margin, and this is quarter sustainable.

Bhavya Nahar : Okay. Thank you.

Moderator: Thank you. Our next question comes from the line of Krishna Raj K from Ekvity Wealth Management Private Limited. Please go ahead.

Krishna Raj K : Thanks for the opportunity. I just wanted to know why would be our revenue per test and revenue per patient is considerably low. I know we operate in a B2B model, but is there any chance of improvement?

Rahul Guha: See, the revenue per patient has grown largely because of the tests per patient, right, not because of price increase. And that is because we have been adding more and more profiles, Jaanch and all the other new initiatives effectively help a patient comprehensively test when they are feeling well versus -- unwell, sorry, versus doing sporadic testing.

So to that extent, the test per patient has gone up. But we -- our mission statement is to make good quality diagnostics affordable to all. So we typically try not to increase prices and pass that on to the customer. We do our best to maintain the affordability to the customer because we believe that is our duty and our mission. For example, when the GST changes

happened, we

were the first company to pass on the entire GST benefits to our franchisees and partners.

Krishna Raj K : Okay. Got it. And the second question on the -- so would we happen to see more volume in for the likes of GLP -1 weight loss drugs going off patent next year. Is it some opportunity for diagnostics industry? I wanted to know your views on this.

Rahul Guha: I thank you for that question. I genuinely believe that is going to be a huge opportunity. For those who know my profile, I have spent almost 20 years in the healthcare industry. I have never seen a molecule or a brand reach INR100 crores in such a short period of time. And obviously,

when it comes to weight loss drugs, it has to be taken with caution and with regular testing to ensure that your -- while you're losing weight, your health parameters are all on track.

So we are also in the process of launching complementary packages with the GLP -1 therapy, both pre therapy, during therapy and post therapy. So that as you get on these weight loss drugs, you preserve your health along with the weight loss. So it's a huge opportunity. Thank you for calling it out.

Krishna Raj K : That's it from my side. Thank you and all the best.

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Moderator: Thank you. Our next question comes from the line of Harsh Kataria from RRR Investments. Please go ahead.

Harsh Kataria: Hello, sir. Good evening. So my first question is, sir, I want you to throw some light on the revenue mix based on the online and the offline channel, specifically including the Thyrocare app and the PharmEasy platform because as we mentioned, there is 36% growth year-on-year from the PharmEasy. So can you please share the revenue mix based on the online and offline platform?

Rahul Guha: Sure. So a large part, the way you can think about it is our franchisee business is largely representative of our offline revenue mix, that you have seen has grown about 20%. The partnerships business, you can actually take as representative of the online revenue mix. That has grown at about 36% for overall and for API as well.

So those are the best bellwether. Thyrocare is wonderfully placed because we have our leg in online and offline and we get benefits of tailwinds in both. But to answer your question specifically, offline would be 20%, online would be 35%.

Harsh Kataria: Okay, sir. And mostly I think that the prices for the same plans are different at both the platforms. That is the PharmEasy and the Thyrocare app. Sir, is the lower prices of one affecting the volumes from the other app?

Rahul Guha: See, the prices now I think if I look at our most online players, when it comes to pricing, they have become quite rational. So what used to be a very market -marked price differential maybe 1.5 years, 2 years ago is now more or less in the same range. That being said, Thyrocare being primarily a B2B player, we don't dictate how our partner's price.

So in that way, I have no control on the pricing on that front. But our offline players typically look at the Thyrocare rate on the website and then use that as a benchmark. So it's actually very difficult for us to discount the Thyrocare website because that creates competition for our offline players. So we don't actually compete in this HealthTech. We would much rather partner in that space.

Harsh Kataria: Okay, sir. That's it from my side. Thank you.

Moderator: Thank you. We have our next question from the line of Raman K. V. from Sequent Investments.

Raman K. V: Hello, sir. Thank you for the opportunity. I have a few questions. First starting to -- starting with the B2G business. So you said that we execute TB projects under the B2G business, can you quantify the number like how much of the top line comes from the B2G. And on that note also, recently there has been a price hike with respect to the CGHS rate. So

will this -- will this provide

us any traction going forward with the B2G businesses?

Rahul Guha: So I would say B2G is not a very large focus for us. So if you look at our overall revenue mix, I think it's about 1% of our total revenue. Last quarter, we would have done about INR2 crores of government business in the INR200 crores of revenue that we showed. So it's not a very large focus area for us, except in very specialized technologies like TB and others that we look at. The Thyrocare Technologies Limited

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price realizations in the B2G business tend to be much, much lower and margin dilutive for us as a company. So unless it's a very specialized segment, we normally don't participate in that segment.

Raman K. V : Okay. So basically, the CGHS rate hike will not be an impact on the business operations, right?

Rahul Guha: Not to a large extent, though it does -- as they raise the prices, everyone raises the prices. So to that extent, there will be some tailwinds, but I don't see any meaningful impact for us.

Raman K. V : And sir, my second question is with respect to the GLP -1 drugs. So there is a new wave of with respect to the preventive care and wellness segment. Can you -- and the company also grew 19% during the quarter with respect to its wellness segment? Can you give any guidance or the market opportunity which Thyrocare can aim for going forward?

Rahul Guha: Sure. I just answered this question with the previous participant. I will just say, in summary the opportunity with GLP -1 for diagnostics is huge and we are well geared up for it.

Raman K. V : So this -- the mid -teen volume growth, we are comfortably doing mid-teens at volume growth every quarter. And with the GLP -1 coming into action in the coming quarters, I'm assuming our volume growth will be much higher. So can we expect a high-teen volume growth going forward like in FY '27, '28?

Rahul Guha: Difficult to say at this point in time, it's still very early days for GLP -1. So I think when it comes to guidance, probably closer to the end of the year is when we will firm up our guidance for next year. It's too early to say at this point.

Raman K. V : And sir, okay...

Moderator: Sorry to interrupt, sir. We request you to please rejoin the question queue if you have further questions. Thank you. Our next question comes from the line of Siddhant K , an Individual Investor. Please go ahead.

Siddhant K : Yes. Thank you, sir, for the follow -up questions. Sir, like you mentioned, the only differentiator

between the partnership and franchise model is online and offline or is there something else as well?

Vikram Gupta: No, I just said it is representative of the growth rates of offline and online. They are otherwise very different businesses, but what is your question?

Siddhant K : Sir, just -- so what categorizes as a partnership versus a franchise. So I just wanted to understand the difference between what -- how we categorize our franchise versus a partnership?

Rahul Guha: So a franchise is when someone walks into a store or contacts a Thyrocare franchisee for home collection. A partnership is where the partner doesn't have a storefront or a -- broadly a storefront and relies on Thyrocare to complete the entire phlebotomy collection as well as reporting. So that's the difference.

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It's basically a franchisee does the collection himself. Our partnerships rely on Thyrocare to do the labs. Our partnerships are of three major types. There's Health Tech of which PharmEasy, as I mentioned earlier, is a very large Health Tech partner. But equivalent to PharmEasy, we have as many other Health Tech partners. Then our Corporate segment and our Insurance segment comprises mostly the partnerships.

Siddhant K: Got it, sir. So that means that the franchise model can have both our own br

and as well as non-

branded partner -- franchise models also?

Rahul Guha: The franchise business can have both branded and non-branded. Partnerships can also be branded and non-branded.

Siddhant K: Got it, sir. Got it. Thank you, sir.

Moderator: Thank you. Our next question comes from the line of Abdulkader Puranwala from ICICI Securities. Please go ahead.

Abdulkader Puranwala : Hi, sir. Thank you for the opportunity, and congratulations on good set of numbers. Sir, my first question is, for the quarter, what would be your organic growth that is excluding Polo, Vimta and Think, how would the business have grown?

Rahul Guha: So, our organic growth -- inorganic growth contribution is 2%. So of 24%, 22% is organic and 2% is inorganic.

Abdulkader Puranwala: Understood. And sir, if I look at your presentation, it talks about we having close to, say, 38 labs now, and we have added a couple of that in this quarter and the prior quarter as well. Sir, have we also closed a couple of labs? And if you could help us understand, why would we do that as compared to last quarter?

Rahul Guha: I'll ask Vikram.

Vikram Gupta: No, no, we have not closed any lab. In last quarter, we reported 39 labs. What we mentioned --

Rahul mentioned in his opening remarks that we are integrating Polo and Vimta Labs into our labs. So we have integrated two of Polo Labs into our network and hence you are seeing a reduction in count.

Abdulkader Puranwala: Got it. Got it.

Vikram Gupta: Hope this answers it.

Abdulkader Puranwala: Yes, sir, that clarifies. Thank you. And sir, just one follow-up on the GLP -1 packages. So, sir, I mean, could you also help us understand how the strategy is going to be, whether we associate ourselves directly to the hospital clinics which are already our franchisee or this is going to directly reach out to the pharma companies, how exactly we are going to work here?

Rahul Guha: See, that is a matter of internal strategy. If I reveal everything on the call, my competitors will get a leg up. So, I would request we keep it close to our chest. All I can say is that we have recognized the opportunity and are executing towards it.

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Abdulkader Puranwala: Understood, sir. And sir, just one bookkeeping question. On the margin front, this quarter, we saw a significant improvement. And, sir, how should we look at for the rest of the year, that is the second half and next year? And on an adjusted basis or on a reported whatever you are comfortable sharing?

Rahul Guha: See, this is a seasonal business. Q2 is always strong. Q3 tends to be soft, costs remain fixed, right? And then Q4 tends to be strong again, and we will recover. So, I think we can expect margin to dip a bit in Q3 and pick up in Q4.

I think H1 to H2 will be similar, unless some large investment opportunity comes our way in which we'll then stick to our original guidance. But otherwise, H1 to H2, one can expect similar range, unless some, as I said, large investment opportunity comes our way.

Abdulkader Puranwala: Sure, sir. Understood. Thank you.

Moderator: Thank you. We have Sudarshan Agarwal from Axis Capital with the next question. Please go ahead.

Sudarshan Agarwal : Yes, hi. I think my last question on margins was already answered. Just on the top line front as well, I think we had called out earlier that H2 base is strong, so we are expecting mid -teens.

Would that guidance continue for FY '26? And my second question is, I think your annual increments are still to be done for this year? Or, you know, when -- in which quarter do you take annual increments?

Rahul Guha: When you say salary increments, are you talking about price?

Sudarshan Agarwal: Yes, yes, yes. Salary increments. And if you can answer the additional -- okay.

Rahul Guha: From 1st April, we do the increments. That was done in 1st April. With

regards to the second

half guidance, my general philosophy and life is plan for the worst, hope for the best. All that you have said still holds true. We are sitting on a very large base in H2. We have our work cut out for us. The current performance is very encouraging.

But as I said in my opening comments also, fever was down quite substantially versus the last year. And October, November, December tends to be the peak time for fever. So, multiple headwinds, I'd say. At this point, I would be hesitant to revise guidance, and let's see as the next quarters evolve.

Sudarshan Agarwal: Yes. Got it. Got it. And lastly, just one...

Moderator: The current participant seems to have dropped from the queue, sir. We will proceed to the next participant. Our next question comes from the line of Anshul from Emkay. Please go ahead.

Anshul : Hi. Thank you for the opportunity. My first question is sort of a clarification required. Could you help me understand what explained the 10 tests, 10.7 tests per patient that we conduct. This number seems higher versus other B2C operators at least. Is this because of bundling, wellness or anything that you can call out?

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Rahul Guha: You're right. It's largely because of bundling. See our average -- our highest selling Aarogyam package is Aarogyam C, which has about 70 tests per patient, right? And that as I said in the opening comments, is close to about 33% of my revenue. So, a large part of that test per patient is explained from the packages, both Aarogyam as well as Jaanch.

Anshul: Got it. Second question is, you mentioned that pricing has become rational on the online front.

And despite that, our partnership business seems to be outstripping franchisee business. Are you seeing any shift in customer preference? Or was this probably some seasonality due to heavy monsoons in the current quarter that online players sort of got a leg up or anything that you would want to call out in terms of customer preference from online -- from offline to online testing?

Rahul Guha: That's a very good question. It will maybe take me a couple of minutes to help you break it up, right. If I look at my offline business, right? A large part of the growth comes from Tier 2 markets and beyond, right? Actually, if you look at the metro market, I would say, growth would be in the single digits. But as you go outside the metros is where you start to see the real uptick in growth in the offline space, right?

On the partnership side, which is mostly online players, most of them are metro and Tier 1 city predominant, right? And so you would see them expanding and taking share in the metro market. So, if I had to break out the growth, right, online players, largely metro and maybe a few Tier 1 cities, right, where the growth even in offline tends to be muted. But as soon as you leave the metro and, I would say, select Tier 1 city, then the growth is -- in offline is much stronger because online is -- well has not yet reached that level of penetration.

Moderator: Thank you. Our next question is from the line of Yogesh Soni from InCred. Please go ahead.

Yogesh Soni: Thanks for the opportunity, sir. My first question is just to get a clarity on the GST impact. Did you say that we have passed on complete GST impact to the franchisees. That means the prices for them have been revised downwards?

Rahul Guha: That is correct. That is correct.

Yogesh Soni: Second question is just if you would like to discuss more on your expansion of the field force and the investments on the infrastructure that you are doing on logistics front, investments that you are doing on the logistics?

Rahul Guha: Sorry, I could not follow the question.

Yogesh Soni: Can you help us understand, I mean, what kind of expansion you are doing in the overall field force? I mean what is the quantum of expansion

that has been happening? And what kind of investments are we making in the logistics front?

Rahul Guha: So just to answer your question, we have passed on the GST benefit, but we have also got the

GST benefit. So effectively, margins are expected to be stable, right? So that's just one important clarification. On the investments, we have a field team. The field team is, I think, about 40 people right now.

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And we have a call center team of another 50 -odd people. So it's almost 100 people that have been invested in franchisee expansion. And on average, they are adding between 100 to 150 net franchisees every month, and that has -- they have been doing consistently, I would say, for almost the last 12 to 18 months. So that is going very well for us.

On the logistics front, we have invested very heavily in cold chain. Now every sample transportation box in Thyrocare has a data logger where we are able to track the end-to-end adherence to the cold chain, not just at the start point and the endpoint, but actually across the entire transportation. We believe we are the first diagnostics chain nationally to be able to achieve this milestone. And that has yielded a lot of good results in terms of our cancellations, our ability to report more accurately as well as customer satisfaction.

On the other front, we continue to expand labs and improve our turnaround time. It used to be almost 40 hours plus. We are now down to 18 hours or less pan-India. And so that has yielded good results for us. And I think those are the large investments we are making.

Yogesh Soni: Got it, sir. If I can ask 1 more question on the radiology front. Couple of quarters, we had -- in the past, you had told that you weren't expecting to grow -- Radiology business to grow. Have you seen any change in stance on the Radiology business, given the margins have improved in the business?

Rahul Guha: Yes. Maybe Vikram can add. But the Radiology business last year was loss making, right, and a drag on our overall bottom line. So we said let's forego unprofitable growth, and let's at least focus on ensuring these businesses are contributing to the bottom line. In that, Vikram can elaborate a little bit more on what exactly we have done there.

Vikram Gupta: So, this quarter, radiology growth for active centers was 3%. This excludes because we have closed 1 non-profitable center and taken some action on the -- there was some temporary halt in one or two centers because of the issues which will get again started in QND quarter.

But having said that, as Rahul mentioned that we have foregone the non-profitable growth, and we have increased our realization per scan. So that goodness will continue to come. And with all the centers fully operational in the QND quarter, we expect revenue growth to also come back in the next quarter.

Moderator: Our next question comes from the line of Raman K. V. from Sequent Investment.

Raman K. V. Thank you, sir. All my questions have been answered.

Moderator: Thank you. We have our next question from the line of Abdulkader Puranwala from ICICI Securities.

Abdulkader Puranwala: Sir, could you help us quantify the quantum for the overall contribution of the test on which there was a GST cut taken? And when we talked about earlier that the CGHS rate hike can lead the industry to increase test prices. So what percentage of our portfolio would overlap there, these two quantification, if possible?

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Rahul Guha: So I'll take the CGHS question. I'll let Vikram take the GST question. See, CGHS, there's no specific tests that we have in our menu that is exclusive to CGHS or vice versa. I mean, what gets written as a prescription in CGHS, almost all those steps are there in Thyrocare. So there's no such overlap.

If you are asking how

much of our business is CGHS based, it's very little, you can say, negligible overall. So to that extent, we won't see a benefit on our overall business because of CGHS rates. But what I was saying is as CGHS increases its rate, the competitive pressures in the industry also ease off, right? And therefore, there is some benefit on that front. Vikram, you want to take the GST question?

Vikram Gupta: On the GST rate cut, basically there was a GST rate change as an input only on a couple of technologies. So most of the other technologies, which we are using are already on a 5% GST as an input. So like if I have to quantify close to 20% of the total input, which we are purchasing, there was a GST rate change, which was majorly biochemistry-related tests. So there, we have - in the biochemistry-related tests we have reduced our prices, which we have passed on from B2C and B2B.

Abdulkader Puranwala: Okay. So what was the contribution to revenue of this biochemistry test?

Vikram Gupta: This would be close to -- I think that the same 20% of revenue -- 20% to 25% of revenue.

Abdulkader Puranwala: Understood, sir. And just one...

Vikram Gupta: At the same time, you should also understand that this is 20%, 25% of the revenue, where the - there is a GST rate cut on the input prices. And since our COGS is 30%, let's say, so the benefit would be 30% on that 30%, right? So it would be up 0.9% on that 20% or 25%. So that the benefit was very negligible, but whatever is the benefit that we have passed on, on those specific tests.

Abdulkader Puranwala: Sure, sir. Understood. And sir, one final one, if I may. Sir, on margins, so in the last couple of quarters we have been opening new laboratories. So is there any EBITDA burnout, which is happening there? And if you could call out what is the impact of these new centers on your margins currently?

Rahul Guha: So, see, we have a very limited lab network, right, of roughly about 39 labs as we said, and so therefore, we service a very large area of the country. And we are able to see where demand is coming from and accordingly take decisions on setting up labs.

For example, in Bhagalpur, we knew already the workload that was coming from Bhagalpur to our Patna lab. So when we set up Bhagalpur, on day one, it was full, right? So there was no real opex dilution or no material opex dilution and a much better service level to the customers in Bhagalpur. So our lab expansion is very thought-through with data where we are able to see where the demand is coming from to one of our, let's say, existing labs and then invest accordingly.

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Moderator: Thank you. As we have no further questions from the participants, I would now like to hand the conference over to Mr. Rahul Guha for closing comments. Over to you, sir.

Rahul Guha: Thank you. Thank you, everyone, for joining us and spending the time with us this evening. As always, we continue to remain focused on our strategy, which is to be the most affordable, good quality diagnostic center -- testing partner for anyone in the healthcare business, and we continue to execute on that strategy.

We have been investing in improving our quality, improving our reach and ensuring our turnaround time is as close to best-in-class, and we've made substantial progress on all of this and that is what is driving the results that you see. And thank you all for your support in this journey, and I wish you all a good evening and a very happy Diwali in advance. Thank you.

Moderator: Thank you. On behalf of Thyrocare Technologies Limited, that concludes this conference. Thank you all for joining us. You may now disconnect your lines.

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Sub: Transcript of p ost r esults earning c onference call h eld o n May 12, 2026, with
Analyst/Investors .

Ref: D isclosure under Regulation 30 and other applicable regulations o f the SEBI
(Listing O bligations and D isclosure R equirements) R egulations, 2 015.

Dear S ir/ M adam,

Pursuant to Regulation 30(6) read with Part A of Schedule III of the Listing Regulations, please
find enclosed herewith the transcript of the earnings conference call with Analysts
and Investors held on M ay 12, 2 026.

The same has al so been made available on the website o f th e Company

<https://investor.thyrocare.com/>

We request you to p lease take the same on record.

Yours F aithfully,

For Thyrocare Technologies Limited,

Bri

jesh Kumar

Company Secretary and Compliance Officer

En

cl. A/a

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"Thyrocare Technologies Limited

Q4 FY '2 6 Earnings Conference Call "

May 12, 202 6

MANAGEMENT : MR. RAHUL GUHA – MANAGING DIRECTOR AND CHIEF
EXECUTIVE OFFICER – THYROCARE TECHNOLOGIES
LIMITED

MR. RAJDEEP PANWAR – CHIEF COMMERCIAL
OFFICER – THYROCARE TECHNOLOGIES LIMITED

DR. RAMESH KINHA – CHIEF OPERATING OFFICER –
THYROCARE TECHNOLOGIES LIMITED

MR. VIKRAM GUPTA – CHIEF FINANCIAL OFFICER –
THYROCARE TECHNOLOGIES LIMITED

MR. PREET JOSHI – STRATEGY AND INVESTOR
RELATIONS – THYROCARE TECHNOLOGIES LIMITED

MR. ALOK KUMAR JAGNANI – NON-EXECUTIVE
DIRECTOR - THYROCARE TECHNOLOGIES LIMITED

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Moderator: Ladies and gentlemen, good day, and welcome to the Thyrocare Q4 FY '26 Earnings Conference
Call, hosted by Thyrocare Technologies Limited.

As a reminder, all participant lines will be in the listen -only mode and there will be an
opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call,
please signal an operator by pressing s tar then zero on your touchtone
phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Preet Joshi from Thyrocare. Thank you. And over to you,
sir.

Preet Joshi: Thank you, Swapnali. Good morning, everyone, and thank you for joining us today. I'm Preet

Joshi from Strategy Team at Thyrocare. It is a pleasure to welcome you all to the Q4 FY '26 earnings call for Thyrocare. Joining me on the call today are Mr. Rahul Guha, our MD and CEO; Mr. Rajdeep Panwar, our Chief Commercial Officer; Dr. Ramesh Kinha, our Chief Operating Officer ; and Mr. Vikram Gupta, our Chief Financial Officer.

I hope you have gone through our results release, the quarterly earnings presentation and the press release, which has been uploaded on the stock exchange website. A transcript of this call will be available in a week's time on the company's website.

Please note that today's discussion may be forward-looking in nature and must be viewed in relation to the risks pertaining to our business. After the end of this call, in case you have any further questions, please feel free to reach out to the Investor Relations team.

I now hand over the call to Mr. Rahul Guha to make the opening remarks. Over to you, Rahul.

Rahul Guha: Thank you, Preet. Good morning, and welcome to the call. Thank you for taking out time from your busy schedules to join us this evening. As in all my calls, I will start with a quote from Nelson Mandela in recognition of our foray into Africa. It is in your hands to make a better world for all who live in it. We believe Thyrocare can take its business model to Africa and make affordable, high- quality diagnostics accessible to all.

Before I get into specifics, I want to take a step back and talk about how we see the diagnostics industry evolving. In today's health care landscape, diagnostics is no longer a back-end support function. It is central to clinical decision -making and increasingly to preventive health management.

Patients today expect convenience, speed and reliability as a baseline and are actively looking for trusted partners who can deliver end-to -end diagnostic solutions seamlessly. This shift aligns closely with our own mission at Thyrocare to make preventive health care easy to deploy, consistent in quality and accessible to all.

Against this backdrop, FY '26 has been a year of meaningful progress for us, not just in terms of growth, but in terms of strengthening the foundation of the business. We are very proud of some of the key milestones that we achieved in FY '26. Thyrocare was certified as a Great Place to Work by Great Place to Work Institute.

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Work, a recognition by the global authority on workplace culture, reflecting our commitment to fostering a positive, inclusive and empowering environment for our people.

We were recognized at the 4th National Diagnostic Forum & Awards by Voice of Healthcare with two prestigious awards, Best Diagnostic Lab Chain of the Year, National, and more importantly, Patient-Centric Diagnostic Company of the Year, National. During the year, we onboarded Madhuri Dixit as the brand ambassador for Thyrocare, which is an important step in strengthening brand recall and consumer trust at a national level.

Our diagnostic portfolio continues to do well, Aarogyam growing in line with the company growth rate. But the highlight was Jaanch, which grew at 66% this quarter year -on-year, showing the move towards more specialized disease-specific testing even in the annual health checkup space.

We expanded our diagnostics portfolio significantly into the specialty space. One key addition was allergy testing, which was introduced using the Phadia platform and currently has over 250 SKUs. We started our foray into genomic testing on May 1, a segment that is nascent but directionally significant for Indian diagnostics.

The global genomics market is projected to exceed \$100 billion by 2030. And India with one of the highest burdens of genetic disorders, including thalassemia, sickle cell disease and hereditary cancers represents a structurally underpenetrated opportunity . Carrier screening, pharmacogenomics and cancer pre-disposition panels are seeing early but accelerating demand, particularly amongst urban health aware consumers with a franchise network of almost 11,000 collection points and a deep brand trust in preventive testing.

Thyrocare is well positioned to bring genomic testing to a broader patient base at accessible price points, democratizing a category that has largely remained confined to metro hospitals and specialty clinics. We are approaching this methodically, starting with defined panel offerings, particularly non -invasive prenatal testing, building clinical validation and layering physical education before scaling aggressively.

Now we have more than 10,800 franchisees. As a result, we processed 210 million tests in FY '26, which grew by 23% year -on-year. It's important to contextualize that this volume is higher than some of our biggest competitors combined. We serve 19.2 million patients in FY '26, which increased by 15% year -on-year.

As we continue to scale, our focus remains anchored on delivering high- quality diagnostics with consistently faster turnaround times. In Q4 FY '26, 97% of our samples are processed in NABL - accredited owned laboratories, with every reports being reviewed by qualified MD Pathologist.

We have also made significant progress on our quality metrics, with complaints reducing to 3.06 per million tests, thus sustaining a Six Sigma level of performance. Also, during the quarter, we delivered an average turnaround time of 3.43 hours from sample receipt, supported by our laboratory operations and a strong well-integrated logistics backbone.

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This allows us to ensure timely access to critical diagnostic information across the country. Thus, overall, FY '26 has been about building scale with consistency, while continuing to invest in capabilities that will drive the next phase of growth.

With that, I will now hand it over to Rajdeep to take you through the business and commercial updates. Over to Rajdeep.

Rajdeep Panwar: Thank you, Rahul, and good morning to everyone joining us today. I'll take a few minutes to walk you through how the business performed during the year and the key drivers behind this performance. At an overall level, we delivered strong growth this year, with consolidated revenue delivering 21% year-on-year growth in FY '26 and 20% year-on-year growth in Q4 FY '26, primarily driven by our core pathology business.

For the year, our flagship brand, Aarogya, continues to lead preventive health care segment, growing at 20% year-on-year this quarter, complemented by Jaanch, which caters to curative and chronic health needs. Though just being 2% of overall pathology revenue, Jaanch has also grown at 66% year-on-year this quarter and is becoming a strong pillar of our lifestyle offering. Let me now break this down across our key business segments. In FY '26, franchisee business delivered 18% Y-o-Y growth and in Q4 FY '26, 21% Y-o-Y growth. It has now been almost 3 years since we implemented the pay-for-performance structure, which has led to renewed energy and motivation within our franchisee network to move up v

olumes and enter higher slabs.

Supported by this renewed energy, along with strong brand equity and market demand, our franchisee base has reached its highest ever level of 10,800 active franchisees in Q4 FY '26. To

further accelerate growth, we have expanded both our field and central teams, enabling us to penetrate deeper into India and open high transacting stores more quickly.

The activation of our specialty division will complement our broader growth strategy by strengthening our omnichannel presence, enabling us to become a one-stop solution for our partners and drive higher wallet share within our existing network. To further strengthen engagement and relationship with our franchisees, we regularly conduct structured interactions. This year, to further drive franchisee engagement, we hosted 13 franchisee meetings across different locations in India, with a healthy participation from our franchisee partners, where 774 partners attended. Insights and feedback from these interactions directly inform how we strengthen process, service delivery and quality outcomes.

Moving to our partnership business. As we know, this continues to be a strong growth engine for us. In FY '26, partnership business grew at 32% year-on-year. And in Q4 FY '26, the growth was 23%. This growth has been led by strong momentum in insurance and healthtech segments. Along with continued scaling of existing accounts, our API-based integration are also enabling partners to expand diagnostics offering across multiple cities seamlessly, which is strengthening our positioning as a preferred B2B partner. In Q4 FY '26, growth at 23% in our partnership has been lower than expected versus previous quarter in this financial year. But it is important to note that this is a one-off dip.

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This dip is primarily because last year, we had a one-off boost in our partnership business, primarily due to an aggressive push on camps, especially during the Maha Kumbh period and the initial investment to build traction in the insurance segment as well. This year, we have normalized our pricing into the insurance segment, and therefore, the growth has been slower in this segment.

And of course, the scale of camps this year has been much lower. The extent of the one-off was approximately INR4 crores in Q4 FY '25. And normalizing for that, the partnership segment still grew at 32%, which is in line with the historical growth across the previous 3 quarters in the year. Thus, our overall focus has been on building depth across channels, improving partner productivity and expanding the value we deliver through a wider test portfolio and stronger service capabilities.

With that, I will hand it over to Dr. Kinha to walk you through operations and lab initiatives.

Ramesh Kinha: Thank you, Rajdeep. Good morning, and a warm welcome to everyone joining us today. I'll

focus on three areas today. First, our lab network expansion; second, operational capabilities and research; and third, on how we are improving patient experience. So let me start with our first area of the lab network.

Our lab network now stands at 40 labs in India and one in Tanzania. This year, we expanded our geographic footprint further across India and opened seven new labs, each at Bhagalpur, Mandi, Roorkee, Kashmir, Davanagere, Vijaywada and Gwalior. This helped us expand our reach in significant geographies and add capacity there where nearby labs capacity was about to get exhausted and meet better TAT commitments.

But beyond capacity, lab expansion plays a much larger role. It builds patient trust at the first point of contact, especially in Tier 2 and Tier 3 markets, driving repeat usage and referrals. It also strengthens our B2B proposition, making us a more compelling pan-India partner for corporates, insurers and hospital networks.

And finally, our hub-and-spoke

model ensures strong economics, allowing incremental volumes

to flow into existing infrastructure, driving better utilization and operating leverage as the network scales. Now, let's move on to the second area of operational capabilities and research.

During the year, we continued to strengthen our test portfolio and operational capabilities, including the addition of Phadia-based allergy testing platform and other 20-plus specialized tests, including histopathology, a new HPLC platform, specialized coagulation test, next-generation sequencing and the BioFire PCR platform to the portfolio.

Our genomics expansion is underway, with a strong focus on building a comprehensive specialty test portfolio. Starting with NIPT, we are adding tests in a structured phased manner to enhance clinical depth and drive growth in precision diagnostics. We also continue to translate real-world diagnostic data into actionable clinical insights.

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For example, our fever study based on approximately 3 years of data from our Jaanch fever panels revealed that 1 in 3 fevers are linked to more complex infections such as dengue, typhoid, malaria and influenza. This reinforces the need for timely and comprehensive testing, which we address through our Jaanch fever profile.

One more example is our Bharat Aarogyam Score study, which is based on the data collected between 2023 and 2025 from over 1 lakh patients over the country, reveals 96% of individuals exhibit risk for at least one health condition, indicating that very few people have completely optimal health parameters.

Now, coming to the third focus area of patient experience, which is something I care deeply about. Diagnostics is fundamentally a trust business. Every improvement we made this year was driven by a simple question. Does this make the patient feel more confident on us? To maintain credibility, we continually reinforce the four foundational pillars that underpin trust in medical diagnostics.

First is accuracy and quality. This continues to be non-negotiable for us, supported by NABL-accredited labs and ongoing equipment upgrades, whereas our 90% samples are processed at NABL-accredited labs. Second is faster resolution when things go wrong. I agree no service is perfect every time, but what sets a trusted brand apart is how quickly and transparently it resolves the issues.

We strengthened our complaint management channels, reduced resolution time lines and improved proactive communication with the patients. This year, we achieved Six Sigma levels in complaint management, with the set metric standing at 3.06 complaints per million test for Q4 FY '26.

Third is faster reports. We have worked extensively on improving logistics and lab processes, bringing our turnaround time to 3.43 hours in the quarter. And fourth, strengthening trust with the doctors, who are often the first point of contact for patients. We continue to deepen our engagement through multiple forums, meetings and on-ground interactions.

During the year, we conducted nine doctor meets with participation from approximately 300 doctors. Additionally, we recently started inviting doctors to visit our labs, where 256 doctors had visited our labs across India, where we showcased our quality systems, advanced equipment and strengthened quality control processes, while also addressing awareness gaps around our capabilities. Thus, overall, our focus has been on building an operations backbone that can support scale without compromising on quality or experience.

With that, I'll now hand it over to Vikram for financial performance.

Vikram Gupta: Good morning, everyone, and a warm welcome. Thank you for joining us today. I am pleased to report that we have delivered another strong quarter, along with a solid full-year performance for FY '26, driven by consistent revenue growth. We continue to

deliver growth in the early -20s
with all our key strategic drivers performing well.
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Our stand-alone business remains strong and continues to grow higher than industry average. Both our franchisee and partnership business continues to execute very well. On the international front, our Tanzania operations, though small in base but has also grown by 75% for us year-on-year.

In radiology, as covered in earlier calls also, we undertook some strategic consolidation of centers during the year. While this has temporary impact on the growth, the overall financial performance has improved with a stronger bottom line. Overall, our steady performance reflects disciplined execution, and we remain focused on delivering consistent growth going forward.

Now, let me take you through the key financial highlights for the current quarter. Starting with revenue, our stand-alone revenue came in at INR 210 crores, which is up 21% year-on-year, driven by strong growth across franchisee and partnership business. On a consolidated basis, revenue stood at INR 224 crores, reflecting a healthy 20% year-on-year growth.

Moving to margins and profitability. Our consolidated gross margin has improved to 74.7%, an increase of over 113 basis points year-on-year. This was largely driven by better negotiations and a strong growth in test volumes, which has helped improving operating efficiency. Employee and other overhead cost has increased year-on-year, primarily due to annual inflation, higher volume growth and continued investment in the new growth areas like specialty.

Our EBITDA margin for the quarter was 34%, with EBITDA growing 31% year-on-year, supported by strong revenue growth and improved gross margins. Profit after tax stood at INR 48.7 crores with a PAT margin of 21.7%. This represents a 128% year-on-year growth.

Now moving to our full-year performance. For FY '26, stand-alone revenue reached INR 774 crores while consolidated revenue stood at INR 829 crores, reflecting a strong 21% year-on-year growth. Pathology revenue grew by 22% in FY '26, while radiology revenue saw a decline of 6% during the year.

On profitability, in absolute terms, EBITDA for the year stood at INR 262 crores and profit after tax was INR 163 crores, reflecting strong growth of 38% and 81% year-on-year. Earnings per share adjusted for last year bonus issue came in at INR 2.99, which is up by 64% from INR 1.82 last year.

Our balance sheet remains strong with a net cash and investment of INR 230 crores plus and zero debt as on 31 March, '26. On the cash flows, we generated INR 213 crores from operating activities after tax during the year. Before I hand over, I would also like to share that Board of Directors has recommended a final dividend of INR 7 per equity share for the year ended March 31, 2026.

With that, I now hand over to Rahul for the strategic updates. Thank you.

Alok: In the meantime, if Rahul is unable to connect, can we ask Rajdeep to update the strategic piece on behalf of Rahul. Rajdeep, are you on the call?

Moderator: Rajdeep, sir, your line is unmuted. Please go ahead.

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Rajdeep Panwar: Yes. I'll take it from here.

Alok: Yes, Rajdeep.

Rajdeep Panwar: Thank you, Alok, and thank you, Vikram. Briefly, I would like to take a few minutes to recap to the strategic direction, and then we'll open it up for Q&A. First, I will reiterate our value proposition to customers. We will continue to remain an affordable option to all patients with good quality and on-time reports.

All our efforts on our value proposition is towards ensuring low cost to the patient, assurance on quality of testing through our certifications and engagement with doctors. We have made substantial progress on this, which I updated in my initial comments and is reflected in the

presentation as well.

This will remain at our core, and will continue to guide all that we will do.

Second, our strategy. We continue to maintain our strategy of being the B2B partner of choice to all front-end diagnostic services companies in India, whether it is a small diagnostic center in a semi-urban area, a pharmacy in a metro, a small nursing home, an individual doctor or a leading online diagnostics platform and healthcare marketplaces.

We are happy to work with them through our genomics expansion underway, with a strong focus on building a comprehensive specialty. And we are ensuring to provide a low-cost, robust testing solution so that they can serve their patients in the most effective manner.

If they require phlebotomy, we are happy to mobilize our phlebotomy network of over 2,000

phlebotomists, including our network partners to serve them better. This strategy has been working well for us, with both our franchisee and partnership business posting strong growth. As a natural extension of this strategy and in line with our vision to make quality diagnostics affordable and accessible, we are expanding our specialty segment, that is allergy and genomics by deploying an on-ground technical sales team to drive deeper doctor-led engagement, thus also making our strong entry into the curative segment, where we are extending the same quality standards we have already established in preventive diagnostics, while maintaining affordable pricing for patients, further strengthening our position as a comprehensive partner to our network. That in brief is our mandate as management.

Rahul Guha: I'm back. I'll cover the last.

Rajdeep Panwar: Yes, please. Please, Rahul.

Rahul Guha: Thank you, everyone, for giving us a patient hearing. I will once again end with the quote from the Mahatma. Find purpose. The means will follow. And our purpose remains to provide affordable, high-quality testing to the masses. With that, we will open up for Q&A.

Moderator: We will take the first question from the line of Sucrit D. Patil from Eyesight Fintrade Private Limited. Please go ahead.

Sucrit Patil: Good morning to the team. I have two questions. The first question to Mr. Guha is a forward-looking one. How do you see Thyrocare growing over the next few quarters in diagnostic and Thyrocare Technologies Limited
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preventive space? What are the main priorities on your table, like adding new tests or using digital platforms and improving customer trust?

All of these, what do you feel will help the company stay ahead of the competition and build a stronger position in the market? That's my first question. I will ask my second question after this.

Rahul Guha: Sure. Our strategy remains more or less the same. So, the key drivers of growth for us are franchise expansion. If you see over the last 4 years, we have been able to maintain a 25% CAGR on our franchise expansion, culminating in almost 11,000 franchisees by the end of this financial year.

And if you go through the disclosures, we've also shown how an FY '22 franchisee matures, FY '23 franchisee matures, '24 and so on, right? So actually, if you look at the next 4 quarters, it's actually work that has been done almost 2 years ago because those are the franchisees that were added 2 years ago that will start contributing to the growth in this financial year, FY '27 to speak.

The franchisees that we are adding now will contribute in FY '28 and '29. -- right? So, franchise expansion remains at the core of our execution. The other is our partnerships, right? As I had mentioned earlier, if you look at every major healthtech player, we work with them.

If you look at every major, what we call wellness provider, we work with them, but we have just started to scratch the surface on insurance and other allied segments. So, I believe there's a lot of work to be done to capture the opportunity

in that business because there are many players

now who are in the healthtech or healthcare space that also want to offer their patients diagnostics, right?

And Thyrocare being a B2B partner is actually the only partner who offers them that ability to expand their portfolio of offerings in health care to their patients. So, I think those two have been our traditional drivers of growth and will continue to remain the traditional drivers of growth.

The only new element that we have added for FY '27 is specialty, right? As Rajdeep had covered and I had covered, we have made quite a few investments in genomics, allergy, histopathology and other areas more towards moving the mix of Thyrocare into the specialty space.

Of course, FY '27 will be our first year of the foray into specialty. So, it will be on a very small base. But definitely, if you look at a 3-year out horizon, specialty will be a very large component of our growth going forward.

Sucrit Patil: My second question to Mr. Gupta is looking forward, how will you balance spending on technology, new labs and shareholder return while keeping costs under control? What long-term and short-term measures, maybe such as automation, supply chain improvements or vendor partnerships will be put into place to make sure margins stay strong here, even if the costs rise or if there's any compliance changes that happen?

Rahul Guha: And maybe before I hand over to Vikram, I'll just give you a little bit of our operating philosophy, right, which is more like a Warren Buffett kind of operating philosophy, which is Thyrocare Technologies Limited

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first, savings, then expenditure, right? So we -- or in our case, first, margin protection, then expenditure, right? And so what I have always been guiding is that any operating leverage that comes out of the growth, obviously, we are growing at 20% plus and your cost doesn't increase that much.

Any operating leverage that comes out of the business is what we invest in growth, but we try our best to preserve the base margin. So, very much a philosophy of first margin preservation and then surplus goes towards investment, towards capex, growth, new initiatives and so on. So, that's at a philosophy level. But I'll let Vikram take it at a more detailed level.

Vikram Gupta: Yes. I think you have covered it, Rahul. So basically, though we are investing into new things, but as Rahul said, we are very calibrated into the investments, but we are going into the new things. Let's say, for the genomics, we are starting with the top 10 cities, North of India. So, we are calibrated in the investments, which we are doing. And plus on the supply chain also, as you covered, let's say, you have seen that our gross margin has improved by 113 basis points over the last year.

So the benefit of scale continues to come, and we continue to invest that thing into the growth.

So, we will protect our margin, whatever operating leverage that will come, that is going to be a guiding factor for the investments, which we will do into the new things.

Moderator: We will take the next question from the line of Raman KV from Sequent Investments.

Raman KV: Congratulations on good set of results. I have just 2 questions. One with respect to the margins.

We have seen during the quarter, there is a strong uptick in the gross margin. Can you just help us understand where the gross -- the reasons for the gross margin expansion? And is this the new normal?

And can you also help us understand that whether the Tanzania business has breakeven, which led to the contribution towards the higher gross margin, as well as the diagnostic business has rebounded with sharp margins during the quarter? So, can you also throw some light on that aspect?

Rahul Guha: Understood. See, on the margin front, a large part, if you look at quarter -on-year, I think margins are more or less stable, right?

t? So, I think if you look at it going forward, I think margins will be in that 73% to 74% range.

It's a mixture of basically good negotiation with the vendors to be able to realize volume benefits from the volume that we are able to purchase as well as last quarter, there would have been some, how do you say, catch-up effect of the ROU, but that is now fully baked into Q4 '24.

So, I think you can expect the steady-state margins for the business to be around 73% to 74%, barring any abnormal price hike in reagents that may come out as a consequence of the war or the supply chain side. But otherwise, it should be in that range. I forgot your second question, actually.

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Raman KV: Sir, the follow-up on that was whether the Tanzania business has breakeven, which contributed to the higher margin. And also from the PPT, I can see that your -- even though your diagnostic business revenue declined by 9% to 10%, your margin has increased in the diagnostic business. So with the latest acquisition, which you announced of Think Diagnostics, are you planning to expand your diagnostic business as well?

Rahul Guha: Yes. See, we still haven't broken even in Tanzania, right? So, I would say we've been in Tanzania now about 18 months odd and we still haven't broken even. So, that is not the reason. Though the Tanzania losses are minimal. I think it's less than INR1 crore is the overall Tanzania quarterly losses.

So to that extent, we have come down quite a bit on the Tanzania front. And if you look at it overall, I think our gross margins, as I said, have sustained because this year has been a fantastic year in terms of test volume growth, and we were able to get a significant amount of volume discount from our vendors.

Raman KV: Understood, sir. And sir, my last question is there has been a huge uptick in the test volumes during the quarter. So is it because there -- have you been seeing any GLP -1 traction starting from this quarter? And going forward, can we expect the same like 15% test volume growth in the coming year -- for the coming year?

Rahul Guha: Yes. I think, look, this quarter, the test volume growth was a bit of a strategic investment. So for the quarter, I think we delivered almost 29% test volume growth. And that comes largely from -- we received actually some good deals on the biochemistry side. And we were able to, as a result, reduce our prices a little bit on the biochemistry side. And we became very competitive in biochemistry. Just for background, Thyrocare was always known for thyroid, which is a particular technology called immunoassay and we've been very, very strong in that space. But this quarter, because we were given some good offers in biochemistry, we launched a set of biochemistry parameters at an attractive price. And actually, we were very pleasantly surprised by the test volume uptick that we saw.

Raman KV: And sir, the second part of the question, whether this will be the new normal, like 15% test volume growth in FY '27. Can we expect that?

Rahul Guha: See, we always try to overall guide to a mid-teens kind of number and mostly from volume and mix. I think this year, we are now reasonably confident to revise it marginally upwards. I'm, of course, very cautious. So, I would say a mid- to high teens is a good growth expectation for next year, of which I expect volume growth to be the maximum driver, right? I would say almost 75% of the revenue will come from volume and maybe about 25% from mix. We have no intention at this point in time to increase prices.

Moderator: We will take the next question from the line of Yash Doshi from Unifi Capital Private Limited.

Yash Doshi: Congratulations on a good set of numbers. Actually, while going to your press release, I found that you have mentioned that we will be venturing into allied business of consumables and Thyrocare Technologies

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diagnostics. So, maybe can you just clarify on that? Which type of segment we are planning to enter in near future?

Rahul Guha: I would like to keep it internal to the company for the next 1 or 2 quarters because we are just working on the overall launch plan. So, I think it will become clear to you by June or July of what segments we are entering into, right? But I will tell you the strategy is we have realized in many segments, we are a large consumer of allied diagnostic tests, right, and sufficient that we can also launch our own brand.

And so we are planning to launch in that space. I can't share too many details because it's just in the launch phase. If I could request if you can be patient till June, July, it will become very clear, which areas we are entering by then.

Yash Doshi: Okay. Understood. So basically, it's kind of -- if I infer, it's kind of some backward integration as we are the consumers of those consumer goods?

Rahul Guha: You could say to some extent that. Just think of it as in the allied services space. That's it...

Yash Doshi: Understood. And another part, I wanted to know is during the quarter, samples, I think in the presentation, it was not mentioned that how many samples have we processed?

Rahul Guha: During the quarter, we would have processed -- I have -- it's there in one of the disclosures. We processed 80.3 lakh samples in Q4 of FY'26.

Yash Doshi: Okay. And another part was on the Specialty segment. Can you throw light on the investments made, the field force, how many people have we employed? And what is the strategy around basically communicating to the franchisees, doctors? Just a light on Specialty segment. Can you just...

Rahul Guha: See, the capex investments would not be much. They would be in the tune of about INR5 crores to INR8 crores, right? The real investments are on the opex side. We've put a team of, I think, about 40 people on the Specialty side. Their effective job is to go out and explain to doctors that Thyrocare is in the specialty space.

This is our test menu. And of course, we always approach it with our philosophy of good quality testing made affordable to all, right? So, I think that's the extent of investments that we have made. The large investment, of course, is on the pricing side. We always come in with a disruptive pricing.

So to that extent, the percentage gross margin in Specialty will be lower. But of course, given our realization per test is only INR30 to INR40, specialty test can be in the INR1,000, right? The absolute gross margin per test is of course much, much higher and the overhead costs are not to that extent. So, we should get a decent amount of operating leverage, but we will be very aggressive on price.

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Moderator: Sorry to interrupt in between, Yash. I would request you to kindly rejoin the queue for more questions. Thank you. We will take the next question is from the line of Chintan Sheth from Girik Capital. Please go ahead.

Chintan Sheth: Congrats on a good set of numbers. So, my question pertains to -- we are trying to expand test menu getting into the specialty, increasing the share of allied test, which you guys will be working on over the course of next year. How do you see vials per franchisee move? Because when I look at our past few quarters, we have been in the range of 450 to 550 kind of number vials per franchisee?

With this test menu, does that entail that number to inch up linearly upwards going forward given that we have expanded our test menu over there? And if you can spell out how this scale-up happens between the mature -- from the new franchisee to mature franchisee?

How that vials per franchisee number moves? And if you can spell out what is the current mature vial number, which they are consistently doing? And is there any seasonality to it as

well? That

will be my first question.

Rahul Guha: Sure. See, franchisee addition, I expect to remain at the 500 per quarter run rate, right? Some quarters will be good. Some quarters will be bad. Seasonality -wise, during the festive season, Q3, typically, people do not open franchisees, whereas in Q1 and Q4 is when people open up franchisees. And so therefore, we see a good boost typically Q4 to Q2.

And then, of course, it slows down in Q3 and then picks up again in Q4. But I expect about 500 per quarter is -- because we have resourced that way, right? The constraint is not at the demand side. The constraint is our ability to onboard and scale up the franchisees to make them successful. So, I believe we are resourced for 500 a quarter.

I think that's a good run rate to be able to maintain. Now coming to your second question, which is with the new test menu expansion. I expect our average revenue per franchisee to go up, but I don't expect a huge increase in the franchisee count, right.

If you have -- if you look at it, Thyrocare has always been a routine and semi-specialized company. And so many of our franchisees, they will not turn away a customer because Thyrocare doesn't have the particular test in the menu. They will give it to someone else.

Management: So, it is our hope that by launching these tests in our network.

Rahul Guha: Right, that these franchisees now will not give it to someone else, but actually now move it to Thyrocare. And so therefore, the benefit will be in the average revenue per franchisee, not in virgin franchisees who have never worked with Thyrocare coming in.

To your last question on the scale up, actually, in the presentation, we have shown exactly how the scale-up works. If a franchisee gives us x in year 1, it gives us 2.5 in year 2, 3.5 in year 3, 4 in year 4 and 5x in year 5. So 1, 2.5, 3.5, 4, 5. And that number has stayed pretty stable over the last 4 years.

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And that goes to my opening comments where I said, look, the work that we are doing in Thyrocare and franchise addition is not for this year. It's for next year and the year after. The benefit we are getting in FY '27 is the work that was done in FY '25 and FY '26. And so that momentum continues. We haven't seen a decline in that. And I'm hopeful that as the franchise addition quality increases, that number will only get better.

Chintan Sheth: Got it. And this 500 seems to be a little higher than what we earlier guided around 1,200 annually or 1,500 annually. Now, we are targeting 2,000, means we are kind of aggressively trying to onboard and penetrate the market. That would be the right reading?

Rahul Guha: No. See, you should take 500 into 3, which is about 1,500. So for Q3 there are no additions. So to that extent, yes. So against 1,200, I think we are now comfortably achieving 1,500. So, that's what we are doing. Yes.

Chintan Sheth: Got it. And this quarter, I'm seeing test per vials to inch up to 7.5 versus 6.77 historically. Is this the new normal? Or there is some -- you mentioned some scheme you guys have run in the test menu, which led to higher tests this quarter. So it will normalize to 7 per vial that would be -- that should be going ahead number?

Rahul Guha: Yes. Just to clarify, at least it is my impression that it is closer to 10 in this quarter. Preet, can you clarify?

Chintan Sheth: I'm just dividing the test by vials, number of vials.

Rahul Guha: Okay. That is on the higher side. Because, see the test per patient is about 11. And typically, it's about 2.5 vials per patient. So, I don't know that calculation where it comes from.

Chintan Sheth: I'll check with Preet then.

Rahul Guha: It should be closer. See, if you take 80 million vials and 209 tests, it's roughly 3 tests per vial, which is kind of where I thought the figure would be.

Chintan Sheth: Okay, I'll check and confirm.

rm. I'll turn back. Thank you.

Moderator: Next question is from the line of Surya Patra from PhillipCapital. Please go ahead.

Surya Patra: Congratulations for the great set of numbers. My first question is on the investment priorities. If I see for FY '26, the capex number looks really low, while we have already invested into genomics, the way that we have been talking about and also talking about the new specialty areas. So hence, what is the capex likely to be for FY '27? And what investment priorities that we will be having for the next year?

Rahul Guha: All right. On capex, I think there is some up and down because of the ROU. So, I'll ask Vikram to explain it fully, right? Vikram, you can just take that, okay?

Vikram Gupta: Yes. So, yes, that INR20 crores is the capex payout number, which you are looking in the cash flow. There is

INR15 crores. Total capitalization during the year is around INR35 crores besides the machines which we have taken on lease rental.

So to answer you, we have invested into the capex for the capacity also and we have opened new 7 labs also. So, there is no conservatism or there is no underspend in the capex. We will continue to invest into the capex as per the requirement. So, total capitalization of approximately INR35 crores across new labs, new capacity, etcetera, what we have done in -- which includes genomics .

Surya Patra: Okay. So in fact, that is also relatively low looking at the kind of operating cash flow that we are generating, sir, because we know this is a business where cash requirement is limited, rather it is a cash flowing business. So hence also it indicates that, okay, the kind of investment that we will be making, that will be indicating the quantum of growth that we can see. So from those angle, any investment plans and priorities that we'll be having and the quantum, if you can share for next year?

Rahul Guha: Let me take that, Vikram. Just to explain, see the growth of this business is actually not determined by capex. We have sufficient lab capacity. We are present all over India with 40 labs. Our average lab utilization is roughly about 65%. And you drop up in everywhere -- anywhere in India, you will find a Thyrocare lab within 150 kilometers.

So from that point of view, our growth driver is not lab count and capex. Our growth driver is actually franchise addition and that is very asset -light, as you know, because we don't have any company -owned stores. It's all franchise -owned, franchise operated. So to this business, it has always been asset -light. Maintenance capex is roughly about INR40 crores with new expansion and all of that. We expect the capex to be in that range. And as a result, the cash flows to be available ex the capex as well. Our major focus is franchise addition.

Surya Patra: Sure. Yes. So second question is on the kind of a specialty area that you have mentioned, particularly the genomic investments and genomic opportunity that you have highlighted in your opening remarks. Sir, is it possible to kind of give a sense that what is the quantum or what is the kind of business mix that -- or test mix that we can anticipate, let's say, 3 years down the line in the area of a genomic or specialty as a whole because you have also mentioned in your presentation about the ImmunoCAP BioFire Panels and all those kind of things. So, what is the share of a test mix from the specialty side, let's say, in the next 3 -year time considering FY26 as a base year?

Rahul Guha: Yes. See, our competitors specialty mix is roughly about 15% to 20%. I anticipate in 3 years reaching that level within Thyrocare as well.

Surya Patra: Okay. And regards the genomic, the kind of a strength in terms of the revenue potential that you have mentioned, so it is significantly underpenet

rated as of now, it looks like mainly because of the price point. Is that understanding right, sir?

Rahul Guha: That is correct.

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Surya Patra: And just one question, book-keeping question about the financial year -end, sir. What is the kind of a lab number that we are currently having? What is the wellness mix? And also, if you can share what is the mix of your franchisee in terms of the urban and non-urban region?

Rahul Guha: Understood. I missed the middle question. We have 39 labs plus 1 in Tanzania. I missed the second question.

Surya Patra: Wellness mix, sir.

Rahul Guha: Wellness mix?

Surya Patra: Preventive, Aarogyam.

Rahul Guha: Aarogyam will be about 33% of our overall mix and year -on-year growth it would have grown at about 19%, 20%. So, that is the wellness mix. Jaanch is also in my opinion, considered wellness. That is only 2% of our revenue today, but grew at 66% year -on-year. So, I think these are our two major wellness brands. And then on the mix of franchisees, I would say we are roughly about 20% urban, metro Tier 1 and I would say 80% Tier 2 and beyond.

Surya Patra: And whether this ratio was same in the last year?

Moderator: Surya, I request you to join back the queue, please, as we have participants waiting for their turn. Thank you.

Surya Patra: Yes, sure.

Moderator: Next question is from the line of Lokesh Manik from Vallum Capital. Please go ahead.

Lokesh Manik: Hi, good morning Rahul and team. Am I audible?

Moderator: Yes. Please go ahead.

Lokesh Manik: Rahul, just a clarification on capacity and lab expansion, just to understand it better. So for a given capacity at a lab, as you are seeing your test per patient increasing from 8, 9, 10, 11 touching this year, the number of patients served then reduces for a given capacity either that happens, then you need to open a new lab or you have the technology present in the current lab to accommodate even more patients at an even higher test per patient rate. So if you can just clarify on that because you mentioned 65% capacity utilization, but I'm just seeking a clarification from you on this?

Rahul Guha: Sure. See, the driver of capacity in a lab is the tube, not the number of tests in the tube because once you load a tube in the machine, whether the machine has to process 1 test or 10 tests, right, the capacity doesn't vary that much.

My capacity is largely linked to how much time it takes to load a tube into a machine and unload a tube from a machine and take it to another machine. If you see, actually, our revenue or our tests per patient has increased. So in a way, tests per patient actually doesn't consume any additional capacity or very marginal. So to that extent, we are good on the capacity front.

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Lokesh Manik: Understood. Rahul, my second question was since you are foraying into specialty now, do you foresee an increase in test -- revenue per test because the specialty will be at a much higher pricing. You won't take price hikes, but they'll be at a price higher. So, your blended revenue per test will increase. Would that be a fair understanding?

Rahul Guha: Yes. And that is something we are all tracking to see how that mix moves. But yes, one would expect a material movement in revenue per test over the next couple of years.

Lokesh Manik: So how much can -- any ballpark rough figure, how much in a percentage term increase that we can expect?

Rahul Guha: Difficult at this point to speculate. Give me one or two quarters, and I will come with a concrete number.

Lokesh Manik: Sure. That's it from my side. Thank you so much.

Moderator: Thank you. Next question is from the line of Bino Pathiparampil from Elara Capital. Please go ahead.

Bino Pathiparampil: Hi, good morning gentlemen. Congrats on a great set of numbers. Just a

follow-up on the Middle

East situation and the impact on reagents. In an earlier comment, you highlighted that as a risk.

As of now, what's the situation? Are you seeing any availability issues or price -- significant price increases?

Rahul Guha: No, availability has not been a concern as of now because the supply chain actually doesn't route via the Middle East. It routes mostly from Europe this side. So to that extent, yes, it has taken a little longer, but we always carry 2 to 3 months of inventory. So, we haven't faced any disruption on availability front. Of course, with the dollar being where it is, right, many of our vendors have come back requesting for price increases. But given our volume growth, we have been able to mitigate a large part of that. But should the situation continue, I think, definitely, raw material prices will increase and then we will have to evaluate how we pass it on.

Moderator: Thank you. Next question is from the line of Yash Doshi from Unifi Capital. Please go ahead.

Yash Doshi: Yes, sir. So just one book-keeping question. So, current year, if you see, your tax rate has been around 23%, 24%. So, I get to know that it's based on deferred tax benefit. So, can you just enlighten me on what are components of deferred tax and going forward, whether your tax rate will be close to 27%, 28%?

Rahul Guha: I'll let Vikram take this question. Vikram?

Vikram Gupta: Yes. That is because of the deferred tax timing difference. And going forward, we expect our tax rate of 28% to 29%.

Moderator: Thank you. Next question is from the line of Ibrahim an Individual Investor. Please go ahead.

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Ibrahim: Hi, I just want to know the market opportunity for the specialty testing and allied services, which we are going to announce. So, what will be the market opportunity in next 2 to 3 years? How big will be the opportunity?

Rahul Guha: See, specialty testing, overall pathology market is estimated to be between INR50,000 crores and INR60,000 crores. And of that, roughly 15% to 20% is estimated to be the specialty space.

So if you do that, we are talking about INR7,000 crores to INR10,000 crores market. That's a top-down estimate of the overall industry.

Ibrahim: And what about the allied?

Moderator: Ibrahim only one question please. Thank you. Next question is from the line of Shubham Harne from Purnartha Investment Advisers.

Shubham Harne: Hello. Sir, my question is on insurance business. How the insurance business has done in past year and what are your growth expectations for next year?

Rahul Guha: Yes. Our insurance business overall has done very well. If I look at the year -on-year growth for the insurance business, we started very aggressively last year, but year -on-year, we've grown at almost 45%, right, in the insurance space. Insurance is of two parts. There is pre-policy medical checkup and annual health checkup.

Typically, a lot of insurance companies offer a free annual health checkup with their policy and Thyrocare serves both these business lines. It's, of course, on a very nascent size of business. As I said, we are just scratching the surface and as you know, insurance policies itself has grown by about 25%. That being said, we still managed to grow this segment by about 45% and this continues to remain a focus area for us.

Shubham Harne: And what's your plans for next year, expectation?

Rahul Guha: I don't comment on individual segments. As I said, I have given an overall revenue guidance. At an individual segment, there will always be plus/minus depending on execution.

Moderator: Thank you. We'll take our last question from the line of Yash Doshi from Unifi Capital. Please go ahead.

Yash Doshi: Yes, sir. What will be the EBITDA margins going forward? It will be close to 33%, 34% or due to specialty segments, we expect margin expansion?

Rahul Guha: See

, specialty segment will be at a nascent stage. So, I don't expect -- and I will not build any major upside from the EBITDA margin side on the specialty sales in FY27. We can discuss in FY28 and onwards. But then till then, I think it will be difficult. For the full year, I think we delivered roughly about 32% reported EBITDA and a normalized EBITDA at around 34%. I expect that to be stable into the next year. As I mentioned, any further operating leverage that we get, we will continue to invest in growth. So, I would expect, if you take full -year FY26 as a base, we anticipate to maintain that EBITDA margin.

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Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I now hand the conference over to Mr. Rahul Guha for closing comments. Over to you.

Rahul Guha: Thank you, everyone, for joining us and spending time with us this day. My apologies for having had to cancel the conference call at the last minute on Thursday. That was because the Board Meeting took longer than expected and therefore, we did not have enough time to upload the presentation and the results. So, we had to reschedule.

So, thank you for being patient with us and joining us this morning. As always, we continue to remain focused on our strategy, which is to be the most affordable, good quality diagnostic testing partner for anyone in the healthcare business and we continue to execute on that strategy. We have been investing in improving our quality, improving our reach and ensuring our turnaround time is as close to best -in-class. And we've made substantial progress on all of this, and that is what is driving the results that you see. I'm also very happy to see Rajdeep Panwar, our new Chief Commercial Officer.

Dr. Ramesh Kinha our new Chief Operating Officer and Vikram Gupta, our CFO, leading the call and driving the strategic direction for the company. And I'm sure, we with this core team fully in place, we will continue to drive the exceptional execution that you have seen in the last 2 years. Thank you for all your support in this journey and I wish you all a good evening. Thank you.

Moderator: Thank you. On behalf of Thyrocare Technologies Limited, that concludes this conference. Thank you for joining us and you may now disconnect your lines.